CLINICAL MANUAL & RECOVERY PROTOCOL

Point of Support

A Step-by-Step System for Overcoming PPPD (Persistent Postural-Perceptual Dizziness)

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58 Chapters · 5 Modules · Evidence-Based Neuro-Vestibular Protocols

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Introduction

Introduction

This book is not another article from the Internet. This is a map. The map I drew while I was getting out of hell.

Hello. My name is Maxim.

If you are reading this, it means your world has lost stability. I know what it's like. When the floor floats under your feet. When every trip to the store is a marathon for survival. When doctors look at their pictures, throw up their hands and say: “Well, take a sedative.”

You're not going crazy. You're not dying. And you're definitely not the first to go through this.

I walked this path myself. **I have been completely healthy since January 2021.** But before that I was at rock bottom. Endless MRIs, panicky fear of every step, complete misunderstanding - why my body is betraying me. I visited dozens of doctors. I was given “VAD (autonomic dysfunction)”, “cervical osteochondrosis”, “dyscirculatory encephalopathy” - dummy diagnoses with nothing behind them. They don't heal. They don't even explain.

I was desperate. But I found a way out.

My story - short and without embellishment

In January 2020, I experienced **BPPV** - benign paroxysmal positional vertigo. Classic: lay down in bed, turn your head - and the room spins like a carousel. Treated with vasodilator therapy. A month later, the external dizziness went away. But the **internal** remained - that same “cotton wool”, unsteadiness, the feeling that the floor was floating under your feet. I was diagnosed with PPPD and generalized anxiety disorder.

The doctors offered the standard Russian prescription: a course of antidepressants for a year and a half. I specifically went to the mental health institute to pick up the drug. I couldn’t drink - the side effects were worse than the dizziness itself. They offered another one - same story. Pharmacology didn't work for me.

Then I started digging myself. In the Russian segment of the Internet, there was practically no information on rehabilitation after PPPD. I found sources in English - studies, books on vestibular rehabilitation, neurophysiology. I consulted with doctors in Germany and Israel who work specifically with such dizziness. I spent a lot of money on all this. But I gathered a complete picture: what is happening to me, why and what to do about it.

In July I started studying seriously. My symptoms at that time: my head began to feel dizzy after 20–30 minutes of normal walking, there were sharp “dips” - as if you were losing consciousness for a second, like a lag in a computer game. Everything intensified in public places.

From July to January - six months of daily work. Vestibular gymnastics, CBT, exposure, physical activity. Dull, routine, daily discipline without breaks or days off.

Result: I am completely healthy. Not 90%, not “almost”. 100% The dizziness is gone. The fear is gone. I live a normal life and no longer think about every step.

Some numbers

If you think you're the only one, here are some statistics:

- PPPD is the **second most common** reason for visits to otoneurologists in the world (after BPPV). *According to Staab et al. (2017), PPPD is diagnosed in **15–20%** of all patients attending specialized vestibular clinics.
- The average age of onset is **30–50 years**, but occurs at 18 and 65.
- Women get sick **2-3 times more often** than men (although among my subscribers the proportion is almost equal). *It is estimated that **up to 70%** of people with PPPD have a co-occurring anxiety disorder. *The average time from the onset of symptoms to correct diagnosis is **6 months to 3 years**. Three years of going back and forth from neurologist to ENT specialist and back.

You are not a unique broken specimen. You are part of a huge army of people whom medicine has not yet learned to quickly recognize and treat. But the good news is that **treatments exist**, they work, and I've compiled them in this book.

How to work with this book

This is not a fiction novel that is read from cover to cover in an evening. This is a **workbook**. Here are the rules:

Rule 1: Don't swallow everything in one gulp

Read **one chapter a day**, maximum two. After each chapter, you need time to digest the information and do your homework. If you read everything in an evening, you will gain knowledge, but not skills. And skills are the only thing that will cure you.

Rule 2: Do the homework

Each chapter ends with a specific homework assignment. **Don’t skip it.** I understand that you are tired, that you are shaking, that “I’ll do it later.” But “later” is never. Those who do DZ come out of PPPD in 2–4 months. Those who just read sit in it for years.

Rule 3: Keep a diary

Buy a notebook or create a note on your phone. Write there:

- Test results (HADS, DHI) every month.
- Observations after exercise.
- Your cognitive traps that you caught.
- Level of shakyness/anxiety in the morning (on a scale of 1-10).

The diary is your mirror. In a month you will re-read the first entries and will not believe that you wrote it.

Rule 4: Don't jump over

The chapters are in strict order. Module 0 (base) → Module 1 (body) → Module 2 (panic) → Module 3 (thinking) → Module 4 (exit). Don’t get into “Thinking” until you have worked on “Body”. You can't fix software while the hardware is smoking.

Rule 5: It will get worse before it gets better

When you start doing vestibular gymnastics, in the first days the unsteadiness may **increase**. When you start making exposure, panic may temporarily **grow**. This is fine. This means that the brain is being rebuilt. Don't give up practice at this stage. You are renovating a building, and during the renovation the apartment is a mess. This does not mean that repairs are useless.

Who is this book for?

Let's be honest right away. My channel is not for everyone. And this book is not for everyone. It's for people like me.

For people who have PPPD - persistent postural-perceptual vertigo. It doesn’t matter whether you were officially diagnosed or not. If you are shaking, swaying, the picture is floating, the ground is disappearing from under your feet, and the doctors cannot find anything - you are at the right place.

It's for people who are **ready to work**. Don't look for a magic pill. Don't wander around the forums. Don't expect someone else to do everything for them.

People come to me and say: “I’m ready to eat the earth, just so that this will pass.” These are the people I work with. The rest - understand, forgive, pass by.

What do you get here

This book is not theory for theory's sake. This is a **step by step system** that I pieced together. I translated some of the information from English - from advanced research that simply does not exist in Russian.

Some of it comes from the experience of dozens of people who have already gotten out. Some of it is my personal experience, lived in my own skin.

Here's what awaits you:

Module 0 - Base. We'll figure out what's going on with you. No panic, no scary diagnoses. You will understand the mechanics. As a programmer - not as a patient.

Module 1 - Body. You can't fix the head while the body is stuck in concrete. Muscle armor, spasms, “cotton head” - we will remove this first.

Module 2 - Battery. Let's look at the adrenaline loop, hypochondria, panic. You will understand why you are “covered” - and why it is not dangerous.

Module 3 - Thinking. The deepest. Neuroplasticity, cognitive traps, ego, inner child. We'll get to the root of why you came here in the first place.

Module 4 - Exit. Setbacks, fear of relapse, new identity. How to live after. How not to return.

Each chapter ends with a **specific homework assignment**. Without practice, it's just letters on a screen. Don't fool yourself - do it.

Important rule

Personal growth is always painful. It’s unpleasant to realize that you have driven yourself into this hole. It's not pleasant to get out. I won't feel sorry for you. I'll pull you out.

In war, all methods are good.

To whom I am grateful

I dedicate this book to the memory of my grandmother, who, unfortunately, is no longer with me. When you feel relief and solid ground under your feet, light a candle for her repose. For me this will be the best gratitude.

Disclaimer

This book does not replace medical advice. Before working on this program, make sure that you have completed basic examinations and excluded organic matter. We'll cover this in detail in Module 0.

I'm not a doctor. I'm a person who went through it and came out. The information here is the most advanced in the world. But only two things are needed from you: **desire and action**.

Let's go.

> ***My experience is not "survivor's error."*** *Our community contains dozens of stories of people who have returned to normal life. People who fly on planes again, stand in lines, walk through shopping centers and don't think about every step. I was there, at that very bottom - and I will lead you out.*

Module 0: Foundation

Psychosomatics: why everything you will learn here works not only for PPPD

PPPD is not a separate disease. This is one of dozens of languages in which an overloaded nervous system shouts "Stop!" And all the tools in this book are tailored not to a specific symptom, but to the screaming mechanism itself.

One engine - different light bulbs

Let me start with an inconvenient truth that you may not be ready to hear yet.

PPPD is **not a unique disease**. This is one of the symptoms. One of many. Your brain has chosen dizziness as a channel through which it tells you: "I'm overwhelmed, I can't cope, stop me." But your neighbor in misfortune, the same overloaded brain chose a different channel - and now he has been visiting gastroenterologists with irritable bowel syndrome for the third year. And his wife suffers from chronic back pain, which no orthopedist can explain. And her friend cannot swallow food because there is a "lump" in her throat, although the ENT says: "Everything is clear."

Different lights on the dashboard. Same overheated engine.

Psychosomatics is not "fiction". It's not "it seems to you." This is a **functional disorder** in which the actual physical symptoms are not caused by a broken organ, but by a glitch in the software of the nervous system. And here is the main secret that is worth this entire book:

All the techniques you will learn in this book work for ANY psychosomatic symptom. Because they don't hit the light bulb, but the engine.

What is psychosomatics: neurophysiology without mysticism

The word "psychosomatics" comes from the Greek "psyche" (soul) and "soma" (body). It sounds like something from the realm of esotericism, but in fact there is hard neurophysiology behind it.

Central sensitization

The key mechanism that unites **all** psychosomatic conditions is called **Central Sensitization**. That's what it is.

Normally, your nervous system works like a filter. Of the millions of signals received from the body every second, the brain allows only the significant ones to enter consciousness: acute pain, a sharp sound, loss

of balance. The rest - bowel sounds, muscle microspasms, slight swaying when walking - are filtered out as background noise. You just don't notice them.

But when the nervous system is in chronic stress mode for **months**, the filter breaks down. The sensitivity threshold **decreases**. The brain begins to perceive normal, harmless signals from the body as a **threat**. Intestinal peristalsis, which a healthy person does not notice, becomes a “terrible spasm.” Normal heartbeat turns into “tachycardia”. Slight swaying when turning the head - “terrible dizziness.”

This is not fiction - it is a proven neurophysiological process. In 2011, Woolf published a seminal paper, “Central sensitization: implications for the diagnosis and treatment of pain,” showing that with central sensitization, neurons in the spinal cord and brain **physically change** their excitability. They begin to respond to stimuli to which they previously did not respond. The response threshold drops. The system begins to “scream” from the whisper.

Brain-Body Axis: HPA and Vagus Nerve

The second leg of the stool is the **hypothalamic-pituitary-adrenal axis** (HPA axis). This is the central stress management system. When you are worried, the hypothalamus gives a command to the pituitary gland, which tells the adrenal glands, and they release cortisol and adrenaline. This is fine. The problem starts when this axis is running **24/7/7 days a week**.

Chronically elevated cortisol:

- Suppresses the immune system (hence constant colds and skin problems)
- Disturbs gastrointestinal motility (hence IBS, bloating, nausea)
- Keeps muscles in constant tone (hence pain in the back, neck, jaw)
- Reduces the production of serotonin (hence the depressive background)
- Disturbs sleep (hence the morning hell you already know)

And the **vagus nerve** (vagus) is a cable through which the brain controls internal organs: heart, lungs, stomach, intestines. When the autonomic nervous system is imbalanced, this cable begins to transmit incorrect commands. The result is all those “organ” symptoms with which people have been going to doctors for years.

Catalog of psychosomatic symptoms: one mechanism - dozens of masks

Now I'll give you a list. Long. And for each point you will see: this is the same engine, just a different light bulb.

🧠 Head and nervous system

Symptom	Mechanism	Why doctors don't find anything
Dizziness, unsteadiness (DISG)	Desync of vestibular sensors + hypercontrol	MRI and vestibulogram are clear
Fog in the head, brain fog	Cortisol overload of the prefrontal cortex	Cognitive tests normal
Tremor, shaking of hands/body	Excess adrenaline → muscle activation without purpose	Neurological examination is clear
Tics, eyelid twitching	Magnesium deficiency + sensitization of motor neurons	EMG normal
Numbness, tingling	Hyperventilation → blood alkalosis → vasospasm	ENMG without pathology

💪 Muscles and pain

Symptom	Mechanism	Why doctors don't find anything
Pain in the neck, back, lower back	Muscle armor (Reich) + trigger points	X-ray/MRI - “osteochondrosis” (everyone has it after 25)
Tension headache	Chronic spasm of the frontal, temporal and cervical muscles	CT/MRI clear
Vestibular migraine	Sensitization of the trigeminal system + anxiety	Otoneurological examination is normal
Jaw pain, bruxism	Suppressed scream, clenched teeth 24/7	Dentist: “Well, take some magnesium”
Fibromyalgia	Central sensitization of pain pathways	All tests are clear, no inflammation

💔 Heart and blood vessels

Symptom	Mechanism	Why doctors don't find anything
Tachycardia, extrasystoles	Adrenaline + sympathetic hypertonicity	ECG, Holter - normal variant

Symptom	Mechanism	Why doctors don't find anything
Pressure surges	Imbalance of vegetatives (sympathetic/parasympathetic)	Cardiologist: “This is VAD (autonomic dysfunction)” (a dummy diagnosis)
Feeling of “fading” of the heart	Parasympathetic “brake” after an adrenaline rush	Holter - single extrasystoles (normal)

□ Breathing and throat

Symptom	Mechanism	Why doctors don't find anything
Lump in throat (globus sensation)	Laryngeal muscle conflict: screaming vs. suppression	Gastroscopy and ENT are the norm
Shortness of breath, “I can’t breathe deeply”	Diaphragm clamp + hyperventilation	Spirometry is normal, saturation 98-99%
Roughness, cough without cold	Spasm of laryngeal muscles + reflux from stress	The ENT specialist shrugs

🤢 Gastrointestinal tract

Symptom	Mechanism	Why doctors don't find anything
Irritable Bowel Syndrome	Vagus dysregulation → motor impairment	Colonoscopy, tests - clean
Nausea, loss of appetite	Cortisol suppresses digestion (not before eating when the “tiger is nearby”)	Gastroscopy is normal
Bloating, cramping	Hypertonicity of intestinal smooth muscles	Ultrasound, coprogram - clean

🖐️ Skin & Systemic Symptoms

Symptom	Mechanism	Why doctors don't find anything
Itching, scabies without rash	Histamine response to stress + central sensitization	Dermatologist: “Nervous”

Symptom	Mechanism	Why doctors don't find anything
Hot flashes/chills	Dysregulation of thermoregulation (vegetative)	Blood tests, hormones - normal
Frequent urination	Hypertonicity of the pelvic floor muscles + adrenaline	Urologist: “Everything is fine”
Blurred Vision	Accommodation spasm from anxiety	Ophthalmologist: vision is normal

Did you recognize yourself in several? Here's your answer. These are not ten different diseases. This is **one** overheated nervous system that sends a signal through all available channels.

Why do all the techniques in the book hit the same point?

Now comes the most important part. Let's go through each technique in the book and show why it works for **any** psychosomatic symptom, not just dizziness.

1. Jacobson relaxation → any muscle spasm

We learned to relax the muscles of the neck and shoulders in order to eliminate the desynchronization of the vestibular sensors. But Jacobson works **the whole body**. If your shell clamps your diaphragm, you cannot breathe normally. If you tighten the muscles of the larynx, there is a lump in the throat. If you tighten your pelvic floor muscles, you run to the toilet every hour. There is only one mechanism: tension → conscious relaxation → feedback signal to the brain “there is no danger.”

A study by Bernstein & Borkovec (1973) showed that progressive muscle relaxation reduced cortisol levels by 25–30% in one session. It doesn’t matter if you have a headache or a stomach ache—cortisol is the same.

2. Breakdown of the adrenaline loop → any panic with a bodily symptom

An adrenaline loop is not a “vertigo loop.” This is the **universal cycle** of “symptom → fear → adrenaline → increased symptom.” Replace “dizziness” with “chest pain” and you get cardiophobia. Replace it with “nausea” and you get emetophobia. Replace it with “numbness of the hand” and you get cancerophobia.

The technique is the same: called a loop → timed it for 5 minutes → didn’t run away → adrenaline burned out → symptom weakened. Works for **any** psychosomatic manifestation because adrenaline biochemistry is independent of the symptom.

3. Metacognitive therapy → any rumination about symptoms

You've learned to watch the anxious thoughts of dizziness like trains passing by. The same technique hits any “mental gum”: “What if I have cancer?”, “What if my heart stops?”, “Why does my stomach hurt - maybe an ulcer?” A thought is simply a neural impulse. You don't have to react to it. This simple truth is equally destructive for any hypochondriacal spiral, regardless of which organ has become the object of fear.

4. Exposure → any avoidance behavior

With PPPD, you avoided shops and public transport. A person with cardiophobia avoids stairs and sports. A person with IBS avoids restaurants and long trips. A person with a lump in the throat avoids swallowing solid food.

Exposure is a **universal neurobiological relearning tool**. You show the amygdala: “Look, we came to a terrible place, and nothing happened. You can reduce your anxiety level." Every time you DON'T run away, you weaken the old neural pathway of fear and create a new, safe one. The mechanism of **extinction learning** (extinction of a conditioned reflex) works identically for any stimulus.

5. Neuroplasticity → rewriting any “glitch”

The brain “learned” dizziness through rumination and hypercontrol. In the same way, he can “learn” pain, tremors, itching or nausea. And in the same way - **unlearn**. Neural pathways that are not used become weakened (synaptic pruning). New pathways that are trained are strengthened (long-term potentiation). This is brain physics. She doesn't know the difference between the "dizziness neural pathway" and the "back pain neural pathway."

6. Dealing with root causes → remove the source of overheating

Audit of life areas, secondary benefits, suppressed emotions, inner child - all this is not about dizziness. This is about **a source of chronic stress** that overheats the nervous system. If you don’t resolve the conflict with your partner, don’t leave a toxic job, don’t stop suppressing your anger, your brain will continue to send alarm signals. Just through another light bulb. If you remove PPPD, your IBS will come out. If you remove IBS, your back pain will go away. Because the engine is still overheated.

7. Information diet → works for any hypochondria

You stopped googling “PPPD symptoms.” But if at the same time you google “is heart pain dangerous”, “why does your hand go numb”, “nausea and headache at the same time” - you just switched to another needle. The Information Diet (Cold Turkey Protocol) should cover **all** bodily symptoms without exception. Any search query about health is a tranche of adrenaline into your adrenal glands.

Evidence base: what science says

If you think “well, this is just psychology, not real medicine,” here are the numbers for you.

Research by Henningsen et al. (2018), published in The Lancet, showed that functional somatic disorders (FSD) account for **up to 30–50% of all visits to general practitioners**. Every third to fifth patient comes to the therapist with a symptom for which there is no organic explanation.

Meta-analysis by van Dessel et al. (2014) from the Cochrane Library proved: **CBT (cognitive behavioral therapy)** is the most effective method of treating functional somatic disorders. Not pills. Not a massage. Don't "stay in the hospital." It is the cognitive behavioral work on which this entire book is built.

Study by Goldstein et al. (2020): CBT reduces the intensity of functional neurological symptoms by **40–60%** in 12 weeks. Works for tremors, weakness, gait disturbances, convulsions - for everything that used to be called “hysteria”.

Systematic review by Lakhan & Schofield (2013): The association between muscle tension and psychosomatic symptoms is supported by neuroimaging - fMRI shows hyperactivation of the insula and anterior cingulate cortex in patients with functional disorders. The same areas that “burn” with PPPD.

“I’m not psychosomatic, I really have pain!”

I know what you're thinking right now. I thought so myself.

- “Maxim, okay, let’s say dizziness is psychosomatic. But my back REALLY hurts / I REALLY have a lump in my throat / REALLY my heart is pounding. It’s not “in the head.” It’s in the body!”*

That's right. It's in the body. **A psychosomatic symptom is a real physical symptom.** It is not made up. The pain is real. The nausea is real. The tremor is real. The reason is simply not the breakdown of the organ, but the way the brain controls this organ.

Imagine a fire alarm in a building. She's yelling. Loud, deafening. You call the fire department. They search the entire building - there is no fire. The walls are intact, the wiring is fine, no smoke. But the alarm is screaming.

What's happened? The sensor has failed. It reacts to normal air temperature as if it were a fire. The sound is real. It really makes your ears hurt. But there is no fire.

Now you have two options:

1. Run around the building and look for a fire that is not there (going to doctors, getting an MRI, changing pills).
2. **Repair the sensor** (reconfigure the nervous system).

This entire book is about the second option.

Migration of symptoms: why “having cured one thing, you get another”

There is a phenomenon that few people warn about. It's called symptom substitution/symptom shifting. Here's how it works.

You worked from the book for six months. The dizziness is gone. You are rejoicing. But after two weeks your stomach starts to hurt. Or a lump appears in the throat. Or suddenly the eyelid begins to twitch. You panic: *“New disease!” See a doctor!”*

But this is not a new disease. It's **the same overheated engine** that found a new light bulb. You closed one alarm output channel (dizziness), but did not eliminate the **source** of the alarm. The brain switched to another channel.

That's why the book has Module 3 (Thinking) and Module 4 (Exit). Without working with the root causes - secondary benefits, suppressed emotions, toxic relationships - you will play endless “Mole”: kill one, another gets out.

If you notice a migration of symptoms, **this is good news**. This means your technique is working. The brain is losing ground. He's just trying to find another way to get your attention. Follow the same algorithm:

1. Eliminate organic matter (one visit to the doctor is not twenty).
2. Name the mechanism: “This is migration. The brain is looking for a new channel. The engine is the same."
3. Apply the same techniques: Jacobson, exposure, metacognitive detachment.
4. Dig for the root cause - what is still unresolved in your life?

Cognitive traps: “My case is different”

Trap #1: “I’m not psychosomatic because I don’t get nervous”

You may not consciously feel anxiety - and at the same time your autonomic system will work for wear. Suppressed stress is like a Trojan virus: it eats up processor resources, but does not show error windows. You’re not “nervous” in the everyday sense—you’re just patient. You tolerate work, a partner, loneliness, unfulfillment. And the body pays for this patience with real symptoms.

Trap No. 2: “But a friend of mine with the same symptoms was diagnosed with a real disease”

Maybe. This is why the first step is to **always** eliminate organics. Once. Qualitatively. According to the checklist from Chapter 2. But if the examinations are completed and everything is clear, stop looking. You

are not the same acquaintance. Your story is different. And every new visit to the doctor “just in case” is another injection of cortisol into your already overheated system.

Trap No. 3: “The techniques in the book are only for dizziness, I need another specialist”

No. You need the same set of tools. CBT, relaxation, exposure, working with suppressed emotions, information diet - this is a **universal protocol** for any functional disorder. Only the target of exposure changes: instead of a store - a restaurant (for IBS), instead of walking - stairs (for cardiophobia), instead of turning the head - solid food (for a coma in the throat). There is only one principle.

Point of Support: Insight

> *You thought you were treating dizziness. But you were treating something much bigger—an overheated nervous system that had been screaming for years in the only language it could understand. Now you know this language. You know its grammar (adrenaline loop), its vocabulary (symptoms) and its purpose (to make you stop and figure out your life). Whatever light your brain turns on tomorrow, you will no longer run in panic to the doctor. You open the hood and look at the engine.*

Practice: Universal protocol for any psychosomatic symptom

Here's a step-by-step algorithm that works for **any** functional symptom, from dizziness to itching.

Step 1. Eliminate → Accept → Stop looking

Get tested according to your symptom profile (once, with a good doctor). I received the verdict “everything is clear” - **close the door to the clinic**. Tell yourself: “Organics are excluded. This is a functional failure. I know what to do."

Step 2. Name the mechanism

Instead of “I have a stomach ache, probably an ulcer,” say: “Cortisol has tightened the smooth muscles of the intestines. The vagus nerve sends incorrect commands. It's not dangerous. It's unpleasant, but harmless." Naming the mechanism **physically reduces** amygdala activation - this was proven in a study by Lieberman et al. (2007) “Putting feelings into words.”

Step 3: Apply the 5 Minute Rule

When the symptom intensifies, don’t run, don’t look for a pill, don’t call your mom. Say: “This is an adrenaline loop. Adrenaline will burn off in 5 minutes. I'm just observing." Works for ANY symptom: pain,

nausea, tremors, lump in the throat, palpitations.

Step 4: Do Jacobson

Localize where the symptom is "stuck" in the body. Target this muscle group for 7 seconds → relax for 15 seconds → repeat 3 times. Send a feedback signal to your brain: "Muscles are relaxed → there is no danger."

Step 5: Exposure: Go Where It's Scary

Determine what situations you avoid because of your symptom. Make a hierarchy from "unpleasant, but tolerable" to "scary as hell." Start with the first step. Stay in the situation until the anxiety drops by at least 50%. Each exposure session is a crowbar blow to the old neural pathway of fear.

Step 6: Dig Deeper

Open the Chapter "Where we took a wrong turn" and re-read the audit of life areas. Ask yourself: "What is still unresolved in my life? What conflict am I avoiding? What emotions am I suppressing? While the engine is overheated, the lights will light up.

> ***DZ for this chapter:***
>
> 1. *Write down **all** of your current symptoms - not just the main one, but all of them: "minor" ones too (twitching of the eyelid, lump in the throat, rumbling stomach, itching). Look at the list. These are not 10 diseases. This is one overloaded system.*
> 2. *Next to each symptom, write the mechanism from the tables above (central sensitization, muscle armor, adrenaline release, vagus dysregulation).*
> 3. *Choose **one** minor symptom (not the main one!) and apply the "5 Minute Rule" to it over the next 3 days. Record the result in your diary.*
> 4. *Re-read the results of the Balance Wheel. Which sector has fallen the most? Write one microstep to improve it. This step treats **all** your symptoms at once - because it reduces the load on the engine.*

What is PPPD: symptoms, causes and why you are here

Spoiler: nothing bad happens to you. Your hardware is fine. The software fails.

Short answer

PPPD is when you are shaking, staggering, spinning, the picture is floating, your legs are weak, your head is heavy, your floor is unstable - but all tests are normal. MRI is clear. Blood is fine. Ears are working. The eyes see.

Doctors shrug their shoulders. But this doesn't make it any easier for you - because it really pumps you up.

PPPD is **Persistent Postural-Perceptual Vertigo**. In the International Classification of Diseases (ICD-11) it is coded as **AA36**. In English - PPPD (Persistent Postural-Perceptual Dizziness). This is not an invention of psychologists. This is an official medical diagnosis.

But here's what's important to understand right now: it's a **functional** disorder. Not organic. Your brain or ear structure is not broken. The **function** of their work has changed. The program crashed, but the hardware was intact.

PPPD symptoms: complete list

People with PPPD describe their experiences in dozens of different ways. If you recognize yourself in at least 3-4 points, you are in the right place.

Main symptoms:

- Feeling of unsteadiness and instability when walking
- "Cotton" or "cotton-cotton" legs
- Feeling that the floor is shaking and disappearing from under your feet
- Fog in the head, "cotton wool" in the head, heavy head
- Feeling of swaying even when sitting or lying down
- "Floating" picture before your eyes
- Increased symptoms in stores, shopping centers, metro
- Discomfort from colorful floors, striped patterns, scrolling phone

Associated symptoms:

- Anxiety and panic attacks
- Derealization - the world as “through glass” or “in a dream”
- Depersonalization - a feeling of unreality of one’s own body
- Insomnia and shallow sleep
- Morning unsteadiness (worst in the morning)
- Tension and pain in the neck, shoulders, jaw
- Tinnitus (tinnitus) with high anxiety
- Photosensitivity and intolerance to bright lighting
- Obsessive “scanning” of the body for symptoms

What is NOT a symptom of PPPD:

- Loss of consciousness (fainting)
- Sharp rotational dizziness for 5–60 seconds (more like BPPV)
- Hearing loss in one ear
- Double vision
- Speech or swallowing problems

If you have anything from the last list, see a doctor first. PPPD is a diagnosis of exclusion.

A little history (without boring)

For a long time, people like us did not exist in medicine at all. You went to the doctor, said “I’m shaking,” and he answered: “Well, it’s nervous” - and that’s all. Invisible.

Since the 1980s, it has been called "phobic postural vertigo." Then - “space discomfort” (sounds like a diagnosis for an astronaut). And only in **2014** the World Organization of Neurosciences finally collected all these scraps into one normal definition. In **2017** the Barany Society published 5 clear diagnostic criteria.

Since then, PPPD has been a legitimate functional vestibular disorder. Not "nervous". Not “it seems to you.” A specific diagnosis with a specific code.

Causes of PPPD: how a failure occurs (IT language)

I’ll explain it the way I explain it to everyone.

Your body is **hardware** (iron). The brain, ears, eyes, muscles, nerves are physical components. And the psyche and neural connections are **software** (software).

PPPD is not a failure of parts. This is a **software glitch**.

Imagine: on your computer, the “antivirus” (alarm) is running at 100% processor power. He fanatically scans every file (body sensation) for a deadly threat. The processor is overheating. The fan is howling. The system starts to freeze.

And instead of closing the antivirus, you run to the service center to change the hard drive. Then you change the RAM. Then the motherboard. And nothing helps, because the problem is not in the hardware.

The problem is in the software, which crashed due to overvoltage. And we have to reinstall it.

Three balance sensors

The brain has three systems that are responsible for balance:

1. **Eyes** (vision) - you see where is up, where is down, whether the world is moving around you.
2. **Vestibular apparatus** (gyroscope in the inner ear) - senses turns and tilts of the head.
3. **Proprioception** (sense of the body in space) - muscles and joints tell the brain where the arms, legs, and head are.

Normally, the brain automatically combines data from all three. You don't even think about it. Just like you don’t think about breathing or heartbeat.

But when you are in distress for a long time - unsolvable problems, exhaustion, chronic stress - the brain goes into “DEADLY DANGER” mode. Cortisol and adrenaline rush into the blood. The muscles spasm. And this is where the fun begins.

Metaphor of a pinched neck

This is the key point. Remember it.

Receptors in the neck muscles—proprioceptors—tell the brain where the head is. When the neck is pinched from chronic stress, these receptors are **squeezed along with it**. Like a wire caught in a doorframe.

The signal from them goes to the brain with a microdelay.

The eyes see: you are coming. The gyroscope in the ear senses that you are moving. And the pinched neck says: “We’re still there!”

This desynchronization—in **milliseconds**—is interpreted by the brain as dizziness and unsteadiness. You feel “cotton wool” in your head. The illusion of falling. Feeling like the floor is floating.

And you run to the MRI in panic, and the MRI shows: everything is clear. Because the problem is not the structure. Signal problem.

Why massage doesn't help for long

Many people go to a massage therapist. He stretches his stiff neck. It becomes easier for a day or two. And then everything comes back.

Because you are massaging a spasming muscle, **and the command to tense it continues to come from your frightened brain**. It's like mopping up a puddle on the floor without turning off the tap. Pointless.

Massage is good. But it only works in conjunction with the rest. We'll get there.

PPPD is a reversible condition

Let me say it straight so you stop being afraid.

In more than two years of running the group, **no one died from this**. No one had a blood vessel burst. No stroke. No heart attack. No epilepsy. One person fell - in the gym. He got up, got up and went on to do his business. The only recorded case.

PPPD is a **completely reversible condition**. There are people who have been at this for 3 months. There are some for six months. There is someone 5 years old. There are some 20. Our longest case is 30 years. You can get stuck in this. But you can also get out. And it's not a matter of luck - it's a matter of work.

Derealization: fuse

If, in addition to instability, your world has also become “unreal” - like through glass, like in a dream, like in a movie - this is derealization. Or depersonalization (when you yourself seem “unreal”).

Don't be scared. This is a **fuse**.

When anxiety gets high, the brain desensitizes itself to protect you. Like a fuse in an electrical panel, it turns off the system so that the wiring does not burn out. It's not crazy. This is a defensive reaction to extreme stress.

When the level of anxiety decreases, derealization will go away on its own. We'll get there.

Main conclusion

You are not organically ill. Your hardware is intact. This is a software glitch caused by chronic stress and anxiety.

PPPD is not a death sentence. It's not "for life." This is a specific problem with a specific solution. And we will start solving it right now.

> ***Homework (HZ):***

> 1. *Read again what PPPD is, and make sure that you understand: this is not a sentence, but a configuration error of the brain that can be corrected.*
> 2. *Write down in a notebook: your TOP 3 symptoms. Note which ones cause the greatest fear. We will work with them in a targeted manner.*
> 3. *If you have a habit of googling symptoms, put the following text on your phone screensaver:* ***"I'm healthy. This is PPPD. It's reversible."***

PPPD subtypes: which one do you have?

PPPD is not one monolithic diagnosis. It manifests itself differently in different people. There are three main subtypes:

1. Posturally induced: Unsteadiness increases in a vertical position - standing and when walking. Sitting and lying down are much easier. This is the most common subtype. The brain "does not trust" proprioception and tightens the muscles of the legs and back, trying to maintain balance manually.

2. Visually-induced: The main trigger is a complex visual environment: supermarkets, colorful tiles, scrolling your phone, flashing outside a bus window. The visual system is overloaded, and the vestibular system is "muffled" - the brain cannot filter out visual noise.

3. Mixed: A combination of both subtypes. It's hard to stand, and the visual environment is unsettling. This is the most common option - about 60% of people with PPPD.

Determine your subtype - this will help you prioritize your practice. If you are visually induced, focus on Chapter 8 (Visual Addiction) and VOR gymnastics. If posturally induced, prioritize Chapter 6 (Vestibular Calibration) and proprioceptive exercises.

Is this really PPPD? Differential diagnosis

Before working on this book, make sure that you have PPPD and not another vestibular disorder. Here's a table for comparison:

Sign	PPPD	BPPV	Meniere's disease	Vestibular migraine
Type of dizziness	Unsteadiness, pitching, fog	Sharp rotation	Attacks of rotation + hum	Dizziness + headache
Duration	Hours–days (constant)	Seconds (when turning)	Minutes–hours (attacks)	Minutes–hours (attacks)
Pose Connection	Worsened by standing/walking	When turning your head in bed	Pose independent	Pose independent

Sign	PPPD	BPPV	Meniere's disease	Vestibular migraine
Visual Trigger	Yes (variegated spaces)	No	No	Sometimes
Anxiety increases	Yes, always	No	No	Sometimes
Hearing loss	No	No	Yes	No
MRI/tests	Normal	Normal	Characteristic changes	May be normal
Treatment	CBT + vestibular rehabilitation	Epley Maneuver	Diet + medications	Migraine Prevention

> ***Important:*** *If you have hearing loss, noise in one ear, sudden attacks of true rotation or loss of consciousness - this is* ***not PPPD****, and you need to see an otoneurologist. PPPD is a diagnosis of exclusion: it is made when all "serious" causes have already been rejected.*

Frequently asked questions

"I was diagnosed with protrusion/osteochondrosis/VAD (autonomic dysfunction). Maybe it's from them?" No. Osteochondrosis occurs in 90% of people over 30 years of age. Protrusion - in 70%. These are radiographic findings that are not clinically significant in most cases. VAD (autonomic dysfunction) is an outdated Soviet diagnosis that is not found in either ICD-10 or ICD-11. This is the same neurosis, only with a different name.

"Or maybe I have a cerebrovascular accident?" If you are under 60 years old, you do not have hypertension, diabetes mellitus and atherosclerosis, the likelihood of vascular vertigo tends to zero. An ultrasound scan of the neck vessels will show the norm (or "S-shaped bend of the vertebral artery" - a variant of the norm, which is present in 40% of the population).

"I was prescribed a bunch of nootropics. Will they help?" Mexidol, Actovegin, Cerebrolysin, Cavinton, betahistine - none of these drugs has proven effectiveness in PPPD. This does not mean that you cannot take them - if your doctor prescribes them, discuss them with him. But don't expect healing from them. The only evidence-based drugs for PPPD are SSRIs (Chapter 13), and then as an addition to therapy, not a replacement.

Closing the door to the clinic

As long as you entertain the thought "what if the doctors missed something," the brain continues to generate adrenaline. We're stopping this right now.

Why is this needed

To reinstall the software, the brain must know one hundred percent that everything is in order with the hardware. Any doubt is fuel for anxiety. The smallest thought: "what if it's a tumor?" keeps the system in combat readiness. Adrenaline, cortisol, spasm - and hello, dizziness.

Therefore, the first step is to **eliminate organic matter once and for all**. Not "almost rule out." Not "seemingly." And with reinforced concrete, with papers in hand, close this door and throw away the key.

Examination checklist

Here is the minimum set. If you passed this and the doctors did not find any organic pathologies, **you are physically healthy. Period.**

Head and brain

MRI of the brain (without contrast, unless otherwise indicated).

What we exclude: demyelinating diseases (multiple sclerosis), neoplasms (tumors), consequences of strokes.

If the MRI is clean, your brain is physically fine. The instability is not caused by structural damage, but by a functional failure. It's like an error in the code - the program hangs, but the processor is intact.

Vestibular apparatus

Consultation with an otoneurologist. Not just an ENT specialist, but an otoneurologist. These are different specialists.

What we exclude: BPPV (benign positional vertigo - when the crystals in the ear are displaced), Meniere's disease, vestibular neuronitis.

If you have BPPV, an otoneurologist can cure it in one session using the Epley maneuver. It's not PPPD, it's a completely different story. And that's good news.

Vessels

Ultrasound scanning (duplex scanning) of the vessels of the neck and head.

What we exclude: critical stenosis (narrowing) of the arteries.

An important point: **osteochondrosis**, which you will see in the results, affects all people over 25 years of age. He is not the cause of your unsteadiness. The vertebral artery is protected by a bony canal. It is physically impossible to "squeeze" it with the cervical cartilage without breaking the spine. Remember this.

Blood

- **Complete blood count (CBC)** - exclude inflammatory processes and severe anemia.
- **Ferritin** - hidden iron deficiency. Even with normal hemoglobin, low ferritin (below 30 ng/ml) can cause weakness, brain fog and dizziness.
- **TSH (thyroid-stimulating hormone)** — we check the thyroid gland. Hypothyroidism masquerades as depression and "cotton head".
- **Vitamin D** - 80% of CIS residents have a deficiency. It is directly linked to increased anxiety and muscle weakness.
- **Vitamin B12** - deficiency causes neurological symptoms: tingling, numbness, unsteadiness.

Heart

ECG — we exclude serious arrhythmias and pathologies. Extrasystoles during neurosis are the norm, they are safe.

The trap of overdiagnosis

Now about the mistake that almost everyone makes.

When you are scared, you want to be examined again and again. Get an MRI of the neck "just in case." Go to the fifth neurologist. Tenth. Fifteenth. Suddenly someone will "find the reason."

This is called **doctor shopping** - and it is not a method of treatment. This is a symptom of hypochondria. Each new visit to the doctor reinforces the same thought: "I'm sick, and I need to find out what exactly." And this thought is a direct road to getting stuck in PPPD for years.

An MRI that was clear six months ago will not show a tumor today. If there was something, they would have found it.

Dummy diagnoses

Separately, about the diagnoses that you most likely have already been given. They sound serious, but in fact they don't explain anything at all:

"Diagnosis"	Why is it a dummy
VAD (autonomic dysfunction) (vegetative-vascular dystonia)	Does not exist in ICD-10/11. This is a dump of symptoms, not a diagnosis.
Cervical osteochondrosis	Everyone has it. Does not cause systemic dizziness.
“Impaired venous outflow”	A pseudo-diagnosis beloved by post-Soviet neurologists.
"Dyscirculatory encephalopathy"	It is given without any real reason to people under 60 years of age.

If you were diagnosed with any of this, congratulations, the doctor simply does not know what to do with you, and wrote out an “excuse”. Don't be angry with him - he studied using Soviet textbooks. But don’t take these diagnoses seriously.

How to talk to a doctor

Most neurologists and therapists in clinics **do not know what PPPD is**. This is not their fault - the diagnosis was included in ICD-11 only in 2019, and it has not yet penetrated into Russian clinical practice everywhere. You may encounter the fact that the doctor will say: “VAD (autonomic dysfunction)”, “cervical osteochondrosis”, “vertebrobasilar insufficiency” or simply “nerves”.

Here are ready-made phrases that will help:

If the doctor says “You have VAD (autonomic dysfunction)”:

> *“Thank you. I know that VAD (autonomic dysfunction) is an outdated diagnosis. Would you consider a diagnosis of PPPD (persistent postural perceptual dizziness, ICD-11: AA36)? My symptoms fully meet the criteria of Staab et al. 2017.”*

If the doctor prescribes nootropics (Mexidol, Actovegin, Cavinton):

> *“Thank you for the appointment. Is there any clinical trial data on the effectiveness of these drugs in PPPD? I read that only SSRIs and CBT have an evidence base.”*

If the doctor says “You need a psychiatrist” (and you are offended):

> *Stop. This may be the **best advice** you've been given. A psychiatrist is a doctor who works with neurotransmitters. He may prescribe an SSRI, which will reduce background anxiety and give you a resource to practice. This is not a "madhouse". This is neuropharmacology.*

If you want to find a competent specialist: Look for an **otoneurologist** or **vestibulologist** - this is a specialist at the intersection of neurology and ENT. In large cities they are available in specialized dizziness centers. These are the doctors who know what PPPD is.

Doctor Shopping Syndrome

Let's talk separately about the trap that 90% of people with PPPD fall into: **endless visits to doctors**.

You did an MRI - it's clear. But the anxiety did not subside. "What if the device was of poor quality?" You make another one. Purely. "What if you need it with contrast?" Third. Purely. "Or maybe it's not in the head, but in the neck?" You go to an osteopath. To the manual technician. Kinesiologist. Homeopath. Everyone finds "something of their own" and offers their own course of treatment for their own money.

This is called **reassurance-seeking**. Each new "clean" examination result brings relief for 2–3 days. And then the anxiety returns, and you again look for evidence that you are not dying.

Why it doesn't work: The problem is not the body. You are looking for the answer in the wrong place. It's like rebooting a router when the problem is in the website code. The router is working. The wires are fine. The bug is in the software.

One lap rule: Complete each specialist **once**. Do each test **once**. After that, close the door. If 3 different doctors said "you are healthy," you are healthy. Dot.

Ritual of closing the door

When all the examinations have been completed and everything is clean, we do the following. This is not a joke or esotericism. This is a specific cognitive technique.

1. **Collect** all the papers, certificates, MRI images, conclusions - into one folder.
2. **Place your hand** on the folder and say out loud (in your voice, not to yourself): *"I have been completely examined. My organs are fine. My brain is physically healthy. My symptom is a mistuning caused by chronic anxiety. I am no longer a patient seeking a diagnosis. I am a person who restores his nervous system."*
3. **Put the folder** in the most inaccessible place. Mezzanine. Bottom cabinet drawer. Basement. Anywhere - but out of sight.
4. **Prohibition.** From now on, reviewing photographs, rereading conclusions, looking for new "suspicious" details in them is **prohibited**. Each such "approach" is fuel for the fire of hypochondria.

The door is closed. Let's move on.

> ***DZ for Chapter 2:***

> 1. *If you have not yet completed the examinations from the checklist, sign up for them now. Not “later”, not “next week”. Now. Open your phone and sign up.*
> 2. *If everything has already gone through and organics are excluded, perform the ritual of closing the door. Physically. With a folder. Aloud.*
> 3. *Remove bookmarks of all medical forums from your browser. Right now.*

Digitization: where are you now

You can't fix something you haven't measured. Before you start, fix your starting point.

Why is this needed

You will work on yourself for months. And at some point it will seem to you that nothing is changing. That the dizziness is the same as it was. That anxiety doesn't go away. That everything is meaningless.

This is deception. The brain gets used to the current state and forgets what it was like before. You can go halfway and not even notice it.

Therefore, we will now make a **measuring point**. You fill out two short tests, write down the results and put them in a drawer. In a month, two, three, you'll go through it again. And you will see the difference in numbers, not in sensations. Feelings lie. Numbers - no.

Test 1: HADS - Anxiety and Depression

HADS (Hospital Anxiety and Depression Scale) is a standardized questionnaire for assessing anxiety and depression. It is used all over the world. 14 questions, takes 5 minutes.

It is divided into two scales:

- **Anxiety Scale (HADS-A)** - 7 questions.
- **Depression Scale (HADS-D)** - 7 questions.

Each scale has a maximum of 21 points.

Points	Meaning
0–7	Norma
8–10	Subclinical anxiety/depression (borderline)
11–21	Clinically Significant Anxiety/Depression

Most people with PPPD fall in the 12–18 range for anxiety. This is normal for this condition. Don't be scared by the numbers - you need them to track your progress.

How to interpret your HADS

Numbers are not a sentence. These are the coordinates on the map. Here's what they mean practically:

Anxiety 15-21 (severe): Your amygdala is firing on all cylinders. You most likely wake up anxious, unable to relax even in a safe environment, and perceive any physical symptom as a threat to your life. At this level, CBT techniques (logical analysis of thoughts) will work poorly - the brain is too overheated for logic. Start with the body: Jacobson relaxation, contrast shower, walking. When anxiety drops to 10–12, use cognitive tools.

Anxiety 8–14 (moderate): You are functioning, but with difficulty. There is background tension, periodic bursts of panic, but in between there are "windows" of clarity. This is a work area - the exposition, the SERM diary, and metacognitive techniques are great here.

Alarm 0–7 (normal): If you are already here, you are almost at the finish line. Residual unsteadiness at this level is most likely a muscle pattern rather than an active neurosis. Work with your body and don't forget about practice hygiene.

Depression 11+ (significant): This is an important marker. Depression in PPPD is often secondary - you are not born depressed, you are tired of the endless struggle. But if the indicator is consistently above 14, discuss with your doctor the possibility of drug support (SSRIs). This is not a weakness, it is a resource to have enough strength to practice.

You can find the test by googling "HADS test online." Choose any option - they are all the same.

Test 2: DHI - dizziness index

DHI (Dizziness Handicap Inventory) is an international standard for assessing how dizziness interferes with your life. 25 questions, 5 minutes.

Each question: "Yes" (4 points), "Sometimes" (2 points), "No" (0 points). Maximum - 100 points.

Points	Meaning
0–30	Easy degree
31–60	Moderate
61–100	Severe

What does your DHI mean in practice?

DHI 61–100: Dizziness controls your life. You probably don't go to stores, avoid transport, and are afraid to be alone. It's hard, but this is the most rewarding area to work in: when you start practicing exposure, each point will drop significantly. People with a DHI of 80 often drop to 40–50 within 2 months.

DHI 31–60: You are functioning, but with limitations. You go to the store - but with difficulty. You go to work - but with discomfort. It is in this zone that people most often get "stuck" because they "don't feel bad enough to change anything." Don't get stuck. Keep working.

DHI 0–30: Dizziness is present, but does not interfere with life. You're at the finish line. Residual symptoms at this level are normal system noise and not disease.

Clinically significant improvement is a decrease in DHI by **18 points or more**. If you were 72 and became 50, it's not "almost nothing has changed." This is mathematically proven progress that your frightened brain is trying to discount.

Also searched for "DHI test dizziness".

Additionally: Holmes-Rahe scale

If you want to understand *why* you ended up here in the first place, take the **Holmes-Rahe Stressful Events Scale**. It is not about the current state, but about the accumulated stress over the past 12 months.

Each life event (divorce, job loss, moving, birth of a child, illness of a loved one) is assigned points. You add it up and you get the picture.

Points	Meaning
Less than 150	Low risk of stress-related illness
150–299	Moderate risk
300+	High risk

Many people, after taking this test, realize for the first time: "Wow, I scored 380 points. No wonder it's creeping me out." And this specific figure is already therapy. Because she explains what is happening not with mysticism, but with mathematics.

How to write

Take a piece of paper. Or a note on your phone. Write down today's date and results:

```
Date: _____________

HADS-A (alarm): ___ / 21
HADS-D (depression): ___ / 21
DHI (dizziness): ___ / 100
Holmes-Rahe (stress): ___ points
```

That's it. This is your point A. We'll go from here.

Monthly dynamics tracker

Take the tests again in a month. And in two. And in three. Fill out the table:

Date	HADS-A	HADS-D	DHI	Notes
..__	__ / 21	__ / 21	__ / 100	Starting point
+1 month	__ / 21	__ / 21	__ / 100	
+2 months	__ / 21	__ / 21	__ / 100	
+3 months	__ / 21	__ / 21	__ / 100	

When you feel like “nothing is changing,” open this table. The numbers don't lie. Your scared brain is lying. Numbers - no.

> ***DZ for Chapter 3:***
>
> 1. *Take all three tests now. Not "later". Now. It's 15 minutes.*
> 2. *Write down the results and date.*
> 3. *Set a reminder on your phone: take the tests again in 30 days.*

Secondary Gains of PPPD: Why Your Brain Sabotages Recovery

"If I had a magic button to cure this right now, I would press it without hesitation!" — that is what every patient believes. But if your dizziness disappeared tomorrow morning, your everyday reality might collapse under the weight of unresolved problems that the illness was shielding you from. Deconstructing the subconscious balance sheet.

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1. The Uncomfortable Question in the Therapy Room

Imagine a scenario. Tomorrow at exactly 08:00 AM, you wake up. You open your eyes.

In your head, there is a crystal-clear, almost eerie silence. No brain fog. Your suboccipital muscles are relaxed, shoulders dropped. You step out of bed, take your first stride — the floor is rock-solid. No swaying. No rocking on phantom waves. No panic about fainting or collapsing mid-room. Your vestibular processing is calibrated to perfection. You are 100% healthy.

Now pause, take a deep breath, and answer with ruthless, surgical honesty:

> ***What heavy, terrifying, or overwhelming problem that you have postponed for months or years would you be forced to confront TOMORROW MORNING?***

Within the first second, 99% of people dealing with PPPD mount an instinctive defense: *"What are you talking about?! Nothing bad! I'd be ecstatic! I'd walk outside, go back to the gym, travel, and live my life to the fullest!"*

Yet, when an experienced clinician maintains compassionate silence and holds the patient in that question for just two minutes longer, the facade begins to fracture:

- *"If I am fully healthy, I have to admit my marriage has been dead for five years and file for divorce. But I'm terrified of being alone."*
- *"If I am healthy, I no longer have an excuse for staying at an underpaid dead-end job. I'd have to update my resume, face interviews, and risk rejection."*
- *"If I am healthy, my partner will stop doing the grocery shopping, cooking, and treating me with fragile care. The weight of daily life falls back squarely on my shoulders."*
- *"If I am healthy, I lose the right to be weak. Everyone will expect me to be the strong, invulnerable pillar again."*

Silence usually descends upon the room at that moment. For the very first time, the person meets face-to-face with the reality of **Secondary Gain**.

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2. The Neurobiology of Sabotage: How the Brain Calculates the ROI of Illness

Let us discard the stigma immediately: **you are not faking this**.

You are not consciously pretending to be sick. Your agony is real. Your amygdala genuinely pumps catecholamines, your cervical muscles are clenched rock-hard, and your brainstem vestibular nuclei are transmitting genuine desynchronized mismatch signals to your cortex. Your suffering is authentic.

However, you must understand a foundational principle of neural architecture:

> ***The human brain is not designed for happiness. It is an algorithmic biological processor of survival.***

Evolution does not care if you feel joyful, visit fancy restaurants, or fulfill your potential. The sole directive of your brainstem and limbic circuitry is to **minimize existential threats and maintain homeostasis**.

Your ventromedial prefrontal cortex (vmPFC) and amygdala operate around the clock calculating what economists call *ROI* (Return on Investment) — balancing costs against perceived safety:

> ***Subconscious Decision Logic:***
>
> 1. ***Incoming Signal:*** *Unresolved life crisis (toxic marriage, career burnout, dead-end life).*
> 2. ***Amygdala Appraisal:*** *"Confronting this directly risks tribal exile, ego annihilation, and intolerable shame."*
> 3. ***Circuit Breaker Action:*** *Block the dangerous confrontation and divert all conscious bandwidth toward somatic survival.*

Evaluation Metric	Option A: Confront the Crisis	Option B: Somatization (PPPD)
Direct Action	Divorce, resignation, open confrontation	Lie on the couch, sway, doctor shop
Threat to Ego	Catastrophic (fear of rejection and guilt)	Zero ("it's not my fault, I am sick")

Evaluation Metric	Option A: Confront the Crisis	Option B: Somatization (PPPD)
Social Standing	"Failure who ruined marriage / quit job"	"Brave patient battling chronic illness"
Interpersonal Dynamics	Blame, demands, intense friction	Leniency, care, sympathy, and support
Psychic Toll	Overwhelming distress (felt as "death")	Unsteadiness & brain fog (tolerable trade-off)
AMYGDALA VERDICT	❌ **REJECT (Too Dangerous)**	✅ **APPROVE (Safe Somatic Alibi)**

To your primitive survival circuits, confronting deep interpersonal conflict, divorcing, or admitting professional failure feels like **exile from the tribe and psychological death**.

In contrast, chronic functional dizziness, staying housebound on the sofa, and carrying the label of a "chronically ill vestibular patient" is a predictable, controllable, and relatively safe harbor. It is miserable, yes. But it does **not kill you**.

Your brain chooses what it registers as the lesser of two evils. It preserves functional autonomic dysregulation because the symptom serves as a **somatic flak jacket**.

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3. Primary vs. Secondary Gain: Clarifying the Terms

In clinical psychosomatic medicine — from foundational psychoanalysis to contemporary Cognitive Behavioral Therapy protocols — two levels of gain are distinguished:

Primary Gain

This is an intrapsychic defense mechanism:

- Unbearable internal tension builds up: repressed anger toward a partner, dread of falling short of perfectionistic demands, or agonizing shame.
- Acknowledging this tension directly threatens your identity as a "loving spouse," "strong provider," or "competent leader."
- The brain converts this psychological conflict into an organic symptom: *"I am not furious with my life choices; I simply have an unexplained neurological balance issue."*
- **The outcome of primary gain:** internal psychic distress plummets because conscious focus shifts entirely toward MRIs, balance tests, and physical symptoms.

Secondary Gain

These are the tangible, real-world interpersonal and environmental payoffs that crystallize once the illness is established:

- Complete immunity from harsh criticism and high expectations.
- Being excused from unpleasant family obligations and housework.
- Receiving unconditional gentleness and attention from partners.
- A socially acceptable ticket to decline any stressful demand.

Primary gain shields you from an internal psychic explosion. Secondary gain anchors you into chronic disability, making full recovery biologically disadvantageous to your subconscious.

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4. The Five Masks of Secondary Gain in PPPD

Let us strip the deceptive packaging from the condition and examine the five recurring scenarios where unsteadiness functions as a subconscious lever.

Mask 1: The Ironclad Alibi Against Adult Life

The most prevalent pattern among individuals with functional dizziness is using illness as **an official pardon from life-altering decisions**:

- *"I would have walked away from this toxic marriage long ago, but how can I move out when I sway just walking down the hallway?"*
- *"I hate this draining corporate job, but in this economy, who would hire someone with PPPD? I have to stay."*
- *"I was going to launch my own business, but then this condition struck. Now my sole job is survival."*

Illness becomes the ultimate shield. Neither friends, family, nor your internal hyper-critical voice can blame you for inaction. You are not a coward, you are not procrastinating, you are not weak. You are *ill*. And one does not fault an ill person.

Mask 2: The Monopoly on Care and Coerced Affection

A significant percentage of PPPD patients grew up in environments characterized by conditional warmth:

> *"When you were healthy, productive, and obedient, you were taken for granted. You received demands, not hugs. But the moment you lay sick in bed with a high fever, your mother sat beside you, stroked your hair, brought hot tea, and spoke with tenderness."*

The developing nervous system hardwires an equation: **Love = Sickness & Helplessness**.

As an adult, asking directly for comfort feels humiliating to your ego. The somatic body takes over negotiations:

- The unsteadiness ramps up; legs turn to jelly.
- A distant spouse is forced to rush to the pharmacy and hold your arm in the grocery store.
- Relatives check in multiple times a day with genuine concern.
- You finally receive the validation and care you starved for over decades. But the price tag is your independent mobility.

Mask 3: The Legitimate Ticket to Rest Without Guilt

The classic profile of a PPPD sufferer is a hyper-controlling, hyper-responsible perfectionist. The chronic rescuer.

Inside lives a ruthless "Inner Critic" that outlaws relaxation:

- Lying down in the middle of a Sunday just to rest? *"Get up, you slacker! The house is dirty, emails are waiting, and you haven't achieved anything today!"*
- Saying "no" to an exhausting favor? *"Selfish! You must be dependable!"*

Your system is depleted to absolute zero. Your battery is depleted. You urgently need weeks of deep nervous system downtime. Yet taking it voluntarily triggers agonizing guilt.

The autonomic system trips the circuit breaker:

- It switches on PPPD.
- Now you lie flat on the couch with your eyes closed.
- And nobody — not even your tyrannical Inner Critic — can reproach you.
- You are not "lazy." You are *gravely unwell*; the ground is rocking beneath you. To a chronic overachiever, PPPD is the only legally approved medical leave.

Mask 4: The Alibi for an Unlived Life

You look around at peers who took career gambles, built businesses, ended stale relationships, and lived boldly. Inside you fester bitter regret and silent envy.

Admitting: *"I was terrified. I played it too safe. I wasted years in comfort zones"* destroys your fragile self-esteem.

It is vastly less painful to construct a tragic narrative:

- *"I had immense potential. I was going to conquer the world! But this mysterious vestibular condition robbed me of everything. I am bravely fighting an invisible enemy."* The illness transforms your stagnation from a series of personal choices into an honorable, tragic battle against biology.

Mask 5: The Safety Cocoon Against a Hostile World

The outside world is overstimulating, loud, demanding, and unpredictable. Out there, you can be rejected, criticized, or humiliated.

Home is safe. But an adult cannot simply stay in pajamas and refuse to engage with society without being judged as immature.

When PPPD induces secondary agoraphobia, home confinement becomes medically certified:

- *"I am not avoiding social exposure. I literally cannot navigate supermarket aisles because the fluorescent lights and visual motion trigger my vestibular system."* The fortress door locks shut. Subconscious mission accomplished.

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5. The Shadow Audit: 7 Diagnostic Questions

Prepare a notebook and a pen. Do not answer these questions in your head — your intellect will instantly sanitize your responses with rationalizations. Write them by hand:

#	Diagnostic Question	What This Uncovers
1	What do you immediately stop doing when your unsteadiness peaks?	Your hidden zones of chronic resentment and burnout
2	Who is the first person you turn to during an attack?	Your primary source of coerced emotional co-regulation
3	What major life decision has been shelved under: *"Once I recover, I will deal with it"*?	Your ultimate somatic excuse
4	How would your relationship dynamics change if you became 100% self-sufficient tomorrow?	Hidden dread of losing partner attention or resuming burdens
5	Which daily duty or professional role makes you feel internally sick?	The root stressor fueling your central sensitization
6	What do you envy about healthy peers, besides physical health?	Your actual, suppressed desires and blocked ambitions
7	If your dizziness were a bodyguard character, what is it protecting you from?	The core defensive purpose of your functional illness

If your stomach churns or you feel sudden irritation reading these questions, pay close attention. You have struck the core wire. That is precisely where the pathway to recovery is hidden.

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6. The Real Balance Sheet: The Disastrous Cost of the Armor

Your subconscious believes it negotiated a brilliant bargain: exchanging dizziness for relief from intolerable demands.

Now examine the actual balance sheet. You are not paying with currency; you are paying with the only non-renewable asset you possess: **years of your life**:

- To avoid divorcing an incompatible spouse, you pay with the daily sensation of walking on a tilting ship in a storm.
- To avoid confronting your Inner Critic for taking an afternoon off, you pay with the inability to attend concerts, travel, or walk freely in nature.
- For a teaspoon of coerced affection from family, you surrender your romantic vitality and assume the role of an invalid.
- To avoid professional risk, you pay with chronic financial vulnerability.

> ***Secondary gain is a high-interest predatory loan taken out by your subconscious.***
> *You borrowed five minutes of temporary escape, and the autonomic collectors arrived to seize your balance, your autonomy, and your dignity.*

See the math for what it is. The bargain is an utter disaster.

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7. The Protocol for Dismantling Secondary Gains

Until you discover how to fulfill the underlying needs **through adult, conscious, healthy channels**, your nervous system will cling fiercely to PPPD. No amount of vestibular rehab exercises or SSRIs can override a brain convinced that health is more dangerous than sickness.

Here is the four-phase protocol to dismantle subconscious sabotage:

Phase 1. Legitimizing Needs Without a Doctor's Note

Review your audit answers. Note what the illness was providing:

- The right to rest.
- The right to say "no."
- Warmth, gentle attention, and care.
- Solitude and quiet.

Now engrave this into your belief system: **you are entitled to all of these simply by virtue of being alive**.

You do not need an ICD-11 diagnosis or a vestibular crisis to tell your employer: *"I will not answer emails over the weekend."* You do not need unsteadiness to tell your partner: *"Please sit with me and hold my hand; I had a hard day."*

Phase 2. Decontaminating Decisions from Symptoms

Institute a non-negotiable mental rule: **major life decisions are made completely independent of dizziness levels**.

- Stop waiting for "calm seas in your head" before you begin altering your reality.
- If a relationship is toxic, communicate your boundaries today, even if your visual field is swimming.
- If a job is destroying your nervous system, begin updating your CV today, through the brain fog. When the subconscious realizes that unsteadiness **no longer stalls adult action**, its biological utility evaporates.

Phase 3. Direct Verbal Communication

Stop using your somatic body as an emotional megaphone.
Approach the people from whom you seek warmth and speak plainly:
"I have been frightened, and I need your emotional support. But I need you to support my health, not my disability. Do not do my errands for me. Simply walk beside me while I do them myself."
Relinquish the patient identity. Reclaim your agency.

Phase 4. Embracing the Discomfort of Adult Freedom

Recovery is not a regression into a stress-free infantile paradise. It is a return to the messy, demanding, real world where you must choose, risk, set boundaries, and endure occasional failure.
Yet this world offers the single greatest reward denied to a person trapped in functional dizziness: **freedom, self-respect, and solid ground beneath your feet**.

💬 The Core Insight

> *"Your dizziness is not trying to destroy you. It is a clumsy, agonizing attempt by your subconscious to protect you from a life you could no longer bear. Thank your brain for keeping you alive during the emergency. Then tell it clearly: 'Thank you, but I will take it from here. I am an adult, and I can walk on my own two feet.'"*

📝 Practice & Homework

Reading without implementation changes nothing in neural wiring:

1. **The Sabotage Inventory:** Divide a sheet of paper into two columns:
    - *Left:* "What does my dizziness give me or protect me from?" (write at least 3 points).
    - *Right:* "What specific, adult action will I take this week to meet this need without illness?"
2. **One Direct Refusal Without Medical Excuses:** This week, decline one request, chore, or invitation that you genuinely do not wish to accept. It is **strictly forbidden** to mention headache, dizziness, blood pressure, or exhaustion. Simply state: *"Thank you for asking, but I cannot take this on."*
3. **Tracking the Somatic Relief:** Notice your body's reaction. Your anxiety will initially spike for 10–15 minutes. Soon after, a profound wave of autonomic relief will wash over you — because your brain no longer needed to simulate a neurological illness to protect your boundaries.

Fear of Dying and Going Crazy in PPPD: Why You Won't Faint or Lose Your Mind

Beneath every wave of rocking, heavy legs, and brain fog lie two primal terrors: "I am about to pass out or die right here on the pavement" or "My mind is snapping, I am going to lose control and end up in a psychiatric ward." Let us dismantle both scenarios using clinical physiology and psychiatry.

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1. The Ultimate Bluff of Your Neurosis

Let us be completely honest: the gentle swaying sensation itself is not what terrifies you.

When you drank a glass of champagne at a celebration or rode a carousel as a child, your equilibrium swayed five times harder than it does in PPPD — yet you laughed and enjoyed it. Why? Because you knew you were safe.

In PPPD, the slightest sensation of a "floating floor" in a supermarket or at a crosswalk triggers an icy drop in your stomach for one reason alone — the **catastrophic prediction** your imagination projects in 0.2 seconds:

1. **The "Death / Fainting" Script (Thanatophobia):** *"The dizziness is about to peak, my vision will go black, I'm having a stroke or cardiac arrest, and I'm going to collapse face-first onto the floor in front of strangers."*
2. **The "Insanity / Psych Ward" Script (Lyssophobia):** *"This brain fog and derealization have gone too far. My mind is about to snap, I'll stop recognizing my family, start screaming in public, and get locked in a psychiatric hospital forever."*

As long as even 1% of you believes that dizziness can culminate in fainting, stroke, or psychosis, your amygdala will keep sounding the five-alarm siren 24/7.

Let us demolish both myths using hard facts from clinical medicine.

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2. The Physiology of Fainting vs. The Physiology of Panic: Why You Physically Cannot Collapse

The most common fear in PPPD is: *"My legs feel like jelly and my vision is swimming — I am seconds away from passing out!"*

Now let us look at the laws of hemodynamics (blood flow and pressure):

How True Fainting (Syncope) Actually Works

For a human being to lose consciousness, there must be a **sudden, steep drop in arterial blood pressure** (for example, dropping to 60/40 mmHg) accompanied by a slowing heart rate (bradycardia). Gravity pulls blood away from the cerebral cortex toward the legs, causing brief cerebral hypoperfusion that "turns off the screen" so the body falls horizontally and blood flow returns to the brain.

What Happens in Your Body During a PPPD & Panic Surge?

When you stand in a checkout line or walk down the street and get scared of your dizziness, your sympathetic nervous system floods your bloodstream with **epinephrine (adrenaline) and norepinephrine**. What does adrenaline do to your cardiovascular system?

- It **accelerates your heart rate** (to 90–130 bpm).
- It **constricts peripheral blood vessels** and **raises arterial blood pressure** (e.g., to 130/85 or 145/90 mmHg).
- It forcefully pumps maximum oxygenated blood directly into your brain and major leg muscles!

> ***Foundational Medical Fact:*** *Fainting at the peak of an adrenaline panic surge is like freezing to death inside a blazing sauna. Syncope and panic are* ***diametrically opposed*** *physiological states!*

Physiological Metric	True Fainting (Vasovagal Syncope)	PPPD & Panic Surge (Sympathetic Drive)
Active Autonomic Branch	Parasympathetic (vasovagal drop)	Sympathetic ("Fight or Flight" mobilization)
Arterial Blood Pressure	Plummets sharply (↓ 70/40 mmHg)	Rises or stays firm (↑ 130/85 mmHg)
Heart Rate	Slows down (bradycardia)	Speeds up (tachycardia)
Leg Muscle Tone	Complete atonia (muscles go limp)	Hypertonicity ("jelly legs" from co-contraction)
Cerebral Perfusion	Temporarily reduced	Maximized for survival
Probability of Passing Out	High	**Virtually Zero (0%)**

"Then Why Do My Legs Feel Like Jelly and My Vision Blur?!"

- **"Jelly Legs"** in PPPD are not muscle weakness! They occur because you are simultaneously contracting both the flexor and extensor muscles of your thighs and calves in a desperate attempt to

"grip the floor." Your legs aren't weak — they are rigid like two over-wound steel springs. Next time your legs feel like jelly, try doing a squat or rising onto your tiptoes — you will do it effortlessly!

- **"Blurry / Swimming Vision"** occurs because adrenaline dilates your pupils (*mydriasis*) to let in more light and spot peripheral threats. Wide pupils reduce depth-of-field sharpness and create a sensation of glare or visual haze. That is predator-hunting vision, not pre-syncope!

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3. Why It Is Not a Stroke or Heart Attack

The health-anxious mind counters: *"Okay, maybe not fainting! But what if these blood pressure spikes and dizziness cause a blood vessel to burst in my brain and give me a stroke?!"*

Let us look at clinical neurology:

1. **Healthy cerebral arteries withstand pressures up to 250–300 mmHg.** When an athlete performs a heavy squat in the gym, their blood pressure briefly spikes to 220/120 mmHg — and zero vessels rupture because healthy arterial walls are built like reinforced Kevlar hoses. Your anxious 140/90 is a gentle warm-up for your vasculature.
2. **A stroke does not last for months.** A stroke is an acute vascular event producing focal neurological deficits (facial droop, unilateral limb paralysis, slurred speech). If you have been swaying for three months, six months, or two years, yet you are reading this page, walking on two legs, and typing messages on your phone — **you are not having a stroke**.

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4. Lyssophobia: Why a Neurotic Person Can NEVER Go Crazy

The second primal terror in PPPD — especially when unsteadiness triggers derealization (feeling like the world is "behind glass," "dreamlike," or "unreal") — is:

> *"I am losing my mind! I'm developing schizophrenia! My sanity is about to snap!"*

In clinical psychiatry, there is an ironclad diagnostic law taught in the very first year of medical school:

> ***Neurosis and psychosis exist on two completely separate biological tracks that never cross into one another.*** *An anxiety disorder cannot "escalate" into schizophrenia any more than nearsightedness can escalate into colorblindness.*

Compare the brain architecture of neurosis (PPPD, panic, derealization) against true psychosis (schizophrenia, delusional disorder):

Clinical Criterion	Neurosis / PPPD / Derealization	True Psychosis (Schizophrenia)
Clinical Insight	**100% Intact.** You notice the strange sensations, dislike them, and recognize they are abnormal	**Absent (Anosognosia).** The patient believes their hallucinations and delusions are 100% real
How Symptoms Are Described	"It feels **AS IF** the world is behind glass," "It's **LIKE** I'm in a dream" (metaphorical awareness)	"The CIA replaced the sky with a hologram," "Aliens are controlling my legs" (literal conviction)
Fear of Going Crazy	Intense! Constant self-checking for sanity	Zero. Psychotic patients believe they are fine and the world is conspiring against them
Neurochemistry	Excess norepinephrine, cortisol & glutamate (hyper-vigilance)	Mesolimbic dopamine dysregulation & thought disorder
Control Dynamics	Fear of losing control caused by **excessive self-control**	Actual loss of reality testing with zero fear of insanity

Read this truth three times: **The very fact that you are afraid of going crazy and Googling symptoms of psychosis is your 100% clinical certificate that your reality testing and sanity are completely intact!**

A person losing their mind never worries about losing their mind. They do not sit in PPPD forums asking, *"Hey guys, does anyone else get brain fog in grocery stores?"*

What About Intrusive Thoughts of "Losing Control and Doing Something Crazy"?

People with PPPD frequently experience terrifying harm/control intrusions (an OCD-spectrum phenomenon):

- Standing on a balcony or subway platform and having the sudden flash: *"What if I lose control and jump?"*
- Sitting in a quiet meeting and thinking: *"What if I suddenly snap and start screaming?"*

Understand this: those thoughts horrify you precisely because they are **100% opposite to your character and values** (*ego-dystonic*)! A hyper-responsible perfectionist fears losing control so intensely that their brain runs a worst-case simulation just to double-check: *"Are the brakes still working?"* Not a single neurotic person with intrusive control thoughts has ever "snapped" and acted on them. You do not suffer from a lack of control — you suffer from an **overdose of control**.

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5. The "Existential Defusal" Protocol: 3 Real-Time Drills

The next time a wave hits you in public whispering, *"You're about to faint / die / go crazy,"* execute this 60-second protocol instead of fleeing:

1. The "Calf-Raise Reality Check" (Against Fear of Fainting)

Feel like your legs are turning to jelly and you're about to collapse? Stop and perform **3 brisk half-squats** or **rise up onto your tiptoes 5 times**. Feel the tremendous muscular power currently coursing through your calves and quadriceps. A person in true pre-syncope cannot stand on their tiptoes! Your body is supercharged with adrenaline like a Formula 1 car.

2. Viktor Frankl's Paradoxical Command (Calling the Bluff)

Is your amygdala threatening you with fainting? Call its bluff on the spot. Stand still on the sidewalk and command your body with total sincerity:

> *"Go ahead! Faint right now! Collapse onto this pavement! I give you full permission! One, two, three — pass out!"*

Try to faint on command. You will realize you physically cannot do it — and in that exact second, the panic bubble bursts because you stopped running from the bluff.

3. The "As-If" Anchor (Against Fear of Insanity)

When derealization hits and the room feels surreal, repeat this exact clinical formula:

> *"My mind is not breaking. My tired nervous system simply turned on its energy-saver dimmer switch to reduce sensory overload. As long as I can use the words 'as if,' my sanity is 100% rock-solid."*

💬 Core Insight

> *"You are afraid of falling because you are gripping too tightly onto thin air. You are afraid of losing your mind because your intellect is working overtime trying to micromanage every neuron. Your body is not dying — on the contrary, it is defending your life with such fierce intensity that you are mistaking its armor for an attack."*

📝 Homework Assignment (Action Plan)

1. **The 60-Second Collapse Challenge:** The next time your legs feel weak or dizzy in a store or on the street, strictly forbid yourself from sitting down or leaning on a wall. Stand tall, soften your knees slightly, and tell your brain: *"You have 60 seconds to make me faint right here. Go."* Stand there for one full minute and prove to yourself that you remain standing.
2. **The Scoreboard Audit:** Write down the answer to one question on paper: *"How many times during my PPPD have I been convinced I was about to faint, have a stroke, or go insane — and how many times did it actually happen?"* Write it as a fraction (e.g., **300 / 0**). The scoreboard is 300 to 0 in favor of your survival.
3. **Shattering the Fragility Myth:** Tonight, perform 15 brisk bodyweight squats to deliberately raise your heart rate to 120 bpm, then stand calmly and watch how your autonomic nervous system brings your pulse back to baseline within 2 minutes without any conscious effort from you. Trust your hardware — it was engineered with a tenfold safety margin.

Module 1: The Body

Muscle armor in PPPD: how chronic tension causes dizziness

Why do you feel dizzy when you get out of bed - and what does it have to do with the fist clenched since birth.

Frozen Beast Pose

When any animal gets scared, the first thing it does is pull its head into its shoulders. Instinct. Protect your neck from a predator's bite. Have you ever been scared by a sharp sound? Remember - your shoulders fly up to your ears, your jaw clenches, your breath freezes. It happens in milliseconds, you don't even have control over it.

Now imagine that you live in this state. Not for a second. Not for a minute. **For months.**

Chronic stress keeps the body in constant microspasm. You don’t notice this, just like you don’t notice that you’re carrying a heavy bag - your hand dries up, but you don’t shift it. Because I'm used to it. The spasm became the norm.

In psychology there is a cruel but absolutely accurate phrase: **“If you can’t scream with your mouth, your body starts screaming”**. All suppressed emotions, everything that you cannot say out loud or express with action, does not evaporate. Stress energy simply hides deep in the body and comes out as a physical illness. Chronic stress always hits the weakest link of the body - where it is thin, it breaks:

- **Lump in the throat** is not a beautiful metaphor, but a literal conflict of the muscles of the larynx: they try to open for a scream and at the same time contract in order to hold back this scream.
- **Stomach problems** are a classic reflection of the inability to “digest” some difficult life situation.
- **Back pain** is often the same unbearable burden of hyper-responsibility.
- **Dizziness with PPPD** is a signal from an overloaded nervous system that cannot cope with an unresolved internal conflict.

Back in the 1930s, psychoanalyst Wilhelm Reich described this and called it **"muscular armor"**. He identified seven segments where the body blocks emotions:

1. **Eyes** - tense forehead, “frozen” eyes
2. **Mouth** - clenched teeth, clenched jaw
3. **Neck** - muscles like stone
4. **Chest** - shallow breathing, “lump in the chest”

5. **Diaphragm** - stiff, breathing does not reach the stomach
6. **Stomach** - spasm, irritable bowel syndrome
7. **Pelvis** - tension that you are not aware of at all

Every repressed emotion gets stuck in one of these blocks. You don't scream - your throat is blocked. You're not angry out loud—your jaw clenches. You endure - your shoulders turn to stone.

Why neck = dizziness

Now comes the most important part.

The neck muscles are not just muscles. They are densely packed with **proprioceptors** - tiny sensors that tell the brain where the head is. This is the third "balance sensor" of the three we talked about.

When the neck is in spasm, these sensors are clamped along with it. The signal from them goes to the brain with a **microdelay**.

You get out of bed in the morning. What's happening:

- **Eyes** see: you got up.
- **The gyroscope in the ear** senses: you are moving.
- **Clamped neck** says: "We are still lying!"

The brain interprets this desynchronization—in milliseconds—as a loss of balance. Dizziness. Unsteadiness. "Vata" in the head.

But there is a nuance that makes everything even worse. Anxiety turns you into an **iron bucket with an alarm clock inside**. A healthy person also experiences a slight sway when turning their head - this is normal vestibular physics. He just doesn't pay attention to it. And you, being in an anxiety disorder, literally listen to every micromovement. The alarm clock is the same as that of a healthy person. But you hear it 100 times louder, because the bucket is iron and the ringing is reflected from the walls. You turn your head 10 degrees: *"Oh, I'm dizzy!"*. What if it's 60? *"That's it, it's over, God forbid!"*. You are afraid of dizziness itself - and it is this fear that intensifies it. Vicious circle: a dog chasing its own tail.

And you run to the MRI in horror. And the MRI is clean. Because the problem is not the structure. **The problem is the signal.** More precisely, it's how your frightened brain interprets a completely normal signal.

Trapezoidal trigger

90% of people with PPPD have tender spots in the trapezius muscles. Press your fingers into the muscles between your neck and shoulders. Hurt? Here.

These points give **referred pain to the back of the head**, which doctors confuse with "impaired blood circulation in the brain." They prescribe Cerebrolysin, Actovegin, Mexidol - dummy drugs that do not work. Because they are treating the wrong problem.

The problem is tight muscles. And the muscles are tense - because the brain is afraid.

Temporary relief: massage and acupuncture

You go to a massage therapist or an acupuncture session (acupuncture, dry needling). The specialist kneads the neck or sticks needles into trigger points. At this moment a miracle happens.

Physiologically it works like this:

1. **Local release:** The needle or strong pressure physically breaks the stagnant microspasm at the trigger point, restoring blood flow and flushing out accumulated waste products (lactic acid).
2. **Gate Control Theory:** A strong tactile or painful signal from a needle/massage travels to the spinal cord faster than a chronic background signal from a tight neck. The brain switches to a new stimulus and temporarily "forgets" to send the command to spasm.
3. **Endorphin Release:** Needles stimulate the release of natural analgesics, which provide a feeling of lightness and relaxation.

After the session you come out looking like new. The head is clear, the unsteadiness is gone. You rejoice: "This is it! The cause has been found, osteochondrosis has been defeated!"

But two days later, everything returns to normal. Sometimes it gets even worse.

Why? Because **the command to tense your muscles continues to come from your frightened processor (brain)**. You cleaned the local port, but the malicious alarm code continues to send endless packets of data to the spasm. It's like mopping up a puddle on the floor without turning off the tap.

Does this mean that needles and massage are useless? No. They perfectly relieve the acute phase and help restore blood circulation in degraded muscles. But they work **only as a preparation for Jacobson's relaxation and brain retraining**. If you do not reset the alarm generator itself, your visits to massage therapists will become an eternal subscription to temporary relief.

Why does the shell come back after sleep

You noticed: in the morning the unsteadiness and stiffness of the neck is often worse than in the evening. It would seem that you lay there for 8 hours, resting, your muscles should relax. But the opposite happens. Why?

First of all, **sleeping during anxiety is not rest**. When you fall asleep with high cortisol levels, the brain does not fully enter deep sleep mode (N3/SWS phase). He is "half asleep" - vigilant. The muscles do not relax completely because the amygdala continues to send background signals: *"We are in danger, hold*

the line". You wake up with a "stone" neck, not because you slept uncomfortably, but because your brain kept your muscles on alert all night.

Secondly, **morning cortisol peak (CAR)**. In the first 30–45 minutes after waking up, cortisol levels physiologically soar by 50–75%. For a healthy person, this is simply a "power button" - the body prepares for an active day. But in a neurotic person, this surge falls on the already overheated limbic system, and the brain interprets it as an alarm signal. Hence the morning stiffness, the "wobbly" head and the instantaneous activation of the internal scanner.

What to do: Don't lie in bed after waking up "listening to your body." The first 60 seconds - get up, wash your face with cold water, take 5 slow belly breaths. This switches the brain from internal monitoring mode to external monitoring. Morning Jacobson (3-5 minutes) immediately after getting up is a powerful tool against the morning shell.

Right now: body audit

Sit comfortably. Close your eyes for 30 seconds. And scan:

- **Jaw.** Are your teeth clenched? Unclench. Is your tongue pressed against the roof of your mouth? Let go.
- **Shoulders.** Are they pulled up to the ears? Put them down. Straight down. Even lower.
- **Forehead.** Are you frowning? Smooth it out.
- **Belly.** Retracted? Let him be soft.
- **Breathing.** Superficial, in the chest? Take one deep breath into your belly. Exhale slowly.

Open your eyes.

You just dropped your shell for a second. The brain received a signal: "The muscles are relaxed → there is no danger → anxiety can be reduced." This is called **feedback** (bottom-up signal). We will use it as a weapon.

> ***DZ for Chapter 4:***
>
> 1. *Set a reminder on your phone - every 2 hours. Text:* ***"Where are my shoulders? Are your teeth clenched?"***
> 2. *When it works, as you exhale, release the tension from your jaw and shoulders. 3 seconds. No more.*
> 3. *Do this 7 days in a row. Without missing. In a week you will begin to catch the spasm automatically.*

> 4. *Bonus: Press your trapezius (the muscles between your neck and shoulders). If it hurts, remember these points. We will work with them in the following chapters.*

Self-massage of trigger points of the neck and shoulders

If you pressed on the trapezius and felt pain, that means there are trigger points there. These are knots of muscle spasm that can persist for weeks or months, maintaining background unsteadiness. Here's how to work with them yourself:

Point 1: Trapezius muscle (superior fascicle)

- **Where:** Muscle between the neck and shoulder, halfway.
- **How to find:** With the opposite hand, feel the “mound” on the trapezium. Press. If you suddenly get sick, you found it.
- **What to do:** Use the ball of your thumb to apply moderate pressure (5-6 out of 10 on the pain scale, no more). Hold the pressure for 30-60 seconds until the pain begins to “spread” and decrease. Repeat 3 times on each side.

Point 2: Suboccipital muscles

- **Where:** At the base of the skull, on either side of the spine.
- **How to find:** Lie on your back. Place 2 tennis balls (or 2 walnuts in a sock) under the back of your head so that they press on the muscles on either side of the spinal column.
- **What to do:** Just lie on the balls for 3-5 minutes. Gravity does the work for you. If it hurts a lot, put a towel on it to soften it.

Point 3: Sternocleidomastoid muscle (SCM)

- **Where:** Lateral surface of the neck - the muscle goes from the ear to the collarbone. Feels like a dense cord.
- **How to find:** Turn your head to the left - a “rope” will appear on the right side of your neck. This is GKSM.
- **What to do:** Gently grab the muscle with your thumb and forefinger. Gently “roll” it between your fingers from bottom to top. Don't press too hard - this is where the carotid arteries pass. 30 seconds on each side.

Regularity: Do self-massage every evening before Jacobson. In 2 weeks, the volume of muscle tension will decrease by 40–60%.

Fascial chains: why do your legs hurt when your neck is tight?

You may have noticed that it's not just your neck that hurts. Pulls the lower back. The calves are buzzing. Feet feel like lead. And you think: “Well, that’s it, in addition to dizziness, my legs are also falling off. Probably multiple sclerosis!

Calmly. This is the **fascial chain** - and it explains everything.

Muscles in the body do not exist in isolation, like individual parts in a construction set. They are connected by **fascia** - a thin but incredibly strong connective tissue membrane that envelops each muscle and connects them in continuous chains from head to toe. Anatomist Thomas Myers called them **"anatomical trains"**.

The most important circuit for a PPPD practitioner is the **posterior superficial line**. It goes like this:

Pole of the foot → calf muscle → hamstring → sacrum → erector spinae → occiput → aponeurosis (forehead fascia)

This is one continuous “cable channel”. When the upper end (neck and back of the head) is locked in a chronic spasm, the tension is transmitted **down the entire chain**, as if you were pulling one end of a tight rope. Result: you feel “lead” in your legs, nagging pain in the lower back and stiffness in your feet - and all this is **from a tight neck**, and not from “sore legs”.

Practice: roller myofascial release

Buy a foam roller or use a regular plastic water bottle (1.5 liters). Lie down on the roll and slowly roll the back line from bottom to top:

1. **Feet:** Place your foot on the roll, roll back and forth. 1 minute on each leg.
2. **Calves:** Sit on the floor and place the roll under your calves. Raise your pelvis and roll. 1 minute.
3. **Ham:** Roll under your hips, roll slowly. 1 minute.
4. **Lumbar and shoulder blades:** Lie with your back on the roll, hands behind your head, slowly roll from the lower back to the shoulder blades. 2 minutes.

When: In the evening, before self-massage of the neck and Jacobson. 5 minutes. After a week, you will notice that the “lead legs” have become lighter - because you have removed the tension from the entire chain, and not just from the top link.

Diaphragmatic block: dizziness due to breathing

Wilhelm Reich identified 7 segments of the shell. We examined the neck (3rd segment) in detail. But there is another critically important one - **diaphragm** (5th segment).

The diaphragm is the muscular partition between the chest and abdomen. She is the main engine of breathing. When you're stressed, the diaphragm spasms and gets stuck in a half-retracted position.

Breathing becomes **shallow and chesty**: the chest rises up and down, but the stomach does not move.

Here's what it does to your wobbliness:

1. **Hyperventilation.** Shallow breathing becomes more frequent. You're exhaling too much CO_2. The level of carbon dioxide in the blood drops.
2. **Respiratory alkalosis.** Blood pH shifts to the alkaline side. The blood vessels in the brain narrow (blood carries less oxygen to the brain).
3. **Result:** Dizziness, “fog” in the head, tingling in the fingers, feeling of “cotton legs”, ringing in the ears.

You run to a neurologist. He does an ultrasound scan. He says: “The blood vessels are normal.” And he’s right - the vessels are normal. But they are **functionally narrowed** because you are not breathing correctly. But you're breathing incorrectly because your diaphragm is tight. And she is clamped because her brain is afraid.

Test: where do you breathe?

Sit up straight. Place **right palm on chest**, **left palm on stomach** (below navel). Take a normal breath. Don't try, just breathe as usual.

- If the **right** (chest) rises → the diaphragm is blocked. You breathe from the tops of your lungs.
- If the **left** (stomach) rises → the diaphragm works. You are breathing correctly.
- If both → you’re already doing great, but check if you’re forcing your breath.

Practice: “Book on your stomach”

1. Lie on your back. Place a heavy book on your stomach (below your navel).
2. Inhale through your nose so that the book **rises**. The chest remains motionless.
3. Exhale through your mouth, the book **falls**. Exhale 2 times longer than inhalation (inhale 4 seconds → exhale 8 seconds).
4. 5 minutes, 2 times a day (morning and evening).

A book is visual feedback for the brain. You see your stomach rise → the brain remembers the pattern → after 2 weeks, diaphragmatic breathing becomes automatic. Hyperventilation stops. The “fog” in my head goes away.

Jaw: bruxism and dizziness

The second segment of the shell according to Reich is **mouth and jaw**. And this is a blind spot that neither neurologists nor ENT specialists know about.

Chronic teeth clenching (bruxism) is a reaction to suppressed aggression. You can't scream → your jaw clenches. You endure → teeth clenched. You get angry in silence → your chewing muscles turn to stone.

This happens during the day (micro-compression that you don’t notice) and at night during sleep.

Why is it associated with dizziness:

- **The temporomandibular joint (TMJ)** is located millimeters from the vestibular apparatus (inner ear). Chronic spasm of the masticatory muscles creates pressure on the joint, which is transmitted to the structures of the middle ear.
- **Referred pain.** Spasm of the masticatory muscles (especially the lateral pterygoid) produces pain that radiates to the temple, behind the ear and to the back of the head. Many people with PPPD have been going to the ENT for years with “ear pain”, but the problem is in the jaw.
- **Tinnitus (tinnitus).** A tight TMJ can cause clicking and buzzing in the ears, which a neurotic person immediately interprets as “Meniere’s disease” or “tumor of the auditory nerve.”

Self-diagnosis of TMJ

1. Open your mouth as wide as possible. Do **3 fingers** (index, middle, ring) fit vertically? If not, the joint is limited.
2. Open your mouth slowly. Is there a **click** or crunch? Does the lower jaw move **to the side** when opening? These are signs of TMJ dysfunction.
3. Press your fingers on the point **in front of the tragus of the ear** (the depression in front of the auricle) with your mouth closed. Hurt? This is the joint capsule of the TMJ.

Practice: Exercise “Fish”

1. Open your mouth, relax your lower jaw. Let it “hang” under its own weight.
2. Place the tip of your tongue on the roof of your mouth (behind your upper teeth).
3. Exhale slowly through your mouth. The jaw is as relaxed as possible, like a fish.
4. Hold for 30 seconds. Repeat 5 times.

Bonus: Before going to bed, place a warm compress (towel soaked in warm water) on both sides of your jaw for 3 minutes. This relieves nighttime bruxism and reduces morning stiffness. If bruxism is severe (teeth wear, morning jaw pain), contact your dentist for a custom-made mouthguard.

Workplace: “text neck” and 8 hours of spasm

You do Jacobson morning and evening. Massage trigger points. And then you sit down at the computer for 8 hours with your neck stretched forward towards the monitor. And you wonder why the stone shoulders return in the evening.

This is called **forward head posture** - “text neck”. Every 2.5 centimeters of moving the head forward from the axis of the spine adds **5 kg of load** to the neck muscles. The average office worker's head is moved forward by 5–7 cm. This is 10–15 kg of constant load on the already tight trapezius. 8 hours a day. 5 days a week. No Jacobson can compensate for that.

Workplace checklist (5 minutes once)

- **Monitor:** The top edge of the screen is at eye level. If you have a laptop, buy a stand (or place it on a stack of books) and an external keyboard.
- **Distance:** Extend your arm - your fingertips should touch the screen. No closer.
- **Chair:** The back is pressed against the backrest. Feet completely on the floor. Knees bent 90°.
- **Phone:** Don't bow your head to the phone. Raise the phone to eye level (yes, your arms will get tired, but your neck will thank you).

Microbreaks: every 45 minutes

Set a timer. Every 45 minutes, get up and do:

1. **Chin tuck:** While sitting, without tilting your head, move your chin back, as if you were giving yourself a "double chin." Hold for 5 seconds, repeat 5 times. This stretches the suboccipital muscles and returns the head to its axis.
2. **Circular movements of the shoulders:** 5 forward, 5 backward. Slowly, with maximum amplitude.
3. **Inhale-exhale:** One deep breath in the stomach, long exhale.

15 seconds. But there are 10 such breaks per day. This is 150 seconds of targeted release of neck spasm that accumulates from the pose.

Thermal contrast therapy for the neck

A simple but powerful tool for losing muscle mass every night. Alternating heat and cold on the neck and trapezius:

1. **Warm compress:** Wet a towel with hot water (not boiling water!), wring it out, wrap it around your neck and shoulders. Hold **3 minutes**. Heat dilates blood vessels, increases blood flow, and relaxes spasmodic fibers.
2. **Cool compress:** Replace with a towel soaked in cold water. Hold **1 minute**. Cold tones the walls of blood vessels and relieves swelling in inflamed trigger points.
3. **Repeat 3 cycles**, finish warm.

When: Every evening, **before** self-massage and Jacobson. Thermal contrast prepares the muscles - after it the massage goes deeper and relaxation is more effective. Total time - 12 minutes.

Progressive muscle relaxation according to Jacobson: step-by-step technique

Why it is impossible to relax "just by force of will" and how to return the body to the basic setting of rest through controlled tension.

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1. The essence and problems: the "relax" trap

When people advise you from all sides "just relax, don't be nervous," it only causes quiet rage. If you could relax, you would have done it a long time ago. But your body doesn't listen.

In a state of PPPD and chronic anxiety, your muscles are tight 24/7. You wake up already tired, with a wooden neck, clenched jaw and raised shoulders. This is called sensory-motor amnesia: the brain is so accustomed to keeping its muscles toned to protect itself from an imagined threat that it has simply **forgot** what it feels like to be completely relaxed. You try to lie down and let go of control, but your body remains taut. You are lying on the bed, and your shoulders and lower back are still levitating a millimeter from the mattress, ready to jump.

2. Physiological explanation: how to deceive a pinched wire

The nervous system is built on the principle of a pendulum. It has two modes:

- **Sympathetic** (fight or flight) - tension, mobilization, adrenaline rush.
- **Parasympathetic** (rest and digest) - restoration, relaxation, peace.

In PPPD, the sympathetic system is overheated. She constantly sends signals to the muscles: "Keep your tone, now we will save ourselves!" The muscles are clamped, sending a return signal to the brain: "We are clamped, which means the danger is real!" The cycle is completed.

To break this loop, physiologist Edmund Jacobson discovered a simple law at the beginning of the 20th century: **after maximum tension of a muscle there is always a period of deep automatic relaxation**. This is pure physiology, a reflex. We don't have to force the brain to calm down directly (that's difficult). We'll go in through the back door - through the skeletal muscles. By straining them to the limit for several seconds, we deplete the supply of transmitters in the synapses, and the muscle simply has no choice but to give up and go limp. The brain sees: "The body is relaxed, which means we are safe," and turns off the alarm siren.

3. Cognitive traps: “I’m trying to relax”

The main mistake beginners make is trying to control relaxation. “Okay, I have to relax my neck... Why doesn’t it relax? Oh God, I can’t even relax, I’m hopeless!” - this is classic rumination, which only adds adrenaline.

Remember: **relaxation is not an action. This is a lack of action.** There is no need to *do* relaxation. Your task is to create high-quality *tension*, and relaxation will happen on its own, according to the laws of physics. You just need to take your hands off the steering wheel and watch the warmth and heaviness return to your limbs.

4. Point of Support: Insight

> *Your body is not an enemy that swings and spins to torment you. This is a frightened beast that has cowered in a corner from endless mental lashing. Stop hitting him with your thoughts. Give him the physiological command “hang up.”*

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5. Practice: Progressive muscle relaxation (PMR)

Perform this complex every evening before going to bed. It will take exactly 20 minutes.

Instructions for performing the cycle:

1. **Pose:** Lying on your back, arms extended along the body, eyes closed.
2. **Breathing:** Take a deep breath, hold your breath and while holding it, strongly tense the target muscle group for **7 seconds**. Strain until you tremble, but without pain.
3. **Reset:** Exhale sharply through your mouth (“Haaaah”) and instantly release tension. Imagine that the strings that held the muscle were cut.
4. **Observation:** Just lie still for 20-30 seconds and listen to the sensations in your relaxed zone. Look for a feeling of warmth, tingling or heaviness.

Step-by-step protocol:

1. **Feet:** Clench your toes into a fist and pull your feet away from you (as if you were gripping your toes in the sand). Tighten up, hold your breath... Exhale - reset.
2. **Calves:** Pull your toes towards you, pressing your heels to the floor. Feel the tension in your shins... Reset.
3. **Hips and Glutes:** Squeeze your glutes and tighten the front of your thighs, straightening your legs like strings... Reset.

4. **Belly:** Tighten your abs as if you are preparing to take a blow or pull your stomach under your ribs... Reset.
5. **Hands:** Clench your fists tightly until your knuckles are white... Reset.
6. **Forearms and Biceps:** Bend your elbows, pressing your fists to your shoulders, contracting your biceps... Reset.
7. **Chest and shoulder blades:** Bring your shoulder blades together at the back, open your chest... Reset.
8. **Shoulders and Neck:** Raise your shoulders high towards your ears, pull your head into your shoulders... Reset.
9. **Face:** Close your eyes, wrinkle your nose, grit your teeth (without fanaticism) and put your whole face into a "grimace"... Reset.

At the end of the exercise, lie down for 2 minutes, feeling how your body "spreads" across the bed. If everything is done correctly, you will feel a pleasant heaviness, as if you are being pressed into the mattress. This is the normal tone of a healthy body.

Homework (DZ):

1. Perform a complete Jacobson 9-zone PMR **every evening before bed** for 7 days in a row.
2. In the morning, do the express version (5 minutes) immediately after waking up.
3. Before your first practice, read the detailed guide in the template 1.2_jacobson_guide.pdf.

Additional technique: Body Scan

Bodyscan is the second line of defense for those who have a particularly dense muscle shell. Unlike PMR, where you actively tense and relax your muscles, bodyscan is a **passive observation** of sensations in the body.

How to do it

1. Lie on your back. Close your eyes. Take 3 deep breaths.
2. Bring your attention to the **top of your head**. Don't try to relax - just observe: is there tension here? Warm? Tingling? Heaviness?
3. Slowly "move the beam of attention" down: forehead → eyes → jaw → neck → shoulders → arms → chest → stomach → hips → knees → feet.
4. Stay in each zone for 20–30 seconds. If you find tension, don't fight it. Mentally say: * "I see you. I let you be."*

5. After completing the full scan, lie down for 2 minutes and observe the general sensation of your body.

Duration: 15–20 minutes.

Why it works: Bodyscan trains interoception - the brain's ability to sense the body without fear. In people with PPPD, interoception is distorted: any sensation in the body (pulsation, tingling, slight tension) is interpreted as a threat. Bodyscan teaches the brain: "*Feelings in the body are just information, not a danger signal*".

Express-PMR: 5-minute version for the office

It is not always possible to lie down and do a complete PMR. Here's a shortened version that you can do while sitting at your desk:

1. **Jaw (10 sec):** Clench your teeth tightly for 5 seconds → relax. Notice how the warmth spreads.
2. **Shoulders (10 sec):** Pull your shoulders towards your ears for 5 seconds → drop down.
3. **Fists (10 sec):** Clench your fists for a maximum of 5 seconds → unclench and straighten your fingers.
4. **Feet (10 sec):** Press your toes inside the shoes for 5 seconds → relax.
5. **Breathing (60 sec):** Close your eyes, do 5 square breathing cycles (inhale 4 sec → hold 4 sec → exhale 4 sec → hold 4 sec).

Do a quick PMR every time you notice that you have clenched your jaw or raised your shoulders towards your ears. Especially useful before the lunch break and at the end of the working day.

Vestibular exercises for PPPD: exercises for retraining the brain

Why you stagger out of the blue and how to teach your brain to trust your inner gyroscope again, and not just your eyes.

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1. Essence and problems: life in the “pitching” mode

The most debilitating symptom of PPPD is the feeling that the ground is falling away from under your feet. You are standing on a hard floor, but it seems that you are slightly rocking from side to side, as if on the deck of a ship. In supermarkets with long colorful rows or in a crowd, this feeling turns into hell: the picture floats, your head becomes “cast iron”, and you want to quickly grab the cart or the wall.

You look at your feet as you walk. You control your every step, constantly checking your balance. You are afraid to turn your head again or shift your gaze sharply. The nervous system is in constant visual supercontrol mode - you are trying to “maintain” balance with the power of your gaze.

2. Physiological explanation: gyroscope failure

Normally, our balance is maintained by three independent systems:

1. **Vestibular apparatus** (inner ear) - acceleration and head tilt sensor.
2. **Vision** - tells where the horizon line and surrounding objects are.
3. **Proprioception** (muscle sense) - pressure sensors in the feet and tension in the joints, reporting contact with the support.

The brain collects signals from these three channels and glues them together into a single picture of a stable world. When you get really stressed (or really dizzy), the calibration fails. The brain got scared and “turned off” trust in the vestibular apparatus and proprioception, considering their sensors unreliable. Instead, he turned up the vision sensitivity to 100%. This is called **visual addiction**. Now, when you walk, the brain tries to stabilize the body only due to the visual image. As soon as you go into a colorful supermarket, where the picture is constantly moving, or just close your eyes, your vision stops helping, and you start to sway wildly.

The vestibular apparatus is absolutely physically healthy. He is simply “sleeping” because his brain has stopped listening to his signals. Our task is to forcefully wake him up and force the brain to trust the inner ear and feet again.

3. Cognitive traps: avoidance strategy and gaze fixation

The main cognitive trap here is the idea that if your head is spinning, you need to freeze and look at one point in order to stabilize the world. When you fix your gaze and avoid turning your head, you confirm the brain's hypothesis: * "Yes, moving is dangerous. If we turn our heads, we will fall."* The brain reinforces this mistake. To retrain the system, it must be purposefully introduced into a state of slight mismatch. **The brain only learns when it makes mistakes.** If you don't give it a reason to make mistakes and adjust its balance, it will never re-calibrate.

4. Point of Support: Insight

> *Your vacillation is not a symptom of illness. This is a desperate attempt by the brain to maintain balance with closed eyes and pinched ears. We will open his eyes to reality.*

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5. Practice: Vestibular calibration

Each exercise should be done according to the principle: **"A little uncomfortable"**. If you start to feel nauseous or feel very stormy, reduce the pace or amplitude. We need a gentle challenge, not a panic attack.

Perform the complex 2 times a day (morning and evening). To be safe, stand next to a wall or a stable chair.

Level 1: Statics (Turn on proprioception)

1. **Romberg stance:** Place your feet close to each other (toe to toe, heel to heel), lower your arms along your body. Take a deep breath. First, stand like this for 30 seconds with your eyes open. Then close your eyes for 30 seconds. Notice how the body begins to make small vibrations - it's the feet and legs that are "groping" for balance. Don't bother them.
2. **Single Leg Stand:** Standing in the corner of the room (for safety), lift one leg and hold your balance for 30 seconds with your eyes open. Repeat for the other leg. When it becomes easy, try doing this with your eyes closed for at least 10 seconds.

Level 2: Dynamics of the eyes and head (Separating vision and vestibular tissue)

1. **Look at a Point (Horizontal):** Extend your arm in front of you, raise your thumb to eye level (or choose a bright point on the wall). Without taking your eyes off the finger/point, begin to smoothly turn your head left and right (as if you were saying "no-no"). The finger should remain absolutely focused, and the background may be slightly blurred. Do it for 1 minute. Select the speed so that the picture does not double.

2. **Gaze at the Point (Vertical):** Same thing, but nod your head up and down (as if you are saying “yes, yes”) while keeping focus on your finger. Do it for 1 minute.

Level 3: Gait Retraining

1. **Tandem Step:** Walk along an imaginary line (or along a tile/carpet joint), placing the heel of one foot close to the toe of the other. Keep your head straight, looking forward in front of you, not down at your feet. Walk 10–15 steps forward and backward like this.
2. **Walking with head turns:** Walk at a normal pace along the corridor. At every second step, smoothly turn your head to the left, then after two steps - to the right. This trains the brain to ignore dynamic noise from turning the head while walking.

Homework (DZ):

1. Perform the complex **twice a day for 5 minutes**.
2. Get a progress tracker. Your goal for the first 2 weeks is to confidently hold the Romberg stance with your eyes closed for 30 seconds without panicking.
3. Stop looking at your feet when walking on the street. Lift your chin, look at the horizon, let your feet do the work.

Level 4: Advanced exercises (join in 2-3 weeks)

When levels 1–3 become comfortable, add:

1. **Walking on uneven surfaces:** Walk onto a dirt path in a park, gravel path or lawn. Walk slowly, look at the horizon (not at your feet). The feet must adapt to uneven surfaces without visual guidance. Start with 5 minutes, increase to 15.
2. **Cushion Stand:** Stand with both feet on a sofa cushion or soft rug. Hold your balance for 30 seconds with your eyes open, then 15 seconds with your eyes closed. An unstable surface forces the brain to engage proprioception and the vestibular system.
3. **Walking with head turns + counting:** Walk at a normal pace and turn your head left and right every second step. At the same time, count backwards from 100 (100, 99, 98...). This loads cognitive resource and prevents the brain from getting stuck on controlling balance—putting it on autopilot.

Progression table by week

Week	Exercises	Goal
1–2	Level 1 (static): Romberg + single leg stand	30 seconds with eyes closed without losing balance
3–4	Level 2 (eyes/head): VOR horizontal + vertical	1 minute without blurring the picture
5–6	Level 3 (gait): tandem step + walking with turns	15 steps without support
7–8	Level 4 (Advanced): Rough Surface + Cushion	30 seconds on a pillow with eyes closed

Integration into everyday life

Vestibular calibration is not just a 5-minute exercise in the morning and evening. You can train your brain **background** throughout the day:

- **In line at the store:** instead of standing tense and controlling your every step, shift your weight to one leg, wait for 20 seconds, change. Invisible stand on one leg.
- **In the kitchen:** when washing dishes, close your eyes for 15 seconds. Feel how your feet maintain balance. This is a micro-Romberg.
- **When walking:** consciously stop looking at your feet. Every time you catch yourself "looking at the asphalt," lift your chin and look at the farthest building. In a week it will become a habit.

Exercise machines: how to give your brain the right load

Why gymnastics alone is not enough - and what to do next when the basic exercises have already been mastered.

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1. The essence and problems: the brain does not trust its sensors

In the previous chapter, we started the process of calibrating the vestibular apparatus. Basic exercises are the foundation. But with PPPD, there is another layer of the problem that Kavtorn-Kuksi gymnastics does not completely cover.

The brain with PPPD is in two interconnected states:

1. **Sensory conflict** - vision is overloaded with trust, vestibular and proprioception are ignored. The brain refuses to redistribute weight between channels.
2. **Sympathetic hyperexcitability** - the nervous system is constantly "on edge". In this state, neuroplasticity is weakened: the brain does not learn, it survives.

Standard vestibular gymnastics trains the first. This section is about how to solve the latter, and how to enhance the former through more specific techniques.

2. Physiological explanation: three channels and one distrustful editor

Three systems maintain balance:

- **Vestibular** - acceleration sensor in the ear
- **Visual** - picture of the outside world
- **Proprioceptive** - pressure, tension, body position in space

Normally, the brain constantly weighs all three signals. With PPPD, a recalibration occurred: vision received a disproportionately large weight, and the other two channels were frozen. The brain has become "visually dependent."

Result: any moving visual field becomes destabilizing. Supermarket, crowd, escalator, scrolling - the brain interprets this as a threat, produces motion and anxiety. The circle is closed.

> ***Point of Support:*** *The brain is not broken. He retrained in the wrong direction. It can be retrained*

> *back. It will take time, but the mechanism works.*

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3. When NOT to do these exercises

Before you start, check the list. This is not insurance, this is elementary logic: you cannot train a system that is currently overloaded.

Hard stop:

- You may have BPPV (benign paroxysmal positional vertigo) - first Epley/Semont maneuvers, then VOR
- Acute phase of PPPD - the first 1-2 weeks after the onset of symptoms
- Any acute illness: ARVI, high fever, severe lack of sleep (less than 5 hours last night)
- Yesterday there was a strong pullback - take a break for 1 day

Situational stop:

- Anxiety right now is 8+/10 - first HRV breathing to 5/10, then training
- You just ate - wait 40–60 minutes
- My head is “exploding” at 9/10 right now - this is not a “good load”, this is an overload

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4. Practice: Four Techniques for Advanced Recalibration

Technique 1: VOR Training - Stable Gaze with Head Movement

How is it different from regular gymnastics: These are not just “nods”. This forces the vestibulo-ocular reflex (VOR), a reflex that keeps the gaze stable when the head moves. In PPPD it is deregulated.

How to do:

Basic level (weeks 1–2):

1. Choose a point on the wall at eye level - a letter, a cross, a corner of a frame.
2. Fix your gaze firmly. Don't tear it off.
3. Move your head left and right - the point should remain **clear**.
4. Blurred - slow down. Clarity = criterion for correct tempo.
5. Start with 20-30 movements per minute, 1-2 minutes, pause 30 seconds.

Intermediate (weeks 3–4):

- The same movements while standing.

- Add a vertical plane (nod up and down).
- Tempo up to 60–80 movements per minute.

Advanced level (weeks 5+):

- Walking with simultaneous turns of the head left and right.
- Look at the horizon, not at your feet.

Technique 2: Optokinetic training - habituation to a moving background

The essence of the problem: A crowd, a supermarket, a flickering screen, an escalator—all these are "optokinetic stimuli." With PPPD, the brain reacts to them as a threat: it produces nausea, unsteadiness, and anxiety. This is **not weakness**. This is a trained reflex.

How to do:

You need visual movement - a moving pattern, streaming video, a moving stream of objects.

- Look **relaxed**, without trying to follow individual objects
- Don't "hold on" with your gaze - let the background move
- Start with 30 seconds, add 15–20 seconds every 2–3 days
- Discomfort 3–4/10 – work area ✓
- Discomfort 7+/10 - stop, too early

Progression by level:

Level	Source of stimulation	Difficulty
1	Stripes/dots moving slowly (horizontal)	Low
2	Fast moving pattern	Average
3	First person video (movement through space)	High
4	Real environments: escalators, crowds, transport	Exposure

Technique 3: Saccades - restoration of rapid gaze switching

The essence of the problem: Saccades are quick, precise jumps of gaze from object to object. With PPPD, the oculomotor system is overloaded: the brain is accustomed to holding its gaze motionless, afraid of losing its visual "fulcrum." Switching gaze between objects causes a short-term loss of landmarks. Saccade training restores confident and rapid switching without disorientation.

How to do:

1. Hang two objects on the wall at a distance of 40–60 cm from each other (letters, numbers).
2. Move your gaze from one to another - **quickly and accurately**, without “floating” between them.
3. The head is **stationary** - only the eyes work.
4. 20–30 translations, pause, repeat.

Complication:

- Three or more points in a chaotic order - name the object out loud before looking at it.
- Add head movement simultaneously with saccades.

Technique 4: HRV breathing - transferring the NS to training mode

Why is this necessary: Neuroplasticity—the brain's ability to change neural connections—is significantly suppressed when cortisol levels are chronically high. When the nervous system is constantly on alert, the brain saves resources for survival rather than for learning. If you train the vestibular system in a state of high anxiety, the effect is minimal.

HRV (heart rate variability) breathing is a technique that directly activates the vagus nerve and switches the NS to parasympathetic.

Resonance frequency: 5-6 breaths per minute.

Protocol:

1. Inhale through the nose - 4 seconds
2. Exhale through the mouth - 6 seconds
3. 10 cycles = ~1.5 minutes

Do this **before** every VOR or optokinetics workout. This is not meditation or affirmations. This is a direct physiological switching of the nervous system into a mode in which the brain is able to retrain.

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5. Weekly progression table

Week	Technique	Volume
1–2	HRV breathing + VOR sitting (horizontal)	15 min/day
3–4	+ standing VOR + saccades	20 min/day
5–6	+ Optokinetics (slow patterns)	25 min/day

Week	Technique	Volume
7–8	+ Optokinetics fast + POV movement	30 min/day
9+	+ Exhibition in real environments (shop, street, transport)	Feels like

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6. What progress looks like - improvement markers

People quit after 2 weeks because they don't know what to consider as an improvement. Here are concrete signals that the system is working - even if it seems that "nothing is changing."

Weeks 1-3: Microsignals

- Optokinetics for 30 seconds gave 8/10 → now 4/10 with the same duration
- After VOR, the head "comes to its senses" in 5 minutes instead of 20
- I began to look less at my feet on the street - I noticed this after the fact

Weeks 4-6: noticeable changes

- You can spend 5 minutes longer in the supermarket without increasing anxiety
- Saccades have become more accurate - objects "do not float"
- In the morning, when you open your eyes, the motion goes faster

Weeks 7-12: Functional Wins

- Riding in a vehicle with objects moving outside the window is no longer a disaster
- You can turn your head in a conversation without thinking
- Scrolling the phone feed is no longer a trigger

> *If after 4 weeks none of the microsignals have appeared, check: are you doing exercises in sympathy (anxiety above 6)? This is the main reason for zero progress.*

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7. Dosage rules

80% Rule: Do 80% of your limit. Mild discomfort 3–4/10 is normal. Severe dizziness for more than 30 minutes after - too much, reduce the volume.

Rule of regularity: 15 minutes every day is better than an hour once a week. The brain learns through frequent repetition, not through one-time intensity.

Parasympathetic rule: if anxiety is now above 6/10 - first HRV breathing, and only then training. In high anxiety states, exercise has minimal effect and can provide negative reinforcement.

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8. Morning ritual: 7 minutes that set the tone for the day

Instead of training randomly, build it into the morning. The optimal time is 45–60 minutes after waking up: the peak of the cortisol response (Cortisol Awakening Response) occurs in the first 20–30 minutes after waking up, after which the level begins to subside and the brain enters a more stable operating state.

Sequence (7 minutes total):

1. **Stand up → 3 minutes of HRV breathing** (inhale 4 sec, exhale 6 sec, sitting on a bed or chair)
2. **2 minutes VOR** (sitting, horizontal, point on the wall opposite)
3. **1 minute of saccades** (two objects on the wall)
4. **1 minute optokinetics** (moving pattern when you reach the desired level)

This is not a replacement for a full workout - it is a daily “drip” for the vestibular system. The sequence is important: first parasympathetic, then load.

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9. Frequently asked questions

"It got worse after the first workout - did I break something?" No. A temporary increase in symptoms in the first 2-3 days is a normal reaction to an unusual load. The brain has received a new signal that it does not know how to process - while it is learning, symptoms may temporarily increase. Post-session discomfort that resolves within 15–30 minutes is normal. If more than an hour does not pass, reduce the volume, do not stop completely.

"How long will it take for the results to appear?" The first microsignals occur in 2–3 weeks. Noticeable changes occur within 4–6 weeks. Sustainable functional result – 3–6 months. This is not a course - it is brain retraining. The brain is slow but reliable.

“Can I do all four techniques at once?” No. One new technique every 2 weeks. Overloading the vestibular system is not “working hard”, it is negative reinforcement. The brain will remember the workout as a stressor rather than as a safe experience.

“I’m fine when I’m lying down and bad when I get up - is it the same story?” Most likely yes. In a vertical position, the vestibular system works at full capacity, plus visual flow is added. If there are no symptoms while lying down, this is a good sign: start training while sitting, move on to standing exercises only after 2 weeks of confident progress. If there are strong symptoms while lying down, you need to rule out other causes by a doctor.

10. What these techniques DO NOT do

The physical part is only half of the recovery. These techniques:

- ✓ Reduce sensory conflict
- ✓ Restores VOR and optokinetic tolerance
- ✓ Switch the NS to parasympathetic
- ✗ **Does not eliminate fear** - this is the work of exposure and CBT
- ✗ **Do not remove the belief "there is something wrong with me"** - this is the work of cognitive restructuring
- ✗ **Do not work at full capacity** without parallel psychological work

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Homework (DZ):

1. Add to your morning vestibular exercises **3 minutes of HRV breathing at the beginning**.
2. Choose **one** of the four techniques and do it daily for the first 2 weeks - not all at once.
3. Create a scale: before training, rate your discomfort from 0–10. After training - again. After 2 weeks, look at the dynamics.
4. Write down **one specific progress marker** - what you want to get easier first. You can compare in 4 weeks.

The next step is to figure out why the brain is so heavily addicted to vision and how this addiction works from the inside. That's in the next chapter.

Basic neurophysiology settings: fill the car with the right fuel

Why it's useless to meditate if you sleep for 5 hours and drown your anxiety in coffee, and how to set up your body's hardware for recovery.

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1. The essence and problems: morning hell and a dead battery

If you have PPPD, your typical morning looks like this: you barely open your eyes, and you are already starting to feel stormy and swaying. Even before your feet touch the floor, a sticky anxiety grows in your chest, your heart is pounding, and your head seems empty and cottony. You drag yourself to the kitchen, brew a cup of strong coffee in the hope of "cheering up and collecting your brains," but instead of vigor, all you get is increased internal tremors and tachycardia.

All day long you feel like a phone with a worn-out battery: the charge is draining before your eyes, you have no strength for anything, and every small task seems like an insurmountable obstacle. In the evening you go to bed in the hope of rest, but instead of sleep you get endless mental chewing gum (rumination) and wake up at 4 am in a cold sweat.

2. Physiological explanation: morning peak of cortisol and adrenaline reservoir

Why is morning the hardest time? This is pure physiology. Our body has a natural awakening mechanism called the Cortisol Awakening Response. 30 minutes before you wake up, the adrenal glands release a surge of cortisol (the stress hormone) and adrenaline to mobilize the body and raise blood sugar.

In a healthy person, this slight hormonal surge goes unnoticed. But in a neurotic person with PPPD, the "reservoir" of anxiety is already filled to the brim. The morning's natural surge of cortisol hits an already overheated nervous system. The brain interprets this physiological arousal as an alarm signal: * "It's bad again, it's shaking again!" * - it gets scared, makes a second release of adrenaline, squeezing the neck and causing that very "cotton" head.

To stabilize the system, we need to tweak the body's basic biology:

1. **Contrast shower** - trains the autonomic nervous system (vascular tone) and helps to quickly "wash away" morning cortisol, switching the system from sympathetic to parasympathetic.
2. **Movement (10,000 steps)** - evolutionarily, our brain burns adrenaline only through the work of the leg muscles. While walking, you literally give your brain a bodily signal: * "We ran away from the tiger,

the danger is behind us, we can reset our tone"*. If you are sitting on the sofa, adrenaline does not go away and continues to circulate in the blood, spasming the muscles of the neck.

3. **Sleep hygiene and melatonin** - the sleep hormone melatonin is the main antioxidant and restorer of the nervous system. It is produced only in complete darkness. Blue light from smartphone screens an hour before bedtime reduces its production by 50%, depriving your brain of the resource for nightly repair of neural connections.

3. Cognitive traps: "I'll get cured first, and then I'll get into a routine"

The main cognitive trap of an excellent student in neurosis: * "I feel so bad now, what are 10,000 more steps? When it stops pumping, then I'll go for a walk/do exercises/sleep normally."*

It's an illusion. You're trying to pour 92-octane gasoline into a Formula 1 racing car and wonder why it sneezes and won't drive. Neuroplasticity and psychological CBT techniques will only work at 30% if your "hardware" (body) is physiologically depleted. The mode and basic settings are not a "bonus" to treatment, it is a foundation, without which the walls will collapse with any gust of wind.

4. Point of Support: Insight

> *Stop demanding complex analytics from an overheated processor if you haven't cleaned it of dust and replaced the thermal paste. Your body is a biological machine. Start with basic maintenance.*

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5. Practice: "Basic Settings" Protocol

Implement these three rules every day, without compromise. Your brain needs a stable and predictable environment to believe it is safe.

1. Morning Cortisol Reset Ritual

- **Water balance:** Immediately after waking up, drink a glass of clean, warm water. This will reduce blood viscosity and ease the work of the heart.
- **Contrast shower:** Get under the shower. Make the water comfortably warm. Then switch to cool (not ice!) for 15 seconds. Return warm water for 30 seconds. Repeat the cycle 3 times, ending with cool water. This instantly tones the blood vessels and "washes away" the morning stagnation of adrenaline.
- **Morning meditation (10 minutes):** Perform strictly according to the safe vestibular protocol from Chapter 47: Meditation for PPPD (with half-open eyes and external audio accompaniment, so as not to cause an attack of unsteadiness from loss of vision). Harvard research (Hölzel, 2011) has shown that 8 weeks of regular practice physically thickens the **prefrontal cortex** (rational control area) and

reduces gray matter density in the **amygdala** (fear center). You are literally growing a panic brake. Slow, rhythmic breathing stimulates the **vagus nerve**, switching the autonomic system from fight or flight mode to recovery mode. The activity of the **Default Mode Network**, the brain network where rumination and "mental chewing gum" live, also decreases. 10 minutes a day, every morning, no phones for the first 30 minutes of the day.

2. Physical adrenaline burn (10,000 steps)

- Buy a fitness bracelet or put a pedometer on your phone.
- Your task is to walk 10,000 steps outdoors every day. Walk at a medium pace. If you sway, keep your back straight and apply the "Horizon Rule" (look forward, not at your feet). Leg movement is a physiological stress reliever.

3. Circadian Hygiene (Rule of Darkness)

- **1 hour before bedtime:** No phones, tablets or TVs. The blue spectrum of the screen tricks the pineal gland into thinking it is daytime.
- **Bedroom:** Hang blackout curtains or use a sleep mask. The room should be completely dark and cool.

Homework (DZ):

1. Implement a **morning protocol**: contrast shower → 10 minutes of meditation (using the technique from Chapter 47) → breakfast. Do it 7 days in a row without skipping. Record the level of unsteadiness in your diary in the morning (before the protocol) and 30 minutes later (after).
2. Start walking **at least 10,000 steps per day**. Not necessarily for one walk - count in total.
3. Read the section on the effects of caffeine and alcohol below. If you drink more than 2 cups of coffee, reduce it to 1 for a week and notice the difference.

Caffeine, alcohol, nicotine: effects on PPPD

Caffeine

Caffeine blocks adenosine receptors, which are responsible for inhibiting the nervous system. Essentially, it "turns off the brakes" on vegetation. Result: *Elevated cortisol levels 2-3 hours after the cup.

- Increased muscle tension (especially neck, jaw).
- Deterioration in the quality of sleep (even if you drink coffee only in the morning - caffeine is eliminated within 6-8 hours).

Recommendation: Maximum 1 cup in the morning. Ideally, replace it with green tea (contains L-theanine, which softens the effect of caffeine) or cocoa (contains theobromine, a mild stimulant without affecting the vegetative system).

Alcohol

Alcohol works as a GABA antagonist - for the first hour it relaxes, reduces anxiety, and removes unsteadiness. You feel like a "normal person." But after 4–6 hours, a **ricochet effect** begins: the central nervous system, which was suppressed by alcohol, becomes compensatory excited. Result: morning anxiety, unsteadiness, panic - 2 times stronger than before drinking.

Recommendation: Complete abstinence from alcohol for at least 2 months of working on the book. Then - no more than 1-2 glasses of wine per week, only with food, not on an empty stomach. If you notice that after even 1 glass, unsteadiness returns in the morning, it means that your vegetative system is still too labile for alcohol. Wait another month.

Nicotine

Nicotine is a sympathomimetic. It directly stimulates the release of adrenaline and norepinephrine. Every cigarette is a mini-panic attack that you give yourself. Plus, nicotine constricts the blood vessels in the neck (already tight in people with PPPD), impairing cerebral blood flow.

Recommendation: Quit. Not "someday," but now. If you can't do it suddenly, switch to nicotine patches (they give a stable dose without adrenaline peaks from smoking). Vapes are not an alternative: they contain the same nicotine plus glycerin, which irritates the respiratory tract.

Visual addiction: unhooking from eye anchors

Why supermarkets, subways and colorful asphalt cause panic and how to apply the "Horizon Rule" to restore stability.

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1. The essence and problems: the nightmare of supermarkets and colorful floors

Every person with PPPD fears the same thing: large, open spaces with lots of detail. A trip to the supermarket turns into torture. As soon as you enter the hall with high shelves littered with colorful boxes, the world begins to "float". The motley tiled floor seems uneven, your legs feel weak, and your head feels foggy.

The same story happens on the subway, at busy intersections, or even at home when scrolling through the news feed on your phone. You notice that your body instinctively shrinks, and your gaze is firmly fixed on the asphalt right in front of the toes of your shoes. You are afraid to look up, because it seems that if you stop looking at your feet, you will immediately stumble and fall.

2. Physiological explanation: optical flow and video card overload

This phenomenon in medicine is called **visual dependence**.

When the brain, due to high levels of anxiety, stopped trusting signals from the inner ear and feet, it transferred 100% of balance control to the eyes. Your gaze has become a crutch. As long as you are in a small, familiar room with white walls, everything is fine. But as soon as you step outside, your retina is bombarded with a huge array of moving information. In science this is called **Optic Flow**.

Cars are driving, people are walking, supermarket shelves are full of goods, tiles are flashing under your feet. In a healthy person, the brain filters out this "noise", understanding: *they* are moving, not *I*, because the vestibular apparatus is working stably. But your vestibular signals are blocked by your brain. The brain focuses only on vision. Seeing the chaotic movement around, he falls into a stupor. It seems to him: "Everything around is moving, which means I'm moving too!" We are falling! An instant vegetative crisis occurs - a surge of adrenaline, nausea, unsteadiness and panic. Your internal computer simply freezes due to the overload of the "video card".

3. Cognitive traps: a saving glance at the asphalt

The logic of the frightened brain is simple: * "Since the world is spinning around me, I will look strictly at my feet on the ground in order to control every step. It's safer this way."*

This is a major cognitive trap. Looking at your feet, you shorten your visual horizon to one meter. Your brain is deprived of a three-dimensional, three-dimensional perspective and ceases to see stable landmarks in the distance (buildings, trees, the horizon line). Without these reference points, your brain has an even harder time calculating balance, and it tightens your neck muscles even more. It turns out to be a vicious circle: you look at your feet so as not to fall ➔ your brain loses its bearings ➔ your unsteadiness intensifies ➔ you shrink and look at your feet even more closely.

4. Point of Support: Insight

> *Your eyes are sensors for collecting information about the world, not anchor bolts with which you must fasten yourself to the asphalt. Let your legs move on their own; they have been able to do this since they were one and a half years old. Raise your head.*

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5. Practice: Breaking visual dependence

Our goal is to retrain the brain to process optical flow without panic and teach it to let go of visual control.

1. Horizon Rule

Every time you go outside:

- **Be aware of the clamp:** Notice how your head is tilted down, and your gaze scans the asphalt in search of holes and cracks.
- **Do a reset:** Relax your shoulders, open your teeth, gently lift your chin so that it is parallel to the ground.
- **Find the distant anchor:** Move your gaze 10–15 meters ahead or to the horizon line (roofs of houses, treetops). Walk, keeping your attention on this long-term perspective. Let your peripheral vision blur what's happening under your feet. Your feet will adapt to the unevenness of the road on their own thanks to proprioception if you stop disturbing them.

2. Display in a supermarket

There is no need to avoid stores. We will use them as a simulator:

- Go to the supermarket. As soon as you feel a little dizzy from the diversity of the rows, stop. Don't run towards the exit.
- Look for a static sign at the far end of the hall (for example, “EXIT”, “MILK” or a clock).
- Fix your gaze on this inscription. Exhale slowly. Stand and look at it for 1 minute, ignoring the flashing of people and goods on the sides. The picture around the periphery will begin to stabilize. When the

hesitation subsides, keep moving, keeping your focus on global reference points rather than on individual boxes on the shelves.

Homework (DZ):

1. **For 7 days, apply the "Horizon Rule"** whenever you walk outside. Catch yourself automatically looking at your feet and force your gaze back to the horizon.
2. Do one training session in a store: go there for 10 minutes solely to practice fixing your gaze on distant landmarks. Record your anxiety level before and after practice on a scale from 1 to 10 in a notebook.

Protocol for working behind the screen

If you work on a computer, you are at risk. The screen creates an intense optical flow in a confined space: scrolling text, switching tabs, animations. For the visually dependent brain, this is a continuous vestibular trigger.

Rules for working behind the monitor:

1. **20-20-20 Rule:** Every 20 minutes, look at an object 20 feet (6 meters) away for 20 seconds. This switches vision from near to distant focus and relieves spasm of the ciliary muscle.
2. **Font Size:** Increase the font in your editor/browser by 20-30%. Small text causes the brain to "clean" its eyes to the screen, increasing visual dependence.
3. **Smooth scrolling:** Turn off the scrolling animation in the settings (or scroll page by page rather than smoothly). Flashing lines is a powerful optical trigger.
4. **Dark theme:** Switch your IDE, browser and instant messengers to dark mode. Less brightness means less load on the visual cortex.
5. **Monitor position:** The top edge of the screen is at eye level or slightly below. Tilt of the head downward pinches the neck and increases unsteadiness.

Exercise: Optokinetic Desensitization

This is an advanced technique for those who have a strong visual addiction (hard in stores, the subway, when scrolling the screen):

1. Open the "optokinetic stimulation" video on YouTube (black and white stripes moving horizontally or vertically).
2. Sit 1 meter away from the screen. Look at the center of the video, allowing the stripes to move around the periphery. Don't track them with your eyes - look "through" the screen.
3. Start with 30 seconds. If you don't feel nauseous, increase to 1 minute, then to 2 minutes.

4. Do it once a day for 2 weeks.

Why it works: You train your brain to ignore a moving visual background by relying on vestibular and proprioceptive signals. This is essentially exposure for the visual cortex.

Sleep and PPPD: why the night is a battlefield, and how to regain your rest

Insomnia in PPPD is not a separate disease. This is a symptom of the same overheated "software" that rocks you from side to side during the day. But it is at night that the brain must reboot - and if the reboot does not happen, you get up in the morning feeling like you never went to bed.

Why the night turns into a nightmare

You will probably recognize it: in the evening your body is tired, your eyes are stuck together, you go to bed - and **bam**. The dream evaporates. Instead of relaxation, the scanner turns on: * "What if an attack happens at night? What if I wake up and start shaking? The heart somehow beats quickly... Or not? Let me listen..."*

Sound familiar? This is not just a "bad dream". This is an **adrenaline loop in a horizontal position**.

Here's what happens at the neurochemical level:

1. **Cortisol does not decrease.** Normally, the level of cortisol (stress hormone) drops in the evening and reaches a minimum at midnight. In a person with PPPD, the adrenal glands work in emergency mode - cortisol remains elevated 24/7. The brain perceives this as a signal: *"The danger has not gone away, don't fall asleep"*.
2. **The reticular formation does not turn off.** This is an ancient structure of the brain stem that is responsible for the level of wakefulness. In an anxious person, it is chronically overexcited. Imagine trying to turn off your computer, but 47 programs display a "Are you sure?" window. - and each one blocks the completion of work.
3. **Vestibular sensations are enhanced in the dark.** When you lie with your eyes closed, the brain is deprived of visual information. For the PPPD student, this is a disaster - because his brain is accustomed to **relying excessively on vision** (Chapter 8). Without visual "support," vestibular signals come to the fore: the head begins to "float," the bed seems to sway, and a feeling of sinking appears.

Morning hell: why the first hour is the hardest

If you have noticed that you are more unsteady in the morning than in the evening, this is not a coincidence. There is a precise neurophysiological explanation for this:

- **Morning peak of cortisol (CAR - Cortisol Awakening Response).** Normally, cortisol jumps sharply in the first 30-45 minutes after waking up - this is a physiological "alarm clock" that raises blood pressure and accelerates metabolism. In anxious people, the CAR peak is **2-3 times more powerful than normal**. Result: you wake up with a pulse of 100, wobbly legs and the feeling of "I'm going to fall."
- **Muscle rigidity.** During the night, the muscles of the neck and trapezius become stiff (remember the muscle shell from Chapter 4?). Tight neck muscles distort proprioceptive signals - the brain literally receives "broken data" about the position of the head in space.
- **Orthostatic load.** The transition from horizontal to vertical is a vestibular stress test. For a healthy brain it is invisible. For an overheated PPPD brain, this is like running a heavy application on a computer with 99% CPU load.

> ***Insight:*** *Morning wobbliness is not a "worsening illness." This is normal physiology, coupled with an alarming superstructure. It goes away within 1-2 hours, when cortisol returns to normal, the muscles warm up and the brain "warms up". You haven't gotten worse overnight—you just haven't "loaded up" yet.*

Three types of insomnia with PPPD

Not all insomnia is the same. Determine your type - this is important for choosing tactics:

1. Presomnia (difficulty falling asleep)

You lie there for an hour, two, three. Thoughts spin like a squirrel in a wheel. If you try to "force yourself to sleep," it gets even worse. The harder you try, the further you sleep.

Mechanism: Trying to control sleep is the same CAS trap (Chapter 10). Monitoring the process of falling asleep makes it impossible. Sleep is the only process that happens **only when you stop controlling it**.

2. Intrasomnia (frequent awakenings)

You fall asleep normally, but wake up 3-5 times a night. Every awakening is accompanied by a mini-panic: * "Where am I? What's happened? Why is my heart beating?"*

Mechanism: The adrenal glands continue to secrete adrenaline in micropulses. Each impulse is a mini-awakening. Normally, you would roll over and go back to sleep, but your anxious brain interprets waking up as a threat.

3. Post-somnia (early awakening)

You wake up at 4-5 in the morning and can't fall back to sleep. The head has already "turned on" and started scanning the symptoms.

Mechanism: Early awakenings are often a marker of elevated cortisol and/or subclinical depression. If this is your type, pay special attention to the section on sleep hygiene below.

Information diet before bed

Your brain processes information for another 2-3 hours after you "stop thinking." This is why what you consume 2 hours before bed is **critically important**.

Strictly prohibited 2 hours before bedtime:

- ❌ Medical forums and groups about PPPD/dizziness
- ❌ News feeds (negative content activates the amygdala)
- ❌ Bright screens without blue light filter
- ❌ Scrolling social networks (dopamine loops do not allow the brain to "complete work")
- ❌ Difficult conversations, conflicts, clarification of relationships

What you can and need:

- ✅ Fiction (paper, not from the screen)
- ✅ Calm or ambient music
- ✅ Light stretching / relaxation according to Jacobson (Chapter 5)
- ✅ Breathing "4-7-8" (inhale 4 sec, hold 7 sec, exhale 8 sec)
- ✅ Warm shower (lowering body temperature after a shower = signal to fall asleep)

Sleep hygiene: 10 rules of the central nervous system

This is not "tips from a magazine." Each point is a specific neurophysiological lever that you can switch:

1. **Fixed rise time** (±15 minutes, including weekends). The circadian rhythm is not an alarm clock, it is a pendulum. If you "sleep in" on Saturday until 12, you knock down the pendulum, and on Sunday evening it will be impossible to sleep.
2. **Bed = sleep only.** Don't work in bed, don't scroll on your phone, don't watch YouTube. The brain must associate the bed with one action - falling asleep. This is called **stimulus control**.

3. **20 minute rule.** If you don’t fall asleep within 20 minutes, get up. Go to another room, do something boring (sort through your socks, read the microwave instructions). Return to bed when you feel sleepy. It sounds counterintuitive, but it works: you break the association “bed = torture.”
4. **The temperature in the room is 18–20°C.** The brain falls asleep when the body temperature decreases. This does not happen in a hot room.
5. **Total darkness.** Even minimal light from the charging indicator suppresses melatonin production. Use a sleep mask or blackout curtains.
6. **Caffeine - stop before 14:00.** Caffeine half-life is 5-7 hours. A cup of coffee at 16:00 = half the dose of caffeine in the blood at 22:00. It's not a "feeling alert" - it's a chemical that blocks the adenosine receptors responsible for making you sleepy.
7. **Alcohol is not a sleeping pill.** Yes, it helps you sleep. But it disrupts the REM (rapid eye movement) sleep phase, in which the brain processes emotions and “digests” the stress of the day. In the morning after drinking alcohol, anxiety **increases** - this is called “rebound anxiety.”
8. **Physical activity - no later than 4 hours before bedtime.** Sports increases body temperature and adrenaline levels. It takes time for the system to cool down.
9. **Ritual for ending the day.** Create a sequence of 3-4 actions that you repeat every evening: shower → tea → 10 minutes of reading → relaxation → lights off. After 2 weeks, the brain will begin to perceive this ritual as a command to “start shutting down.”
10. **Don't look at the clock.** Every glance at the clock at night starts a calculation: "*Only 4 hours left... I haven't slept for 3 hours... tomorrow will be a terrible day*". This is rumination. Remove the clock from your sight.

Paradoxical intention for sleep

Remember Viktor Frankl (Chapter 12)? Its paradoxical intention works ideally with insomnia.

Instead of: *“I have to sleep, I NEED to sleep, why am I not sleeping?!”*

Tell yourself: *“Great. I will lie with my eyes closed and try NOT to fall asleep for as long as possible. Let's see how long I can hold out.*"

Sounds absurd? That's exactly how it works. You take the pressure off. You stop “trying to sleep.” And sleep is a process that comes **only when you stop waiting for it**. Like a bus that always arrives when your back is turned.

> ***Insight:*** *Insomnia in PPPD is not “just another disease.” This is the same brain bug: hypercontrol. You are trying to control a process that by definition requires a loss of control. The only way to sleep is to*

> ***give yourself permission to stay awake***.

Melatonin, magnesium and what really works

Briefly about supplements, because everyone asks:

- **Melatonin (0.3–1 mg).** Works as a chronobiotic - helps shift the circadian rhythm if you go to bed late. Take 30-60 minutes before desired bedtime. Important: a dose of more than 3 mg **does not make it more effective**, but causes lethargy in the morning and disrupts its own production.
- **Magnesium (glycinate or threonate, 200–400 mg).** Participates in the functioning of GABA receptors, the "braking" system of the brain. Magnesium deficiency = overstimulation. Glycinate is better absorbed and does not cause diarrhea (unlike citrate).
- **L-theanine (200 mg).** Amino acid from green tea. Increases alpha waves in the brain - a state of calm wakefulness. Does not cause drowsiness, but reduces anxiety.
- **Hypnotics (zopiclone, zolpidem) - only in short courses under the supervision of a doctor.** They do not treat insomnia. They mask it, like a painkiller masks a fracture. When discontinued, insomnia returns with a vengeance (rebound insomnia).

"Sleepy Diary": tracking that works

Create a simple table for 2 weeks. Fill it out every morning. This has two powerful effects: (1) you see the real picture instead of the subjective "I didn't sleep at all", (2) you notice patterns.

Date	lay down	Fell asleep ≈	Woke up	Got up	Total sleep	Morning unsteadiness (0–10)	Notes

> ***Fact:*** *Research shows that people with anxious insomnia systematically* ***overestimate*** *their time awake. It seems to you that you have not slept all night - in fact, you slept for 5-6 hours intermittently. The diary will show this.*

##Homework

> ***1. Sleepy diary.*** *Download or draw the table above. Fill out every morning for 14 days. Don't miss a single day—the pattern will become visible on days 7–10.*

> ***2. Information diet.*** *Set a curfew for screens: 1.5 hours before bedtime - no medical forums, news or social networks. Replace them with a ritual for ending the day (see point 9 of sleep hygiene).*

> ***3. 20-minute rule.*** *If you don't fall asleep within 20 minutes, get up and get out of bed. Yes, it's difficult. Yes, it works. Try 5 nights in a row.*

> ***4. Morning protocol.*** *In the first hour after waking up: (1) 5 minutes of neck and trapezius stretches, (2) a glass of water, (3) go out to the balcony/window in the sunlight (suppresses melatonin, speeds up "loading"). Don't check your phone for the first 30 minutes.*

> ***5. Paradoxical intention.*** *Tonight try to lie down and say to yourself: "I won't fall asleep. I will lie with my eyes closed and listen to my breathing. I'll fall asleep - it's good. If I don't fall asleep, that's also normal, the body is still resting." Write down the result.*

Focus for anxiety and PPPD: how to stop listening to the body

Why anxiety tightly rivets attention to the body, how hyperfocus increases unsteadiness and panic tenfold, and evidence-based attention training techniques (ATT, Open Focus, exteroception) to regain control.

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1. Essence and problems: attention is held hostage by anxiety

Think back to your typical day with constant dizziness or anxiety.

You open your eyes in the morning, and before your feet even hit the floor, your internal processor is already running a background self-diagnosis script:

- *"So, is your head heavy or not?"*
- *"Does the picture float before your eyes?"*
- *"Is your neck strained? Is your pulse even?"*

You get out of bed, go to the bathroom - and your entire consciousness narrows to a microscopic point: you listen to every millimeter of your body. You scan the way your foot steps on the tile, whether there is any swaying in the hallway, whether your vision has darkened when you lean over the sink.

When you go outside, this mode does not turn off for a second. In a supermarket, you don't choose products—you watch how your eyes react to the rows of colorful packages and fluorescent light. In a conversation with a colleague, you are half absent, because the other half of your brain is busy with every second measurement: * "It seems that the floor under your feet has swayed. If I fall now, will I have time to grab the table?"*.

In clinical psychology and psychiatry this is called **interoceptive hyperscanning** (*body checking*) and **loss of flexibility of attention**.

Trap "Get distracted!"

Every person with neurosis, PPPD or panic disorder has heard this "brilliant" advice at least once from loved ones or incompetent doctors: * "Just don't think about it! Take a break from work, read a book, watch a TV series."*

And you honestly tried. You open the book, start reading the page... and after two paragraphs you catch yourself with your eyes running over the lines, and all your attention focused on the pulsation in your temples or the wobbly legs.

Why does the attempt to "just get distracted" fail miserably?

Because a fundamental law of neuropsychology, discovered by Harvard professor Daniel Wegner, is working in your head - **the theory of ironic processes** (the famous *"don't think about the white monkey" effect*).

When you tell your brain: *"Stop paying attention to the dizziness!"*, the brain is forced to launch two parallel processes:

1. **Operation process:** tries to find a third-party object (book, movie, work).
2. **Monitoring process (supervisor):** checks with high frequency - *"Are we sure we're not thinking about dizziness right now?"*.

For the second task, the supervisor must constantly keep the image of the symptom in working memory! As a result, an attempt at volitional suppression only **concretes the symptom** in the focus of your consciousness. You are trying to put out a fire with gasoline.

If you have already read the chapter on CAS trap, you will recognize this pattern: symptom monitoring, rumination, attempt at suppression - three gears of one mechanism. This chapter gives you specific tools to jam the main gear - monitoring.

To stop being a hostage to symptoms, you need to stop fighting them and understand the physics of how attention works in your brain.

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2. Neurophysiology: hardware architecture of attention

Attention is not an abstract philosophical concept. This is a strictly localized physiological process of managing the resources of the central nervous system. It's a **hardware traffic router** in your brain.

Where your attention is directed, the brain throws up to 90% of its processing power, metabolic resources and blood flow there:

> ***How your attention beam works:***
>
> - ***Focus to body:*** *Natural noise* ➔ ***Hyperfocus (lens)*** ➔ *Signal amplified 10x*
> - ***Outward Focus:*** *External World* ➔ ***Exteroception (radar)*** ➔ *Background symptom fades*

Let's look at four key neurobiological nodes that determine your state of anxiety:

1. DMN vs CEN: Brain mode switch

In neurobiology, there are two competing global neural networks:

- **DMN (Default Mode Network):** activates when you are self-absorbed, ruminating ("mental chewing gum"), remembering past attacks, worrying about the future and scanning the body. The DMN is an incubator for anxiety disorders.
- **CEN / TPN (Central Executive Network / Task-Positive Network):** activated when your attention is directed outward to solving specific sensory or motor tasks (looking at parts, solving an engineering problem, actively listening to street sounds).

Main law: **DMN and CEN are in a reciprocal (mutually exclusive) relationship**. They work like a swing. When you activate attention externally (CEN), the activity of the area of rumination and fear (DMN) **automatically drops at the level of functional MRI**. You physically cannot panic and deeply scan your body at a time when your attention is 100% occupied by a complex external sensory task.

2. Thalamus and sensory gating

The **thalamus** is located in the brain stem - the main firewall of the nervous system. Every second, gigabytes of sensory noise enter the body: micro-oscillations of the endolymph in the cochlea, tension of the fascia of the neck during breathing, pulsation of the carotid arteries, contractions of the smooth muscles of the intestines.

Normally, the thalamus operates in strict spam filter mode: it cuts off 99.9% of this technical noise and does not allow it to enter the cerebral cortex. A healthy person does not feel how his vestibular nuclei make micro-corrections to the tone of the calf muscles every second.

But when there is anxiety, the **amygdala** sounds the alarm and starts the adrenaline loop: *"We are in danger! Failure of the vestibular apparatus is possible!"*. On this command, the thalamus opens the gates (sensory gating breaks down). All the noise under the hood of vegetation spills straight into consciousness.

You begin to physically feel the normal mechanics of the vestibular system. And your frightened mind labels this natural noise: * "I'm stormy, I'm having a stroke, I'm dying"*.

3. Neurosensory amplification (Macro lens effect)

Attention works exactly like a macro lens with 10x zoom.

If you put a coin under a microscope, its micro-scratches will appear as deep canyons. The same thing happens with the body:

- Light tension in the trapezius when focusing turns into a feeling as if the neck was chained in concrete.
- The natural micro-oscillation of the center of gravity when standing (which all people have from birth) when fixing attention is felt like a force nine storm on the deck of a destroyer.

The neurons of the somatosensory cortex have the property of plasticity: the more time you spend listening to a certain part of the body, the wider the representation of this zone in the brain becomes and

the lower the threshold of its excitability falls. You train your brain to feel dizziness more acutely. Read more about how chronic spasm of the neck muscles enhances this effect in the chapter on muscle armor.

Vicious circle of attention:

> 1. ***Body sensation*** *(microvibration, neck spasm, pulsation)* ➔
> 2. ***Amygdala:*** *alarm signal "We are in danger!"* ➔
> 3. ***Narrowing of attention:*** *focus is firmly tied to the symptom* ➔
> 4. ***Neurosensory amplification:*** *the signal is inflated 10 times* ➔
> 5. ***Adrenaline surge:*** *vasospasm and panic intensify* ➔ *the circle closes.*

4. Narrow focus vs Open focus: Sympathetic and Parasympathetic

The nature of your attention directly switches the autonomic nervous system:

- **Narrow, tunnel focus (Focus on Threat):** when the gaze is fixed at one point, the eyes are motionless, all attention is clenched into a fist on the symptom. For the evolutionary brain, this is a signal: * "We see a predator! Turn on your sympathy!"*. There is an instant release of norepinephrine, spasm of the neck vessels, dilation of the pupils - the unsteadiness intensifies.
- **Open/Panoramic Focus:** Soft peripheral vision, perception of space between objects, simultaneous 360-degree coverage of the sound field. For the brain, this is a biological marker of complete safety: there is no predator, the clearing is clean. The **dorsal nucleus of the vagus nerve (n. vagus)** is instantly activated, parasympathetic activity is activated, the heart rate drops, and the spasmodic suboccipital muscles relax.

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3. Why attention training is the basis for recovery

Most people believe that attention is a slave to their thoughts and symptoms. *"I felt bad, that's why my attention is there"*.

This is a fundamental mistake.

Attention is not a thought. Attention is a **muscle** (more precisely, a network of the dorsolateral prefrontal cortex) that can and should be consciously controlled.

If this muscle is weak and atrophied, any physiological sneeze, any vascular spasm instantly takes consciousness hostage. You become a passive spectator of a horror film that is spinning in your body.

Attention training is a gym for the prefrontal cortex. You teach your brain:

1. **Notice the moment of capture:** Instantly register when attention fell into the body and stuck to unsteadiness.
2. **Unstick the beam:** With an effort of will, tear the spotlight away from the somatic sensation, without fighting with it.
3. **Redirect and Hold:** Voluntarily direct processor resources to neutral or external stimuli.

When you master this skill, dizziness is deprived of its main fuel - **your emotionally charged energy**. A symptom, deprived of attention, quickly fades out, like a fire into which they have stopped throwing dry branches.

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4. Point of Support: Main insight

> *Anxiety and somatoform dizziness cannot exist in an empty space. They are parasites that feed solely on your attention. Your symptoms do not grow from physical illness, but from the spotlight directed at them.*
>
> *Take away the light from them - and they will have nothing to cling to in your nervous system.*

Andrey, 34 years old, programmer: "I walked around the apartment with a pulse oximeter on my finger. I checked my pulse every 15 minutes. When my wife was cooking soup, I heard not the sound of water, but the beating of my own heart. Three cardiologists said everything was normal. When I started doing ATT 12 minutes a day, after 10 days I forgot to check my pulse for the first time in a year and a half. Not because I forced myself - but because my brain just switched."

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5. Evidence-based attention training techniques

Below are 4 of the most powerful, clinically validated techniques for working with focus. They are used in metacognitive therapy (MCT), neurofeedback and body-oriented regulation protocols.

Technique No. 1. ATT (Attention Training Technique) by Adrian Wells

Developed by Adrian Wells, professor of clinical psychology at the University of Manchester. This is the gold standard metacognitive therapy for breaking cognitive attention syndrome (CAS).

ATT is **not meditation or Jacobson relaxation**. Your goal here is not to calm down or relax. Your goal is to give the prefrontal cortex a tough training load on coordination and attention switching.

Preparation:

- Sit comfortably in a chair, close your eyes (or keep a soft, defocused gaze at one point).
- Select from 5 to 8 different sound sources:
- **In the room (near field):** the ticking of a clock, the hum of a laptop cooler, the sound of breathing in the nose.
- **Outside (far field):** the noise of cars outside the window, birds singing, footsteps on the stairs, the sound of the wind.

The protocol consists of 3 consecutive phases with a total duration of 10–12 minutes:

Phase 1: Selective Attention (5-6 minutes)

You learn to focus the spotlight on one specific sound and completely ignore the rest. *Focus **only** on the ticking of the clock. Listen to its volume, intervals, timbre for 45 seconds. If your brain is distracted by thoughts or sounds of machines, gently, without self-criticism, return the beam to the ticking of the clock.

- Now turn the beam **only** to the noise of the laptop cooler. Isolate it from the sound stream. Listen to him for 45 seconds only.
- Turn your focus to the most distant sound outside the window - the hum of passing cars. Hold for 45 seconds.
- Go through 5-6 different sounds one by one.

Phase 2. Rapid attention shift (3-4 minutes)

Now you train the speed of moving the focus. Like ping pong:

- Hear the cooler sound (3 seconds).
- Instantly switch to the birds outside the window (3 seconds).
- Instantly when the clock ticks (3 seconds).
- On the breathing of the neighbor behind the wall (3 seconds).
- Repeat fast jumping tricks in a circle for 3 minutes. You should feel your prefrontal cortex actively flicking switches.

Phase 3: Divided/Divided Attention (2 minutes)

The most powerful step that completely turns off the DMN:

- Try to expand your auditory radar so that you can **hold all sounds in your mind at once**.
- You hear the cooler, the ticking of the clock, the cars outside the window, and your own breathing as a single symphony. *Hold this polyphonic perception for 2 minutes. If the focus narrows to one sound, open it again to the entire panorama.

Technique No. 2. Open Focus by Dr. Lester Femi

Lester Femi is a pioneer of neurofeedback who has spent many years studying the electrical activity of the brain using EEG. He found that people with anxiety, panic, and chronic pain are chronically stuck in a "narrow object focus."

The Open Focus technique instantly triggers **synchronous alpha rhythms (8-12 Hz)** in the cortex - a physiological marker of calm concentration without tension.

Execution algorithm (5 minutes):

1. Sit up straight without straining your back. Direct your gaze straight ahead.
2. Stop looking at specific objects. Soften your gaze, let your eyes "blur" slightly, turning on your **peripheral vision**.
3. Without turning your pupils, notice what is to your left at the extreme limit of visibility. At the same time, notice what is on the right.
4. Ask yourself a series of metacognitive questions while maintaining an unfocused gaze:

- *"Can I feel the space between my eyes and the screen/wall?"*
- *"Can I become aware of the space between my ears inside my head?"*
- *"Can I feel the space between my fingertips and the floor?"*
- *"Can I be simultaneously aware of the space around my body and the silence between sounds?"*

Focusing on **space** (on "nothing" instead of "something") makes it impossible for the brain to grasp onto a threat. The muscles of the face, eyes and suboccipital area instantly relax. Breathing deepens on its own.

Technique No. 3. Field exteroceptive radar "360°" (For street and shops)

This is a first aid technique when going outside, to a shopping center or when walking, when the floor begins to "float" and your legs become weak. Goes well with vestibular gymnastics - use both techniques together when walking.

When unsteadiness sets in, your natural reflex is to stare at the floor a meter away and tensely control your step. This guarantees increased pitching.

Instead, enable the **Radar 360°** protocol:

> ***Protocol Express Route:***

> - ***Step 1. Sensory anchor (Vision):*** *find 3 micro-details of textures (asphalt cracks, weave of jacket fabric).*
> - ***Step 2. Panoramic hearing (Hearing):*** *move your ears 10 meters outside (people's steps, rustling tires, wind).*
> - ***Step 3. Support on gravity (Body):*** *feel the stable contact of your soles with the surface of the planet.*

1. **Take your eyes off the floor:** raise your chin parallel to the ground. Switch your eyes to wide lens mode (peripheral view).
2. **Scan Microtextures (Vision):** Look for three interesting details in the environment with your eyes. Not just "wood" or "car," but specifics: *"what is the exact shade of rust on the fence?"*, *"what is the shape of the crack on the tile?"*, *"what is the font on the store sign?"*.
3. **Move the sensors outside (Hearing):** Intentionally move the point of auditory perception 10 meters away from you. Listen to the sound of a passing bus or the conversation of passers-by.
4. **Ground Your Soles (Proprioception):** Instead of checking your balance, simply feel the solid ground firmly pressing upward on your feet. The earth itself holds you. You don't need to hold her down with your thoughts.

You don't fight the unsteadiness. You allow your body to sway, but the entire processor is busy receiving external signals. After 60–90 seconds, the unsteadiness subsides because the adrenaline spasm subsides.

Technique No. 4. Decentering: "Train on the Platform"

With anxiety disorders, the illusion of complete fusion arises: *"If I feel dizzy and scared, it means that I am this dizziness and this fear."*

Decentering creates a saving gap between you (the observer) and the content of your consciousness.

1. Sit quietly and imagine that your mind is a train station platform.
2. Your symptoms, anxious thoughts and body sensations are like passing freight trains.

- A train passed by * "What if I faint now?" *.
- Here is the car *"Wady legs and compression in the back of the head"*.
- Here is the composition of *"I will never be cured"*.

3. Your usual automatic reaction is to jump on each of these trains at full speed and rush along with it into the abyss of panic.
4. New reaction: **stay standing on the platform**. *Tell yourself: *"I notice the thought of fear. I notice a feeling of unsteadiness in my body. But I am not a train. I am the platform on which they pass. The*

platform remains motionless, no matter what trains rumble along it."

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6. Three mistakes when training attention

Mistake 1. "I meditate, that means I'm training my attention"

Classic mindfulness meditation teaches you to observe sensations in your body. This is beneficial for a healthy person. But with PPPD and anxiety disorder, this can **intensify** interoceptive hyperfocus: you're already scanning your body 24/7, but meditation tells you to do it "consciously."

ATT is the opposite approach. You don't watch the body. You purposefully train your brain to switch to **external** stimuli. The difference is like between an x-ray and a rearview mirror.

Mistake 2. "You need to concentrate as hard as you can"

Overdoing the ATT creates additional muscle and cognitive stress. Clenched teeth, a frowning forehead, an attempt to "press" attention into the sound - all this activates sympathy instead of unloading it.

Correct metaphor: you don't grip the steering wheel until your knuckles turn white. You gently turn the sailboat's steering wheel. Easily, without strain, with curiosity.

Error 3. "I'll check after the exercise - did it help?"

You have completed 12 minutes of ATT and the first thing you do is scan your body: * "Well, are you wobbly? Is your pulse normal? Did it help?"*. Congratulations - you just restarted the CAS loop that you spent 12 minutes trying to break.

When I finished my practice, I got up and went about my business. No revision. No rating. The result will come as a side effect of regularity, and not as an instant pill.

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7. Practice and Homework (HW)

The theory of attention without daily practice is worth nothing. Neural pathways are rewired only through regular physical repetition.

Your attention training program for the next **14 days**:

Daily checklist (15–20 minutes per day):

Time of day	Technique	Duration	Purpose of practice
Morning (after rising)	**ATT according to Wells**	10–12 minutes	Pumping up the strength and flexibility of the prefrontal cortex, warming up the "attention muscle"
Day (on the move/outdoors)	**360° Radar**	3-5 minutes (2-3 times)	Ripping the Velcro of attention from the body in the field, destroying agoraphobia
Evening (before bedtime)	**Open Focus**	5 minutes	Relieving sympathetic tension of the day, switching the brain to alpha rhythm

Execution rules:

1. **No self-abuse.** If during ATT practice you find yourself thinking about work or listening to your neck for 2 minutes, great! The very fact that you **noticed** is already a victory. Calmly, without irritation, return the beam back. Each return of focus is one repetition of the barbell press for your prefrontal cortex.
2. **Prohibition on checking "it helped/didn't help."** Do not evaluate your condition immediately after the exercise: *"Well, has the wobbly stopped?"*. This check is the same old scanner. Your task is not to remove the symptom immediately, but to improve your attention management skills. The result in the body will come as a natural side effect.
3. **Keep a training diary:** mark each approach completed in a notepad. After 7-10 days, you will be surprised to notice how easily and naturally you will begin to pull yourself out of somatic loops throughout the day.

Scientific base

1. Wegner, D.M. (1994). *Ironic processes of mental control.* Psychological Review, 101(1), 34–52.
2. Wells, A. (1990). *Panic disorder in association with relaxation induced anxiety: An attentional training approach to treatment.* Behavior Therapy, 21(3), 273–280.
3. Wells, A. (2009). *Metacognitive Therapy for Anxiety and Depression.* Guilford Press.
4. Fehmi, L.G. & Robbins, J. (2007). *The Open-Focus Brain.* Trumpeter Books.
5. Raichle, M.E. (2015). *The Brain's Default Mode Network.* Annual Review of Neuroscience, 38, 433–447.

Frequently asked questions

Why can't I just stop feeling dizzy?

Because Daniel Wegner's theory of ironic processes works: the command "don't think about it" causes the brain to constantly check whether you are thinking about it. What is needed is not a “distraction,” but systemic attention switching training (ATT), which restructures the neural circuits of the prefrontal cortex.

What is ATT - attention training technique?

ATT (Attention Training Technique) is a 12-minute daily protocol developed by Professor Adrian Wells within the framework of Metacognitive Therapy (MCT). Includes three phases: selective attention, rapid switching and divided attention. Clinically proven to be effective for generalized anxiety disorder, panic disorder and PPPD.

Does meditation help with PPPD and anxiety?

Classical mindfulness meditation can enhance interoceptive hyperfocus in PPPD because it teaches one to observe sensations in the body. ATT is the opposite approach: the focus is purposefully drawn outward, to external sounds. However, specialized techniques (NSDR, audio grounding, Open Focus) perfectly relieve autonomic arousal - for a detailed analysis and ready-made audio protocols, see Chapter 47: Meditation for PPPD and anxiety.

How much do you need to train your attention to make it easier?

With daily practice (15–20 minutes a day), the first lasting changes will be noticeable after 7–10 days: you will begin to automatically “catch” the moment when attention sticks to the symptom. Sustained rewiring of neural circuits requires 4-6 weeks of regular practice.

What is Open Focus and how does it relieve anxiety?

Open Focus is a neuroregulation method developed by Dr. Lester Femi based on years of EEG research. The transition from a narrow object focus to a soft peripheral perception of space instantly triggers synchronous alpha rhythms (8-12 Hz) in the cortex - a physiological marker of relaxed composure. The parasympathetic system is activated through the dorsal nucleus of the vagus nerve.

Meditation for PPPD, anxiety and neurosis: evidence-based protocols for rewiring the brain

Why classical meditation can provoke a panic attack and increase muscle movement, neurobiology of alpha rhythms and the vagus nerve, 4 safe vestibular techniques and step-by-step audio protocols.

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1. Essence and problems: the “Sit down and close your eyes” paradox

Let's start with an honest clinical confession that popular mindfulness gurus and mobile relaxation apps are stubbornly silent about.

When a person with PPPD, chronic dizziness, panic disorder or severe neurosis is advised: * “You just need to meditate - sit up straight in silence, close your eyes and watch your breathing”*, in 8 out of 10 cases this ends in **acute vegetative catastrophe**.

You sit down. You close your eyes. And at the same second:

- The external visual world disappears, and you are left alone with the raging ocean of sensations in your body.
- You begin to hear deafeningly loudly how your heart is pounding in your throat and your temples are pulsating.
- Without visual support, the vestibular system instantly loses the horizon: there is an eerie feeling that the floor is falling in, the chair is tilting 45 degrees, the body is being twisted by a screw or thrown up on the waves.
- Each breath seems shallow and “incomplete”, spasm of the larynx and lack of air sets in.
- After 2-3 minutes, the level of panic goes off scale so much that you open your eyes in horror and jump up from the mat in a cold sweat with one thought: * “Meditation is charlatanism, it only makes me worse” *.

In evidence-based clinical psychiatry, this phenomenon has a precise medical definition - **Relaxation-Induced Anxiety (RIA)** (*relaxation-induced anxiety*). According to clinical studies (Heide & Borkovec), RIA occurs in 17–30% of patients with generalized anxiety and **more than 75% of people with PPPD and vestibular deficits**.

Why do people with PPPD feel dizzy when they close their eyes?

The reason lies in the biomechanics of maintaining balance. Balance is maintained by a triad of sensory systems:

1. **Vestibular apparatus** (inner ear - semicircular canals and otoliths).
2. **Proprioception** (pressure receptors in the feet, joints and muscle spindles of the neck).
3. **Visual analyzer** (eyes as the main stabilizing horizon).

In PPPD, the vestibular filters fail and proprioception is distorted by permanent muscle armor. Under these conditions, the brain develops a pathological visual dependence: **vision becomes the only reliable sensor of the horizon**.

When you close your eyes in a quiet room:

1. You instantly **cut out the visual crutch** on which the brain balances.
2. The thalamus, deprived of external signals, redirects all attention inward (the interoceptive radar turns on).
3. The brain interprets any micro-oscillation of blood vessels or muscle contraction as a disaster: * "We are falling!" Get rid of the adrenaline immediately!"*.

> ***Main engineering rule:*** *In case of PPPD and autonomic neurosis, classical introspective breathing meditation with closed eyes in the initial stages is* ***strictly contraindicated****. What you need is not abstract "spiritual silence," but* ***physiologically verified recalibration of sensory systems*** *through external auditory supports and panoramic attention.*

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2. Neurobiology: what proper meditation does to the brain

If meditation is built taking into account neurophysiology, it ceases to be esoteric and becomes a powerful biochemical regulator of the central nervous system.

```
[Chronic stress / PPPD]
  ➔ High Beta rhythm (18–30 Hz)
  ➔ Spasm of vertebral arteries & neck muscles
              |
              ▼
[Specialized Audio Meditation]
  (NSDR / Somatic Audio Grounding)
              |
              ▼
[Inhibition of Thalamus & DMN]
  ➔ Alpha rhythms (8–12 Hz)
  ➔ Ventral Vagal activation
```

1. Switch from Beta waves to Alpha rhythm (8-12 Hz)

In a state of constant unsteadiness, the brain gets stuck in the high-frequency beta rhythm (18-30 Hz). This is a mode of threat scanning, combat alert and continuous tension of the suboccipital muscles.

Specialized protocols (NSDR, soundscapes, Open Focus) synchronize neural ensembles of the cortex in the **alpha range (8–12 Hz)**:

- The inhibitory transmitter GABA (gamma-aminobutyric acid) is activated, reducing the excitability of the sensory nuclei.
- The activity of the locus coeruleus of the brain stem (*locus coeruleus*) is inhibited, stopping the uncontrolled release of norepinephrine.
- Pathological vasospasm of the vertebrobasilar vessels is relieved, improving the nutrition of the stem vestibular centers.

2. Inhibition of the DMN (Default Mode Network) and unloading of the thalamus

The Default Mode Network (the network of the passive mode of the brain) is responsible for rumination: endless mental chewing gum, scrolling through symptoms and catastrophic thoughts (“What if it’s a stroke?”, “What if I never recover?”).

Harvard Medical School research led by Sarah Lazar (Lazar et al., 2011) has proven that 8 weeks of regular meditation practice leads to:

- Physical thinning of gray matter in the **amygdala** (the center of fear generation).
- Thickening of gray matter in the **dorsolateral prefrontal cortex** (the area of rational inhibition of panic).
- Restoring **sensory gating in the thalamus**: the brain regains the ability to filter background bodily noise and stops sending false signals to the conscious mind.

3. Ventral Vagus Stimulation (Stephen Porges Polyvagal Theory)

The vagus nerve (*nervus vagus*) has two fundamentally different branches:

1. **Dorsal vagal complex** (ancient unmyelinated pathway): freeze reflex, drop in blood pressure, sudden weakness in the legs, body wobbliness and dissociation/derealization. It is this that is activated when an attempt to “relax through force” fails.
2. **Ventral vagal complex** (evolutionarily young myelinated pathway): system of safety, social involvement, conscious rest and cardiac inhibition.

Correct meditation with a slow expiratory pattern (exhalation is longer than inhalation) and soft audio accompaniment stimulates the **ventral vagus**, normalizing heart rate variability (HRV) and returning the brain a fundamental sense of ground control (support).

---

3. Four types of meditation that are safe for PPPD

Forget about sitting in the lotus position in absolute silence. Four scientifically proven protocols are used for vestibular rehabilitation:

1. NSDR (Non-Sleep Deep Rest) and Yoga Nidra

The concept of NSDR, popularized by neuroscientist Andrew Huberman (Stanford University School of Medicine), is based on the principles of classical Yoga Nidra:

- **Mechanics:** You are in a lying position on your back with reliable tactile support (floor, carpet, mattress). The brain understands: it is physically impossible to fall. Attention is directed by the voice of the leader along the anatomical zones of the body and external landmarks without fixating on disturbing thoughts.
- **Clinical Effect:** Research shows that a 20-30 minute session of NSDR restores dopamine stores in the striatum (*striatum*) to the level of 2 hours of sleep at night and reduces systemic cortisol by 30-45%.

2. Acoustic Meditation (Soundscape/Audio Grounding)

Instead of the frightening silence in which the brain begins to listen to the pulse and tinnitus, a continuous layered sound environment is used: the sound of rain, sea surf, white/pink noise, soft sound waves of 432 Hz or binaural alpha rhythms (10 Hz).

- **Why it works:** The auditory analyzer takes on the role of **external spatial reference**. The brain localizes sounds in 3D space, relieving the visual cortex and equalizing vestibular tone.

3. Open Focus (Open Attention Meditation)

Method of Dr. Lester Femi (Open Focus Technique). Instead of narrowing your attention to the breath in the nostrils, you shift your focus to the **space between objects**:

- Distance between temples.
- The space between the eyelids and eyeballs.
- The volume of air in the room around your body.
- **Result:** Instant transition of EEG activity into the phase of synchronous alpha rhythm of the occipital regions of the brain.

4. Detached Mindfulness

Development by Professor Adrian Wells (metacognitive therapy, MCT):

- You don’t fight your thoughts and don’t try to “clear your head.”
- You take the position of a passive observer: thoughts about illness, dizziness and fear are perceived simply as clouds in the sky or a news ticker on the TV screen. You observe the work of the mind without entering into dialogue with it.

4. Author's project: YouTube channel Calm Mind Meditation

In order for meditative practice to bring results, the **quality of audio accompaniment** is crucial: spectral purity of frequencies, absence of sharp percussive sounds, monotonous velvety voice timbre and correct pausing.

Many commercial meditations from the Internet suffer from sudden sounds of Tibetan bells, intrusive music or sugary intonations, which in an anxious person only cause irritation and vegetative spasm.

That is why the author of the book is developing a specialized audio project:

> 🎧 ***Calm Mind Meditation YouTube channel*** *(Link: https://www.youtube.com/@Calm_mind_meditation)*

Sessions on the channel are created with a direct focus on neurophysiological rehabilitation:

- **Neuroacoustic Calibration:** Sound textures are designed to induce high-frequency beta desynchronization and gently stimulate alpha activity.
- **Safety for the vestibular system:** No sharp panoramic jumps in sound between the left and right ears that can provoke an attack of pseudo-vertigo.
- **Voice guidance for the ventral vagus:** Speech rate 45–55 words per minute with pauses synchronized with an extended exhalation cycle.
- **Versatility:** The tracks are divided by purpose - morning launch of stability, daytime anti-stress reset and evening immersion into deep sleep without anxiety.

Subscribe to the channel **@Calm_mind_meditation** and use its audio tracks as a basis for performing the daily protocols below.

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5. Three step-by-step daily meditation protocols

Protocol No. 1. Morning “Neuro-start” (10 minutes)

This protocol covers the practice included in the homework Chapter 7: Neurophysiology.

- **When:** Immediately after waking up, a glass of warm water and a contrast shower. Before breakfast, before opening instant messengers and news feeds.
- **Pose:** Sitting on a chair with a hard back. The back is straight, but without spasm. **Feet are completely on the floor** (we activate plantar mechanoreceptors for grounding).

- **Eyes:** Half-open 70–80%, soft, defocused gaze directed forward and slightly downwards (to the floor at a distance of 1.5–2 meters). Don't close your eyes completely if you have morning shakyness!
- **Audio:** Play the morning track from the @Calm_mind_meditation channel in your headphones at a moderate volume.
- **Step-by-step algorithm:**

1. *Minutes 0–2 (Anchor Sound):* Listen to the volume of sound. Select the three quietest background sounds in the track.
2. *Minutes 2–6 (Physiological Sigh):* Breathe according to Andrew Huberman's scheme - take a deep breath in through your nose, then take a short breath up to the top of your lungs, and long, smooth exhale through closed lips (inhale 4 sec ➔ exhale 6–8 sec). Repeat 5-6 cycles, then return to natural, calm breathing.
3. *Minute 6-8 (Support Scanning):* Feel the firm pressure of your feet on the floor, the weight of your hips on the seat of the chair, the touch of fabric on your skin. The brain reads the signals: "I'm sitting. The support is motionless. The ground below me is solid."
4. *Minutes 8–10 (Exteroceptive exit):* Gently sweep your gaze around the periphery of the room - corners, ceiling, walls. Take a deep breath, stretch and go have breakfast.

- **Result:** The morning surge of cortisol is gently stopped without a vegetative storm, the neck remains relaxed.

Protocol No. 2. Daytime "Emergency reset" during an attack of motion (5 minutes)

Used in the office, store, subway or at home for a sudden increase in unsteadiness, derealization or anxiety.

- **Pose:** Sit in any stable chair or lean your back against a hard wall.
- **Audio:** Put on your headphones and turn on an afternoon relaxation track with @Calm_mind_meditation or white noise.
- **Radar 360° technique:**

1. Don't close your eyes. Look at one neutral point in front of you.
2. Divide auditory perception into 3 layers:

- Layer 1 - the voice of the presenter or the leading melody;
- Layer 2 - background sounds of nature in the recording (rain/wind noise);
- Layer 3 - real external sounds of the room around you (the hum of a fan, the steps of colleagues).

3. Keep all 3 layers in focus at the same time for 3 minutes.

- **Physiological meaning:** The capacity of the RAM of the working zone of the cortex is 4±1 information blocks. Holding three external audio channels completely overloads the attentional processor. The brain simply does not have the resource to monitor the vestibular apparatus and promote panic. The symptom immediately subsides.

Protocol No. 3. Evening Deep Sleep Entry (NSDR, 20–25 minutes)

For those who experience increased motion in the dark before bed, muscle twitching (myoclonus) or insomnia.

- **When:** 20–30 minutes before you want to fall asleep.
- **Preparation:** Ventilate the room (fresh cool air is critical for vasodilation), turn off all overhead lights and gadgets.
- **Pose:** Lying on your back in bed. Place a small pillow or bolster under your knees (this completely relaxes the iliopsoas muscle and relieves tension on the dura mater). The arms lie freely along the body, palms up.
- **Audio:** Play a long session of Yoga Nidra or deep relaxation on the @Calm_mind_meditation channel through your bedside speaker or comfortable sleep headphones.
- **Algorithm:** Follow the voice of the presenter. Mentally draw a "ray of attention" through your body: relax your lower jaw (open your teeth!), release the root of your tongue, lower your shoulders away from your ears, feel the heaviness in your palms, calves and feet.
- **Important:** If during practice you fall asleep, do not resist. This is a sign that the nervous system has safely exited the alarming guard post mode.

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6. Table of errors: why meditation fails

Error	Physiological mechanism of failure	How to fix
Close your eyes during severe motion	The brain instantly loses visual support, the thalamus sounds the alarm	Keep your eyes half open (soft gaze), fixing your gaze on a static object
Trying to "turn off thoughts"	Wegner's paradox is triggered: the brain checks whether it is thinking by increasing the arousal of the cortex	Accept thoughts as background radio. Keep your attention only on the audio track
Meditate in the stuffiness	Hypoxia and excess CO_2 instantly stimulate brainstem chemoreceptors, provoking tachycardia	Always open the window for micro-ventilation before the session

Error	Physiological mechanism of failure	How to fix
Jump up at the first wave of anxiety	Avoidance behavior reinforces the conditioned reflex in the amygdala: "Meditation = death threat"	Lower your shoulders, exhale twice through your mouth and feel the floor with your feet. Anxiety will subside in 90 seconds
Use random gong tracks	Sudden acoustic changes cause a startle (*startle reflex*) and a surge of adrenaline	Listen only to proven, acoustically verified tracks @Calm_mind_meditation

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7. Homework (HW)

1. **Subscribe** to the YouTube channel with specialized audio sessions: Calm Mind Meditation (@Calm_mind_meditation).
2. **Integrate morning protocol No. 1 (10 minutes)** into your routine for the next 7 days: do it strictly after a contrast shower and a glass of water, closing the task Chapter 7.
3. **Test the evening protocol NSDR No. 3** at least 4 times a week before going to bed.
4. **Keep a diary of how you feel:** Rate the level of unsteadiness and anxiety on a 10-point scale immediately BEFORE meditation and 15 minutes AFTER. Put the difference in numbers.

Questions and answers (FAQ)

Why do dizziness and nausea increase during meditation?

This is a classic manifestation of Relaxation-Induced Anxiety (RIA) and visual dependence decompensation. When trying to relax with your eyes closed, the brain loses visual reference and becomes startled by interoceptive signals. Solution: Never meditate with your eyes closed in the first stage. Use a soft, defocused look at the floor and external audio accompaniment.

How is NSDR different from regular napping?

During daytime sleep, you often fall into the slow-wave phase, awakening from which is accompanied by sleepy inertia ("cast iron head", weakness and increased pitching). NSDR keeps the brain in a controlled sleep state at the border between alpha and theta waves. You remain conscious while your body restores neurotransmitters, and you emerge from the session feeling fresh and alert.

Is it possible to meditate while lying down if you feel dizzy while sitting?

Yes, absolutely. At the stage of exacerbation of PPPD, the lying position on a hard surface is preferable: the contact area of the body is maximum, the receptors of the back, pelvis and legs send a powerful

stream of information about immobility to the brain, which eliminates the risk of falling. As you stabilize, move on to practicing while sitting on a chair with your feet on the floor.

How long does it take to practice for the symptoms to subside?

The first vegetative changes (pulse reduction, relief of spasm in the suboccipital zone, deepening of breathing) occur already in the first 10 minutes of a high-quality audio session. A persistent structural restructuring of the sensory gating of the thalamus and the extinction of hyperfocus on pumping takes from 3 to 6 weeks of daily adherence to the morning and evening protocols.

Are headphones with active noise cancellation (ANC) suitable?

With moderate sensitivity - yes. However, in some people with PPPD, the powerful antiphase noise reduction (ANC) algorithm creates a sensation of “pressure on the eardrums” and may slightly disorient the otolithic system. If you feel discomfort, switch your headphones to *Transparency Mode* or use regular open-back headphones/external speakers.

Module 2: The Battery

Adrenaline Loop: How to Break the Vicious Cycle of Panic

The anatomy of an attack of unsteadiness and the physiological secret of the "5 Minute Rule" that deprives panic of its power.

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1. The essence and problems: an endless nightmare in a circle

Anyone who has experienced PPPD knows that sudden moment: you are just walking, and suddenly there is a micro-shock. Slight momentary unsteadiness. Your instant reaction: *"Oh my God, it's started again!"*.

A second later, your heart begins to pound like mad, your hands instantly become cold, your legs turn to jelly, and your staggering from mild turns into a hurricane. You feel like you're about to collapse right there and no one will help you. You frantically grab the railing, squat down, or run home to hide in a safe place. It seems to you that these attacks occur chaotically and uncontrollably, and you are simply their defenseless victim. But the truth is that you yourself are the main generator of this storm.

2. Physiological explanation: the law of adrenaline combustion

What you think is a "sick episode" is actually a classic **adrenaline loop**. This is pure biology, which works according to a strict algorithm:

1. **Trigger (harmless event):** Micro push in the body, turn of the head, dappled light. Your body reacted normally, but your frightened brain was on alert.
2. **Frightening thought (Rating):** *"I'm going to fall!" I'm dying! I'll feel bad!"*.
3. **Adrenaline Rush:** The amygdala (fear center in the brain) perceives this thought as a real physical threat (as if a tiger is running at you). She gives a command to the adrenal glands, and they instantly release adrenaline into the blood.
4. **Increasing the symptom:** Under the influence of adrenaline, the blood vessels narrow, and the neck muscles spasm even more. Because of this, the desynchronization of the sensors increases, and the instability **really increases by 3 times**.
5. **New frightening thought:** *"You see, I'm swaying more!" My worst fears are confirmed!"*.
6. **New adrenaline rush:** ...and so on in an endless spiral.

The secret of survival lies in one fundamental law of neurophysiology: **adrenaline physiologically burns out in the blood in 3–5 minutes**.

The body cannot constantly secrete this hormone - this would lead to overheating of the autonomic system. The amygdala releases a portion of adrenaline and waits. If within the next 3 minutes you do NOT throw coal into the furnace (that is, new frightening thoughts like * "Oh God, I'm going to die" *), the adrenaline wave will fade away on its own. Your heart will calm down, your muscles will relax, and your unsteadiness will return to its original baseline level. Panic cannot last forever. It lasts a long time only because you constantly restart it with new portions of fear.

3. Cognitive traps: "My fear is justified, because I'm really shaking!"

The main cognitive trap: confusing cause and effect.

You think: *"I'm afraid because I'm shaky."*. But the reality looks exactly the opposite: *"I'm shaking more because I'm afraid"*. You evaluate an increase in symptoms as evidence of a fatal disease, although this is just your body's normal reaction to a hormonal surge. Your fear does not save you, it creates your instability.

The second trap is escape. Every time you run home from the supermarket at the onset of panic, your brain checks the box: * "We ran away and survived. This means that the supermarket is truly deadly. Next time we'll turn on the panic even earlier!"*.

4. Point of Support: Insight

> *Panic is just special effects in your blood. An adrenaline wave hits you headlong, but it has no teeth. She can't kill you. She just needs time to burn out. Time it for 5 minutes and don't let the chemistry do its job.*

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5. Practice: Breaking the loop step by step

Our task is to meet the adrenaline wave consciously and not let it go for a second round.

Loop termination algorithm:

1. **Localization of thoughts:** As soon as you feel the beginning of panic, say to yourself out loud or mentally: * "Stop. The adrenaline loop started. I know this algorithm. It's not the disease that has worsened, it's just a frightened brain spitting out adrenaline."*
2. **Start the timer:** Take out your phone and start the stopwatch. Your goal is to simply observe time and sensations in your body for **5 minutes**.
3. **No Escape:** Stay where you are. Don't sit down, don't run home, don't call your loved ones, don't reach for pills. Let the wave pass through you. Tell your fear: *"Okay, you have 5 minutes. Shake me, spin me, do whatever you want. I'm standing here."*

4. **Registration of the ending:** Make sure that at the 4th or 5th minute the intensity of the fear begins to drop. Your body will get tired of being afraid, your breathing will become even. Record this experience. Each loop rupture physically destroys the old fear highway in your brain.

Emergency valve: when "just standing" is impossible

There are situations when the adrenaline goes off scale so much that it is physically impossible to sit still. The body is shaking, the hands are trembling, any attempts to "breathe calmly" only intensify the panic. At such moments, you need to give the body **exactly what it was prepared for** - physical release.

30 seconds of intense exercise: squats, push-ups or jumping jacks. Not 30 minutes - exactly 30 seconds. A short but powerful burst of activity physically burns excess cortisol and adrenaline in the blood. After this, the nervous system finally gets the opportunity to decline. And you are an opportunity to return to calm observation.

Homework (DZ):

1. Draw your personal "adrenaline loop": what sensation in the body you notice first → what thought comes → what emotion arises → what the body does.
2. For 3 days, every time you have an anxiety attack, time it on your phone and watch how the symptoms subside in 3-5 minutes.
3. Practice **physical adrenaline release** (30-second protocol) at least 3 times.

Chronology of adrenaline rush: minute by minute

So that you stop being afraid of panic waves, here is the exact chronology of what is happening in your body:

0-10 seconds (Trigger): The brain has registered a "threat" (thought, sensation, memory). The amygdala instantly activates the adrenal glands. The release of adrenaline and norepinephrine begins. Before you have time to think, your body is already reacting. This is not a bug, this is a feature: evolution has set up this mechanism so that you run away from a tiger faster than you have time to think "is this really a tiger?"

10-60 seconds (Rise): Adrenaline reaches its peak. The pulse jumps by 20–40 beats. The muscles of the neck and shoulders contract (the body prepares for impact). Vision narrows (tunnel vision). Blood is redistributed from the skin and gastrointestinal tract to the leg muscles. Nausea, a lump in the throat, and a feeling of unreality may appear. **This is the peak. There will only be a decline further.**

1–3 minutes (Plateau): If you don't create new disturbing thoughts, your adrenaline levels begin to decrease. The body includes the enzyme **monoamine oxidase (MAO)**, which breaks down adrenaline in the blood. You still feel discomfort, but its intensity stops growing.

3-5 minutes (Disposal): Adrenaline is utilized by 80%. The pulse begins to return to normal. The muscles relax. Slight weakness or trembling appears - this is absolutely normal, this "spent fuel" leaves the muscles.

5-15 minutes (Recovery): Parasympathetic takes over. Drowsiness is possible - this is a signal: "The body has switched to recovery mode." Don't fight with him, if possible, sit quietly.

Key conclusion: Panic **physically cannot** last longer than 5 minutes if you don't add new fear wood. Every time you get scared by your own symptoms, you give a new adrenaline rush, restarting the 5-minute timer again. This is the "adrenaline loop".

Emergency Breathing Protocols

1. Box Breathing

Used by special forces, pilots and surgeons to instantly reduce the stress response.

- Inhale - 4 seconds
- Delay - 4 seconds
- Exhale - 4 seconds
- Delay - 4 seconds
- Repeat 5 cycles

Why it works: Holding your breath after inhaling increases the level of CO_2 in the blood, which reflexively dilates blood vessels and activates the vagus nerve (parasympathetic).

2. Physiological Sigh

Discovered by neuroscientist Andrew Huberman. The fastest technique - works in 1 breathing cycle.

- Take a **double breath** through your nose: the first breath fills the lungs 80%, the second short breath "dry" the remaining 20%.
- Exhale **exhale as long as possible** through your mouth (8-10 seconds).
- Repeat 2-3 times.

Why it works: Double inhalation opens the alveoli of the lungs, which stick together during shallow "anxious" breathing. A long exhalation instantly activates the vagus nerve.

CAS trap: why the internal scanner prevents you from recovering

What is cognitive-attention syndrome, how Ukhtomsky's law of dominance concretes your symptoms, why the "firewall" of the thalamus breaks down and how to turn off the internal seismograph.

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1. Essence and problems: life under the gun of a seismograph

You wake up in the morning. Your eyelids are still closed, but your operating system has already launched the morning self-diagnosis script. Before you have time to take your first breath, you scan your body: * "So... it shakes? Is your head heavy? Wobbly in place? Are there microshocks?"*. You listen to the signals in your neck as if the survival of humanity depends on it.

Your whole day is subject to this endless monitoring. You go outside and instantly activate the internal seismograph:

- *"How did I step on the asphalt? Didn't your leg fall in?"*
- *"What if I turn my head 45 degrees to the left? My vision is blurred again!"*
- *"The supermarket has high ceilings and colorful shelves. I'm shaking more. You need to fix your gaze on the price tag."*
- *"I'm standing in line at the checkout. The floor under your feet begins to float backwards, like an escalator. I think I'm falling over."*

You live not in the outside world, but inside your head. You are locked in an eternal cycle of testing your autonomic system. In clinical psychotherapy, this condition is called **CAS (Cognitive Attentional Syndrome)** - cognitive-attentional syndrome, or simply "internal scanner".

The biggest trap is that you sincerely consider this scanner to be your salvation. Your amygdala whispers: *"Control your balance, listen to your body, otherwise you will fall, embarrass yourself or lose consciousness."* But the reality is diametrically opposite: **it is this scanner that makes you dizzy**. This is not a diagnosis of a breakdown - it is the breakdown itself in action.

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2. Physiological explanation: thalamus, Ukhtomsky dominant and selective attention

To understand how attention physically generates symptoms, let's look under the hood of our brains.

Brain Firewall: Thalamus and the Salience Network

Every second of time, your body is bombarded with millions of sensory signals. The skin feels clothes, the ears catch the background hum, the vestibular apparatus constantly adjusts the position of the body when breathing, the eyes record the micro-vibrations of objects.

Normally, the brain does not allow all this “garbage” into your consciousness. The **thalamus** is responsible for this - the main control center and system firewall of the brain. It filters incoming traffic. A healthy person does not feel how his vestibular apparatus makes thousands of micro-corrections per second to keep the body upright. For him, this is a background process.

But when you are in a state of chronic anxiety, the **Salience Network** turns on. For your brain, signals from the neck and vestibular system receive the highest safety priority. The Amygdala gives the command: * “There is a virus in the system!” Open all ports for testing!”*.

The thalamus obediently turns off filtering. The entire technical log file of vegetatives breaks into consciousness. You begin to feel physiological noise that a healthy brain simply rejects: micro-fluctuations of fluid in the inner ear, natural muscle tremors, proprioceptive noise. And because you haven't felt it before, you interpret this natural noise as "catastrophic vertigo."

Ukhtomsky's law of dominance

Neurophysiologist Alexey Ukhtomsky discovered a fundamental law of brain function: in the central nervous system there is always a **dominant** - a temporarily dominant focus of excitation, which attracts signals from other zones and inhibits their work.

When your "scanner" is turned on, the dominant **"Search for threat in the body"** is 100% active. Attention is a lens that focuses electromagnetic radiation. Where this spotlight is directed, the sensitivity of the receptors is physically turned to its maximum. *Are you listening to the back of your head? Neurons in this zone go into superconductivity mode. You will begin to feel the pulsation of every microartery and the tension of every muscle fiber. *Are you scanning your gait? The brain lays out a multi-lane neural highway there. The more you scan, the stronger this dominant becomes. It literally “concrete” the symptoms in your brain.

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3. Cognitive traps: manual autopilot mode

The main misconception of an excellent neurotic student: * “Control equals safety. If I stop thinking about the steps, my legs will give way.”*

Let's look at this from a system architecture perspective. Body balance is a deeply automated process. It is regulated by ancient structures: the cerebellum, brain stem and vestibular nuclei. This is the “firmware” of your body. It operates at submillisecond speeds.

When you try to control your step consciously (through the cerebral cortex), you switch the system from automatic to manual. This is the same as trying to code a complex web interface by manually rearranging bits in RAM using CPU interrupts. It's too slow!

- **Autopilot:** The signal from the foot came to the cerebellum ➔ The cerebellum instantly adjusted the tone of the lower leg muscles. The reaction speed is hundredths of a second. The movement is smooth.
- **Manual control:** The signal came to the conscious cortex ➔ The cortex thought: * "Okay, it swung to the left, you need to tense your right leg."* ➔ The cortex sent an impulse to the motor zone ➔ The muscle tensed. The reaction speed dropped 10 times. The step became ragged, jerky, artificial.

As a result, your unsteady gait is not a consequence of the disease. This is the physical result of the fact that you are trying to consciously control something that should work fully automatically. You tighten your muscles yourself, creating an artificial pumping.

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4. Point of Support: Insight

> *Your brain is an overloaded web server. Having launched the "internal scanner", you hung an infinite loop on it `while(true) { checkBalance(); }`. This process consumes 90% of RAM and processor resources. The server does not have the capacity to process real life. Turn off this debug loop. Your autopilot will do just fine without manual control.*

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5. Practice: Tools for managing the attention spotlight

We cannot tell the brain to "not think about the symptoms" (hello to the polar bear that cannot be imagined). Fighting a thought is also paying attention to it. Our task is to teach the brain to switch the spotlight of attention at our request.

Tool 1: Sensory Overload "5-4-3-2-1"

This tool is your emergency patch. It physically clogs the RAM with external signals, leaving no resources for the amygdala to generate panic.

As soon as you catch yourself scanning your neck or asphalt, firmly command yourself: ** "STOP. The log is full. Switch"** and find in the outside world:

1. 👁 **5 items of different textures/colors** (wood, glass, metal, blue sign, red brick). Describe their shape.

2. 👂 **4 separate sound sources** (air conditioner noise, tire rustle, conversation of a passer-by, your own step).
3. ✋ **3 physical sensations** (pressure of the bag strap on the shoulder, contact of the foot with the sole, warmth of the wind on the face).
4. 👃 **2 smells** (the smell of coffee, dust, your perfume).
5. 👅 **1 taste** (take a sip of water or just taste the air).

This algorithm temporarily switches the brain from the Default Mode Network, where anxious rumination lives, to the Central Executive Network. The brain is physiologically incapable of panicking when it is busy processing 15 external sensory streams.

Tool 2: Spotlight Training (Daily Debug)

Attention is a muscle. In a neurotic person, it is atrophied by clip thinking and an anxious search for symptoms on Google. We will download it every day.

1. **Pure Focus Practice (2 minutes per day):**
   - Place any simple object in front of you (key, coin, pen).
   - Start the timer for **2 minutes**.
   - Keep 100% attention on the object. Study its micro-scratches, glare of light, shadow.
   - **Critical important:** as soon as a thought enters your head (about symptoms, about work, about exercise) - don't be angry with yourself and don't try to "erase" it. This is a normal background process. Just notice its existence and gently, unemotionally return your gaze to the object. Treat thoughts like pop-up windows in your browser - just close them with a cross and continue reading the main text.
2. **Practice "Reading avidly" (20 minutes a day):**
   - Read exclusively fiction (no non-fiction or articles on medicine!). You need an exciting plot - detectives, science fiction, adventures.
   - Read without your phone. Your goal is to catch a state of flow ("narrative transportation"), when you forget where you are.
   - *Physiological meaning:* Reading a complex literary narrative forcibly transfers the brain from threat scanning mode to image generation mode. This is a natural detox for the amygdala.

Tool 3: Somatic Tracking and Mindfulness (Perceptual Retraining)

If previous tools were aimed at *switching* attention outward, then this one is aimed at **directly reprogramming the reaction to a symptom**.

With PPPD, your brain is in a closed loop: you feel dizzy ➜ you get scared ➜ adrenaline is released ➜ the spasm of the digitalized neck muscles intensifies ➜ the dizziness increases. Somatic tracking breaks this cycle at the very beginning. This is the practice of conscious, non-judgmental observation of sensations in the body.

Your task is to prove to the amygdala that these symptoms themselves are safe. That this is not a "critical system failure", but just noise on the line.

Execution algorithm:

1. **Take a safe position:** Sit or lie down. Relax. You are not in danger, you will not fall.
2. **Close your eyes and direct the spotlight of attention directly to the epicenter of the symptom:** To unsteadiness, "cotton wool" in the head or heaviness in the neck. Stop running away from this feeling.
3. **Turn on the "Data Analyzer" mode:** Start describing symptoms in dry, technical language from the third person, discarding any emotions.
   - *Forbidden words: * "terrible", "rocking like a ship in a storm", "I'm about to fall", "unbearable".
   - *Allowed words (physical parameters):* "I feel vibrations with a frequency of approximately 1-2 Hz", "I feel pressure in the back of my head weighing 200 grams", "swaying goes from left to right", "warm/cold sensation, blurry at the edges".
4. **Breathe slowly and hold this observation for 3-5 minutes:** Let the symptom be. Don't try to stop it, reduce it or fix it. Mindfulness is the acceptance of reality "as it is." Whisper to yourself: * "Yes, I'm shaking right now. This is simply a sensor calibration failure. Signals are sent, but there is no harm to the body. I'm safe."*

By doing this, you deprive the amygdala of food - your fear. Without fear, the dominant threat begins to fade, and the brain gradually erases this symptom from priority tasks.

What to do if you panic while trekking?

If, when concentrating on a symptom, your pulse soars and fear sets in, apply the **"Pressure Relief"** algorithm:

1. **Tell yourself:** *"This is just an adrenaline rush. The physiological half-life of adrenaline in the blood is 3-5 minutes. I don't need to be saved. I'll just wait until this cocktail wears off."*
2. **Open your eyes slightly** and transfer 50% of your attention to a solid object in front of you (floor, chair leg). Feel the ground with your feet: * "The ground is solid. I sit firmly. There is no physical fall."*
3. **Make your breathing "square"** or with an extended exhalation (inhale for 4 counts - hold for 4 - exhale for 6 - hold for 2). This will forcefully activate the vagus nerve and inhibit the amygdala.
4. When the storm subsides, gently return to the non-judgmental description of the symptom. Your victory is not that the symptom disappears, but that you stop being afraid of it.

Tool 4: 4-7-8 Breathing - Physiological Emergency Braking Hack

When panic is in full swing and you can't switch your attention, there is one reliable physiological trick. The whole secret is that **exhalation is longer than inhalation**:

1. Inhale calmly through the nose for **4 counts**.
2. Hold your breath for **7 counts**.
3. Slowly exhale smoothly through the mouth for **8 counts** - as if the air is coming out through a thin straw.

The abdomen inflates as you inhale and deflates as you exhale (diaphragmatic breathing). This rhythm forcibly activates the parasympathetic nervous system - your internal brake. The brain reads a clear signal: * "Since breathing is so slow, it means the danger has passed" *. 3-4 cycles - and the pulse will begin to decrease.

Tool 5: Contrast Muscle Relaxation (Micro-Jacobson)

If you feel that your body is one continuous spasm, use the principle of **contrast**:

1. Clench your fists **as hard as possible** for 5-7 seconds - until you tremble slightly.
2. **Release sharply** and direct all your attention to the pleasant warmth that spreads through your hands.
3. Walk like this from hands to feet: shoulders, stomach, buttocks, thighs, legs.

The brain cannot keep a spasm in a muscle that has just received maximum load. This is the dismantling of even the oldest muscle blocks in which stress has accumulated over the years.

Frequently asked questions about CAS and internal scanner

"What if I lose control of my balance and actually fall?" In all the months (or years) of illness, how many times did you actually fall? Zero. Because the autopilot works. Even when you are "stormed", you stand. Even when the "floor is floating," you walk. You never fell apart. Your cerebellum is doing its job every second while you are trying to take control from it. Stop disturbing him.

"I tried not scanning, but it didn't work. Thoughts come on their own" That's right. Thoughts about symptoms will come - this is a normal product of an overstimulated limbic system. The goal is not to block them (this is impossible), but to not get involved. A thought came → you noticed it → you didn't "think" about it → switched to the outside world. This is a muscle. The first 2 weeks will be difficult. In a month it will become easier. After 3 months - automatically.

"In the morning, the scanner turns on before I even open my eyes. How to deal with this?" The morning scanner is CAR (Cortisol Awakening Response). In the first 30 minutes after waking up, cortisol levels soar by 50-75%. It's biology, not your weakness. Antidote: Don't lie in bed "listening to your head."

Set the alarm for one minute - and during that minute, get up, wash your face with cold water, drink a glass of water. Switch your brain from internal scanning mode to external mode from the first second. Over time, the CAR surge will stop triggering panic because you won't feed it.

“The scanner is amplified in certain places - shops, transport. I'm fine at home" This is called a "contextual trigger." Your amygdala has linked certain locations to danger. At home, the thalamic firewall is partially restored because the brain considers the home a safe zone. In new or visually challenging environments (high ceilings, colorful shelves, flashing people), the brain goes into hyper-vigilance mode and turns off filtering. There is only one solution - exposure. The more often you visit these places without a catastrophe, the faster the amygdala removes the “dangerous” label from them.

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6. Homework (HZ):

1. **Do the “5-4-3-2-1” practice at least 5 times a day** for a week. Do this preventatively, without waiting for strong pitching. You need to train your switching skill until it becomes automatic.
2. **Morning ban on scanning:** When waking up, forbid yourself to lie down and “listen to your head.” I caught the scanner ➔ instantly get out of bed and switch to physical actions (washing my face, brushing my teeth, drinking a glass of water). Switch your brain to external mode from the first second of the day.
3. **Somatic tracking (1 time per day):** Do the practice for 3-5 minutes in the afternoon, when fatigue accumulates. Keep a short diary on your phone:
    - *Date/Time:*
    - *Dry description of the symptom (without emotions):*
    - *Level of fear before tracking (from 1 to 10):*
    - *Fear level after tracking (from 1 to 10):*
4. **Fiction Reading:** Read a book of your choice for at least 20 minutes every night before bed. No screens 30 minutes before bed.

Hypochondria: why you won’t faint and how Google medicine is killing your nervous system

We destroy the main fear of all alarmists, explain the physiology of pressure during panic and announce an information detox.

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1. Essence and problems: fear of fainting and dependence on white coats

If you have PPPD, your constant, paralyzing fear is passing out in the middle of the street, in the supermarket, or on the bus. It seems to you: * “My head is so weak and my legs are so weak that one more step and I’ll pass out and fall face down on the asphalt.”* Because of this, you are afraid to go out alone, you constantly check where the benches are, or you only walk along the walls, ready to lean on.

Your second faithful companion is hypochondria. Your day starts with reading medical forums, PPPD groups on Telegram and articles on Google. You compare your feelings with symptoms of multiple sclerosis, brain tumor, amyotrophic lateral sclerosis and stroke. You have accumulated a giant folder with MRI, blood tests, ultrasound examination of neck vessels and neurologists’ reports. But the most paradoxical thing is: every visit to the doctor that brings the verdict “You are healthy” calms you down for exactly two hours. Then you go back to the Internet to look for “what the doctors missed.”

2. Physiological explanation: why you physically cannot faint when you panic

Let's break down the physiology of fainting down to the cogs.

What is classic fainting (syncope)? This is a protective reaction of the brain to a **sharp drop in blood pressure**. When the pressure drops below a critical level, blood stops flowing to the brain in the required volume. The brain says: * “I don’t have enough oxygen. I move my body to a horizontal position so that it is easier for the heart to pump blood at the same level”* - and turns off consciousness. The body falls, the blood flow is restored, the person comes to his senses.

Now let's see what happens to you during an attack of unsteadiness and a panic attack:

1. The amygdala gets scared, the adrenal glands release **adrenaline**.
2. Adrenaline instantly constricts peripheral blood vessels and makes the heart beat faster (tachycardia).
3. Because of this, blood pressure **rises sharply** (for example, from 120/80 to 150/90).

Physiologically, with high pressure, blood flows under great pressure to the brain. The brain is literally flooded with oxygen. Fainting in a state of panic and high blood pressure is **absolutely impossible**. Your feeling of "I'm going to fall" is just an illusion created by hyperventilation (fast breathing) and muscle spasms in the neck. Your "hard" is overloaded with nutrition, but not deficient in it.

3. Cognitive Traps: Confirmation Bias on Google

Your main trap is the illusion of control through information. You think: *"I have to find the exact reason for my unsteadiness on the Internet so that I know what to fight"*.

But the algorithms of Google and search engines work according to the laws of marketing. Articles with terrible diagnoses (tumors, sclerosis, strokes) collect millions of clicks and reposts. Articles about how "unsteadiness is just an anxiety disorder and a neck spasm" are of no interest to anyone; they hang on hundreds of pages of searches.

When you look for information about symptoms, you fall into the trap of **Confirmation Bias**. Your frightened brain ignores the phrase "in 99% of cases it's a neurosis" and tightly clings to the phrase "in 1% of cases it's fatal." Every medical story you read on forums physically overloads your amygdala, starting a new cycle of the adrenaline loop. You treat the symptoms while simultaneously pouring tanks of informational poison into the brain.

4. Point of Support: Insight

> *Stop playing Dr. House with the Google search bar. Your diagnosis is functional disorder. Your examinations are clear. Every click on a medical forum is a tranche of adrenaline into your adrenal glands. Declare war on those who profit from your fear.*

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5. Practice: Cold Turkey Protocol

We are introducing strict, uncompromising quarantine on any medical information.

Quarantine rules:

1. **Deleting Bookmarks:** Delete all bookmarks with medical forums, self-help groups (where people chew on their symptoms for years) and articles about diseases.
2. **Google Ban:** A complete, 100% ban on Googling any bodily symptoms is being introduced. The finger itched, the eyelid twitched, swayed to the side - **no search**. Tell yourself: * "I passed the examinations. My brain is healthy. All my symptoms are autonomic noise."*
3. **Switching attention:** If your hand reaches out to enter a symptom into the search bar, use the "5-4-3-2-1" technique or do 20 squats.

“Closed Mouth” technique - filtering the information environment

It's not just Google. Your environment can also be a source of toxic information. There are three types of “stressors” that need to be strictly cut off:

1. **News and media.** Negative information background directly feeds the amygdala. Every news about a war, disaster or epidemic is a mini-release of cortisol. Remove news apps. Seriously. The world will not collapse without your participation.
2. **Toxic people.** You probably have friends who, after 10 minutes of communication, leave you feeling like the energy has been sucked out of you. They complain, shift their problems, criticize. “Closed mouth” technique: **do not discuss your condition with those who cannot help**. Don’t tell your mom, friend, or colleague about every bout of unsteadiness—their sympathy feeds your victim position.
3. **Forums and chats of “victims”.** Groups where people have been describing their symptoms for years and competing to see who has it worse - this is pure poison. You go there for support, and come out with three new symptoms that weren’t there before. The brain instantly tries on other people’s diagnoses for itself. **Remove from all such groups.** Leave only channels with specific recovery tools.

Reassurance-seeking: a trap for loved ones

There is a second form of hypochondria that is not talked about much: **seeking reassurance from loved ones**.

You don’t just Google, you call your mom: * “Mom, I feel bad, tell me I’m not dying”. *You tug at your wife: “Look at my pupils, are they the same?”. You ask your friend: * “You see that I’m walking normally? Am I not wobbly?”*.

Every time they answer you *“It’s okay, you’re fine”*, you get instant relief. For 15–30 minutes. And then the anxiety returns, and you ask the same question again. This is reassurance-seeking - a behavioral drug. And it works exactly the same way as googling symptoms: short-term relief → return of anxiety → new dose → addiction.

How to stop:

1. Warn your loved ones: * “I will ask you questions about my health. Please don't answer them. Instead, say: “You know the answer yourself.”*
2. When you want to call your mom and ask, “Am I not dying?” - write this question in a notepad. Next to it, write the answer to yourself: * “No, I’m not dying. It's adrenaline. It will burn out in 5 minutes.”*
3. Please note: 30 minutes after you have NOT received reassurance, the anxiety will go away on its own. The brain will learn that without receiving a “dose”, it will still survive.

How to explain PPPD to loved ones

Your family doesn't understand what's wrong with you. You look fine, the tests are clear, the MRI is fine. They start to think you're making it up or that you're "just nervous." It hurts. But it's not their fault - they don't know what a functional disorder is.

Here are phrases to help explain:

For mom/dad:

> *"I'm feeling dizzy, not because I'm sick, but because my brain is stuck in alarm mode. It's like a broken record. Physically everything is fine, but the nervous system is overheated. I need time and support, not advice "don't be nervous".*

For partner:

> *"I'm having a hard time right now, but I'm working on it. I don't need you to solve my problem. I need you to just be there and not ask every 5 minutes "are you feeling better?".*

For friends:

> *"I'm fine, my nervous system is just malfunctioning. I don't need to Google, I don't need to poke nootropics either. Just invite me for a walk and don't pay attention if I look strange."*

Homework (DZ):

1. **"Cold Turkey"**: delete medical bookmarks, exit neurotic chats, block medical sites.
2. Put on your phone screensaver: **"I'm not allowed to Google. I'm healthy. This is PPPD."**
3. Go 7 days without searching Google for symptoms. Check the box every day.
4. Every time the thought *"What if the doctors missed something?"* appears in your head, answer it with one phrase: *"Organics are excluded. My software is buggy, but my hardware is fine."*

Exposition: we go towards fear and take back our territory

Why avoidance is food for your neurosis, how to build a ladder of fears and use Frankl's paradoxical intention.

1. Essence and problems: life within four walls

Between PPPD and the fear of dizziness, your world gradually becomes smaller. First you gave up traveling on the subway, then you stopped going to large shopping centers. Then you excluded public transport, hairdressers and cafes from your life. As a result, your living area has been reduced to the "bed - kitchen - nearest trash can" trajectory, and every step outside this radius causes wild panic.

You hope that by staying at home, your nervous system will "calm down", and then you will begin to live again. But the opposite happens. The longer you stay at home, the scarier the usual going out becomes. Your brain has declared war on the entire outside world, hanging the label "Danger to Life" on every corner.

2. Physiological explanation: fear extinction mechanism and neuroplasticity

The only way to prove to the brain that a situation is safe is to place the body in this situation and show that no disaster has occurred. This is called an **exposure** (behavioral experiment).

In the brain, the amygdala is responsible for the fear response. When you avoid a place (for example, the subway), the amygdala thinks: * "We didn't go to the subway and survived. This means that the subway is a death trap, and avoidance saved our lives."* This reinforces fear at the level of neural connections.

During the exposure, you are forced to enter the subway and stay there. The Amygdala instantly triggers panic, releases adrenaline and waits for your death or escape. But you stay where you are. 3 minutes pass, 5 minutes, 10 minutes... The adrenaline burns out. You don't faint, you don't die, your heart slows down. Amygdala sees: * "We were in the "trap" for 20 minutes. Pulse decreased. We didn't die. Apparently the metro is not that dangerous."* This process is called **extinction of the conditioned reflex**. By repeating this experiment 5 times, you completely rewrite the neural program of fear.

3. Cognitive traps: jumping over steps and running for safety

The main mistake of beginners is to try to "defeat fear in one fell swoop." A man who hasn't left his house for six months decides to go to the other end of the city during rush hour on the subway. He catches a wild panic crisis at the first station, runs out in horror, calls a taxi and returns home. Result: *"The exposure doesn't work! I felt even worse!"*.

You jumped five steps. Your amygdala experienced extreme stress and only confirmed its hypothesis about the danger of the world. You need to climb the ladder of fears strictly one step at a time.

The second trap is running away at the first discomfort. Remember: **if you started the exposure and ran away at the peak of panic, you just fed your fear**. You must wait until the adrenaline wears off and the panic begins to subside, being inside a frightening situation. Only then will the brain record the experience of victory, not defeat.

4. Point of Support: Insight

> *Avoidance is the tribute you pay to your fear. But this racketeer will never get enough. With every refusal you make to travel or go to the store, he will take new territory from you until he bans you in your own bed. It's time to take back what is rightfully yours.*

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5. Practice: Exposure Protocol and Paradoxical Intention

We will act like engineers: we will break a large problem into small, manageable steps.

Step 1. Ladder of fears

Make a list of situations that you avoid and rate your fear level from 1 to 10.

- **1–2 points (Stage 1):** Go out into the yard, stand at the entrance for 10 minutes.
- **3–4 points (Level 2):** Go to the grocery store near your home, buy one thing.
- **5–6 points (Level 3):** Travel one bus stop, go to the supermarket.
- **7–8 points (Level 4):** Stand in a long line, go to a large shopping center.
- **9–10 points (Level 5):** Take the subway during rush hour, go to another area alone.

Step 2. Assault Rules

1. **Choose a goal:** Start with a step with a fear level of **2-3 points**.
2. **Ban on crutches:** Walk without sedatives in your pocket, without loved ones (if the goal is to learn to walk alone) and without fixing your gaze on the asphalt (apply the "Horizon Rule").

3. **15 minute rule:** Stay in the situation until your fear drops by at least half. Don't leave at the peak of anxiety.
4. **Repetition:** Go to the same point 3-4 times during the week until the fear drops to 1 point. Only then move on to the next step.

Step 3. Frankl's Secret Weapon - Paradoxical Intention

If in the middle of an exposition you feel panicked and feel like you're going to fall, use the paradoxical resolution method. Instead of fighting fear, start demanding it:

> *"Come on, spin me even harder! Throw me onto the asphalt right here, in front of these people! I want to faint, let's do it now! Spin me like a carousel!"*

When you sincerely allow a symptom to happen, you destroy the very essence of fear (resistance). The brain catches a system error: * "Do we want to fall? But then why are we afraid?"* - and the panic circuit instantly turns off.

Step 4. Analysis of "ABC" - dissect your fear on paper

Before each exposure (and after it), describe the situation in three steps:

A - Situation (Activating event): Only bare facts. *"Went to the supermarket"*.

B - Beliefs: Everything that your head screams. First, write down all the arguments **FOR** that a catastrophe will happen: *"I'm going to faint. People will see and call an ambulance. I'll feel bad in public."* Thicken the clouds to the limit, write all the worst things. Then write down all the arguments **AGAINST**: *"I have never fainted in my life. When you panic, your blood pressure rises—fainting is physically impossible. People don't care about me, everyone is on their phone."*

C - Consequences: What really happened? *"Standed for 15 minutes. It rocked. Didn't fall. Nobody looked. Adrenaline burned out in 5 minutes."*

Step 5. Evening conference - unload the garbage from your head

Every night before going to bed, sit down for 10 minutes and write down on paper **everything** that you were afraid of that day. All this mental garbage. Now re-read what you wrote in **third person** as if you were reading someone's log file. For example:

- You wrote: * "I'm afraid to turn around at night - the dizziness will get worse - I'll fall out of bed" *. *Now read this from a distance: *"The object is lying on the bed. He is afraid that when he turns his head he will fall. But it lies horizontally. Where exactly will it roll from a horizontal surface? What physical action must he perform to fall over? None. He lies there like a statue. Absurd*".

When you start looking at your fears in the third person, they become not scary, but **funny**. This is the goal.

Homework (DZ):

1. **Fill out your personal "Ladder of Fears"** of 10 steps.
2. Select target #2 (fear level 2-3) and **do exposure 3 times this week**. Record the results in a diary (time, level of fear before/during/after, final result).

Hidden avoidance: crutches you don't notice

You can honestly go to exhibitions every day, but not get results. Why? Because you are using **hidden crutches** - subtle forms of avoidance that sabotage the process. Your amygdala is not stupid: it has found a way to "survive" the exposure without changing.

Test yourself:

- **Phone-rescuer:** You are walking down the street, but you are clutching your phone and are not looking around. Or you keep a pill in your pocket "just in case" (even if you don't drink). The very presence of a pill is a crutch. The brain knows: "I have an emergency exit." The exposure is counted, but not completely.
- **Fixation of gaze:** You are in a shopping center, but you have buried your eyes on the floor so as not to see the colorful space. Technically, you're "in the mall," but your brain has shielded itself from the visual stimulus. Look up. Look at the horizon, at the shop windows, at the people.
- **Timing hack:** You go to the store, but strictly at 7 am, when no one is there. It's better than staying at home, but the cost of exposure is reduced. Gradually move the time towards rush hour.
- **Crutch Man:** You always take your husband/wife/friend with you. This is normal on the first steps of the stairs. But if you're at stage 5 and still don't walk alone, you're stuck. The presence of a "safe person" prevents the amygdala from truly updating its threat map.

Rule: At each step of the ladder of fears, gradually remove one crutch. Go without the pill first. Then - without headphones. Then - without a partner. Only then does the exposure work 100%.

Two exposure strategies: Gradual vs Flooding

Gradual (gradual) - recommended

This is what we did above: the ladder of fears, we start from the lower steps and go up. Suitable for most people. Safe, predictable, does not cause traumatic experiences.

Flooding (flooding) - advanced

This is a jump straight to a high rung of the ladder. For example, instead of “5 minutes in a small store” - immediately “30 minutes in a hypermarket during rush hour.” Works faster, but requires strong motivation and understanding of physiology.

When to use flooding:

- You have already gone through the gradual exposure to the middle of the stairs and are “stuck”.
- You understand that adrenaline will burn off in 5 minutes and it is not dangerous.
- You have a specific task (trip, flight, important meeting) and don’t have time for a gradual approach.

Flooding rule: Once you enter, you don’t leave until the alarm goes down by at least 50%. If you went into a shopping center and ran away 2 minutes later, you reinforced the “shopping center = danger” connection in the amygdala. Stop, breathe, watch. Anxiety will begin to subside within 5–10 minutes. When it has fallen halfway, you can go out. You won.

What to do after exposure

After intense exposure (especially flooding) you may feel:

- Slight trembling in the arms and legs - this is adrenaline leaving the muscles.
- Drowsiness - parasympathetic takes over after the fight.
- Increased appetite - the body wants to replenish spent resources.

This is all **normal**. It didn't "get worse." This body processes experience. Give it 30-60 minutes and the symptoms will go away. The best thing you can do after exposure is drink warm tea, sit in silence for 10 minutes and write down the result in a diary.

Safety Crutches: Subtle Avoidance Stealing Your Final 20% of Recovery

You already leave the house, shop in supermarkets, and ride the subway. Yet an "emergency pill" still sits in your pocket just in case, your hand clutches a water bottle, and your eyes habitually scan for the nearest bench. Here is why these invisible crutches prevent your brain from shutting off dizziness completely.

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1. Stuck at 80%: "I Do Everything Right, Why Does the Background Swaying Remain?"

In my consultations, I constantly encounter people trapped in the exact same phase. Let us call it **the stubborn tail-end of PPPD**.

The person tells me:

> *"Maxim, I've read the foundational chapters, I do my vestibular drills, I walk 8,000 steps a day, I commute to work, and I go into shopping malls. The acute panic attacks are gone. Yet I still have a constant 2-out-of-10 background rocking sensation! Why haven't the final 20% of my symptoms cleared up for three months?!"*

I start asking simple, surgical questions:

- *"Be honest with me: when you walk into a shopping mall, what is sitting in your right jacket pocket or purse?"* — *"Well... half a tablet of Xanax / Atarax. I haven't taken one in two months! I just carry it 'just in case'—it makes me feel safer."*
- *"And when you walk down a busy avenue, what are your hands doing?"* — *"One hand is in my pocket gripping my phone, or I'm holding a bottle of ice water so I can take a sip if I feel floaty."*
- *"And how do you stand in the checkout line?"* — *"I lean my hip against the counter or grip the shopping cart handle with both hands."*

There is the answer. The person believes they are practicing exposure therapy. In reality, they are moving through the world wrapped in an invisible suit of **Safety Behaviors**.

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2. The Neurobiology of the Crutch: Paul Salkovskis' Discovery

In Cognitive Behavioral Therapy, this exact mechanism was identified and clinically proven by **Professor Paul Salkovskis** at the University of Oxford.

His finding is simple and unforgiving: **safety behaviors completely block inhibitory learning in the amygdala**.

Let us examine how your amygdala (your threat detection center) interprets a trip to the supermarket when you carry a "crutch" in your pocket:

> ***What You Think (Prefrontal Cortex):*** *"I went to the grocery store with my emergency pill in my pocket and held onto the shopping cart, and I didn't faint — great job, I'm training my brain!"*
>
> ***What Your Amygdala Records (Limbic System):*** *"Alert! Today we entered a lethally dangerous supermarket! We narrowly escaped fainting and catastrophe ONLY BECAUSE we gripped the shopping cart with both hands, sipped cold water, and had an emergency sedative ready in our pocket! Conclusion: The supermarket is still deadly! Keep the vestibular alarm active!"*

Do you see the brilliance and cruelty of this trap?

Every single "precaution" you take serves as **direct proof to your survival brain that the threat is real**. Why would a firefighter take off their helmet and fireproof suit if they believe the building is still burning? As long as you carry a "fire extinguisher" in the form of an emergency pill or water bottle, your brain remains 100% convinced that the fire is still raging.

Exposure Metric	Pseudo-Exposure (With Crutches)	Pure Exposure (Zero Crutches)
Behavior	Entering a mall wearing sunglasses, gripping a cart, carrying water & pills	Entering a mall empty-handed with unrestricted vision
Attentional Focus	Scanning for exits, benches, and calculating distance to home	Open presence in the environment
Short-Term Effect	Lower anxiety (4/10), illusion of control	Higher initial arousal for 10 minutes (7/10)
Amygdala Appraisal	"We survived only because of our safety gear! Threat is real!"	"We survived with zero protection! The alarm was false!"
Long-Term Outcome	Permanently plateaued at 70–80%, chronic low-grade swaying	Complete extinction of the fear reflex and 100% remission

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3. The Complete Registry of PPPD Safety Crutches

People dealing with chronic dizziness are remarkably resourceful. Over the years, I have cataloged 6 categories of hidden crutches that disguise themselves as "harmless habits":

1. The Pharmacological Talisman ("The Pocket Pharmacy")

A blister pack of benzodiazepines (Xanax, Klonopin, Ativan), hydroxyzine, beta-blockers, herbal drops, or strong menthol lozenges tucked into a wallet or bag. You may not have swallowed one in months, yet the moment you consider leaving it on the kitchen counter, panic flares up by the elevator: *"What if it hits me two miles from home and I have nothing on me?!"*

2. Tactile and Postural Anchors

- White-knuckling the handle of a grocery cart or stroller (using it as a medical walker).
- Automatically leaning a shoulder or hip against a wall, pillar, or handrail while waiting in line or riding the train.
- Walking exclusively along the edge of the sidewalk next to fences or building walls "just in case I need to grab something."
- Stuffing hands deep into jacket pockets or crossing arms tightly across the chest (mechanically bracing the torso into a rigid splint).

3. Sensory Mufflers (Stimulus Shields)

- **Sunglasses or a low-brimmed cap** worn on overcast days or indoors inside supermarkets to cut off peripheral optical flow and fluorescent glare.
- **Noise-canceling headphones** worn outdoors or in stores to mute acoustic spatial volume.
- **Staring down at a smartphone screen** while walking or standing in line — you aren't actually reading messages; you are using the phone as a fixed visual anchor to avoid looking at the open room.

4. Ritual Props

- **The Water Bottle:** Taking micro-sips of water every 3 minutes (because swallowing briefly stimulates the vagus nerve and interrupts sympathetic panic).
- **Pulse Oximeters & Smartwatches:** Checking heart rate on your wrist every 15 minutes (*"What's my pulse right now? 96 bpm? Oh no, I need to sit down"*).

5. The Social Guide Dog ("I Don't Sway When Someone Is With Me")

Being willing to walk anywhere — provided you are holding your partner's arm or walking alongside a friend. Or a subtler variation: walking alone while **talking on the phone the entire way**, using the person on the line as a "live lifeline" in case you collapse.

6. Geographical Escape Geometry

- Plotting walking routes strictly past pharmacies, urgent care clinics, or subway stations.
- Parking the car right next to the store entrance so the "retreat sprint" is under 30 yards.
- Choosing seats in movie theaters, restaurants, or airplanes strictly on the aisle closest to the exit or restroom.

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4. Why the Final 20% of Dizziness Clings to Your Crutches

Recall the fundamental law of neuroplasticity from Module 3: **a neural fear circuit extinguishes only when a "Prediction Error" occurs**.

Your amygdala predicts: *"If we stand in the middle of a bright, crowded hypermarket without a cart, without water, and without a pill in our pocket, we will lose control and collapse onto the floor."*

As long as you push a cart and keep a sedative in your pocket, **that prediction is never tested**! The amygdala never receives disconfirming evidence.

To erase the final 20% of background unsteadiness, your brain must experience raw, unbuffered reality:

1. I am standing in the center of a bright, busy space.
2. My hands are empty, my shoulders are dropped.
3. My emergency pills are sitting in a drawer 10 miles away.
4. My phone is zipped inside my bag.
5. A wave of rocking hit me — I did absolutely nothing to protect myself — and **nothing catastrophic happened**.

Only in that exact moment does a powerful inhibitory signal fire from the ventromedial prefrontal cortex to the amygdala: *"The threat is zero. Stand down."*

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5. The "Burn the Boats" Protocol: Disarming in 14 Days

Dropping your crutches feels uncomfortable at first — during the first 48 hours without them, your anxiety and swaying may temporarily tick up by 20–30%. Expect this and welcome it: you are taking the training wheels off the bicycle.

Follow this structured 14-day disarmament schedule:

Phase 1. Emptying the Pockets (Days 1–3)

Take every "emergency" pill, sedative, herbal drop, mint, and water bottle out of your pockets, purse, and car glovebox, and leave them at home.

- Walk out the door empty-handed.
- When your mind bargains (*"What if a massive panic attack hits?!"*), answer firmly: *"Then we will experience the adrenaline surge raw and process it on the spot. My tools are in my head, not in a blister pack."*

Phase 2. Releasing Postural Props (Days 4–7)

- In the supermarket, swap the wheeled shopping cart for a **handheld basket** (or carry items in your hands).
- When standing in a checkout line or subway car, stand **in the open center**, without leaning your hip against a counter or bracing your back against a wall. Let your ankles and cerebellum micro-adjust your balance naturally.
- Take your hands out of your pockets while walking — let your arms swing freely in a natural cross-crawl gait pattern (essential for vestibular calibration!).

Phase 3. Removing Sensory Filters (Days 8–10)

- Take off sunglasses and hoods indoors and on cloudy days.
- Remove headphones when walking down busy streets.
- Keep your phone in your pocket while waiting in lines and let your gaze rest naturally on the horizon and surrounding space.

Phase 4. Reclaiming Geometry (Days 11–14)

- Sit in the middle of the row at a theater or café, with your back to the exit.
- Walk straight across the center of open plazas rather than hugging building walls.
- Take off your smartwatch or disable heart-rate notifications.

💬 Core Insight

> *"That 'just-in-case' pill in your pocket is not protecting you from panic — it is guaranteeing that your PPPD never fully ends. A crutch is meant for a broken bone. Once the bone has healed, continuing to lean on the crutch is what keeps you limping. Throw away the safety net so your brain can finally see that you can stand on your own."*

📝 Homework Assignment (Action Plan)

1. **The Contraband Audit:** Right now, empty your pockets, bag, and car console, and write down on paper every single “safety crutch” you use (objects, body postures, phone habits, seating choices). Be ruthlessly honest.
2. **Operation Empty Pockets:** Tomorrow, go for a 40-minute walk or grocery run without a single safety item (no emergency meds, no water bottle, no mints, no phone call to a partner).
3. **The “Center of the Aisle” Drill:** Walk into any supermarket or mall, stand right in the middle of a wide aisle empty-handed (no cart, no phone) for 3 full minutes. Soften your knees, drop your shoulders, and allow your body to sway slightly without bracing. Lock in the realization: your point of support is inside you, not in your pocket.

Sports and reboot: burning adrenaline fuel

How cardio restores the brain on a physical level, why a neurotic needs Kuznetsov's applicator, and how to reset daily internal dialogue.

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1. Essence and problems: scorched earth syndrome

With PPPD, you are in a paradoxical state: on the one hand, your body is wildly tired and exhausted, on the other, a constant background tension is raging inside, which does not allow you to relax. You feel like a motor is clamped in a vice, idling at maximum capacity.

Because of this, you begin to fear any physical activity. As soon as you speed up a little, climb the stairs or make a sudden movement, your pulse jumps, your head starts to spin, and your unsteadiness intensifies. Your frightened brain whispers: * "Sport is dangerous. Physical activity will make me worse, my heart will fail or a blood vessel will burst."* You lie down on the sofa and reduce your activity to zero, hoping to preserve your strength. But lying on the couch only preserves the adrenaline inside your muscular shell, burning out your organs from the inside.

2. Physiological explanation: BDNF effect and adrenaline utilization

Evolutionarily, adrenaline and cortisol are created for the same purpose - **physical action**.

When primitive man saw a saber-toothed tiger, his adrenal glands released adrenaline so that he could run as fast as he could or fight. The muscles of the legs and arms worked actively, burning this hormonal cocktail. When the tiger was left behind, the adrenaline was dissipated and the body returned to rest.

You sit on the couch or at the computer and scroll through frightening scenarios in your head. Adrenaline is released, but no physical action occurs. The hormone continues to circulate in the blood for hours, causing chronic vasospasm, vegetative storms and that very "cotton" head.

To break this stagnation, we need physical activity. And here the balance between two types of activity – cardio and strength training – is extremely important:

1. **Therapeutic Cardio (Aerobic Exercise):**

- **Recycling of stress hormones:** Muscles working at a cyclical pace literally "eat" excess adrenaline and cortisol from the blood. After cardio, blood vessels dilate, improving brain microcirculation.
- **BDNF production:** Brain-derived neurotrophic factor is a protein that stimulates the birth of new neurons and synapses. It restores the structure of the brain (in particular, the hippocampus), making it plastic.

- **Heart rate desensitization:** Cardio teaches the amygdala that a high heart rate (HR) and deep breathing are a normal physical response to exercise, and not a panic attack.

2. **Strength training (anaerobic exercise):**

- **Postisometric relaxation:** Forceful contraction of large muscle groups (quadriceps, buttocks, core, back) is replaced by a phase of deep relaxation. The brain cannot hold the muscle shell after the muscle has received an extreme but controlled load.
- **Proprioceptive Frame:** Strength exercises for the legs and core (squats, lunges, planks) load joints and ligaments, sending powerful, clear signals to the brain about the position of the body in space. This dense flow of proprioceptive input from the legs "clogs" vestibular interference and reduces visual dependence. The brain remembers: *"We have solid support on the ground"*.

In addition, an overloaded body requires **external relaxation triggers**. Since your brain has forgotten how to send the command "relax" to your muscles, we will act from the outside. A warm bath, massage or Kuznetsov's applicator creates a powerful blood flow to spasmodic areas (neck, shoulder blades, lower back), forcibly removing muscle blocks and switching vegetatives to parasympathetic mode.

3. Cognitive traps: fear of high heart rate

Main cognitive trap: *"If my heart rate rises and my breath stops during activity, this is the beginning of a panic attack. My heart can't stand it."*

This is an error of interpretation. A rise in heart rate, slight shortness of breath and warmth in the body during exercise is a **normal, healthy reaction of the heart to work**. Your brain has forgotten how to distinguish between healthy physical excitement and panic. Every time the pulse increases from walking or squats, the brain becomes afraid of the pulse itself and triggers a real panic. Our task is to accustom the brain to the fact that a high heart rate while moving is absolutely safe.

About pills - honestly and without illusions

If you were prescribed antidepressants (SSRIs) and they helped, this is not a reason for joy, this is **proof**. Proof that your problem is functional and not organic. Antidepressants for PPPD work like painkillers for a fracture - they immediately relieve pain, but the bone will not heal. If an SSRI eliminates dizziness, then it's all about neurotransmitters and an anxiety disorder, which means **you can cope without them, through psychotherapy and physical work with the body**. To drink or not to drink is the decision of you and your doctor. But know this: the pill will not reflash your neural connections. Only your daily actions will do this.

4. Point of Support: Insight

> *Adrenaline is gasoline. If you press the gas pedal all the way but stand still with the handbrake, the engine will explode. Take off the handbrake. Let your body move to burn off the fuel that is poisoning it.*

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5. Practice: Forced reboot protocol

We introduce four therapeutic tools into your routine:

1. Therapeutic cardio (2 times a week)

- **Type of activity:** Brisk walking, swimming, exercise on an elliptical trainer or exercise bike. The main thing is the absence of shock load on the neck (for now we exclude jumping and classic running on asphalt).
- **Pulse:** Keep your heart rate in the range of **110-130 beats**. The load should be moderate: you can talk while moving, but you can't sing.
- **Duration:** Start with 20 minutes and gradually work up to 40 minutes.

2. Strength training with an emphasis on the core and legs (2 times a week)

- **Type of activity:** Exercises with your own weight, fitness bands or light dumbbells. Emphasis on legs (squats, lunges) and core (planks, glute bridge). We exclude exercises with axial load on the spine (heavy barbell on the shoulders) and strain on the neck.
- **Principle:** Do the exercises slowly, concentrating on muscle contraction and subsequent relaxation. Give your brain a sense of solid physical support.
- **Duration:** 25-35 minutes.

3. Physical grounding (Kuznetsov Applicator)

- Every evening before going to bed, lie with your back and neck on the Kuznetsov needle applicator (or Lyapko applicator).
- **Time:** 15–20 minutes. The first 2 minutes will hurt, but then a powerful wave of heat will spread throughout the body. This expands the spasmodic capillaries of the neck and back, releasing endorphins into the blood - natural painkillers and relaxants.

4. Reflash of internal dialogue (Conscious affirmations)

Here's what most people don't realize: your brain is already spinning affirmations. Right now. 24/7. Only negative ones. "I'm broken." "I'm shaking again." "This will never go away." "I won't be able to walk

normally." You don't notice them, just like you don't notice the background noise of an air conditioner. But they exist - and they program the brain for illness.

Affirmation is not magic or "positive thinking." This is a **conscious replacement of malicious code**. Neuroplasticity works both ways: if you've been telling yourself "I'm sick" for years, your brain has built a thick neural highway underneath it. Our task is to pave a new one.

Key Principle: Affirmation only works in conjunction with physical action. Don't sit on the couch and mumble "I'm healthy" - your brain won't believe it. You walk down the street and say to yourself in the rhythm of your steps:

- *"My body is physically healthy. My pulse works for the benefit of vegetarianism."*
- *"Unsteadiness is just a spasm, it's harmless. My feet firmly hold the ground."*
- *"With every step I burn off excess adrenaline and restore my neurons"*.
- *"Dizziness is noise on the line, not a signal of disaster."*

Why it works: When you say an affirmation while walking, your brain receives a **dual signal**—from your leg muscles (proprioception: "we are standing, we are walking, we are stable") and from your speech (cortex: "we are safe"). This double stream of data physically builds new neural connections, replacing old disturbing ones. The principle is the same as with London taxi drivers: you repeat the route - the brain grows neurons for it. The old path of fear is overgrown. The new path of peace is being trampled down.

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4 Week Workout Program for Someone with PPPD

Below is a specific plan that you can start implementing today. Each week increases the load by ~20%. If some week is too hard for you, stay there for another 7 days, don't jump over steps.

Week 1: Soft Start

Day	Workout	Details
Mon	Cardio (walking)	20 minutes brisk walking, heart rate 100–115. Familiar route, no visual triggers
W	Holidays	Kuznetsov applicator 15 min + neck stretch
Wed	Power (at home)	3 sets: squats ×12, glute bridge ×15, plank 20 sec, knee push-ups ×8
Thu	Holidays	Applicator + Jacobson
Fri	Cardio (walking)	25 min, same route, a little faster (heart rate 110–120)
Sat-Sun	Active recreation	Easy walk 30 min, no speed control

Week 2: “Building the Base”

Day	Workout	Details
Mon	Cardio	30 min walk, pulse 110–125. Add one unfamiliar section of the route
W	Holidays	
Wed	Power	3 sets: squats ×15, lunges ×10 on each leg, plank 30 sec, push-ups ×10
Thu	Cardio (lung)	20 minutes walking at a calm pace - restorative
Fri	Power	3 sets: squats paused 3 sec at the bottom ×12, side plank 20 sec, single leg glute bridge ×10
Sat	Cardio	30 min, heart rate 115–130, new route
Sun	Holidays	Full day without stress

Week 3: “A Confident Step”

Day	Workout	Details
Mon	Cardio	35 min walking/light jogging (if your neck allows), heart rate 120–130
W	Power	4 sets: squats ×15, lunges with a step back ×12, plank 40 sec, “superman” (lying on stomach, lifting arms/legs) ×10
Wed	Holidays	
Thu	Cardio	30 min, add intervals: 3 min fast / 2 min calm
Fri	Power	Tuesday repeat + side lunges ×10, wall chair 30 sec
Sat	Cardio	40 minutes at a steady pace
Sun	Holidays	

Week 4: Consolidation

Day	Workout	Details
Mon	Cardio	40 min, heart rate 120–135, route with climbs

Day	Workout	Details
W	Power	4 sets: squats with dumbbells/bottles 2-3 kg ×15, lunges ×12, plank 45 sec, deadlift with dumbbells ×12
Wed	Cardio (recovery)	20 min easy walk
Thu	Power	Repeat Tuesday + add 2 sets of “chair” for 40 seconds
Fri	Cardio	35 min interval walking: 4 min fast / 1 min easy
Sat	Active recreation	Swim, bike or long walk 60 min
Sun	Complete rest	

After week 4 go back to week 3 or 4 and alternate between them, gradually increasing weights/time/intensity by 10% every 2 weeks. Don't chase results. Your goal is not sports records, but regular utilization of adrenaline and rewiring of the brain.

Homework (DZ):

1. Do **2 20-30 minute cardio workouts** and **2 leg/core strength workouts at home** this week. Record the sensations in your body after exercise (after 15-20 minutes of rest).
2. Buy a Kuznetsov applicator (if you don’t have one) and **lie on it every night before bed**.
3. Write 3 of your own wordings for reflashing the dialogue and repeat them every time you go for a walk or start training.

Vestibular migraine: when the head explodes and the brain panics

Why vestibular migraine and PPPD are twin brothers, how anxiety turns rare attacks into a chronic disability, and what exactly to do to turn off this pain generator.

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1. Essence and problems: two monsters in one head

You have PPPD. The picture rocks, sways, and floats. You already know that this is a functional failure, and not a hardware failure. You work from the book, do Jacobson, walk 10,000 steps, break the adrenaline loops.

And then - bam. Attack. Not just unsteadiness, but a **real blow**: splitting headache, nausea, light hurts your eyes, sounds hit your temples. Or - even worse - dizziness so strong that you lie on the floor, afraid to move, because any movement of your head causes the room to rotate. And it lasts not 5 minutes, but 4, 8, 72 hours.

Welcome to the world of **vestibular migraine** (VM).

You go to a neurologist. He says, “You have a migraine. Here are the pills." You go to an otoneurologist. He says, “You have PPPD. Do gymnastics." And you stand between them and think: “So I have a migraine or PPPD? Or am I even dying?”*

The truth is that you most likely have **both**. And they feed each other, like two parasites in one body. Vestibular migraine is one of the most common triggers of PPPD. And PPPD, in turn, aggravates migraines through anxiety. A vicious circle that we will now break.

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2. What is vestibular migraine: neurophysiology without fog

Migraine is not “just a headache”

A common tension headache is a dull pressure from tight muscles. Migraines are a completely different beast. It is a **neurovascular disorder**: a disruption in the electrical activity of the brain that causes a cascade of events.

Here's what happens during a migraine attack:

1. **Cortical Spreading Depression (CSD).** A wave of abnormal electrical activity sweeps across the cerebral cortex, temporarily shutting down neurons. If the wave passes through the visual cortex, you

see an aura (flickering zigzags, blind spots). If through the somatosensory, the hand or face goes numb. If through the **vestibular cortex**, the world begins to rotate.

2. **Activation of the trigeminovascular system.** The trigeminal nerve releases the neuropeptide CGRP (calcitonin gene-related peptide), which dilates the vessels of the dura mater. The walls of the blood vessels swell, putting pressure on the nerve endings - and here you have a throbbing, hellish pain.
3. **Central sensitization.** After several attacks, the brain "learns" pain. Neurons of the trigeminal nucleus become hypersensitive. Now even a normal pulse in the vessels is perceived as a pain signal.

Why vestibular migraine is special

In classic migraine, the main symptom is pain. Vestibular - **dizziness**. And it happens like this:

- Dizziness **instead of** pain (painless migraine - yes, that exists)
- Dizziness **along with** pain
- Dizziness **to** pain (like aura)
- Dizziness **after** pain (postdromal tail)

This is why CM is so difficult to diagnose. You go to the doctor complaining of dizziness, and he looks for problems in your ear. The problem is in the brain. Not in the structure of the brain (clean MRI), but in its **electrical activity**.

According to the International Headache Society (IHS), diagnostic criteria for vestibular migraine are:

- At least 5 episodes of vestibular symptoms between 5 minutes and 72 hours *Current or past diagnosis of migraine (with or without aura)
- During an attack: headache, photophobia, phonophobia or visual aura
- Other causes excluded (Meniere's disease, BPPV)

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3. PPPD + Vestibular migraine: a toxic alliance

Here are some statistics that will make your spine go cold:

*According to Staab et al. (2017), **up to 30%** of patients with vestibular migraine develop PPPD as a complication. *According to Furman et al. (2005), **60–70%** of patients with PPPD have a history of migraine.

- A study by Bisdorff (2011) found that people with migraine were **3 to 5 times more likely** to develop chronic functional vertigo than people without migraine.

Why? Because vestibular migraine is the **perfect trigger** for PPPD.

Triple strike mechanism

1. **The first blow is biological.** A migraine attack hits the vestibular cortex. The brain has a powerful, frightening experience of complete loss of control: the world is spinning, you feel sick, you can't get up. This experience is recorded in the amygdala as a **traumatic event**.
2. **The second blow is cognitive.** After the attack, you begin to fear the next one. *"When will this happen again? What if this happens at work? On the subway? Are you driving?"* Anticipatory anxiety triggers an **anticipatory panic loop**: you are so afraid of the next attack that you live in constant tension.
3. **The third blow is behavioral.** You begin to avoid triggers: bright light, noise, movement, public places. But the more you avoid, the more sensitized your brain becomes. The sensitivity threshold drops. Now even a weak stimulus causes an attack.

Bottom line: migraine triggered PPPD. PPPD worsened the migraine through anxiety. Migraines have become more frequent. PPPD has intensified. Snowball.

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4. Anxiety is like gasoline for migraines: the biochemistry of a vicious circle

Here's what chronic stress does to the migraine brain:

Cortisol and migraine threshold

Each migraine sufferer has a so-called **migraine threshold** - the level of neuronal excitability above which an attack is triggered. The lower the threshold, the easier it is to cause a migraine.

Chronic stress and anxiety **systematically lower** this threshold:

- **Cortisol** increases the excitability of cortical neurons, making CSD more likely.
- **Sleep disturbance** (a classic for anxiety) deprives the brain of the deep sleep phase, in which the neurotransmitter systems "reboot" occurs.
- **Muscle spasm of the neck** (muscle armor, Chapter 4) compresses the occipital nerves, causing the cervicogenic component of migraine.
- **Excess adrenaline** causes instability of vascular tone - the same constriction/expansion surges that trigger the trigeminovascular cascade.

Serotonin trap

Serotonin is a key neurotransmitter in both anxiety disorders and migraines. With anxiety, serotonin is **depleted** (this is why SSRIs help with both anxiety and migraines - they increase the availability of serotonin). A drop in serotonin lowers the migraine threshold. Migraine → stress → serotonin is depleted even more → the threshold is even lower → migraines are even more common.

This is not a metaphor. It is literally a biochemical loop that closes without external intervention.

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5. Cognitive traps: three pits into which you have already fallen

Trap No. 1: “I have an organic migraine, psychology has nothing to do with it”

Migraine is a neurobiological disorder. No doubt about it. But the **triggers** of migraines are 60-80% behavioral and psychological: stress, lack of sleep, skipping meals, muscle tension. You can't change your migraine brain (genetics), but you can **raise your migraine threshold** by reducing your chronic stress levels. This is not "psychology". This is the control of biochemistry through behavior.

Trap #2: “I just need to find the right pill.”

Triptans (sumatriptan, zolmitriptan) relieve acute attacks. Prevention (beta blockers, topiramate, antidepressants) reduces the frequency. But no pill will solve the problem of **anxious overcontrol** that keeps your migraine threshold on the floor. The tablet is a tactic. Dealing with stress is a strategy. You need both.

Trap #3: “I can’t do exposure, stress gives me migraines.”

This is the most insidious trap. Yes, acute stress can trigger an attack. But **chronic stress from avoidance** provokes attacks even more often. Paradox: the more you hide from life, the lower the threshold falls, the easier it is to cause a migraine. Correct exposure - **graded**, slow, controlled - does not increase the risk of an attack. It reduces it because it reduces background anxiety.

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6. Point of Support: Insight

> *Migraines have a genetically low ceiling for stress. You can't raise the ceiling higher than genetics allows. But you can clean up the trash from the floor. The less junk of anxiety, muscle spasms and lack of sleep accumulated in your body, the further your head is from the ceiling. Your task is not to cure migraines (this is genetics), but to create conditions under which attacks become rare and short. And it's completely in your hands.*

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7. Practice: Protocol for reducing migraine load

We combine pharmacology (work with a neurologist) and behavioral techniques (your area of responsibility). The pills are not my territory; the doctor selects them. My territory is **everything else** that you can do yourself.

Technique 1: Headache Diary

Without a diary, you are a blind kitten. With a diary is an engineer who sees patterns.

Write down every day:

- **Date and time** of attack (start → end)
- **Intensity** on a scale of 1–10
- **Symptoms**: pain, dizziness, nausea, aura, photophobia
- **Triggers 24 hours before**: sleep (how many hours), food (did you skip), stress (score 1-10), weather, caffeine, alcohol, menstruation, exercise
- **What helped**: pill, sleep, darkness, relaxation

After 4-6 weeks you will see **patterns**. 90% of migraine sufferers have 2-3 key triggers. Once you figure them out, you will gain control. Not absolute, but enough to stop being a victim.

Technique 2: Three Pillars Protocol

Three things that are **proven** to raise the migraine threshold (Buse et al., 2016):

Pillar 1: Dream Anchor. Go to bed and get up at the same time every day. Yes, and on weekends too. The migraine brain **hates** irregularity. Oversleeping on a Saturday morning is a classic "weekend migraine" trigger. The body gets used to the rhythm. Any deviation is stress for an already labile nervous system.

Pillar 2: Food Anchor. Eat every 3-4 hours. Don't skip meals. A drop in blood sugar (hypoglycemia) is a powerful trigger. The brain consumes 20% of the body's total glucose. When you skip lunch, your brain literally starves. And a hungry brain is an irritable brain.

Pillar 3: Water Anchor. Dehydration reduces blood volume → blood vessels narrow → the brain does not receive enough oxygen → the threshold drops. Minimum 1.5–2 liters of water per day. Not coffee, not tea - pure water. Caffeine is additionally dehydrating.

Technique 3: Adapted Relaxation for Migraines

The standard Jacobson you do for PPPD needs to be **modified** for vestibular migraine:

The tension phase is softer. With migraines, severe tension in the muscles of the face and neck can trigger an attack. Tighten to 4-5 out of 10 (not 8-9, as with the usual Jacobson). Voltage time is 5 seconds instead of 7.

Focus on the cervical-occipital zone. Add a separate block:

1. Slowly press your chin to your chest (stretching the back muscles of the neck) - 5 seconds → release → 15 seconds of relaxation
2. Gently tilt your head back - 5 seconds → release → 15 seconds

3. Tilt your head towards your right shoulder - 5 seconds → release → 15 seconds
4. Tilt your head towards your left shoulder - 5 seconds → release → 15 seconds

Completion - visualization of coolness. At the final stage, imagine that a cool, damp cloth is being applied to your forehead. This image activates the parasympathetic system and physically reduces blood flow in the frontal arteries. A study by Blanchard et al. (1990) showed that the cool visualization group showed a 35% reduction in migraine frequency compared to the control group.

Technique 4: Diaphragmatic breathing with extended exhalation (4-7-8)

This technique is a nuclear weapon against the anxiety of anticipating an attack.

1. **Inhale** through the nose - 4 seconds (stomach inflates)
2. **Delay** - 7 seconds
3. **Exhale** through the mouth - 8 seconds (the stomach is deflated, the lips are folded into a tube)

Repeat 4 cycles. Physiology: an extended exhalation activates the **vagus nerve**, which switches autonomics from the sympathetic (combat) mode to the parasympathetic (restorative) mode. This lowers your heart rate, relaxes vascular smooth muscle, and reduces the release of CGRP, the same neuropeptide that triggers the migraine cascade.

Do 4-7-8 twice a day: in the morning (immediately after getting up, before the contrast shower) and in the evening (before bed). Plus - at any moment when you feel the growing anxiety of anticipation.

Technique 5: Biobehavioral Desensitization to Light and Sound

In migraine sufferers, the brain is **sensitized** to sensory stimuli: bright lights, loud sounds, flashing screens. With PPPD, you already practice visual desensitization (Chapter 8). For migraine we add:

Light exposure (graded):

1. Start with a room with dim lighting (40-50% brightness)
2. Every 3 days, increase brightness by 10%
3. After 2 weeks, go outside without sunglasses for 5 minutes
4. Gradually increase the time

Sound Exposure:

1. Listen to background white noise at low volume for 10 minutes
2. Every 3 days, turn up the volume by 5–10%
3. Move on to more “irritating” sounds: city noise, music, conversations

The goal is **not to get used** to pain, but to **raise the firing threshold** of sensory neurons. You teach the brain: “This level of light/sound is safe. There is no need to turn on the emergency siren.”

Technique 6: Cognitive Restructuring of Attack Fear

Fear of the next attack is **anticipatory anxiety**. It is a trigger in itself because it keeps cortisol on a constant rise.

Working with catastrophic thoughts:

Automatic thought	Rational answer
"The attack will happen at the most inopportune moment and I will be embarrassed"	*"In the entire history of migraine, not a single person has died from an attack. If it happens, I'll take a pill and wait it out. People around saw people with headaches"*
"If migraines become more frequent, it means I have something serious"	*"Increased migraines = decreased stress threshold. This is a functional process. My examinations are clear"*
"I can't live with these attacks, it's unbearable"	*"I've been living with them for N months now. I didn't die. The attacks last a finite amount of time. Between them I function"*
"I can't be nervous, otherwise I'll have an attack"	*"Stress is one of the triggers, but not the only one. Avoiding stress creates more stress than stress itself."*

Write down your top 3 fears about migraines. Next to each, write a rational answer. Reread this list every morning for 2 weeks.

Technique 7: "Calm Protocol" - what to do DURING an attack

When the attack has already begun, it's too late to train. We need a clear algorithm:

1. **Take a pain reliever within the first 20 minutes.** The sooner, the better. A triptan or NSAID (as prescribed by the neurologist). Don't wait "what if it goes away on its own." If you endure it, central sensitization will turn on, and the pill will no longer help.
2. **Dark room, horizontal position.** If you can, lie down. Minimum light and sound. Eye mask, earplugs.
3. **Cold on the forehead + heat on the neck.** A cool bandage on the forehead narrows the dilated meningeal vessels. A warm compress on the neck relaxes the cramped muscles that put pressure on the occipital nerves. This contrast is a powerful physiological intervention.
4. **4-7-8 breathing.** 4 cycles. Switch vegetatives.
5. **Forbid yourself from catastrophizing.** Say: *"This is an attack. He is finite. The tablet works. After 2-4 hours I will feel better. I've already experienced this."*
6. **After an attack, don't analyze.** Don't look for "why now," don't Google, don't complain in chats. Write down the facts (date, duration, triggers) in your diary and **close the topic**. Chewing on an attack is rumination, and rumination is fuel for the next attack.

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8. How the techniques from the book “Point of Support” reduce the frequency of migraines

Let's put it all together. Here's how each module of the book hits the migraine threshold:

Module/Chapter	What does it do for migraines
Muscle Armor + Jacobson	Removes chronic spasm of the neck and trapezius → eliminates the cervicogenic component → fewer triggers
Vestibular gymnastics	Desensitizes the vestibular cortex → the brain reacts calmer to movement → fewer vestibular migraine auras
Basic settings (sleep, caffeine, mode)	Stabilizes the circadian rhythm → normalizes serotonin and melatonin → raises the migraine threshold
Adrenaline Loop	Breaks the cycle of “fear of attack → adrenaline → vasoconstriction → attack”
CAS + Metacognitive Therapy	Turns off constant monitoring of symptoms → reduces background cortisol → fewer attacks
Exhibition	Desensitizes the brain to triggering situations → expands comfort zone → less avoidance → lower anxiety
Root Causes	Eliminates the source of chronic stress → the autonomic system calms down → the migraine threshold rises
Suppressed emotions	Unlocks muscle armor through anger work → physically relaxes neck and jaw

Study by Holroyd et al. (2010): CBT combined with preventive pharmacotherapy reduces migraine frequency by **60–70%**—significantly more than pills alone (40%) or CBT alone (50%). Combination is the key.

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9. Specific plan for 8 weeks

Here's a clear protocol for you. Not “I’ll start someday.” Today.

Weeks 1–2: Foundation

- Keep a migraine diary (every day, without skipping)
- Implement the “Three Pillars”: sleep-anchor, food-anchor, water-anchor
- In the morning: 4-7-8 breathing (4 cycles) → contrast shower → breakfast
- In the evening: adapted Jacobson (15 minutes) → 4-7-8 → sleep

Weeks 3–4: Decreased sensitization

*Add light/sound exposure (graded)

- Continue the vestibular exercises from Chapter 6
- Start cognitive restructuring: write down 3 fears about migraines, write rational answers

Weeks 5–6: Behavioral Activation

- Start general exposure: go to places that you avoid due to fear of an attack
- Increase your physical activity (10,000 steps + light cardio 20 minutes 3 times a week)
- Analyze your diary: identify 2-3 key triggers

Weeks 7–8: Consolidation

- Compare the frequency and intensity of attacks with the first weeks
- Re-read rational answers to fears - adjust based on new experience
- Conduct an audit of the “Three Pillars” - where were the failures? Shut them down

---

> ***DZ for this chapter:***
>
> 1. ***Download or create a migraine diary.*** *Start filling it out today. Write down EVERYTHING: even days without an attack (write “there was no attack” - this is also data).*
> 2. ***Implement the Three Pillars*** *now. Set your alarm for the same wake-up time. Have a bottle of water ready for the morning. Plan all your meals for tomorrow.*
> 3. ***Do an adapted Jacobson*** *(15 minutes) with a focus on the cervical-occipital area. After - 4 breathing cycles 4-7-8 with visualization of coolness on the forehead.*
> 4. ***Write down the 3 main fears*** *associated with migraines. Write a rational answer to each. Post this list in a visible place.*
> 5. *If you haven’t seen a neurologist yet about migraines,* ***make an appointment****. There are effective medications to prevent frequent attacks (>4 per month). The techniques in the book work* ***in addition*** *to pharmacotherapy, not instead of it.*

Derealization and depersonalization: the world behind glass

When reality seems unreal, and you feel like an actor in someone else's film. Why the brain turns on this "fuse" and how to safely remove it.

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1. Essence and problems: "I am here, but I am not here"

You probably know this. You wake up in the morning, open your eyes, and the world looks... **wrong**. It seems to be the same room, the same walls, but everything seems to be behind a glass wall. The colors are slightly muted. Sounds come as if from the next room. You look at your hands and don't fully recognize them. They are *yours*, you understand this with your mind, but between you and them there is an invisible film.

You're walking down the street and you feel like you're moving like a video game character: your legs are moving, your body is moving, but *you* are watching it from the outside. Like a drone operator flying somewhere far away. Familiar faces seem like masks. The streets you've walked along a thousand times look like stage sets. You hear your voice and it sounds alien.

And here comes the most deadly fear: * "I'm going crazy. I'm losing touch with reality. This is schizophrenia. This is a tumor. This is the end."*

Stop. You're not going crazy. You are experiencing derealization and depersonalization - one of the most frightening, but also **one of the safest** symptoms of anxiety. And now I will explain why this is so.

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2. Physiological explanation: fuse in the electrical panel

Let's look at two terms:

- **Derealization** - the feeling that **the world around us** is unreal, "like in a dream," "like behind glass."
- **Depersonalization** - the feeling that **you yourself** are unreal, disconnected from your body, thoughts, emotions.

They often come together, but they can also come separately.

What happens in the brain

Imagine an electrical panel in an apartment. When the load on the network exceeds the norm (I connected a washing machine, an iron, a heater and a kettle at the same time), the **automatic** switches

on and cuts off the electricity. Not because the wiring burned out, but to **protect** the wiring from burning out.

Derealization is that same automaton. Here's the exact mechanism:

1. **Chronic stress overloads the amygdala.** Months of anxiety, constant monitoring of symptoms, lack of sleep, panic - the amygdala is working to the limit 24/7. Cortisol and adrenaline are flowing like crazy.
2. **The prefrontal cortex "slows down" the amygdala.** When the stress level goes off scale, the frontal lobes of the brain send the command: * "Stop, we are overheating. Reducing sensitivity"*. Endorphins and GABA are released - the brain's natural painkillers.
3. **The emotional filter is turned on to maximum.** The brain literally **muffles incoming signals**: emotions, tactile sensations, perception of color and depth. This creates that same feeling of "glass" between you and the world.
4. **Vestibular disintegration.** In people with PPPD, derealization often goes hand in hand with unsteadiness - because the same mechanism that suppresses emotions also suppresses sensory integration. The brain stops normally "stitching" signals from the eyes, vestibular apparatus and proprioceptors. Hence the feeling of a "floating floor" and "unreal movement."

> ***Key Fact:*** *Derealization is a* ***defensive reaction****, not a disease. Just like pain in a muscle after overload is a signal "enough, give it a rest," and not "the muscle is dying." The brain does not break down - it is* ***protected from overheating****.*

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3. Cognitive traps: "This means I'm going crazy"

Trap #1: "Derealization = schizophrenia"

This is the most common fear. And it is completely false.

With schizophrenia, a person **does not realize** that his perception is distorted. He believes the hallucinations are real. He cannot distinguish nonsense from reality.

You do the exact opposite: you **clearly understand** that the world around you is unreal, and this scares you. The very fact that you are aware of the distortion and are afraid of it **proves** that you are mentally healthy. A schizophrenic is not afraid of his distorted perception - he considers it the norm.

Trap #2: "If I don't control this, I'll shut down completely."

Derealization is a scary feeling of "losing control." It seems to you that if you let your guard down, you will fall even deeper and completely lose touch with reality.

Truth: You cannot “fall deeper.” Derealization is an automatic, self-limiting process. The brain cannot disconnect itself from reality completely - this contradicts the basic survival instinct. You are safe, even when you think otherwise.

Trap No. 3: “This hasn’t happened before, so I’m degrading”

Derealization does not appear because you have become worse. It appears because anxiety has **finally** reached a threshold at which the brain has turned on its defenses. This is an indicator of long-term stress, not degradation. And this is **reversible** - as soon as the stress level decreases, the “fuse” will turn off on its own.

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4. Point of Support: Insight

> *Derealization is not the beginning of madness. This is the end of your brain's patience. It doesn't break - it saves you from overload, like an automatic fuse cuts off the electricity so that the wiring doesn't catch fire. You're still here. You are still you. The brain just turned down the volume of the world for a while while you were safe.*

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5. How is derealization related to PPPD

For the PPPD specialist, derealization is a double blow:

- You already feel unsteady, and now a feeling of unreality is added to it.
- Unsteadiness in an unreal world is **twice as scary** - because you don’t trust your body or your perception.

But here’s what’s important to understand: **both symptoms come from the same source**—chronic hyperactivation of the nervous system. These are not two diseases. This is the same overheated “software”, which manifests itself in two ways:

Symptom	Mechanism
Shakiness (WSPG)	The brain processes vestibular signals incorrectly due to an anxiety superstructure
Derealization	The brain mutes all incoming information due to stress overload

Conclusion: There is no need to treat derealization separately. It will go away with PPPD when you lower your anxiety level. The same tools—relaxation, exposure, metacognitive therapy—work for both.

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6. Practice: what to do right now

When derealization hits

Step 1: Name it. Say out loud: *"This is derealization. The brain turned on the fuse. I'm safe. This will pass."* Naming an emotion activates the prefrontal cortex and reduces activity in the amygdala (Lieberman study, UCLA, 2007).

Step 2: Grounding through the body. Derealization disconnects you from bodily sensations. Return the connection:

- Squeeze the ice cube in your palm. Cold is a powerful sensory anchor.
- Wash your face with cold water. This activates the "diver's reflex" - the vagal nerve slows down the heart rate.
- Stomp your foot on the floor. Feel the hard surface.
- Smell something with a pungent odor: coffee, ammonia, orange peel.

Step 3: Don't fight. Remember paradoxical intention (Chapter 12)? She works here too. Instead of *"Make it go away!"* say: *"Okay, the world looks unreal. Let it be. I'll still do what I did. Let the world be unreal - I live in it."*

Long-term strategy

Derealization is an **indicator of stress level**, like body temperature during inflammation. To make it go away, you need to work with the reason:

1. **Reduce overall cortisol levels:** regular physical activity (Chapter 13), Jacobson relaxation (Chapter 5), sleep hygiene (Chapter 26).
2. **Stop monitoring derealization.** Every time you check "if the world is real," you add firewood to the amygdala. Apply the metacognitive principle (Chapter 15): the thought "the world is not real" is just a thought. Don't react to it, don't analyze it, don't check it. Let her pass like a train past the station.
3. **Exposure to derealization.** Yes, that's right. Don't run away from her. When she comes - **let her be**. Time it and you will see: the wave passes in 10–30 minutes if you don't add fuel to the fire.

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7. Timeline: When will this pass?

The question that torments everyone: * "How much longer?"*

Honest answer:

- **Acute episodes** (10–60 minutes) pass if you do not support them with monitoring and fear.

- **Background derealization** (a slight constant feeling of “not quite there”) goes away more slowly as the overall level of anxiety decreases. Usually 2–4 months of system work.
- **Full restoration** of perception - when you stop paying attention to it. One day you will simply catch yourself thinking: * “Wait, when was the last time the world seemed unreal?”*. And you won't remember.

> ***Fact:*** *In a study by Hunter et al (2003), 82% of patients with depersonalization associated with anxiety disorder fully recovered after treatment for baseline anxiety. Derealization is a symptom, not a diagnosis.*

##Homework

> ***1. Derealization diary.*** *For 7 days, record the moments of derealization: time, duration, what preceded it (stress, lack of sleep, conflict, scrolling). On the 7th day, you will see a pattern - and you will understand that this is not “random”, but a reaction to specific triggers.*

> ***2. Grounding kit.*** *Collect a small “emergency case” of derealization: ice cube (in the freezer), peppermint essential oil, sour candy, wrist band. When it covers, use any of the anchors.*

> ***3. Paradoxical surrender.*** *The next time the world seems unreal, instead of fighting, say out loud: * “So be it. The world is unreal - but I still go to the store. Or for a walk. Or cook dinner. I don’t need the world to be real in order to live.”* Write down what happened to the sensation after 15 minutes.*

> ***4. Stress Audit*** *Re-read Chapter 17 and answer honestly: What unresolved problems in life are you avoiding? Derealization is a signal “enough to endure.” Perhaps it's time to look at the cause rather than the symptom.*

PTSD and physical trauma: EMDR technique for PPPD

Why the first attack of dizziness or panic turns into post-traumatic syndrome, how the amygdala gets "stuck" in that very day, and how to rewrite the traumatic trace using bilateral eye stimulation.

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1. Essence and issues: "Event No. 0" and somatoform PTSD

Remember the day when it all began.

For some, it was acute vestibular neuronitis: the room suddenly turned around, uncontrollable vomiting began, and an ambulance arrived. Someone had a severe panic attack on the subway or at work: the heart jumped out, air did not enter the lungs, it seemed that a stroke or death would occur right now. Someone has a hypertensive crisis or severe poisoning.

We will call this day **"Event #0"**.

Most people think: * "Well, I got sick and got over it. The ambulance arrived, the doctors checked everything, the organic matter was restored. Why have I been reeling for six months now?"*

The answer lies in psychotraumatology: you have developed **somatoform PTSD (Post-traumatic stress disorder)**.

In classic PTSD (for example, in combat veterans or accident victims), the traumatic factor is an external threat. In PPPD, the traumatic factor was **your own body**. An acute attack of dizziness or panic was so shocking, helpless and frightening that your brain perceived it as an **imminent threat to survival**.

Bottom line: the body has been healthy for a long time, but the nervous system continues to live as if "Event No. 0" is happening right now.

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2. Neurophysiology of trauma: why memory is not "archived"

Normally, any stressful experience goes through the following conveyor belt:

1. **Amygdala (amygdala)** registers danger and turns on the alarm.
2. **Hippocampus** assigns a time stamp and context to the event: *"This happened on May 15, 2023 at 14:00, it's already over, we survived"*.
3. **Medial Prefrontal Cortex (mPFC)** packages experiences and stores them in long-term memory.

During psychotrauma, this conveyor **breaks**:

- **The shock release of adrenaline and cortisol** at the moment of “Event No. 0” completely “freezes” the hippocampus.
- The event **does not receive a timestamp**. It is not sent to the archive.
- The neural network containing the original picture, the emotion of horror, muscle tension and the physiology of panic remains **isolated and hyperactive**.

Now any similar sensation (a slight jump in pressure, a stuffy room, eye fatigue, similar light) works as a **trigger**. The amygdala instantly opens this unprocessed container, and you experience a somatic flashback: a sharp surge of adrenaline, muscle armor, increased dizziness and horror.

This is why logic doesn't work. You can repeat to yourself a thousand times in your head: * “I’m healthy, the MRI is clean”*. But the mPFC does not have access to the isolated amygdala network. You need to use a language that the deep brain understands.

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3. What is EMDR and how it rewires the brain

EMDR (Eye Movement Desensitization and Reprocessing) or **EMDR** (*Eye Movement Desensitization and Reprocessing*) is a method of psychotherapy developed by Francine Shapiro in 1987. It is one of the few methods officially recommended by the **World Health Organization (WHO)** and **FDA** for the treatment of PTSD and trauma.

How does bilateral stimulation (BLS) work?

When you rhythmically move your eyes left and right, **bilateral (two-way) stimulation** of the brain hemispheres occurs. This is analogous to the natural mechanism that occurs every night during the **REM sleep** (rapid eye movement) phase.

During REM sleep, the brain processes emotions during the day. Rhythmic eye movements during waking EMDR trigger the following physiological processes:

1. **Decreased activity of the amygdala.** Bilateral eye movements activate the orientation reflex and simultaneously inhibit the amygdala through activation of the parasympathetic system.
2. **Synchronization of the hemispheres.** Connections between the emotional right hemisphere and the logical left are activated.
3. **Re-preservation of the traumatic network.** The stuck memory is retrieved from the amygdala, combined with the neural networks of the present experience ("*I am safe now*") and rewritten by the hippocampus with the label "*This is in the past*".

After a successful EMDR session, the memory of “Event No. 0” itself is not erased, but the emotional and somatic charge **completely disappears from it**. You remember that there was an attack, but your body

no longer reacts to it with panic and dizziness.

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4. Cognitive Trauma Traps

Trap #1: “I didn’t have a war or an accident, what else is PTSD?”

The brain absolutely doesn’t care what causes mortal horror - from an explosion or from the feeling that the floor is disappearing from under your feet and your heart is stopping. Loss of control over your own body is one of the biggest triggers of trauma.

Trap No. 2: “I’ve already thought about this a thousand times, it didn’t help me”

Normal “chewing” (rumination) occurs in the left hemisphere and only strengthens the adrenaline loop. EMDR does not work through thinking, but through **combining a focus on somatic imprinting with bilateral eye movement**.

Trap #3: “I’m scared to remember that day, in case it gets worse”

In EMDR you work in the so-called **"window of tolerance"**. You stand with one foot in the safe present (watching the movement of your eyes/finger), and with the other foot you only touch the past. This makes the process safe and controlled.

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5. Point of Support: Insight

> *PPPD is not just a vestibular disorder, it is a **stuck startle reflex** from the First Attack. Your body froze on the day you felt sick. EMDR is a surgical tool that removes Event #0 from being a current threat and sends it back to where it belongs—to the archives of the past.*

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6. Practice: Step-by-step EMDR self-help protocol

Note: For severe PTSD, the protocol is conducted with a certified EMDR therapist. However, to independently process the somatic trigger “Event No. 0” in PPPD, an adapted self-help protocol is successfully used.

Preparation: Grounding and “Safe Place”

Before working with trauma, you need to create a resource anchor state in the brain:

1. Close your eyes. Remember a real or imaginary place where you feel completely safe and at peace (seashore, quiet room, forest clearing).

2. Immerse yourself in the details: what do you see, hear, feel on your skin?
3. Place your hands crosswise on your chest (butterfly hug) and make 10-12 slow pats with your palms on your shoulders (left-right-left-right). This is a light bilateral stimulation that “bakes” a state of rest.

Event No. 0 Rework Protocol

Step 1. Define the target

Remember the very day/attack that triggered your dizziness and anxiety.

- **Picture:** Select the brightest, most frightening still frame from that day (for example: *“I’m lying on the sofa, the room is spinning, an ambulance has arrived”* or *“I’m standing in a store, it’s getting dark before my eyes”*).
- **Negative belief (NB):** What thought about yourself arises in this frame? ("*I'm helpless*", "*I'm dying*", "*I have no control over my body*", "*I'm going crazy*").
- **Positive Belief (BP):** What would you like to think about yourself now when you remember this? ("*I'm safe*", "*My body can handle it*", "*I can handle this*").
- **Emotion and somatics:** What emotion arises? (Horror, fear, hopelessness). Where in the body does it respond? (Tightness in the chest, lump in the throat, lump in the stomach, unsteadiness).
- **SUDS Scale (Level of Discomfort):** Rate from 0 to 10 how much of a storm you feel right now when looking at this frame (0 - complete peace, 10 - maximum horror).

Step 2. Starting Bilateral Stimulation (BLS)

Choose one of the methods of bilateral stimulation:

- **Option A (Video/App):** Turn on “EMDR ball tracking” on YouTube or on your phone (a ball moving left and right across the screen).
- **Option B (Manual):** Hold your index finger in front of your face at a distance of 30–40 cm. Move your finger left and right as wide as your eyes, following it only with your eyes (head still).
- **Option B (“Butterfly Hug”):** Hands cross across the chest, rhythmically patting the shoulders alternately with the left and right hand.

Step 3. Desensitization Set (Rework)

1. Hold the selected **freeze frame of “Event #0”** in your mind and the sensation in your body.
2. Start eye movements (or taps) in a rhythm of approximately 1-2 movements per second.
3. Do **25–30 passes** (movements left and right).
4. Stop. Take a deep breath in through your nose and out through your mouth.

5. Ask yourself the question: * “What is coming now? What picture, thought or sensation in the body?”* (Don’t judge, just notice - the picture may fade, change, other episodes may emerge).
6. Do the next set of 25-30 passes.

Repeat sets (usually 8-15 sets required). **Signs of successful recycling:**

- The still image loses its brightness, becomes cloudy, or is deleted.
- Muscle tension in the body relaxes. *SUDS scale is reduced from 8-10 to 0-2.

Step 4: Installing a Positive Belief

When the level of discomfort (SUDS) has dropped to 0–2:

1. Connect the original still image with a **Positive Belief** (“I’m safe, my body can handle it.”*).
2. Do a set of 25–30 passes of bilateral stimulation.
3. On a scale of 1 to 7, rate how true this belief feels in your body (7 = absolutely true). Repeat sets until you reach 6-7.

Step 5: Body Scan

Close your eyes. Mentally run the scanner from head to toe, keeping the original event and new belief in mind. If there is tension left somewhere (in the neck, stomach, chest), do 1-2 more sets of EMDR, focusing specifically on this somatic residue.

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7. Practical plan for working with EMDR for PPPD

1. **Don’t try to process everything at once.** Take the most striking “Event #0” first.
2. **Sessions 2-3 times a week** for 15-20 minutes.
3. If severe anxiety arises during a session, immediately switch to the **Safe Place** technique and 4-7-8 diaphragmatic breathing (Chapter 42).
4. EMDR is ideally combined with **exposure** (Chapter 12): first you process the fear of a past attack through EMDR, then you take a real step into the feared situation.

> ***DZ for this chapter:***
>
> 1. *Write down your **“Event No. 0”** in your notebook: the date, what happened, the most vivid still image and a negative belief (“I’m helpless”*).*
> 2. *Rate your current level of emotional charge on the SUDS scale (0–10).*

> 3. *Try the **Butterfly Hug** technique in the Safe Place exercise to get a feel for how bilateral stimulation works.*
> 4. *Conduct your first EMDR session according to the protocol (8-10 sets). Record the changed SUDS level after the session.*

Symptom Migration: Why Dizziness Shifts to Throat Tightness, Palpitations, or IBS

You finally stopped fearing the rocking sensation, started walking through supermarkets calmly — and out of nowhere, your heart skips a beat, your breath catches, your stomach cramps, or a high-pitched ringing starts in your ears. Do not rush to the ER: your amygdala just changed costumes.

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1. The Recovery Paradox: "My Head Cleared, But Something Else Started"

Picture this: you have put in a solid month of disciplined work. You practiced your vestibular exercises, stopped white-knuckling the shopping cart, and stopped caring whether the floor felt like a boat deck. Your unsteadiness has receded by 70%. For the first time in six months, you exhale and think: *"I'm actually making it out of this."*

And on that exact evening, as you lie peacefully in bed, a sudden thud flips inside your chest — **as if your heart stopped for a full second and then slammed against your ribs**.

Or in the middle of a workday, you suddenly realize you **cannot take a satisfying deep breath** — the air feels blocked mid-chest, and a dense, choking lump locks into your throat.

Or every morning before stepping out the door, your abdomen seizes with **violent intestinal cramping**, bloating, and nausea.

What goes through the mind of an anxious person in that split second?

> *"Okay, the dizziness really was functional neurosis. I get that now. But THIS?! This cannot be psychosomatic! This is a real cardiac arrhythmia / asthma / esophageal tumor! I need an EKG, a Holter monitor, and an endoscopy immediately!"*

And someone who was two steps away from complete freedom sprints right back into the medical labyrinth — only this time trading the neuro-otologist for a cardiologist, pulmonologist, or gastroenterologist.

Welcome to the clinical phenomenon of **Symptom Migration** (*Somatic Shifting* / *The Symptom Imperative*).

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2. The Neurobiology of the Chameleon: The Vagus Nerve and Insular Cortex

Why does the nervous system behave like a street corner shell-game artist? Why does it deal you a brand-new bodily symptom the moment you devalue the old one?

The answer lies in the neuroanatomy of two structures: the **Insular Cortex** (*Insula*) and the **Vagus Nerve** (*Nervus Vagus*).

The Insular Cortex — The Central Hub of Interoception

The insula is the brain's control tower where every internal visceral signal converges: heartbeats, gastric wall tension, bronchial tone, and spatial equilibrium. In a regulated nervous system, the insula filters out 99% of this background biological noise so it never reaches conscious awareness.

In PPPD and chronic hyperarousal, your insula is locked in **central sensitization** — its gain amplifier is turned up to maximum volume.

When you use exposure and CBT to prove to your brain that *"rocking is safe, I no longer care about dizziness,"* your prefrontal cortex inhibits the alarm in the vestibular channel. However, **your amygdala is still hyper-vigilant**! Your baseline threat arousal has not fully cooled down, and its favorite lever for controlling your behavior (dizziness) has stopped working because you no longer fear it.

> ***The Overheated Amygdala's Algorithm:***
>
> 1. *"The host stopped reacting to unsteadiness! They are stepping into the dangerous world unprotected!"*
> 2. *"We need a fresh alarm signal that they haven't habituated to and will definitely fear!"*
> 3. *"Rerouting autonomic discharge down the vagus nerve to a new target organ!"*

The Vagus Nerve — One Master Cable for Every Special Effect

The vagus nerve (*Cranial Nerve X*) is a massive bi-directional highway that exits the brainstem medulla (directly adjacent to the vestibular nuclei!) and innervates:

- The pharynx and larynx (throat);
- The trachea and bronchi (lungs);
- The heart (SA and AV pacemakers);
- The esophagus, stomach, and entire gastrointestinal tract.

Vagal Target Organ	Autonomic Discharge	Subjective Sensation	What You Panic-Google
Throat & Esophagus	Cricopharyngeal muscle spasm	Tight "lump in the throat" (*Globus*), fear of choking	Throat cancer, anaphylaxis
Heart	Premature contraction (PVCs / PACs)	Heart "flip-flop," skipped beat, chest thump	Lethal arrhythmia, heart attack
Respiratory Center	Hyperventilation / sighing dyspnea	Inability to get a "full breath," constant yawning	Pulmonary embolism, asthma, hypoxia
GI Tract	Enteric spasm & hyper-motility	Urgent cramping, IBS, solar plexus knot	Ulcer, pancreatitis, GI malignancy
Auditory / Cervical	Tensor tympani spasm & central gain	High-pitched ringing or pulsing in ears (tinnitus)	Brain aneurysm, impending stroke

Grasp this foundational truth: **these are not five separate medical diseases**. They are the exact same electrical current traveling down the exact same vagus nerve, simply switching tracks from the vestibular nuclei to your heart, throat, or gut.

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3. The Five Favorite Understudy Masks When PPPD Recedes

Let us deconstruct each of these decoys down to the nuts and bolts so your brain loses the element of surprise.

Mask 1: Ectopic Heartbeats & Cardiac Neurosis ("Skipped Beats")

The number-one replacement for dizziness. You feel a sudden pause in your chest, followed by a heavy thud and a warm rush of adrenaline.

- **Physiology:** Driven by catecholamines and shifting vagal tone, the heart muscle fires an extra, early beat — a *benign premature ventricular or atrial contraction (PVC/PAC)*. A brief compensatory pause follows (allowing the ventricles to fill with slightly more blood), making the very next normal beat feel unusually forceful.
- **Clinical Fact:** A completely healthy person records 200 to 500 ectopic beats per day on a Holter monitor without ever noticing them. A sensitized insula magnifies a single benign extra beat into "impending cardiac arrest."

Mask 2: Air Hunger & "Lump in the Throat" (Globus Pharyngeus)

The sensation that the room lacks oxygen, that your inhale "stops short of filling your lungs," accompanied by compulsive sighing, yawning, and an invisible collar tightening around your neck.

- **Physiology:** Chronic background anxiety shifts your breathing to rapid, shallow upper-chest respiration. This blows off too much carbon dioxide (CO_2), inducing *respiratory alkalosis*. Ironically, your blood oxygen saturation is 99% (verify with any pulse oximeter!), yet your brainstem respiratory center misinterprets low CO_2 as suffocation. Meanwhile, the "lump" is simply tonic tension in the upper esophageal sphincter.

Mask 3: Irritable Bowel Syndrome (IBS) & Functional Nausea

Just as you prepare to attend an event without fearing dizziness, your stomach knots up.

- **Physiology:** Your gut houses the *Enteric Nervous System* (over 100 million neurons), wired directly to the brainstem via the vagus nerve. Under threat, sympathetic and parasympathetic signals collide: the stomach delays emptying (causing nausea and fullness) while the colon spasms to "dump ballast before battle."

Mask 4: Somatosensory Tinnitus & Ear Fullness

Your balance stabilizes, but in the quiet bedroom before sleep, you notice a high-frequency ringing or humming in your ears.

- **Physiology:** Chronic clenching of the jaw (masseter/temporalis) and sternocleidomastoid muscles alters tension in the *tensor tympani* muscle of the middle ear. Simultaneously, when your brain stops hyper-monitoring balance, the auditory cortex removes its noise-cancellation filter and amplifies the baseline electrical firing of its own neurons.

Mask 5: Paresthesias, Tingling, Burning Skin & Muscle Twitches

Sudden numbness in your pinky finger, tingling across your cheek, a "tight cap" around your scalp, or benign muscle twitches (fasciculations) in your eyelid or calf.

- **Physiology:** A direct byproduct of mild hyperventilation (which shifts ionized calcium and magnesium levels in the blood) combined with chronic neuromuscular hypertonicity. It has zero connection to multiple sclerosis or stroke.

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4. The Fatal Mistake: Why You Must Not Doctor-Shop the New Symptom

The moment you tell yourself: *"Okay, the dizziness was PPPD, but I should really get my heart/stomach checked just to be safe"* — **you hand the victory back to your neurosis**.

Running to a new specialist during symptom migration causes three destructive outcomes:

1. **It validates the amygdala's bluff:** You send an official confirmation to your survival brain: *"Great job! Dizziness stopped working, but chest tightness hit the bullseye — the host is panicking again!"*
2. **It carves a new neural groove:** Instead of extinguishing health anxiety, you merely trade the "vestibular patient" badge for the "cardiac" or "GI patient" badge.

3. **It collapses your adult agency:** You regress right back into the helpless reassurance-seeking loop.

> ***The Law of Conservation of Neurosis:*** *As long as baseline emotional tension and body-monitoring habits remain intact, somatic energy will flow toward whichever bodily sensation still triggers your fear.*

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5. The "Mask Reset" Protocol: 4 Steps When a New Symptom Appears

When your body throws a brand-new special effect at you just as your dizziness fades, execute this four-step engineering protocol:

Step 1. Instant Fingerprint Verification

Every psychosomatic migrant symptom leaves three unmistakable fingerprints:

- It surfaces **precisely when** your primary symptom (PPPD) begins receding or loses its power to scare you.
- It **fades or vanishes** when you are deeply absorbed in work, anger, conversation, or sleep, and **flares up** the moment you are idle and monitoring yourself.
- It triggers the exact same catastrophic script as your first vertigo attack: *"What if THIS time it's actually fatal?"*

Spot those three fingerprints — stamp it immediately: **"Amygdala Spam."**

Step 2. Verbal Interception with Irony

Refuse to panic or open Google. Address your nervous system out loud or mentally like an experienced veteran:

> *"Oh, hello there! Unsteadiness stopped selling, so we're pitching skipped heartbeats and throat tightness today? Nice try, amygdala, I know this script. Thump, squeeze, and gurgle all you want — I'm not booking a single scan."*

Step 3. Zero Behavioral Concession

The golden rule of neuroplasticity: **never alter your plans for a migrant symptom**.

- Chest feels tight before a walk? Put on your shoes and walk.
- Throat feels constricted before dinner? Eat your meal calmly with sips of water.
- Stomach cramps before meeting friends? Go to the meeting anyway.

When the amygdala sees that the new costume yields **zero behavioral payoff** (you didn't lie down, didn't Google symptoms, didn't cancel plans), the substitute symptom extinguishes 10 times faster than the original dizziness — typically within 3 to 5 days of total indifference.

Step 4. Audit the Unresolved Life Stressor

Symptom migration often indicates that while you desensitized yourself to dizziness (Modules 1–2), you haven't yet resolved the **underlying life friction** keeping your nervous system overloaded (Module 3). Ask yourself: *"Where in my life am I still tolerating the intolerable, suppressing anger, or betraying my boundaries?"*

💬 Core Insight

> *"Symptom migration is not a sign of relapse. On the contrary, it is the death rattle of your neurosis. Your brain realized that dizziness can no longer intimidate you, so it is frantically firing the last blanks in its magazine. Do not fall for a costume change — behind every mask stands the exact same paper tiger."*

📝 Homework Assignment (Action Plan)

1. **The Timeline of Past Masks:** On a sheet of paper, list chronologically every physical symptom that has scared you over the past 5–10 years (tension headaches, tachycardia, air hunger, lump in the throat, neck pain, IBS, dizziness). See with your own eyes how they **took turns on stage** — never harming you, yet always hijacking your attention.
2. **The 14-Day Moratorium Pact:** Write a note in your phone: *"If my dizziness fades and my chest, throat, ear, or stomach starts acting up, I enforce a strict 14-day moratorium on doctors and symptom-Googling while continuing my normal life."*
3. **The CO_2 Reset for Air Hunger:** Next time you feel you "can't take a full breath," resist the urge to gasp for a deeper inhale. Instead, **exhale slowly and completely**, then pause your breath for 7–10 seconds. Let blood CO_2 normalize and feel how quickly the false suffocation reflex releases.

Module 3: Mindset

Neuroplasticity: how we trained our disease and how to erase it

Why your brain is plasticine, how London taxi drivers prove the possibility of recovery, and how long it takes to trodden new paths of peace.

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1. Essence and problems: professional alarmist

When you've been neurotic for years, you feel like your anxiety and dizziness are part of your DNA. You find yourself thinking: * “I’m just that kind of person. Suspicious, anxious, always swinging. My brain is completely broken, and I will never return to the state of a normal person.”*

You react to symptoms automatically. As soon as you open your eyes, you habitually think about the bad. When you leave the house, you habitually shrink. You have brought your panic reactions to absolute automatism. You react to fear faster than you think. And it seems to you that this program is firmly sewn into your head. But the truth is that you just trained her very well.

2. Physiological explanation: plasticine in your skull

Our brain is not a frozen circuit of wires. The brain has a unique property called **neuroplasticity** - the ability to physically change its structure, grow new neurons and destroy old connections depending on your experience.

In 2000, researcher Eleanor Maguire conducted a famous study of the brains of London taxi drivers. To obtain a license, a taxi driver in London must memorize 25,000 streets and thousands of landmarks. An MRI showed that in experienced drivers, the **posterior hippocampus** (the area of the brain responsible for spatial memory) physically increased in size compared to ordinary people. The brain literally grew additional tissue for a regular task.

When you worry 24/7, scan your body, fear unsteadiness and google symptoms, you do the same thing. You're training your brain to worry. You have trampled a giant, wide, eight-lane neural highway of fear in your head. Signals travel through it at the speed of light. You became an “Olympic panic champion.”

But neuroplasticity works both ways. Neural connections live according to the principle: **“Use it or lose it”** (Use it or lose it). If we stop sending signals along the highway of fear and begin to forcibly direct them along a new, narrow path of calm, the old highway, as unnecessary, begins to be dismantled by brain cells. This process takes time (research shows that physical rewiring of neural networks takes **6 to 8 weeks** of daily practice), but biologically it is inevitable. Your brain will rewire itself.

3. Cognitive traps: “I should get well instantly”

The main cognitive trap: expecting a linear and quick result. You do Jacobson's relaxation for three days, go outside, catch the unsteadiness and shout: * “Nothing works! Everything is gone!”*.

You're trying to grow a century-old oak tree in three days. Neuroplasticity is a slow biological construction. You've been training the disturbing highway for years. It's impossible to disassemble it in a weekend. The lack of an immediate effect is not a sign that the method is bad. This is a sign that your brain cells have not yet physically rebuilt themselves. You just have to keep going.

4. Point of Support: Insight

> *There is a path across the lawn in your yard. If you walk on it every day, it will be bare ground. Stop walking on it, and over the summer it will be completely overgrown with grass, and the ground will be trampled in place of the new path. Your illness is just an old trope. Stop walking on it.*

Why chronic pain techniques work for dizziness

This is one of the most unobvious, but powerful insights. Chronic pain and PPPD are **cousins**. Both conditions are functional neuroplastic disorders. The mechanism is the same:

1. **Initial trigger** - actual trauma, infection or stress. The body reacted normally.
2. **Fear and fixation** - the brain remembered the symptom and began to be afraid of it. For chronic pain: *“This back pain is a hernia, I can’t move”*. With PPPD: *"This dizziness is something in the brain, I'm going to fall*".
3. **Neuroplastic consolidation** - fear → hypervigilance (constant scanning of the symptom) → strengthening of neural dominance → the symptom becomes chronic **without an organic cause**.

Pain Reprocessing Therapy (PRT) from Alan Gordon offers a paradoxical approach: **do not run away from the symptom or fight it, but turn to face it and deprive it of its status as a threat**. Translated to PPPD, this technique goes like this:

- “Well, dizziness, come on, spin me even more! Come on now! Pull me to the floor, I'm waiting! Come on, haven't you killed me yet? Why are you so weak?”*

When you genuinely **mocker** a symptom, you deprive the amygdala of its only weapon: your fear. The brain receives a signal: *“If the owner himself asks for increased dizziness, then it is definitely safe”*. The dominant threat begins to fade. This is not bravado - this is a conscious neuroplastic rewiring.

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5. Practice: Map of new paths

Our task is to create clear alternative routes for brain signals to stop using the old fear highway.

1. **Create a table of alternatives:** Divide a piece of paper into two columns.

- In the left column, write your usual automatic reactions (Old Path).
- In the right column, write a new reaction of calm (New Path).

Situation	Old Trail (Autopilot)	New path (Informed choice)
Swayed on the street	Shrink, look at your feet, get scared, go home.	Raise your head (Horizon Rule), relax your shoulders, continue walking.
Panic in the supermarket	Throw the basket, run out into the street, call a taxi.	Stop, fix your gaze on the distant sign, turn on the timer for 5 minutes.
Morning Clip	Lie down and listen to your head, curse fate.	Get up immediately, drink warm water, and take a contrast shower.

2. **Keep Focus for 60 Days:** Hang this chart in a visible place. Your only goal for the next two months is to catch yourself at the moment of choice and consciously direct the signal along the right column. Even if at the beginning it seems artificial and difficult, continue. We build physical neural bridges in your head.

Homework (DZ):

1. **Write your personal path map** (minimum 5 situations).
2. Every time you manage to choose a "new path" instead of automatic fear, put a plus in your notebook. Your goal is to collect 50 pluses in a week.

PRT: Pain Reprocessing Therapy

PRT is a method developed by Alan Gordon for chronic pain, but it works ideally for chronic dizziness as well. The point: teach the brain to perceive a symptom not as a threat, but as **safe neural noise**.

Step-by-step PRT protocol for PPPD:

Step 1. Somatic Tracking When you feel unsteady, instead of being scared, start **observing the sensation** with curiosity:

- *"I wonder where exactly I feel this?"*

- *"Is it pulsating or smooth?"*
- *"Is it warm or cold?"*
- *"If you give it a form, what is it? Round? Spicy?"*

This switches the brain out of "danger!" mode. to "research" mode. The amygdala calms down when you observe instead of react.

Step 2: Reassess security After tracking, tell yourself (mentally or out loud):

- *"This sensation is neural noise. The brain sends a false signal."*
- *"My body is physically healthy. This has been confirmed by examinations."*
- *"I'm safe. This feeling cannot harm me."*

Step 3: Positive Affect Find something that evokes a slight pleasant feeling - look at a beautiful tree, listen to your favorite song, remember a funny moment. The goal is to give the brain a signal: *"I am observing the symptom AND I AM GOOD at the same time"*. This breaks the symptom = suffering link.

Regularity: Practice PRT with every episode of unsteadiness. After 2-4 weeks, the brain will begin to automatically reduce the intensity of the signal, because it will understand: this signal does not cause a fear reaction → it is useless → it can not be sent.

How to speed up neuroplasticity: BDNF boost

BDNF (Brain-Derived Neurotrophic Factor) is a protein that works as a "fertilizer" for neurons. The more BDNF, the faster new neural connections are formed and the faster the brain is rebuilt.

Here's what boosts BDNF:

- **Aerobic exercise** (running, swimming, fast walking) is the most powerful boost. 30 minutes a day increases BDNF by 200-300%.
- **Sleep** (7–8 hours). BDNF is synthesized primarily during sleep.
- **Meditation** (10+ minutes per day).
- **Omega-3** (fatty fish, flaxseed oil, walnuts).
- **Curcumin** (turmeric) - increases BDNF and reduces neuroinflammation.
- **Sunlight** (30 minutes per day) - via vitamin D.

What reduces BDNF:

- Chronic stress (vicious circle: stress → low BDNF → slow adaptation → stress).
- Sugar and ultra-processed foods.
- Sedentary lifestyle. *Lack of sleep.

Metacognitive therapy: how to stop arguing with your own radio

Why classical CBT does not work for severe anxiety, what is the insidiousness of "mental chewing gum" and how to introduce a timer for anxiety.

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1. The essence and problems: endless debates in your head

When you feel unsteady, your head turns into a courtroom. One voice shouts: * "This is the end! We are staggering, it's a stroke!". *The second voice tries to convince him: "No, I had an MRI, the vessels are clean, it's just neurosis". The first one objects: * "What if the MRI was of poor quality? What if the doctor missed a mini-stroke?"*. The second one answers: * "But the doctor is an authoritative specialist..."*.

You spend hours on this internal debate. You are trying to prove to your brain that you are healthy, arguing with it, analyzing every thought. By the evening, these mental debates give you a headache, you have no strength left at all, and your unsteadiness only intensifies. You are confident that if you "think about the problem" carefully, you will find a solution. But you are simply stuck in the trap of **rumination** - an endless mental chewing gum that grinds your resources to waste.

2. Physiological explanation: MCT revolution and the law of "off attention"

In traditional cognitive behavioral therapy (CBT), it is common to work with the **content** of thoughts: challenge them, look for evidence for and against. But when the amygdala is overheated, this method fails. Your brain is in survival mode, the logic of the cortex is turned off. Any attempt to argue with a thought is perceived by him as a continuation of the struggle. You enter into a dialogue with a thought ➔ the thought becomes important ➔ the brain releases even more adrenaline.

Metacognitive therapy (MCT), created by British professor Adrian Wells, offers a revolutionary approach. MCT states: **the problem is not WHAT you think, but HOW you react to the very fact of the appearance of a thought**.

Thoughts are born in the brain automatically, as a product of the vital activity of neurons. Up to 70,000 thoughts run through your head every day, most of which are complete nonsense. Normally, the brain rejects them. But an anxious person clings to a frightening thought and begins to "think" it - analyze, check, ruminate.

Rumination is a conscious, controlled process. The brain does not distinguish between "thinking about danger" and "being in danger." While you are spinning scenarios of your disability in your head, your

adrenal glands continue to pump stress hormones. If you stop paying attention to a thought, it will physiologically dissolve within a couple of minutes. Thoughts are clouds. They float across the sky. Your job is not to try to blow them away or argue with them, but to simply let them float by.

3. Cognitive traps: the illusion of the benefits of rumination

Main cognitive trap: * "If I stop thinking about my illness, I will miss something important and will not be able to recover. Rumination helps me analyze the situation."*

This is a lie from your frightened mind. Rumination is the illusion of useful work. It's like pedaling an exercise bike: a ton of effort is expended, but there is no forward movement. Analyzing symptoms never makes your head clearer, it only drives you deeper into the adrenaline loop. Thinking does not cure functional failure. Treats - switching attention and physiology.

When to argue and when to remain silent: CBT vs MCT

Practical question: if CBT teaches you to sort out thoughts, and MCT teaches you not to touch them, then which should you use when?

The answer depends on the level of anxiety:

- **Anxiety 3-5 out of 10 (background):** The cerebral cortex is still working. Classic CBT works well here: analysis of thoughts, SERM diary, behavioral experiments. You can logically refute a catastrophic thought, and the brain will hear it.
- **Alarm 7+ out of 10 (acute panic):** Logic disabled. The cortex is suppressed by adrenaline. At this point, any attempt to "argue with thought" is fuel for the fire. Here, switch to MCT: don't touch the thought, just note it (*"I have a thought that I'm dying. There's noise on the line. I'm not listening."*) and shift the focus to the breath or body.

Remember: **CBT is for cold analysis. MKT is for the hot moment.** These are not competitors, but partners. Use the right tool at the right time.

4. Point of Support: Insight

> *Imagine that you are sitting in a cafe, and at the next table two strangers are loudly discussing you: "He is sick, he will never get better, he will fall." You won't run to them to argue, prove the opposite and show them your MRIs? You'll just think, "This is bullshit," ignore them, and continue drinking your coffee. Your anxious thoughts are those very strangers. Don't interfere in their conversation. Let them chat in the background.*

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5. Practice: Detached Observation and Worry Timer

We will learn to let go of thoughts and control the process of rumination using two powerful techniques.

1. Detached Mindfulness Technique

When the thought arises in your head: *"I'm shaking, I'm going to fall"*:

- **Don't argue:** Don't try to calm yourself down with the phrases *"No, I won't fall"*.
- **Identify:** Tell yourself: *"I had the thought that I would fall. It's just noise on the radio. I hear him, but I don't listen closely."*
- **Let it be:** Imagine this thought as a cloud passing by, or as a train passing by. Let it be in your head, but don't touch it with your hands. Turn your attention to what you are doing physically at this second (steps, breathing, cup in hand).

2. Worry Timer Technique

Does your brain need to worry? Great, we will give him this right, but on our terms.

- **Set aside time:** Set a strictly fixed time for alarm. For example, **from 18:00 to 18:15** daily.
- **Postpone thoughts:** If during the day (for example, at 12:00) the thought *"What if it's multiple sclerosis?"* appears in your head, tell it: *"Good thought. I will think about it in detail at 18:00. And now I have things to do."* And switch your attention.
- **Anxiety Session:** At 6:00 pm, sit in a chair, set your alarm for 15 minutes and start panicking as hard as you can. Think the worst thoughts, ruminate, be afraid. As soon as the alarm rings, the session is closed. You will be surprised, but by 18:00 most of the day's "horror stories" will have lost their relevance, and it will be difficult for you to bring yourself to panic about the order.

3. Rumination audit - measure your trash

Most neurotics have no idea how much time they spend ruminating. It seems to them - "well, about 10 minutes a day." Actually, it's a clock. This tool will show you the real picture.

Week 1 (measured): Every night before going to bed, open your notes on your phone and write down:

- How many times have you caught yourself ruminating today? (approximately)
- What thought was on your mind most often?
- How many minutes (approximately) did you spend in "mental chewing gum" throughout the day?

Don't try to change anything this week. Just observe and record. You'll be shocked by the numbers.

Week 2 (intervention): Now that you know your "baseline" level of rumination, start using the Worry Timer and Detached Observation. Write down the same three points every night. By the end of the second week, you will see a 30-50% reduction.

Why metering works: When you record rumination, you automatically move to the meta level - observing the thinking process instead of drowning in it. The very fact of writing "caught yourself chewing gum" is already a metacognitive act that breaks the cycle. In addition, you get an objective metric of progress. Not "it seems to me that it has become easier," but "there were 12 episodes a day, now there are 4."

Homework (DZ):

1. **Implement a Worry Timer** this week. Rigidly put off all anxious rumination until the appointed time.
2. Every time you catch yourself arguing internally with a thought, take a deep breath and practice "Detached Observation." Write down your feelings in a notebook: did you manage to leave the thought unanswered?
3. Practice the "Leaves on the River" visualization (below) once a day for a week.

Visualization of "Leaves on the River" (from ACT)

ACT (Acceptance and Commitment Therapy) offers a powerful technique for working with intrusive thoughts - the Leaves on the River visualization:

1. Close your eyes. Imagine that you are sitting on the bank of a calm river. Leaves are floating along the river.
2. Every time a thought appears in your head, **put it on a sheet** and send it down the river. Don't argue with her. Don't agree. Just put it down and watch it float away.
3. If the thought returns, put it on a new sheet. No irritation. *"Oh, you're back. Here's another sheet for you. Swim*".
4. If you "fell into" a thought (started to argue with it or got carried away by the content) - this is normal. Just notice that you have "fallen into the river", climb back to the shore and continue to observe.

Duration: 5–10 minutes.

Why it works: The technique trains **defusion** - the ability to separate yourself from your thoughts. You are not your thoughts. You are the one watching over them. The thought "I am dying" is not a fact. This is a leaf floating on a river. You can look at it, or you can just let it go.

After 2-3 weeks of daily practice, you will notice that obsessive thoughts come less often and their "volume" decreases. Not because you defeated them, but because you stopped feeding them attention.

Excellent Student Syndrome: How Hypercontrol and Cognitive Distortions Keep You Neurotic

Why perfectionism leads to autonomic failure, an analysis of the five main thinking traps and why the world copes just fine without your indispensability.

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1. Essence and issues: portrait of an excellent student on the verge of a breakdown

95% of people suffering from PPPD have the same psychological profile. These are perfectionists. "Excellent students." People with a heightened sense of duty and hyper-responsibility. These are those who “bear” everything: difficult work, problems of relatives, everyday life, finances. These are people who don’t know how to say “no” and are terrified of letting others down or appearing weak.

Your motto in life: *“If not me, then who?”* or *“I must do everything perfectly”*. When your neurosis began, you transferred this habit to your illness. Now you are trying to perfectly control everything in your body: breathing, neck position, placing your foot on the asphalt, the reaction of your pupils to light. You keep your central nervous system in the grip of total control 24/7. But it is precisely this attempt to control everything that burns the fuses of your brain, causing autonomic failure.

2. Physiological explanation: the cost of hypercontrol of the central nervous system

The human nervous system is not designed for continuous manual control of automatic processes.

In our body, the autonomic nervous system operates in automatic mode (autopilot). You don't need to think about how to contract your stomach after eating, how to expand your bronchi when running, or how to adjust the tone of hundreds of small muscles in your feet and lower legs when you step. This is done by an ancient, autonomous part of the brain.

When you engage hypercontrol, you are trying to control the autopilot manually. The cerebral cortex (consciousness) begins to send hundreds of extra impulses to the motor areas. This creates a constant, colossal overload of the central nervous system. Imagine a computer whose processor is 100% loaded with background processes. Sooner or later the system will start to freeze.

Your brain just overheats from stress. Chronic tone of the cortex keeps the autonomic system in a state of constant combat readiness (sympathetic mode). The blood vessels are narrowed, the muscles are tense,

and proprioception is distorted. Your unsteadiness is a physical cry from your body: *"Turn off manual control, my processor is overheated!"*.

3. Cognitive traps: the five horsemen of the anxiety apocalypse

Your brain is used to processing reality through distorting filters. Let's look at the five main cognitive traps of a neurotic:

1. **Catastrophization:** *"If I get rocked now, I'll fall, break my head, go crazy"*.

- *Truth:* During all the months or years of illness, you never fainted or went crazy. The Amygdala predicts a catastrophe that is biologically impossible (remember the physiology of pressure).

2. **Mind reading:** *"Everyone sees how I'm swaying and thinks I'm drunk or sick"*.

- *True:* People around me don't care at all. 99% of passersby are buried in their phones or chewing on thoughts about their own loans, problems and relationships. Your walk is invisible to them.

3. **Must:** *"I should feel 10/10 every day. Any discomfort is unacceptable."*

- *True:* No living organism functions perfectly. Fatigue, poor sleep, weather - all this affects the tone. 6/10 is an excellent, working result for a person.

4. **Black and white thinking:** *"If I have even a little bit of unsteadiness left, it means I'm completely sick and nothing is working."*

- *Truth:* Recovery is a spectrum. If before you were pumping 24/7, but now only in the evening when you are tired, this is a huge progress of 70%, and not a defeat.

5. **Depreciation:** *"Yesterday I walked normally, but today I'm shaking again. My efforts are useless."*

- *Truth:* The brain is rebuilt nonlinearly. Temporary decline is a normal physiological process of adaptation of the nervous system.

Three horsemen you haven't noticed yet

In addition to the five classic traps above, the neurotic has three more distortions that work more subtly and unnoticed - like a rootkit in an operating system. You don't even suspect that they are active.

6. **Emotional reasoning:** *"I'm scared → that means the danger is real"*.

- This is the most insidious trap. You take your gut feeling as objective evidence. You are afraid to go to the subway - and you decide that the subway is really dangerous. You feel sick in the shopping center - and you are sure that the shopping center is harmful to your health. But emotion is not a fact. A person with arachnophobia is afraid to look at a cross spider, although this spider is absolutely harmless. Fear lies. He's not an informant, he's an alarmist.

- *Antidote:* Every time you catch yourself saying “I’m scared, that means it’s dangerous,” stop and ask: *“What objective facts confirm the danger? Not feelings, but facts.”* In 99% of cases there will be no facts.

7. **Personalization:** *“The colleague frowned → it’s because of me. He noticed that I was shaking and now he thinks I’m an alcoholic/psycho.”*

- A neurotic person attaches to himself everything that happens around him. Did the salesperson at the checkout wince? It's because I saw your unsteady gait. Friend didn't reply to your message? He's probably tired of hearing about your dizziness. In fact, the salesman was thinking that his shift was ending, and his friend simply fell asleep with the phone in his hands.
- *Antidote:* Write down 5 alternative reasons for the other person’s behavior that are not related to you. You'll be surprised how few things in this world revolve around you.

8. **Tunnel vision (Selective abstraction):** *“Today I spent 7 hours at work normally, but for 20 minutes after lunch I was shaking - that means it was a terrible day.”*.

- The brain of a neurotic person works like a bad spam filter: it allows only “bad” messages into the consciousness and discards “good” ones. You can have a great day - walking, working, socializing - but you will only remember those 15 minutes when you felt symptoms. The entire day is coded in memory as “bad.” This is tunnel vision: you look at reality through a narrow gap into which only negative aspects fall.
- *Antidote:* Start a “Diary of Victories”. Every evening, write down **3 moments from the day when you felt normal or good**. This is not positive thinking, this is calibration of perception. You are literally training your brain to notice the good - because there is objectively more good in your day than bad, you just filter it.

Frequently asked questions about cognitive biases

“I know this is a distortion, but I’m still afraid. Knowledge doesn’t help!” It's okay. Knowledge is the cerebral cortex. Fear is the amygdala. They operate at different speeds. The amygdala reacts in 12 milliseconds, the cortex “thinks” in 300-500 milliseconds. Fear will always come first. But knowledge gives you a choice: continue to react to fear or switch. Every time you notice a distortion and don’t follow its lead, you create a new neural pathway. In 3-6 months this path will become a highway, and the old path of fear will be overgrown.

“If I stop controlling my body, will something serious happen and I won’t notice?” Your body is not a monitoring station that needs to be checked every 5 minutes. You have an automatic warning system - pain. If something really serious happens (heart attack, stroke), you will feel it clearly and clearly: sharp pain in the chest, numbness in half the body, loss of speech. These symptoms cannot be missed. But cloudy “swaying”, “cotton wool in the head” and “something is wrong” are just a signature of anxiety, not an organic disease.

“It seems to me that I have all 8 distortions at once. Is this normal?" Absolutely. Most neurotics have 4-6 distortions active at the same time, and they are intertwined. Catastrophizing triggers emotional reasoning, which is reinforced by tunnel vision. Don't try to work with everyone at once. Choose the 2 most common ones (usually catastrophizing and mind reading) and start tracking them in your SERM diary. The rest will catch up.

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4. Point of Support: Insight

> *Remember the last time you had the flu with a high fever. You lay there, didn’t control anything, didn’t solve work problems and didn’t save the world. So what happened? The world didn't collapse. Colleagues coped, the family survived, the sun continued to rise. Your indispensability is an illusion of your ego. Your main task now is not to be a perfect rescuer. Your task is to be alive and give your body a rest. Tell your inner overachiever: “Okay.”*

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5. Practice: SERM Diary and legal imperfection

We will use two tools to treat overachiever syndrome.

1. Keeping a diary SERM (Situation - Thought - Emotion - Reaction)

Every time your anxiety spikes or your unsteadiness increases, record it in the table:

- **S (Situation):** Just the facts. *“Standing at the checkout counter at the supermarket”* (no drama).
- **M (Thought):** Your literal interpretation. *“Now I’ll fall, everyone will see my weakness”*.
- **E (Emotion):** Rate the strength of the emotion (Fear 80%, Shame 70%).
- **R (Reaction):** What the body and you did. "*Clenched his teeth, grabbed the cart, and quickly left*".

After filling it out, look at the thought from column “M”: ask yourself what kind of cognitive distortion (Catastrophizing? Mind reading?) was at work here. Rewrite the thought into a realistic one: * “I’m shaking because of a spasm in my neck. It's unpleasant, but safe. Nobody looks at me."* The diary template is available in the application 3.3_smer_diary.pdf.

2. Practice “Legal Imperfection”

Once a day, do one action that is intentionally imperfect to prove to your brain that the world won’t collapse because of it:

- Leave the unwashed plate in the sink overnight.
- Reply to a colleague's email 2 hours late without apologizing.

- Make a report or task with a C grade (70% of the ideal) and send it.
- Delegate to your loved ones what you usually do yourself (buying groceries, cleaning).

3. Third-person habit decompilation (Distancing)

In cognitive behavioral therapy, this method is called **cognitive defusion**. When you say to yourself: * "I feel bad, I'm stormy, I'm going crazy" * - you completely merge with your condition. You become this state.

To break the automatic alarm script, you need to leave the role of a participant in the process and take the position of an external debugger (debugger), which simply reads the log file of the running program.

How to do it: Every time you catch yourself experiencing symptoms or behavior that is disturbing, begin to mentally (or out loud) describe what is happening **in the third person**, using your name or referring to yourself as the "object."

- **It was (merger):** *"I'm walking through the supermarket, the floor is floating, I'm dizzy, I'm afraid of falling, I'm clinging to the cart"*.
- **Became (distancing):** *"[Name] entered the zone of colorful visual stimuli. The brain recorded a divergence of signals from the neck and eyes. The standard fear routine is running. [Name] tensed his trapezius, clenched his jaw and automatically activated the auxiliary support (grabbed the cart). The system alarm level is 7 out of 10. The object continues to move."*

Why it works: When you refer to yourself as "he" or "she," you activate the lateral areas of the prefrontal cortex and physically "turn off" the emotional overheating of the amygdala. You are no longer a victim, drowning in symptoms. You are a programmer monitoring the execution of crooked system code. The code works clumsily, but the hardware is fine.

Homework (DZ):

1. Complete **at least 3 situations in the SMER Diary** this week.
2. Complete **2 actions as part of the "Legal Imperfection" practice** and write down your emotions.
3. Every time you catch yourself trying to grab a wall, check your pupils, or panic from unsteadiness, **describe this situation in the third person** (make 3 entries in your phone notes in a week). Did you feel like an outside observer?

Where we took a wrong turn: three areas of life and the hidden benefits of your neurosis

A symptom like a red light on a dashboard, a ruthless audit of three areas of life and why the brain chooses illness instead of making decisions.

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1. Essence and issues: the cry of your body

When a person has PPPD, they often perceive it as a sudden catastrophe: *"I was living a normal life, working, making plans, and suddenly — bam! — out of nowhere, intense dizziness hit me. This is just an unfair accident."*

But a neurosis never strikes in a vacuum. It doesn't just fall from the sky. Unsteadiness and panic attacks are the **cry of your body**, which for a long time endured what was intolerable. For years you may have worked against your own interests, sidelined your authentic needs, stayed in toxic relationships, pushed through exhaustion at an unfulfilling job, or refused to allow yourself true rest without guilt. Your autonomic nervous system sent persistent warning signals — fatigue, muscle tightness, mild anxiety — but you brushed them aside and kept running. Eventually, the brain depleted its compensatory reserves and pulled the emergency brake through constant dizziness.

2. Physiological Explanation: Neurosis as an Autonomic Circuit Breaker

The autonomic nervous system cannot operate under chronic overload indefinitely.

When there is a persistent, unresolved background stressor in your life (for instance, an unfulfilling relationship you fear ending, or a career draining you dry that you feel trapped in), your subconscious scans this unsolvable deadlock 24/7. With no resolution in sight, emotional and physiological tension builds up within the limbic system.

When this tension exceeds a critical threshold, the brain triggers a protective failure mode: **functional autonomic deregulation**. It functions exactly like a circuit breaker in an electrical panel. If you plug in a space heater, a washing machine, and an iron into a single overloaded outlet, the circuit overheats and trips the breaker, shutting down power to protect the house from catching fire. PPPD and panic attacks are that tripped circuit breaker. Your central nervous system deliberately slows you down — through unsteadiness and sensory overload — so you physically cannot continue destroying yourself at the same unsustainable pace.

Here lies the clinical phenomenon of **secondary gain**. This is an unconscious coping mechanism: you are not faking illness intentionally. Rather, the brain realizes that somatic symptoms are the only legitimate

way to obtain desperately needed rest or avoid an unbearable reality. For example:

- Illness grants permission to step down from the exhausting role of being "invincible and self-sufficient" without feeling lazy.
- Symptoms postpone frightening life decisions (such as changing careers or ending an unhealthy marriage).
- Physical vulnerability compels loved ones to finally provide care and attention that was previously missing.

The Brain as a Processor: Unresolved Problem = Frozen System Thread

Your brain processes challenges systematically: identify problem → resolve → reset baseline → move forward. But when an **unresolvable conflict** persists — years in a toxic environment, chronic financial dread, or suppressing core values — that task becomes a runaway background thread. It consumes 100% of your cognitive and autonomic bandwidth, running a continuous alarm loop in the background.

And the most insidious part is self-deception. You tell yourself: *"Everything is fine at home,"* while subconsciously dreading returning there. You convince yourself: *"My current paycheck is fine,"* while feeling deep existential burnout. You put on a **social mask** — the selfless parent, the indefatigable executive, the agreeable colleague — and fear taking it off because your sense of identity has become attached to it.

Three core domains where unresolved conflict almost always hides:

1. **Relationships:** With yourself, your partner, parents, children, or superiors. Chronic suppression of boundaries in any core relationship is a constant autonomic trigger.
2. **Career & Financial Fulfillment:** A persistent gap between your values, efforts, and outcomes that you endure without resolving.
3. **Existential Meaning & Self-Realization:** Suppressing your true capabilities and settling into a diminished role out of fear.

3. The Victim Mindset: The Trap of Passive Helplessness

Alongside secondary gain, another psychological trap cements chronic symptoms: **learned helplessness and the victim mindset**.

When chronic symptoms persist, people frequently surrender control to external circumstances:

- *"I would practice somatic tracking, but today the weather is bad / magnetic storms are high / my dizziness is too severe."*
- *"Doctors in my city don't understand PPPD, so recovery is impossible for me."*
- *"Nobody supports me; I am completely alone in this nightmare."*

Adopting the victim role is protective for a frightened brain because **it eliminates personal accountability**. If recovery depends entirely on weather, perfect doctors, or external miracles, you don't have to confront scary exposures, tolerate discomfort, or do the daily work of nervous system regulation.

However, the objective neurological reality is clear: **passive waiting never produces neuroplastic recovery**. Healing begins the precise moment you assume 100% agency over your rehabilitation. Weather does not shake your legs; hyper-sensitized vestibular processing in your brain does. And only systematic retraining — through daily graded exposure, autonomic down-regulation, and CBT protocols — rewires those circuits. No physician can execute vestibular habituation or Jacobson relaxation on your behalf. You must take the pilot's controls of your own nervous system.

> *For an in-depth neurobiological analysis of learned helplessness, dorsal vagal states, and a structured recovery protocol, see **Exiting the Victim State**.*

4. The Cognitive Trap: "First I must heal 100%, then I will fix my life"

A pervasive mental barrier is the belief: *"Now is not the time to address relationship stress or career burnout. I must first become completely symptom-free, and only then will I fix my life."*

This is backwards. PPPD developed as a somatic manifestation of systemic chronic overload. If you temporarily quiet symptoms with medication but return to the exact same toxic patterns, boundary violations, and emotional suppression, the nervous system will inevitably trip the breaker again. Sustained recovery requires changing the conditions that caused your central nervous system to crash in the first place.

5. Point of Support: Insight

> *A car that is driven at full speed without gas and oil will sooner or later seize the engine. Your neurosis is not a breakdown of parts. It's a red light on the dashboard that screams, "Out of gas!" Stop, you're going the wrong way!" Stop being angry at the light bulb. Look under the hood of your life.*

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6. Practice: Ruthless life audit

We will conduct an audit of three key areas of your life to localize the source of tension. Open the file 3.4_life_spheres_audit.pdf and honestly answer the questions in writing.

Step 1. Three Spheres Test

Rate your satisfaction from 1 to 10:

1. **Relationships:** How safe and warm are you with your loved ones? Are there those whom you tolerate out of a sense of duty?
2. **Implementation:** Are you doing what you love? Is the job financially rewarding?
3. **Body and rest:** Do you know how to rest without feeling guilty? Do you get enough sleep?

The source of tension is where the score is below 5 points.

Step 2. Secondary benefit questions

Answer yourself in writing and mercilessly:

1. What difficult decision or action does my illness "save" me from?
2. Whose attention or care do I receive now that I feel bad?
3. What will change in my life tomorrow if I wake up 100% healthy? What unpleasant thing will I have to do?

Step 3. Victim Position Test

Ask yourself two sobering questions:

1. In what situations over the past week have I blamed external circumstances (weather, doctors, loved ones) for my poor health, instead of asking myself: * "What have I personally done or not done for my condition?"*?
2. What rehabilitation actions am I putting off, hiding behind the phrase *"I'm too bad for this"*?

Step 4. Wheel of Balance - a visual map of your life

The Wheel of Balance is a powerful tool for coaching and psychotherapy that allows you to see imbalances in life clearly, at a glance. Draw a circle and divide it into 8 sectors:

1. **Health and body** - physical well-being, sleep, nutrition, activity
2. **Relationships with a partner** - intimacy, trust, support
3. **Family and children** - quality of communication, participation, emotional connection
4. **Job/career** – satisfaction, growth, meaning
5. **Finance** - sufficiency, control, perspective
6. **Friends and society** - is there a circle of support, are you isolated?
7. **Hobbies and pleasure** - what makes you alive, not a "function"
8. **Personal development/meaning** – studies, goals, sense of movement

Rate each sector from 1 to 10 (where 1 is “complete failure”, 10 is “completely satisfied”). Paint each sector to the appropriate level.

How to interpret:

- If the wheel turns out to be even (all sectors are approximately the same) - you have a uniform load, even if the levels are low. It's better than skewed.
- If the wheel is “crooked” (one sector is 2, the other is 9) - here it is, your dominant. The brain continuously processes this imbalance, and it is it that consumes resources that should be used for recovery.
- **Sectors with a score of 3 and below are red zones.** This is where the unsolved problem is buried that keeps your neurosis afloat.

Rule of action: Choose one red sector and write down **three specific microsteps** that you can take this week. Not “change your life” - but three small actions. Sign up for a consultation with a career coach. Write a resume. Talk to your partner about something you have been silent for a long time. A small step destroys the dominant, because the brain sees: “The task is no longer stuck - it is being processed.”

Repeat the Wheel of Balance once a month and compare it with the previous one. You will see how the red zones gradually tighten up - and how the symptoms go away at the same time.

Homework (DZ):

1. **Complete a complete written life audit** using the template in the materials.
2. **Draw the Wheel of Balance** (on paper or in the application) and fill in all 8 sectors. Identify red zones.
3. Write **one specific micro-step** to solve a problem in a sagging area of life in a healthy way. *(For example: if the problem is burnout at work, start devoting 2 hours on the weekend to searching for new vacancies, rather than ruminating about precariousness).*
4. Reread your answers to Step 3 and honestly admit in which areas you have taken a passive victim position. Write how you will regain control over these processes.

Ego: dismantling the false personality brick by brick

You think that you are your name, profession, status and set of habits. What if it’s not you?

Advanced level

If you have reached this chapter, you have already completed the base. You know what PPPD is. You are working with the body. You've broken the adrenaline loop. You're doing CBT.

Now we go deeper.

There is a moment “here and now”. The past no longer exists - it has passed. The future does not exist yet - it has not arrived. There is only this moment. And in this moment I will ask you several questions that can knock down the door in your mind.

Who does your cognitive dissonance come to?

To whom does anxiety come?

Who gets dizziness?

Who are you anyway?

I am not “me”

Not me as a name. Not me as a programmer, manager, mother, accountant. Not me as a “good person” or a “bad person.” Not me as a “family man” or a “loner.”

All these labels are not you. It's **ego**.

The ego is a construct. A set of bricks that other people gave you. My parents gave me some building blocks: “you must be obedient,” “boys don’t cry,” “good girls don’t get angry.” Some were given in kindergarten, school, army, at work. Some - the street, the Internet, advertising.

You didn't choose these bricks. They were **invested** for you.

Coca-Cola advertising

Maybe a dumb example, but it explains it well.

Do you remember the Coca-Cola commercials? New Year, red trucks, Santa Claus, “The holiday is coming to us.” Shown for years, since childhood. And now you have a can of Coca-Cola on your New Year’s table.

Congratulations. It's not yours. This was forced on you.

You didn't want a Coke. You wanted tea with lemon. Or he didn’t want anything at all. But smart guys in advertising agencies have been putting the association in your head for decades: “New Year = Coca-Cola = happiness.”

Now scale that to life.

Imposed paradigms of happiness

Apartment, car, family, career. Helicopter, yacht, penthouse - for those who have “achieved more.” This is the paradigm of life that was invented for us. As per Maslow’s table - satisfy your needs, climb the pyramid, and you will be happy.

Think about it. Is this really true? Is a car really a measure of happiness?

If you talk to people who have a car, an apartment, and a family, many of them are more unhappy than you. Because they were chasing someone else's template, achieved it - and discovered emptiness. And emptiness is anxiety. And anxiety is hello, PPPD.

What does dizziness have to do with it?

And here's what it has to do with it.

When you don’t live your own life - when you follow other people’s attitudes for years, suppress your desires, tolerate what you don’t need to tolerate - your body begins to scream. Because the body doesn't know how to lie.

You can deceive your head: “Everything is fine, I like my job, my marriage, my city.” But the body hears something else. And it sends a signal - first quiet (fatigue, tension), then louder (headaches, cramps), and then screams at everyone (panic, dizziness, PPPD).

Neurosis is the body’s cry when you betray yourself.

Knock out other people's bricks

What to do? CBT, meditation, spiritual practices - all these are tools for the same thing: **knocking other people's bricks out of your ego**.

Here is the attitude: “I must earn a lot, otherwise I am a failure.” Whose is it? Yours? Or a dad who grew up poor? Or a society that measures people by salary?

Here's another: "You can't refuse people, it's selfishness." Whose? Yours? Or a mother who sacrificed herself all her life?

Each brick that you realize is foreign - and you consciously remove - makes you **more whole**. A false belief is replaced by an adequate one. Yours. Not imposed.

"I earn as much as I need for a comfortable life" - instead of a race.

"I have the right to say no" - instead of eternal sacrifice.

"I don't need to be perfect to be worthy of love" - instead of the excellent student syndrome.

How a mask creates a spasm

This is not an abstraction - here is a specific chain that I have seen dozens of times:

You work in an office. The boss is a toxic tyrant who humiliates you in front of your colleagues. Everything is boiling inside of you. The jaw clenches. Fists clench. You want to stand up and say: *"Don't you dare talk to me in that tone"*. But the mask of a "good employee" does not allow it. You smile, nod and swallow silently.

Where does this anger go? It doesn't evaporate. It goes into the **trapezius, neck and diaphragm**. The muscles contract, but do not receive a release. After a month of this regime, you have "inexplicable" unsteadiness and a "cotton" head. And six months later - full-fledged PPPD.

Here are other examples of false bricks that I have seen in people with PPPD:

- **"I must be an ideal mother"** - woman, 38 years old. She devoted 100% of her time to the children, leaving not a single hour for herself. The feeling of guilt for going to a cafe without children was stronger than the fear of dizziness. When she first said in therapy, "I have the right to 2 hours a week for myself," her unsteadiness decreased by 30% in a month. Not from pills. From allowing yourself to be imperfect.
- **"A real man doesn't complain"** - man, 42 years old. I suffered from back pain, insomnia and chronic fatigue for 3 years. He told his wife "everything is fine." When the doctor told him, "You have PPPD, you need psychotherapy," he replied, "I'm not crazy." Left. He returned after 8 months when he stopped leaving the house. The brick "men don't go to psychologists" almost cost him his career.
- **"I have to save everyone"** - woman, 51 years old. Nurse. I took night shifts for colleagues. Helped neighbors, relatives, and stray cats. My health came last. PPPD began after she cared for her sick mother-in-law for 3 months without days off. When she was asked in therapy: * "Who is caring for you?" * she began to cry. For the first time in 20 years.

The world is objectively sunny, clear and red

When you start knocking out false bricks, a strange thing happens. The world is becoming... simpler. Easier. Brighter.

Not because the world has changed. But because you took off a ton of other people's junk and finally see it for what it is. No alarm filters. Without installations through which everything seemed gray.

The world is objectively sunny. Clear. Beautiful.

You just have to take off someone else's glasses.

Practice: Inspection of bricks

Step 1. Settings table

Take a piece of paper. Divide it into three columns:

Installation	Whose is it?	My version
"I must be strong"	Dad. He grew up without a father and decided that boys don't cry.	"I have the right to be weak. Weakness is not a shame, it is a signal to rest."
"You're nothing without money"	Society. Advertising. Instagram.	"My worth is not measured by my salary. I am valuable in myself."
"You can't let people down"	Mother. She sacrificed herself all her life.	"I let myself down every time I say yes against my will."

Step 2. Friend/foe test

For each setting in the table, ask yourself three questions:

1. **When did I first hear this?** If the answer is "in childhood" or "from my parents," there is a 90% chance that it is not yours.
2. **Is this attitude helping me right now?** If it causes anxiety, guilt or shame, it is harmful.
3. **If I grew up on a desert island, would I come to this belief on my own?** If not, it's an imposed code.

Step 3. Strikethrough Ritual

Installations that did not pass the test are crossed out in red. Write your version next to it. Adequate. Healthy. Read it out loud three times. Hang it on the refrigerator.

> ***DZ for Chapter 18:***
>
> *1. Fill out the settings table - **minimum 10 bricks**.*
>
> *2. For each brick, conduct a "friend/foe" test. Be brutally honest.*

> 3. *Cross out the bricks that are not yours. Write **your** version next to it.*
> 4. *Choose one setting that causes the strongest resistance. It is she who is your main brake. Start with her.*

> *[!TIP] The other side of your ideal Ego (Persona) is everything that you forbade yourself to show and pushed into the system basement. We will talk about how this repressed part breaks the vegetative system and how to return it in the chapter **Accepting the Shadow: Integration of the Repressed**.*

Inner child: who is really scared in your body

Why your unsteadiness is the tears of the frightened child inside you, how to stop being your own taskmaster and give yourself security.

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1. The essence and problems: a child locked in a basement

If you suffer from PPPD and neurosis, you are probably used to communicating with yourself in the language of orders and strict demands. Your internal dialogue is an endless spanking: * “Get yourself together, you rag! Why are you unstuck again? You have to go, you have to work, you have to be strong!”*.

You treat your body and your psyche like an evil slave owner treats a tortured horse: you beat it with a whip of demands, forcing it to run further, although it is already falling from fatigue. And when she starts stumbling (indicating dizziness and unsteadiness), you get angry at her for being “ineffective.”

But ask yourself a question: who exactly is afraid inside you during a panic attack? Who cowers in fear when the store floor floats? This is not an adult, smart, 30- or 40-year-old person with a higher education and work experience. This is your **Inner Child**—the scared, vulnerable, emotional part of you. And while you stand over him with a whip and demand to “be a man / a strong woman,” his panic will only grow.

2. Physiological explanation: the neurobiology of self-compassion and stress response

Let's look at this from a neuroscience perspective.

When you criticize yourself, demand the impossible, and scold yourself for your symptoms, your brain perceives your own inner voice as an **external threat**.

The limbic system (amygdala) reacts to self-criticism in the same way as to the cry of a real enemy: it triggers a stress response, releases cortisol and adrenaline, tightening the neck muscles. Your inner taskmaster scares your amygdala 24/7.

But our brain also has another system - the Caregiving system.

When we experience genuine self-compassion, when we hug ourselves or speak calming words to ourselves, the hypothalamus releases **oxytocin** and **endorphins**. Oxytocin is a natural cortisol antagonist: it instantly reduces the activity of the amygdala, dilates blood vessels, relaxes muscles and returns the autonomic system to a state of rest. Your scared “inner child” (your limbic system) doesn’t need orders. He needs the security that only you can give him.

3. Cognitive traps: “If I relax, I will become a doormat”

The main cognitive trap of an excellent student: * “If I start feeling sorry for myself and indulging my “inner child,” I will completely fall apart, lie down on the sofa and turn into a vegetable. My toughness with myself is what keeps me going.”*

This is a fatal mistake. Rigidity led you to neurosis and PPPD. You are already at the bottom precisely because of her. Self-compassion is not self-pity, which is paralyzing. Self-compassion is recognizing the fact: * “I’m having a hard time right now. I'm scared. And that's okay. I will support myself."* A child who knows that his parent will protect him calmly goes out to explore the world. A child who is beaten at home for any mistake sits huddled under the bed. By giving yourself security, you will receive the resource to act, and not eternal paralyzing anxiety.

4. Point of Support: Insight

> *Your unsteadiness is not a gyroscope failure. This is the scared child inside you who cries and tugs at your sleeve, trying to say: “I’m scared, I’m tired, stop!” And you respond by hitting him in the face and shouting: “Shut up and move on!” Stop being a tyrant to yourself. Become a Reliable Adult for him.*

---

5. Practice: Calming the inner child

Every time you are overwhelmed by unsteadiness or panic, instead of the usual anger (“Yes, how much you can spin!”*), use the support protocol:

Step 1: Role switching

Mentally step out of the role of an “evil controller” and take the position of a “Reliable Adult.” Imagine that a small child (you are 5-7 years old) is standing in front of you, shaking with fear.

Step 2. Talk to the body (Focus)

Address the frightened part within yourself with the following words (you can silently):

> *“I see how scared you are. I'm here with you. You are not alone. I won't scold you for what makes you sick. It's just adrenaline, it's about to burn out. I will protect you. We are safe.*"

Step 3. Physical anchor of oxytocin

- Place a warm hand on your chest near your heart or hug your shoulders.

- Take a slow, deep breath and a very long, relaxed exhale. Feel the warmth of your hand. This physical touch stimulates the release of oxytocin, calming the overheated amygdala at hormonal levels.

“Safe Place” technique (visualization)

This technique is used in trauma therapy to create an internal resource. It is especially useful when anxiety is running high and no logical arguments are working - because visualization addresses the limbic system directly, bypassing the rational brain.

How to do it:

1. Lie down or sit comfortably. Close your eyes. Take 5 deep breaths.
2. Imagine a place where you feel **absolutely safe**. This can be a real place (grandmother's dacha, the seashore from a vacation, your children's room) or a fictional one.
3. Start “drawing” this place in detail: *What do you **see**? (colors, light, objects) *What do you **hear**? (silence, sound of waves, birdsong)

- What do you **feel** on your skin? (warmth of the sun, coolness of the wind, softness of the blanket)
- What is the **smell**? (freshly cut grass, sea air, grandma's pies)

4. When the picture becomes bright and voluminous, invite your inner child there. Take his hand. Say: *“It is safe here. No fear can get here.”*
5. Stay in this place for 5-10 minutes.

Why: When you regularly “visit” a safe place in your imagination, your brain forms a neural connection between this image and a state of rest. After 2-3 weeks of practice, you will be able to “turn on” the safe place instantly - like a hot key - even while standing in line at the supermarket.

Self-Compassion Protocol by Kristin Neff

Kristin Neff is a psychologist and creator of the scientifically based Mindful Self-Compassion program. Her method includes 3 components:

1. Awareness (instead of merging)

When you feel bad, don’t run away from the feeling and don’t drown in it. Just say: * “I'm in pain right now. I notice my pain.”* This simple confession is already therapy. It stops rumination and puts you in the position of an observer.

2. Shared humanity (instead of isolation)

Tell yourself: *"Suffering is part of the human experience. Millions of people right now feel the same way I do."* This destroys the illusion "I'm the only one who is so broken", which feeds neurosis.

3. Self-kindness (instead of self-criticism)

Place your hand on your chest (this activates the production of oxytocin) and say: * "I want to be kind to myself. I deserve care and sympathy."* If you can't tell it to yourself, tell it to your childhood photo. You definitely won't refuse sympathy for this child.

Time: 3-5 minutes. Do it every time you catch yourself thinking * "I'm a failure" *, * "Why can't I just get up and live normally" * or * "Others are coping, but I'm not" *.

Homework (DZ):

1. **Find your childhood photo** (up to 5 years old). Look at her. Remember this baby. Tell him out loud: * "You are safe. I'm here. I'm protecting you."* Yes, you will be embarrassed. Do it anyway.
2. Write a dialogue between **you the adult** and **you the child** in a notebook by hand.
3. Practice the Safe Place meditation every evening for 7 days.

Repressed emotions: anger that you forbade yourself to feel

How unspoken anger turns into muscle spasm, why "good people" get sick more often and what to do with an emotion that you have buried alive.

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1. Essence and issues: "I'm not angry. I'm just dizzy."

Ask yourself: when was the last time you were truly angry?

I wasn't annoyed. Not "was a little unhappy." Namely - **angry**. Screamed. Slammed the door. He said "no" so hard that the walls shook.

If you can't remember, welcome to the club. You are the type of person I see in 80% of PPPD cases: **chronic emotional suppressor**.

Here is your portrait. Do you recognize?

*You are a "good person." You don't conflict. You endure.

- If someone steps on your foot, you apologize.
- The boss dumps someone else's work on you - you take it in silence.
- Your partner says something offensive - you swallow and remain silent. *"Well, don't quarrel over nonsense."*
- Mom calls and dumps her problems on you for an hour - you listen, although everything is boiling inside.
- You say "everything is fine" so often that you already believe it.

But your body didn't believe it. Because the body doesn't know how to lie.

2. Physiological explanation: emotion is energy, and it doesn't go anywhere

An emotion is not an abstract "feeling." It is a **specific physiological response** with specific biochemistry and specific muscle script.

When you're angry, your body gets ready for **action**:

- Adrenaline and norepinephrine are released into the blood.

- The muscles of the shoulder girdle, jaw and fists **tense** (preparing for a blow).
- Vocal cords tighten (preparing for screaming).
- Blood pressure rises, pulse quickens.
- The diaphragm contracts (gasping in air for a powerful exhalation/scream).

This is an ancient mechanism, honed by millions of years of evolution. Anger = energy to protect one's boundaries. This is not a "bad" emotion. It's **fuel**.

But what happens when you **suppress** this reaction?

Adrenaline was released, but there was no action. The muscles tensed - but there was no blow. The ligaments were stretched - but there was no scream. All this energy remains **inside**, like steam in a closed boiler.

And the body does the only thing it can: **freezes the energy in the muscles**. This is the mechanism for the formation of the muscle shell, which we discussed in Chapter 4. Only now you understand **where** this shell comes from. Not from "bad posture" or from a "sedentary lifestyle". It comes from **decades of unspoken anger** locked in your muscles.

Map of suppressed anger in the body

Where do you feel	What is suppressed	Typical phrase
Jaw (bruxism, clenched teeth)	Unspoken words, suppressed cry	*"I wanted to say, but kept silent"*
Neck and traps (stone shoulders)	Suppressed aggression, "carrying oneself"	"*I must endure*"
Diaphragm (pressed breathing)	Suppressed crying or sobbing	*"Men don't cry" / "You can't be weak"*
Fists (you clench your hands automatically)	Suppressed impulse to strike	*"I'll kill him" → "No, you can't, I'm good"*
Stomach (lump, nausea, IBS)	Repressed fear or disgust	*"This makes me sick" (literally)*
Throat (lump in throat, soreness)	Suppressed words that are stuck	*"I want to say it, but I can't"*

If you see your symptoms in this table, congratulations. You've just found the root source of 50% of your muscle armor. And he is not in "cervical osteochondrosis." He is in unresolved emotions.

3. Five masks of suppressed anger

Anger is a tricky emotion. When you stop yourself from being angry, it doesn't go away. It **masks** itself as other conditions:

Mask 1: “I'm not angry. I'm just tired"

Chronic fatigue is one of the main symptoms of suppressed anger. Suppressing emotions requires enormous energy expenditure. Imagine that you continuously keep the lid on a boiling pan. Your hands get tired, your body gets tired, but the pan still gurgles. You're not tired of work. You're tired of **holding yourself back**.

Mask 2: “I'm not angry. I'm worried"

Anxiety and anger are two ends of the same stick. When anger is directed outward, it is aggression. When it is directed **inward** (because it cannot be released) - it is anxiety. Many PPPD sufferers who are “just worried” are actually **furiously angry**—at their partner, at their parents, at their job, at themselves. But admitting anger is scarier than admitting anxiety. An anxious person is a “poor, sick little man.” And evil is “an aggressor, a bad person.” And you unconsciously choose anxiety.

Mask 3: “I'm not angry. I'm offended"

Resentment is frozen anger. You're angry, but you don't allow yourself to act. Instead, you “puff up”, become isolated and wait for the offender to figure it out and apologize. He has no idea. You get “offended” even more. The neck turns to stone. My head is spinning.

Mask 4: “I'm not angry. I don't care"

Emotional dullness, apathy, “I don’t want anything” is often a sign that the brain has **frozen** all emotions at once, because you have forbidden one (anger). Unfortunately, it is impossible to suppress one emotion and leave the rest. When you suppress anger, you suppress joy, tenderness, and interest in life. All or nothing.

Mask 5: “I'm not angry. My body hurts"

Psychosomatics. Headache, back pain, irritable bowel syndrome, skin rashes. The body screams what the mouth is silent. This is not "fiction". This is a proven mechanism: suppressed emotion → chronic muscle tension → pain → you go to the doctor → the doctor finds nothing → you worry even more.

4. Cognitive traps: “Good people don’t get angry”

Trap #1: “Being angry is bad”

Where is this from? From childhood. *"Don't shout!"*, *"Behave yourself!"*, *"Good girls don't get angry"*, *"A man should control himself"*. Since childhood, you have been taught that anger = danger, conflict, destruction. And you decided: I will be good. I won't be angry. Never.

Truth: Anger is a **normal basic emotion**, just like joy, fear, or surprise. He signals: * “My boundaries have been violated. I need to protect myself."* Suppressing anger is like putting duct tape over a red light on your dashboard. The light does not come on - but the engine is still overheating.

Trap #2: “If I get angry, I’ll lose control.”

Fear: "*If I open this door, an avalanche will pour out and I will hurt people*".

Truth: You are confusing **anger** and **aggression**. Anger is an emotion (internal experience). Aggression is behavior (external action). There is a choice between them. You can be angry without hitting, screaming or destroying. You can get angry and **talk** about it: *“I get angry when you do X. It makes me feel bad. I ask you not to do that.”* This is not aggression. This is **assertiveness** - a healthy, adult expression of your boundaries.

Trap #3: “If I express anger, I will be rejected.”

Fear: * “If I tell my mom that I don’t like it when she calls every 2 hours, she will be offended and stop communicating with me.”*

Truth: People who reject you for honestly expressing your feelings are people who do not want you, but your **function** (comfortable, flexible, reliable). Healthy relationships can handle anger. Moreover, they **become stronger** after an honest conflict because both sides finally understand each other's real boundaries.

5. Point of Support: Insight

> *You didn't get PPPD because you were “weak” or “nervous.” You got sick because you were **strong** for too long. You held the lid on the boiling pot of your emotions with such force that your fuses blew. Neurosis is not weakness. This is the **price of suppressed power**. It's time to open the lid - carefully, in a controlled, adult way.*

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6. Practice: Safely releasing suppressed emotions

Technique 1: Anger Letter (written expression)

This is a technique from Pennebaker's Expressive Writing (University of Texas). Research shows that 20 minutes of honest writing about suppressed emotions reduces cortisol levels, improves immune function and **reduces doctor visits** by 43%.

How:

1. Take a piece of paper (not a phone - with your hand). Set a timer for 20 minutes.
2. Write down **everything** you feel about the person or situation that pisses you off. No censorship. Without “but he’s a good person.” Without “okay, I’m exaggerating.” Checkmate is welcome. Rigidity is welcome. Nobody will read this.

3. Write something you **never said** to this person. Something that's stuck in your throat. The thing that makes you sick.
4. When the timer rings, re-read the letter once. Realize: these are **your real feelings** that you have been hiding.
5. **Destroy the letter.** Tear it up. Burn it. Delete. This is not for storage. This is unloading poison from the system.

Frequency: 1 time per week, for 4 weeks. You will notice that by the 3rd week the “boiling pot” in your stomach will become quieter.

Technique 2: Body Reset (safe physical expression)

Anger is energy for muscles. If you don't give it a way out through movement, it will remain in the muscles in the form of a spasm.

Safe Reset Options:

- **Pillow.** Lock yourself in the room. Take a pillow. Hit her with your fists. Shout at it. Does it sound idiotic? Yes. Works? Yes. Because you finally allow the body to perform the action that it wanted to do and could not.
- **Towel.** Twist the towel into a rope and twist it as hard as you can. 30 seconds. This is a powerful proprioceptive release for the arms and shoulders.
- **Stomping.** Stand up and stomp your feet hard on the floor. 30 seconds. This grounds and discharges the lower body.
- **Sound.** In the car (windows closed) or in the shower - **growls**. A low, guttural growl from the stomach. Not a squeal, not a scream - just a roar. This activates the vagus nerve and relieves diaphragm spasm.

Important: After any bodily reset - 2 minutes of deep breathing (inhale 4, exhale 8) and relaxation according to Jacobson. Reset → relaxation → integration. Without relaxation after the reset, you will simply overstimulate your nervous system.

Technique 3: “Anger Chair” (adapted from Gestalt technique)

This is a simplified version of Fritz Perls's "Empty Chair" technique:

1. Place an empty chair in front of you.
2. Imagine that the person you are angry with (partner, mother, father, boss) is sitting on it.
3. **Tell him out loud** everything you want to say. Don't worry about politeness. Speak as you would if there were no consequences.
4. When you have spoken, sit on that chair. Be “that person” for 30 seconds. Try to understand how he feels.

5. Return to your chair. Say the final phrase: * “I'm angry with you. And I have the right to this. But I won’t let this anger destroy my body.”*

Sound like therapy? Because this is therapy - only you do it yourself, without a psychologist. This technique does not replace working with a therapist, but it does **break the silence** and give voice to what has been locked away.

Technique 4: Red Light Diary

Create a note in your phone that says “Red light.” Every time during the day you catch yourself receiving one of the following signals:

*Clenched teeth *Clenched fists

- Felt a lump in my throat
- Felt heat in my chest
- Said “everything is fine” when it wasn’t normal
- write down:

Time	Situation	What I suppressed	What did he say	What I wanted to say
14:30	Colleague dumped his job	Anger	“Okay, I’ll do it”	“No, this is not my task”

In a week you will have a **suppression card**. You will see a pattern: in what situations, with what people and how often do you bury your emotions. And you will understand why in the evening your neck is like stone and your head is spinning.

Homework (DZ):

1. **Write an “Anger Letter”** to the person who causes you the most suppressed anger. 20 minutes. No censorship. Destroy after reading.
2. **Keep a “Red Light” diary** and record moments of suppression for 7 days. On the 7th day, re-read it - you will see a pattern.
3. Try **one body reset** from the list above. Write down what you felt before, during and after. Pay attention to your neck and shoulders - have they become softer?
4. Answer yourself honestly: **who** are you most afraid to tell the truth? Write down the name. This is your main “customer” of the muscle shell.

Next level: uninstalling root programs

Suppressed emotions do not come from a vacuum. They have accumulated over the years due to two deep psychological patterns:

1. **Learned helplessness:** the habit of feeling like a passive hostage of symptoms and looking for a rescuer. Read: **Exiting the Victim Position**.
2. **The "good person" syndrome:** fear of admitting one's aggression, strength and selfishness. Read: **Accepting the Shadow: Integrating the Repressed**.

Exiting the Victim Position: The Neurobiology of Learned Helplessness and Taking Back Control

Why the brain chooses to get sick, how the Karpman triangle supports body spasm, neurobiology of the dorsal vagus according to Porges, the hidden benefits of suffering and 5 steps to transition to the position of the Author of your life.

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1. Essence and issues: anatomy of voluntary paralysis

Let's take off the white gloves and talk frankly, without a sympathetic pat on the head.

What does your self-talk sound like on a typical day with PPPD or chronic anxiety?

- *"It hit me again... Why do I need all this?"*
- *"The weather is oppressive today, the atmospheric pressure has dropped, which is why I'm shaking so much."*
- *"Doctors in our city are mediocrities and idiots, they cannot make a normal diagnosis"*.
- *"My loved ones don't understand me, they think I'm being a fool. I am completely alone in this nightmare."*
- *"If only the damned dizziness would go away, I would move mountains. But while it's with me, I'm paralyzed."*

Do you hear that common denominator? In all these phrases, you as an active subject **do not exist at all**.

There are certain external forces: the villainous weather, stupid doctors, magnetic storms, ununderstanding relatives and the insidious PPPD syndrome, which attacks you from around the corner, like a robber in a dark alley. And you, in this structure, are an innocent, suffering martyr tied to the rails, towards whom a train is rushing at full speed.

In psychology and psychiatry this is called the **Victim Mindset**.

> ***The Victim Position*** *is an unconscious refusal to control one's own biological system and handing over the steering wheel to external circumstances for the sake of obtaining an alibi: "I can't do anything about it, which means I'm not responsible for anything".*

Let's use our fingers like an engineer to an engineer. Imagine that a critical service has crashed on a complex server. The system administrator sits down in the middle of the server room, clasps his knees

with his hands, begins to sob and lament: * “Damn bytes! Why are you doing this to me?! This is all because the air conditioner is noisy and the users are angry!”*. Sounds crazy? Absurd?

But this is exactly what you do with your central nervous system every single day. You gave the console with root rights to the frightened autopilot and wonder why the system went into an endless kernel panic.

As long as you remain in the position of the Victim, **you will never be cured**. Not a single pill, not a single vestibular gymnastics, not a single antidepressant can fix a person who sincerely believes that someone else controls his body.

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2. Neurobiology of learned helplessness: the biological cost of surrender

The victim attitude is not just “bad character” or “bad mood.” This is a specific, severe functional state of the neural networks of your brain.

```
[Chronic stress / unexplained somatic symptom]
                        |
                        ▼
   [Illusion of zero control: "There is nothing I can do"]
                        |
                        ▼
  [Dorsal Raphe Nucleus (DRN) activation + Dopamine block in Striatum]
                        |
                        ▼
     [Dorsal Vagal Shutdown (Porges Polyvagal Theory)]
     [Biological Freeze / Play Dead response]
                        |
                        ▼
     [Reduced cortical motor tone ➔ Distorted cervical proprioception]
     [Sensory gating failure: brain stops filtering vestibular noise]
                        |
                        ▼
       ═══════════════════════════════════════════════
       RESULT: Chronic unsteadiness, brain fog,
             heavy/jelly legs, complete loss of energy
       ═══════════════════════════════════════════════
```

Let's look at the hardware part of this disaster.

1. The Martin Seligman phenomenon: the neurochemistry of giving up the fight

In 1967, American psychologist Martin Seligman discovered the phenomenon of **Learned Helplessness**.

The dogs were placed in enclosures where the floor was periodically shocked. The first group could press their nose on the panel and turn off the current (they had *control*). The second group received exactly the same shocks, but pressing the panel did not lead to anything - the current was turned off regardless of their actions (there was no control).

Both groups were then moved to an open enclosure with a low barrier that could be easily jumped over at the first blow.

- The dogs from the first group instantly jumped over the barrier and were saved.
- The dogs from the second group **didn't even try to escape**. They simply lay down on the floor vibrating under the current, whining and passively accepting the pain. Their brains have learned: * "My actions have no effect on anything. Resistance is useless."*

Modern neuroscience has proven that when the brain decides that a situation is out of control, the Dorsal Raphe Nucleus (DRN) flares up.

DRN releases excess serotonin into the amygdala, tightly blocking the basal ganglia (*Striatum*) and dopamine reward centers. The brain physically loses the ability to initiate motor action. It's not laziness. This is a chemical ban on salvation.

When you say, "I've tried everything, nothing works for me, I'm hopeless," your dorsal raphe nucleus blocks any attempt by the prefrontal cortex to retrain the vestibular system. You voluntarily lie down on the electric floor.

2. Stephen Porges' polyvagal theory: Dorsal vagal collapse

Professor Stephen Porges (*Stephen Porges*) described three evolutionary levels of response of the autonomic nervous system:

1. **Ventral Vagal:** social involvement, calm, safety. The brain operates normally, sensory noise filters are turned on to maximum.
2. **Sympathetic system:** "Fight or Flight" response. Adrenaline, tachycardia, readiness to fight.
3. **Dorsal Vagal Shutdown:** The oldest reptilian "Freeze/Play Dead" response.

When an animal realizes that it is impossible to escape from a predator (the sympathetic system has failed), the brain turns on an emergency protocol: a sharp release of pressure, a slowdown in metabolism, dissociation, numbness of the limbs, fog in the eyes. The body fakes death to make the predator lose interest or to avoid the pain of tearing the flesh.

PPV and chronic laxity are pure dorsal vagal collapse.

Your position as a Victim transmits a signal to your brain: * “We are in a trap, there is no way out, we are powerless!”*. The body receives a command to freeze:

- The leg muscles become “wobbly”, the knees buckle.
- The visual analyzer turns off the periphery (tunnel vision, derealization).
- The vestibular nuclei cease to receive a clear proprioceptive response from the feet and neck, because muscle tone chaotically jumps between spasm and lethargy.
- The ground disappears from under your feet.

You are treating the vestibular apparatus, but what you need to treat is learned biological freezing.

3. External locus of control and prefrontal cortex atrophy

American psychologist Julian Rotter introduced the concept of **locus of control**:

- **Internal (internal):** *“What happens to me is the result of my decisions, actions and reactions”*.
- **External (external):** *“I’m a chip in the ocean, everything depends on the boss, doctors, genes and weather”*.

Functional MRI studies show that in people with an external locus of control, the dorsolateral prefrontal cortex (dlPFC) is functionally disconnected from the amygdala.

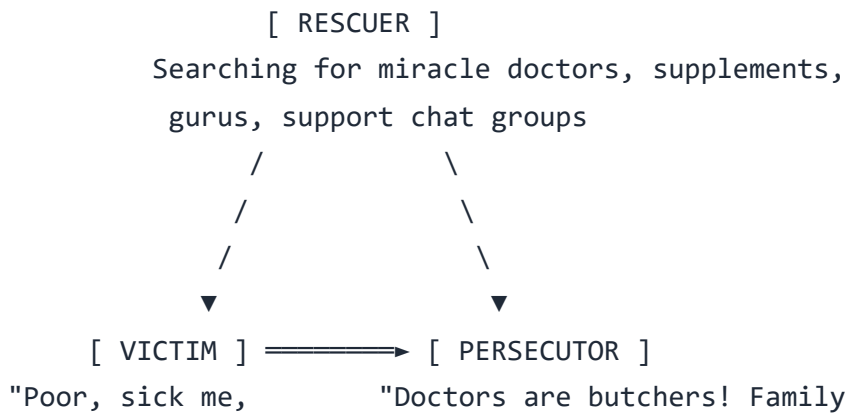
The prefrontal cortex (your inner top manager) literally “sleeps” and does not send inhibitory fear signals. As a result, any fluctuation in the body (slight vascular spasm, turning the head, tinnitus) is interpreted by the amygdala as a global catastrophe.

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3. Karpman Triangle: the neurochemical drug of suffering

You've probably heard about Stephen Karpman's dramatic triangle: **Victim - Persecutor - Rescuer**.

But few people understand that a neurotic person spins around this triangle with dizziness for a reason, but for the sake of **a powerful neurochemical dose**.



```
staggering again,      members are cold & selfish!
 my life is ruined"     I hate my broken body!"
```

Let's see how you navigate this carousel:

Phase 1: Victim

In the morning you wake up, feel a heaviness in the back of your head and immediately fall into the drama: * “Again... Well, as much as possible... I’m disabled, I can’t do anything”*.

At this moment, the adrenal glands produce an even background of cortisol. Cortisol constricts blood vessels, increases neck spasm, and unsteadiness naturally increases. You lie down on the sofa and pick up your phone.

Phase 2. Rescuer (Search for the magic pill)

The victim cannot save himself - by definition. She needs an external Messiah.

You open a forum on PPPD, a Telegram chat with sufferers, or go on YouTube:

- You are looking for a new “luminary otoneurologist” who will “definitely figure it out.”
- You read a diagram with a rare nootropic, microdosing or osteopathic reduction of the atlas for $500.
- You write in the chat: * “Guys, today I’m shaking like in a nine-point storm, who experienced this?”*.

At the moment when they answer you: *“Hold on, honey, I feel the same way too, hugs!” *, your brain receives a microdose of **oxytocin and endogenous opioids**. There comes a fleeting, surrogate relief. But it lasts exactly 15 minutes.

Phase 3. Persecutor (Aggressor)

The new doctor didn't help. The pill caused nausea. Atlas is in place, but the earth continues to float. Your loved ones are tired of listening to your complaints and dryly said: * “Stop whining already, go for a walk”*.

And then the meek Victim instantly puts on wolf fangs and turns into a furious **Persecutor**:

- *“Doctors are creatures and charlatans, they just take money!”*
- *“The husband/wife is a heartless egoist, no one loves me, you want me dead!”*
- *“I hate this stupid job, this country, this weather and my own body!”*.

Rage gives a powerful adrenaline rush. For 20 minutes, the vessels dilate, energy appears, but then the resource is depleted, the prefrontal overheats, and you fall back to the bottom with a roar - into the state of a helpless, crushed Victim.

> *This cycle repeats itself for years. You're hooked on this swing. Your brain has become an **adrenaline-cortisol junkie** who uses the bodily symptom as a bargaining chip to manipulate the*

> *world.*

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4. Shadow accounting: secondary benefits of your illness

Now comes the most unpleasant section of this chapter. If after reading it you do not have a burning desire to close this page and tell the author to go to hell, then you are not reading carefully.

Ask yourself a question that requires extreme, surgical honesty:

> ***What terrible, overwhelming or frightening thing will happen in your life if tomorrow morning you wake up ABSOLUTELY healthy?***

Most people foam at the mouth and shout: “Nothing! I'll be happy! I’ll go for a walk, I’ll work, I’ll enjoy life!”*.

Lie. Your brain is an absolute pragmatist. It never supports an energy-consuming pathological process just like that. Any chronic neurosis has **secondary gains** (*Secondary Gains*) - system bonuses that you receive in exchange for instability.

Let's open your ledger:

1. Reinforced concrete alibi from adult life

While you are sick, you are officially freed from the need to make difficult, life-changing decisions:

- You don’t need to quit your boring, unpromising job: * “Where will I go with my dizziness? Be grateful that I’m at least hanging in here.”*
- You don't need to solve a crisis in your marriage: file for divorce or openly sort things out. After all, you can’t abandon a sick person, and it’s scary to go into the unknown yourself. The disease keeps the family frozen in the status quo.
- You don’t need to separate yourself from authoritarian parents: * “Mom calls me every day to measure my blood pressure, I’ll be lost without her.”*

Illness is your legal, socially recognized sick leave, which exempts you from the matriculation exam.

2. Monopoly of care and forced love

Deep down in your soul, you are deeply convinced (hello from childhood): ** “On my own, healthy and ordinary, no one needs me. Love must be earned."**

As a child, your mother came to your bed, stroked your head, gave you tea with raspberries and spoke kind words only when your temperature rose to 38.5. Your brain flashed: **suffering = love and safety**.

As an adult, you continue to demand care through weakness. When you are unsteady, people start running around you: your husband prepares dinner, your wife takes care of the children, your colleagues do not burden you with complex tasks. You get warmth, but you pay a terrible price for it - your own capacity.

3. The right to be weak without guilt

You are a perfectionist with hypercontrol. Your inner Censor forbids you to simply lie down on the sofa and say: “I’m tired. I don't want to drag this project on anymore. I want to be useless."*

The censor will immediately begin to whip you: *“Get up, you lazy beast, you need to plow, you need to conform!”*.

And then your autonomic system finds a brilliant compromise: it turns off the vestibular analyzer. Now you are lying on the sofa absolutely legally: * “I would be glad to plow, but the ground is disappearing from under my feet! You see, this is not laziness, this is a terrible organic disease! Your body breaks itself to save you from your own inner taskmaster.

---

5. Cognitive traps of the Victim

To cement you into helplessness, the brain uses three classic cognitive errors:

Trap 1. “Recovery first, action later”

“When my neck stops hurting and my vision stops swimming, then I’ll start doing exposure, go to the gym, go to the supermarket and change jobs.”

This is a fatal fallacy of causation.

Neuroplasticity is exactly the opposite: **the brain is retrained only IN THE PROCESS of action**. While you sit at home and wait for calm, your brain strengthens the neural networks of fear and agoraphobia. Health is not a precondition for life. Health is a by-product of proper movement and courage.

Trap 2. The illusion of unique defectiveness

- “Everyone else just has VAD (autonomic dysfunction) and neurosis, but I have a special, atypical, most severe case in world medicine. Nothing in this book will help me.”*

The pride of the Victim is paradoxical. The victim loves to feel special. Being an ordinary person with a standard vegetative spasm is boring and offensive. Being the owner of a “unique incurable syndrome” is tragic, romantic and sublime. This removes any responsibility for the result.

Trap 3: Magical delegation

"I paid the psychotherapist/neurologist/author of the book money - let them cure me. And if they weren't cured in 3 sessions, they are charlatans."

You treat your body like a broken iron that has been sent in for repair. But the body is not an iron. No one can contract your muscles for you. No one can live through a store display for you. The doctor can only be a navigator, but you must press the pedals and turn the steering wheel yourself.

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6. Practical protocol: 5 steps to transition from Victim to Author

Let's move on to uninstalling the bug. Our goal is to completely close root access to external triggers and return the management console to your hands.

This protocol is tough. It will require breaking many years of habits. But this is the only door leading outside.

Step 1. Linguistic refactoring (Speech interface interception)

The language you use to talk about your condition directly programs your limbic system. As long as your speech is dominated by impersonal, passive verbs, you are a biochemical victim.

Go on a hunt for parasitic words and forcefully rewrite your dictionary:

❌ **Language of the Victim (Passive object)**	✅ **Author's language (Subject with steering wheel in hands)**
"I'm feeling really wobbly today"	**"My brain is currently throwing a sensory integration error and I'm straining my neck muscles"**
"I had a panic attack"	**"I myself accelerated my pulse with frightening thoughts, and my adrenal glands regularly released adrenaline"**
"The weather is pressing on the temples"	**"My blood vessels adapt to the pressure difference, and I concentrate on this feeling"**
"The disease ruined my life"	**"I have driven myself into a dead end with overload and unresolved conflicts, and my body is signaling this"**
"I can't help it"	**"I haven't yet learned how to relax my body at the moment of a symptom, but I practice every day"**

Every time you catch yourself saying *"I was overwhelmed"*, stop, take a breath and say out loud the correct subjective formula. You regain ownership of the process.

Step 2. Audit of secondary benefits: ruthless digitization

Take a sheet of paper, divide it into two columns and audit your shadow accounting:

1. **Column A (Cost of Illness):** What took away my dizziness? (Career, travel, joy, money for examinations).
2. **Column B (Benefits of illness):** What does my dizziness free me from? (You don't need to sort things out with your mother, you don't have to look for a difficult job, my husband drives the car, everyone feels sorry for me and is lenient towards me).

Look at Column B with very open eyes.

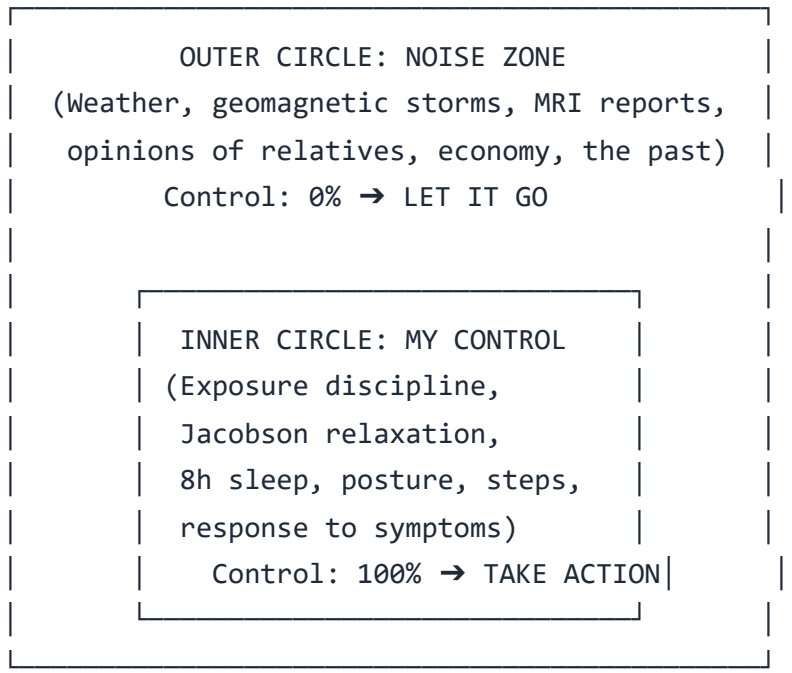
Now make an adult decision: **are you ready to solve these problems directly, with your mouth and actions, without the mediation of dizziness?**

- If you don't want to communicate with a toxic mother, tell her: * "Mom, let's call once a week, I feel more comfortable that way."*
- If you're tired at work, take a vacation or start looking for a new one.
- Stop using your body as a human shield.

Step 3. Control circles (CNS engineering)

All your anxiety is born from one mistake: you spend 90% of your energy on things that you **don't control**, and neglect things that are **your direct control**.

Draw two concentric circles on paper:



Every morning ask yourself the question: * "What circle am I in now?"*. If you read the forecast of magnetic storms or google the symptoms of an auditory nerve tumor, you are draining energy into the

noise zone. Pull yourself up. Shift your focus to the inner circle: do 15 minutes of progressive relaxation, put on your sneakers and walk 5,000 steps down the street to spite the motion.

Step 4. Information blockade protocol (Pity Detox)

The victim feeds on social sympathy. While they pat you on the head in response to complaints, your brain does not see any reason to turn off the disease.

We are introducing strict quarantine:

1. **Prohibition rule for loved ones:** Gather your family and say: *"From today on, we will not discuss my well-being. If I'm unsteady, there's no need to ask: "How are you?" If I need physical help, I will tell you specifically. In other cases, we communicate as if I were completely healthy."*
2. **Total ban of medical chats:** Immediately remove yourself from all Telegram channels, VKontakte groups and forums on PPPD, VAD (autonomic dysfunction) and neuroses, where people savor their suffering. These are toxic cesspools that keep your dorsal raphe nucleus in a state of paralysis.
3. **Prohibition on retelling symptoms:** Not a single complaint to friends over a cup of tea. Your symptoms are your internal technical telemetry, not a topic for small talk.

After 7 days of information hunger, the brain will understand: focusing on suffering no longer brings dividends. We will have to look for other ways to contact the world.

Step 5: Radical Ownership

This principle was formulated by former US Navy SEAL commander Jocko Willink:

> ***There are no guilty parties in your life. For everything that happens within your perimeter, only you bear full, absolute responsibility.***

Not guilty, but **responsible**. Do you feel the difference?

- **Guilt** looks into the past, causes shame and paralyzes: *"I'm a fool, I'm to blame for everything, I ruined my life"* (this is again the position of the Victim!).
- **Responsibility** looks into the present and future and gives power: *"Yes, I drove myself into this state. But since I myself created this muscular shell and this phobia, it means that only I have the keys to dismantle it. And I will do it step by step."*

Each time a symptom occurs, repeat the Author's mantra:

> *"This is my body. These are signals from my brain. I'm not afraid of them. I remain on the captain's bridge."*

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7. Homework (HW)

Words are worthless until they are grounded in muscle contractions and actual behavioral acts. Download and print the finished worksheet: **3.5_victim_and_shadow_worksheet.pdf**.

Here is your weekly protocol for transitioning to the Author position:

1. **Linguistic Purity Week:** Put a sticker on your refrigerator or desk that says: *"WHO RUNS?"*. Every time you catch yourself complaining or the phrase "I'm overwhelmed," reformulate the thought into the language of the subject.
2. **Audit of secondary benefits on paper:** write down at least 3 hidden bonuses that your condition gives you (use the form). Opposite each, write one specific adult action that you can use to satisfy this need without the help of illness.
3. **Information amputation:** unsubscribe from at least 2 chats or public pages on diseases. Warn at least one loved one to stop asking you about your health.
4. **One action in spite of fear:** choose something that you refused due to unsteadiness (a subway ride, a trip to a big store, a walk in the park alone). Go there not to "see if your head will spin," but to tell your brain: * "I am the boss here. You can shake the picture as much as you like - we will still do this job."*

Point of Support: the main insight of the chapter

> *The victim is looking for a savior. The savior always turns into an executioner. The circle closes on the graveyard of your unrealized potential.*
>
> *The only person who can pull you out of the bottom of this swamp is the one you look at in the mirror every morning while brushing your teeth. He doesn't have a white coat, he doesn't have a magic wand. But he has a neuroplastic brain, two legs and the will to rewrite the programming code of his life.*
>
> *Climb out of the hole of learned helplessness. Take away the right of illness to decide how your day will go. Take the helm.*
>
> *And now, when you have stopped being a victim and taken responsibility, we are ready to take the next, deepest step: to go down into the basement of your psyche and get to know the one you have been hiding there for years.*
>
> *Meet your **Shadow**.*

Accepting the Shadow: How the repressed part of the personality explodes the body and creates anxiety

Why the "good person" syndrome leads straight to PPPD and somatization, the neurobiology of repression according to Jung, and 5 practical steps of Shadow integration to turn off background anxiety.

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1. Essence and issues: anatomy of a holy martyr

Look at yourself in the mirror very honestly, without the usual social masks.

You are a "very good person", aren't you?

- You don't make trouble with anyone and smooth out any rough edges.
- When a boor brazenly gets in front of you in line, you lower your eyes and pretend to read a message on your phone.
- When your boss on Friday at 18:00 throws someone else's report on your desk with the words: *"It needs to be done by Saturday morning, you're the most reliable"*, your face stretches into a polite smile: *"Yes, of course, no problem"*.
- You have been listening to toxic complaints from relatives for years, afraid to interrupt the conversation because "it's inconvenient to offend your mother/sister/neighbor."
- You are used to enduring when you are physically or mentally uncomfortable.

And then you go out to the supermarket - and the ground disappears from under your feet. Suddenly a wave of nauseating unsteadiness comes over you, your eyes swim, the picture disintegrates into frames, your neck stiffens, and your heart begins to pound like an alarm bell in your throat. You run in horror to otoneurologists, do three MRIs of the brain, undergo an ultrasound scan of the vessels of the neck, look for multiple sclerosis and a cerebellar tumor, and the doctors shrug their shoulders: * "There is no organic pathology. You have PPPD, hypochondria and vegetative-vascular dystonia. Drink some grass."*

You are indignant: * "What dystonia?! I am an absolutely adequate, calm, kind person! I'm not angry with anyone, I don't have stress!"*.

This is where the disaster lies. Your stress is not in the outside world. Your main source of radiation is what Carl Gustav Jung called **Shadow** over a hundred years ago.

What is the Shadow without esotericism and mysticism

Let's remove the touch of mysticism from this concept. For us, people with engineering and rational thinking, the Shadow is not “dark astral spirits.”

> ***Shadow*** *is a collection of real biological impulses, needs, emotions and character traits that you have outlawed and forcibly locked into your operating system in order to earn love, approval and security.*

Everything “inconvenient” is thrown into the Shadow from early childhood:

1. **Healthy rage and aggression:** the impulse to fight back, defend the territory, bark at an insolent person.
2. **Weakness and vulnerability:** the right to be afraid, to cry, not to cope, to ask for help, to admit: * “I’m tired, I can’t do it anymore”*.
3. **Healthy selfishness:** the right to put your basic needs above the whims of others.
4. **Envy and ambition:** recognition that you want more, better, your own.
5. **Right to refuse:** a pure, firm, unjustifiable word “NO”.

When you screamed and stomped your foot at the age of 5, your mother said: “Ugh, what a nasty boy!” Good boys don’t get angry; mothers leave such children.”* Your childish brain instantly learned a fundamental bug: **“If I get angry or show weakness, I will be kicked out of the pack, and I will die under the fence”**.

From this moment on, the split personality protocol was launched:

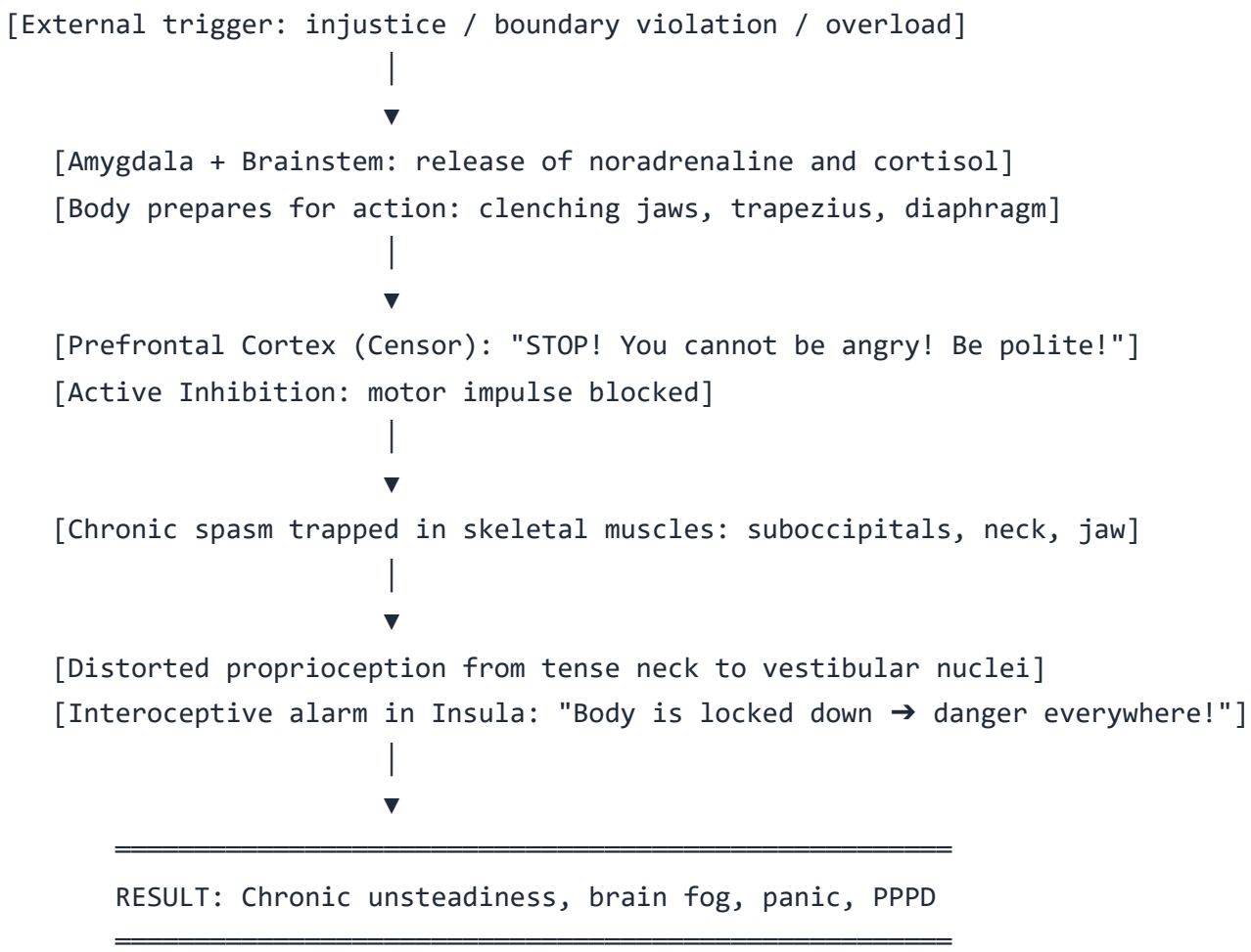
- A **Persona** (your ideal, holy Ego) is posted on the facade - a comfortable, responsive, trouble-free, smiling robot.
- And all biological fire, living anger, natural resistance and fear are pushed into the basement - into the Shadow.

But here is the system law: **any displaced program continues to be executed in the background**. Only now the processor spends a colossal amount of energy not on life itself, but on holding the lid on the boiling pot. And this cauldron ends up tearing your body through the symptoms of PPPD.

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2. Neurophysiology of repression: the biological price of holiness

Let's look at the hardware. What happens in your skull and body when you once again “swallow” an insult or hide a weakness?



1. Active Inhibition and RAM leak

In the brain, the dorsolateral and ventromedial prefrontal cortex (dlPFC and vmPFC) are responsible for controlling social norms.

When you have a natural impulse of anger, the amygdala sends a powerful excitatory signal to the motor centers. But your inner Censor says: *"Leave it!"*. To inhibit physical movement, the prefrontal cortex is forced to produce a continuous, high-amplitude inhibitory signal through GABAergic pathways.

This is equivalent to pressing the gas pedal to the floor in a car and at the same time pressing the brake pedal as hard as you can with both feet. The engine roars, the brake pads smoke, the tires burn, the car stands still.

You call it "I can control myself." Your central nervous system calls this **critical core overheating**. The prefrontal cortex becomes depleted, glucose and ATP levels in neurons drop, and you no longer have the resource to filter out sensory noise. Hello, visual addiction and derealization.

2. Subclinical hypercortisolism and the HPA axis

Experiments in the field of psychoneuroimmunology have long proven that **people who suppress emotions have higher levels of cortisol and norepinephrine than those who openly express anger**.

In an open scandal, the hormonal release is short-term: a flash for 3 minutes, the action is completed, adrenaline is utilized by the muscles, the parasympathetic system (vagus nerve) returns homeostasis.

When displaced into the Shadow, the hormonal tap remains slightly open **around the clock**. The hypothalamus-pituitary-adrenal axis (HPA-axis) is in a state of low-grade sepsis anxiety. Chronic cortisol:

- Blocks the synthesis of BDNF (the brain loses neuroplasticity and cannot retrain from exercise).
- Breaks deep sleep cycles (hence morning grogginess and unsteadiness).
- Keeps vascular tone at maximum, causing microischemia of the vestibular analyzer.

3. Somatomotor trap: where the unspoken goes

An emotion is not a disembodied thought. An emotion is a **specific pattern of muscle contractions**:

- **Anger:** impulse to clench the jaw (bite the enemy's throat), tense the trapezius muscles (pull the head into the shoulders to protect the carotid arteries), clench the fists, fix the pelvis.
- **Fear:** compression of the diaphragm, holding the breath half inhale, tuck the abdominals.

When you smile instead of hitting or screaming, the motor pattern is not canceled. The muscle fibers receive the command to contract, but do not receive discharge through locomotion. A persistent myofascial trigger arises - the same muscle armor.

Spasmed suboccipital muscles (*mm. suboccipitales*) and sternocleidomastoid muscles (*SCM*) compress small vessels and flood the vestibular nuclei of the brain stem with false signals about the position of the head in space.

The brain receives a picture from the eyes: *"We are standing straight"*. And from a spasmodic neck: *"The head is tilted and is experiencing an overload of 3G"*. A sensory conflict arises. The brain panics and turns on emergency mode - **the illusion of rocking and falling**. Your unlived anger is literally making your head spin.

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3. Cognitive traps and myths about the Shadow

Why do smart, well-read people refuse to look into their Shadow for years? Because four reinforced concrete myths stand in the way:

Myth 1. "If I let the genie out of the bottle, I will become a monster"

The most common fear of a neurotic: * "If I allow myself to be angry, I will lash out at my children, kill my boss, destroy my family and end up in prison."*

Reality: Only those who **do not realize their Shadow** become a monster. It is the unaccepted Shadow that breaks out in the form of sudden psychoses, uncontrollable outbursts of rage out of the blue, severe passive aggression, sarcasm and destructive somatic diseases.

A person who knows his Shadow and looks it straight in the eye has **choice**. He feels rage, acknowledges it: *"Wow, I'm ready to crush this person right now."* — and then consciously decides how to direct this force: say a firm "no," break a toxic contract, or go unload the barbell at the gym. Integration of the Shadow makes you not a dangerous psycho, but a calm, confident predator who is well-fed and does not rush over trifles.

Myth 2. "Being angry and afraid is a sign of weakness and spiritual immaturity"

The trap of so-called "spiritual bypassing" and toxic positivity. A person hangs himself with mantras, goes to yoga, breathes heavily with his uterus and repeats: * "I send light and love to everyone" *. And inside him bubbles a poisonous resentment and hatred, which he is ashamed to admit even to himself.

Reality: Anger is the oldest evolutionary sensor. If our ancestors had not had rage, they would have been devoured during the first ice age. Anger is the watchdog of your personal boundaries. If you shoot a dog, any tramp will come into your house and wipe his feet on your bed.

Myth 3. "Good people are loved, but bad people are rejected"

You think that your reliability and sacrifice buy you a guarantee of love and recognition.

Reality: They don't like "comfortable" people. They are **used**. They get used to them as if they were a comfortable bedside rug. And when the rug begins to feel dizzy from overload and can no longer serve, those around them experience not sympathy, but dull irritation: * "Why is it broken? Let it be repaired back."* The fear of being rejected for your imperfections keeps you in bondage.

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4. Evidence base: what science says

If you still consider the concept of the Shadow to be purely philosophical lyrics, look at the numbers and clinical studies:

1. Research by James Pennebaker (University of Texas at Austin)

Professor Pennebaker has been studying the phenomenon of expressive writing protocols for decades. Its randomized controlled trials showed:

- Patients who repressed emotional trauma and negative affect (anger, shame, guilt) demonstrated persistent suppression of cellular immunity (decreased activity of T helper cells and NK cells).
- When participants were given the opportunity for just 4 days to write out their most unsightly, "forbidden" and dirty thoughts on paper without censorship for just 4 days, they had a **43% reduction in the frequency of visits to doctors** over the next 6 months, and markers of autonomic reactivity (heart rate variability) returned to normal.

2. Dr. John Sarno and the PPD/TMS model

New York University rehabilitation medicine professor John Sarno has treated thousands of patients with intractable chronic back pain, fibromyalgia, and unexplained dizziness. His conclusion revolutionized somatic medicine:

> *Pain and dizziness in 80% of cases are a **distraction of the brain** (Tension Myositis Syndrome / Psychophysiologic Disorder).*

The brain deliberately creates a physical symptom (spasm, ischemia, unsteadiness) so that all the person's conscious attention switches to the body, and God forbid the person does not feel the unconscious rage buried in the Shadow at loved ones, children or himself. As soon as the patient recognized his Shadow and realized: * "My dizziness is a screen for suppressed anger" *, the symptom disappeared in a matter of weeks without a single pill.

3. fMRI studies by Matthew Lieberman (UCLA, neuroimaging)

Professor Lieberman's research has proven the phenomenon of **"Affect Labeling"** (verbalization of affect). When a person suppresses a negative emotion, the amygdala explodes like a supernova. But at that moment when a person out loud or on paper directly and accurately names his shadow state: * "I am furious. I hate this situation. I'm jealous. "I'm scared"*, the right ventrolateral prefrontal cortex (rVLPFC) is activated, which sends an immediate inhibitory impulse to the amygdala, reducing anxiety levels in 20-30 seconds at the neuroimaging level.

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5. Practice: 5 steps of Shadow integration

Enough theory. Let's move on to uninstalling the bug. Our task is to pull the Shadow out of the system basement into the light, cleanse it of shame and return the energy it contains back into the energy system.

Step 1. Shadow audit through projections ("What's pissing me off?")

You cannot see your Shadow directly because your Censor covers it with blind spots. But there is a sure-fire marker: **projection**.

> *What irritates you furiously, to the point of white heat, and triggers in other people is 100% your own blocked Shadow.*

Take a sheet of paper and fill out the table of shadow projections:

A man and his behavior that makes me shake	What shadow quality am I condemning in him?	My secret, repressed desire (My Shadow)
Colleague laughs loudly, argues with the boss and leaves exactly at 18:00, having given up the rush	Arrogant, unscrupulous, selfish, lazy	*I desperately want to give up work, put myself first and stop being a victim*
Blogger brags about money, shopping and luxurious life	Upstart, show-off, mercantile	*I want recognition, wealth and not to be ashamed of my material ambitions*
An acquaintance whines, complains about life and openly asks for help	Weakling, weakling, slobber	*I'm dead tired of being strong, I want to lie down, cry and be pitied*

Look at the right column. These are not "sins". These are your **amputated vital functions** that you forbade yourself to show.

Step 2. Black Box Protocol (Affect Labeling without censorship)

You must release the accumulated emotional dirt without harm to others. Your head needs to stop being trapped in a gas tank.

Instructions:

1. Take a blank sheet of paper and a pen (you cannot type on the keyboard - hand motor skills directly activate the limbic areas of the brain).
2. Set the timer for exactly **15 minutes**.
3. Start writing all the most disgusting, evil, unsightly, envious and cruel things that are inside you right now. *Uncensored. No punctuation. Without regard to morality.

- Write in plain text: who you hate, who you want to send, who you wish to fail, how tired you are of family, work, illness, how you hate your dizziness.
- Don't try to be "correct" or "conscious". The dirtier and more violent the flow, the deeper the cleansing.

4. **Important Safety Rule:** After the timer rings, you **do not re-read what you wrote**. You take this leaf, burn it over the sink or tear it into small pieces and flush it down the toilet.
5. Notice what happened to your neck and breathing immediately after disposal. In 90% of cases, the diaphragm makes a spontaneous deep exhalation, and the shoulders drop 2-3 centimeters down.

Step 3. Somatic legalization of impulse (Somatic Unshaming)

When a shadow feeling (anger, irritation, shame, cowardice) comes to you, do not try to suppress it with affirmations. Learn to live it with your body:

1. **Localize the impulse:** Where is it stuck? In the jaw? In the throat? In clenched fists?
2. **Tighten the clamp for 5 seconds:** Clench your teeth even tighter, clench your fists, pull your shoulders up to your ears. Bring the physiological signal to its peak.
3. **Give sound and motor output:**

- Make a powerful, long exhale with a sound (growling, loud moaning, hissing).
- Knock out the pillow, squeeze the towel in your hands and twist it furiously, as if you were wringing out laundry.

4. **Tell your brain a permissive phrase:** *"I see my anger. I have every right to be furious. It's safe for me and others. My body can feel it."*

Step 4. Assertive Firewall: Practice Pure "NO"

Until you learn to refuse with your mouth in the outside world, your body will refuse for you with symptoms of dizziness. PPPD is your only legal way to tell the world: *"Get away from me, I'm sick, I can't do it!"*.

Transfer this process from unconscious somatic sabotage to conscious speech control:

- **5 second pause rule:** When asked for something, never answer immediately. Take a break. Feel your body. If there is a nagging feeling of resistance in your stomach, your answer is "NO."
- **Formula for refusal without excuses:**
- ❌ *Wrong:* "Oh, you know, I would love to help, but my head is spinning, my blood pressure has risen, the cat is sick, please excuse me..." (you make excuses = feel guilty = feed anxiety).
- ✅ *Correct:* **"No, I can't do that"**. Or: **"This doesn't suit me"**.
- Period. No explanation. Your refusal does not require justification before a jury. You have the right to refuse simply because you decide so.

Step 5. Legalization of the right to break (Anti-perfectionist protocol)

Your Shadow retains the right to be imperfect, stupid, awkward and weak. Start returning these rights in small doses:

1. Deliberately do one everyday action with a C minus (leave an unwashed mug in the sink overnight, send a letter to a colleague without proofreading commas five times).
2. Allow yourself to openly say in a conversation: * "I don't know the answer to this question" * or * "I'm confused and now I don't understand what to do" *.
3. Understand: when you allow yourself to be an imperfect person, the world does not collapse. On the contrary, people begin to gravitate toward you because a living, vulnerable person evokes warmth and trust, while a plastic, flawless robot evokes only coldness and suspicion.

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6. Homework (HW)

Theory without practice in our business is zero. For ease of use, download and print the finished form: **3.5_victim_and_shadow_worksheet.pdf**.

Here is your step-by-step weekly Shadow integration sprint:

1. **Fill out the table of shadow projections:** Write down at least 5 people who cause you great irritation, and determine what repressed quality of yours they mirror (use the form).
2. **Conduct 3 "Black Box" sessions:** Monday, Wednesday, Friday - 15 minutes of uncensored writing with mandatory subsequent destruction of the paper.
3. **Make 3 Conscious Nos:** This week, say a firm "NO" to three requests or offers that you don't like, using the no-excuses formula. Write it down in your notebook: no one died, the sky did not fall to the ground.
4. **Exercise "Permission to Weakness":** Take 1 hour on your day off, lie down on the couch and officially forbid yourself to be useful. No self-help books, no cleaning. Just lie there and stare at the ceiling and say: * "Right now I'm worthless, lazy and useless - and I have every right to be so."*

Point of Support: the main insight of the chapter

> *You were so desperate to become an angel in the flesh that your human, biological body simply rebelled against this unnatural vivisection.*
>
> *Your dizziness is not a breakdown. This is a beast locked in the basement, which beats with its paws on the iron door of your ideal Ego and shouts: "Let me out!" I want to live! I'm tired of pretending to be a saint!*
>
> *Stop fighting with your own shadow. Put down the weapon, open the door to the basement, embrace your inner beast, acknowledge its strength, its anger and its right to weakness.*
>
> *Only when you accept your darkness will your internal processor finally release the emergency pressure, your neck will relax, and the ground under your feet will again become a solid, unshakable monolith.*

Mental Boundaries of Consciousness: The Trap of the "Unique Sufferer" and Blindness to the Exit

Why we fanatically believe our personal hell is unprecedented, how the vanity of suffering locks us in a dead end, and why the solution lies in a dimension you stubbornly refuse to acknowledge.

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1. Essence and Problem: The Curse of "Terminal Uniqueness"

Let's strip away all the noble disguises and call things by their real names. No self-pity, no soft-pedaled therapeutic cliches, no sugarcoating.

What is your deepest, darkest, and most sacred internal dogma?

It sounds like this:

> ***"Nobody in this damn world suffers the way I suffer. Nobody truly understands me. All these people on the street, doctors, relatives — they live simple, easy lives. But what is happening to me is cosmically complex, exceptional, and irreversible. My case is unique."***

You may never utter these words aloud. But inside, you wear this belief like an invisible martyr's crown.

- When someone tells you: *"It's a neuro-functional loop; start with vestibular exercises and cognitive desensitization,"* you cringe: *"They don't understand the magnitude of my catastrophe. They just had mild dizziness; my situation is entirely different."*
- When you read stories of people who made a 100% full recovery, your mind instantly devalues them: *"Well, they had it easier. They didn't have THIS level of rocking. They don't know what it feels like to sway 24/7 for six months straight."*
- You look at pedestrians outside your window with a toxic mix of envy and arrogant condescension: *"You oblivious drones just walk around taking the solid ground for granted. You have no idea what real hell is."*

The Narcissism of Suffering

Congratulations: you have walked right into the most suffocating, insidious trap of the human ego. In psychotherapy, this is called **Terminal Uniqueness**.

It is ego-vanity turned inside out. If I cannot be the most successful, wealthiest, or happiest person — I will become **the most severely ill, misunderstood, and uniquely suffering entity in the universe**.

Your brain clings to this illusion of exceptionalism with fanatic devotion. Because admitting that your case is an ordinary, standard, off-the-shelf biochemical glitch of the nervous system that yields to predictable, boring protocols... means stripping off the tragic hero's mantle. And for the ego, that is unbearable.

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2. The Anatomy of "Pigeon Vision": Why You Keep Bashing into Closed Doors

You feel trapped in an inescapable labyrinth not because there is no exit, but because **you are attempting to solve the problem using the exact same thinking that created it**.

How does the boxed-in mind operate?

Stage	What Happens in the Mind & Actions
1. Symptom	Unsteadiness, derealization, neck muscle spasm, brain fog.
2. Level of Thinking	*"I must find hidden structural damage, find a magic pill, and fix my body like a broken machine."*
3. Actions	15th MRI, continuous IV infusions, chiropractors, venting on forums, passive waiting for a miracle.
4. Result	**Zero.** The symptom persists, while anxiety and hyper-focus escalate.
5. False Conclusion	*"I am incurable! My case is uniquely complex! Nothing in modern medicine works!"*

This is the exact behavior of a fly crashing full-speed into a windowpane.

The fly sees light. Hardcoded into its primitive neural circuits is the rule: *"Light = freedom."* It accelerates and slams head-first into the transparent barrier. Again. And again. Until total exhaustion, until it drops dead.

Ten centimeters to the right, a massive window is wide open. Fresh air, breeze, and absolute freedom. But to exit through that opening, the fly would have to do something counterintuitive to its narrow paradigm: **fly backward into the dark room, change its vantage point, and exit through a different plane**.

The fly cannot do that. For the fly, only the glass and the light exist.

> ***You are that fly.*** *For years, you have been bashing your skull against the glass of "medical investigations," "battling symptoms," and "pitying your unique tragic fate." The exit is sitting right*

> *next to you — in a dimension you dismiss as "too simple" or "irrelevant."*

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3. Shattering the Ego's Pride: You Are Not the Chosen One; Your Glitch is Off-the-Shelf

Let's bring you back down to earth. It stings, but it is the only path to genuine recovery.

1. **Your fear is not unique.** The exact same wave of unsteadiness, with cold palms, a tight suboccipital band, and the panic thought *"I am going to collapse,"* was experienced by an ancient hunter 50,000 years ago when ambushed by a predator. Your amygdala runs on Stone Age factory firmware. It possesses zero individuality. It is an uninspired, stamped-out biological reflex.
2. **Your thoughts are not original.** The belief *"There is something deeply wrong with me, and doctors don't understand"* has been thought before you by 100% of humans suffering from health anxiety and PPPD. You haven't discovered a new universe of suffering. You are just replaying a worn-out record, naively believing it's your exclusive track.
3. **Your agony does not make you noble.** Suffering does not inherently elevate a person. Suffering from chronic neurosis is not heroism. It is the natural consequence of tunnel vision, stubbornness, and cognitive blind spots.

And herein lies **the greatest good news**: Because your breakdown is not unique, **the solution is not unique either**. You do not need to invent an unprecedented cure for a cosmic anomaly. You just need to stop outsmarting reality, acknowledge the limitations of your current perspective, and walk down the well-trodden path paved by thousands who recovered before you.

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4. The Flatland Paradigm: Stepping into the Third Dimension

Imagine two-dimensional creatures ("Flatlanders") living on a flat sheet of paper.

If you draw a closed circle around a Flatlander with a pencil, to him it becomes an impenetrable, lifelong prison. He will pace along the perimeter for years, touch the line, weep, curse the cosmos, and declare: *"I am doomed forever; there is no way out."*

Yet if he had the capacity to look upward into the third dimension (3D), he would see: **there is no ceiling**. All it takes is a single step upward in the 3rd dimension — and he is free.

Perceptual Dimension	Focus of Attention & Cognitive Pattern	Actions & Outcome
Flatland (2D) *(The Impasse Trap)*	**“How do I eliminate dizziness? What pill should I take? Who is to blame?”** Constant body scanning, chasing organic damage, self-pity, belief in an "unprecedented severe case."	Running endless loops: doctors $\rightarrow$ neck scans $\rightarrow$ doom-scrolling $\rightarrow$ despair. **Outcome:** Chronification of symptoms.
The Third Dimension (3D) *(Meta-Position & Exit)*	**“Why did I corner myself here? What is the symptom shielding me from? How do I reclaim my life?”** Accepting the ordinary biology of nervous exhaustion, shedding the tragic martyr identity.	Shifting life context, dismantling hypercontrol, somatic exposure, and metacognitive therapy. **Outcome:** Full and lasting recovery.

Your cognitive boundaries have locked you in Flatland:

- **Flatland:** "How do I make my body stop rocking? What supplement should I take? Whose fault is this?"
- **Third Dimension:** "Why did I drive myself into this corner? Why am I terrified of living? What life decisions am I evading behind this symptom? How do I shift my focus from my body back into the world?"

As long as you fight on the 2D plane, there is no solution. The moment you step into the 3rd dimension, the circle ceases to exist.

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5. Step-by-Step Protocol: Breaking Your Mental Boundaries (Homework)

Stop being a hostage to your own intellectual pride. Execute these 4 steps without debating your inner critic.

Step 1. Dismantling the Martyr's Crown

Take a pen, a physical sheet of paper, and write by hand:

> ***"I acknowledge that my condition is NOT unique.***
> ***My brain is glitching according to standard, primitive biological rules.***
> ***My suffering is not a badge of tragic nobility; it is the result of being trapped in my own cognitive loops.***

> ***I resign from the role of the great martyr and become an ordinary student ready to execute simple, proven protocols."***

Read this out loud three times. Notice how your ego tenses and hisses. Let it hiss. That is your prison wall crumbling.

Step 2. The Cosmic Perspective Practice (Dissolving Pain Egocentrism)

Whenever another wave of unsteadiness hits and the automatic script starts running (*"Why me? Why do I have to suffer like this?"*):

1. Close your eyes for 60 seconds.
2. Visualize yourself from the outside: a tiny dot in a room.
3. Zoom the camera out: your neighborhood, your city, your continent, planet Earth.
4. Realize: right now on this planet, 8 billion people exist. Millions of them at this exact second are experiencing the identical vasoconstriction, brain fog, and fear of falling.
5. Remind yourself: *"This is simply ancient biochemical noise of the human organism. I am not the center of the universe. Nothing supernatural is occurring. This will pass."*

Step 3. Blind Spot Inventory (Stepping into 3D)

Answer these 3 confrontational questions in writing:

1. *"If I were strictly forbidden from thinking about my body, symptoms, and doctors — what actual real-life crisis would I be forced to face?"*
2. *"In what area of my life am I acting out of cowardice while using my dizziness as an alibi?"* (Career, toxic relationship, asserting boundaries, family enmeshment).
3. *"What simple, obvious habit do I dismiss as 'beneath me' or 'too trivial,' even though it is exactly what I need?"* (e.g., sleeping 8 hours, walking 5 km daily without a phone, stopping medical searches, saying an unapologetic "NO").

Step 4. Permission to Not Know and Intellectual Surrender

Look directly into a mirror and declare:

> ***"My intellect led me into this dead end. All my brilliant analysis and rationalizations produced chronic rocking and panic. Therefore, my current intellect is incompetent to solve***

> ***this on its own terms. I cease trusting my automatic thoughts and begin strictly following proven, evidence-based recovery instructions."***

Stop searching for hidden doors in the brick wall. The window is wide open. Step into the other plane.

The “Good Person” Syndrome: People-Pleasing, Guilt, and the Inability to Say “No”

When your mouth politely says “Yes, of course I’ll help” for the hundredth time out of fear of offending someone, your nervous system pulls the emergency brake and says “No” through dizziness. Deconstructing the neurobiology of people-pleasing and suppressed auto-aggression.

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1. Profile of the Classic PPPD Hostage

Over years of working with hundreds of individuals trapped in chronic unsteadiness, a striking pattern emerges. Among PPPD patients, you almost never find unapologetic, assertive people who effortlessly defend their boundaries and put their own well-being first.

Instead, 85% of any PPPD support community shares the exact same psychological blueprint:

- **Compulsive “Rescuers”** who carry everyone else’s workloads, family crises, aging parents’ emotions, and friends’ relationship dramas on their backs.
- People with **hypertrophied guilt** who apologize even when someone else bumps into them on the street.
- Conflict-avoidant perfectionists who **swallow resentment**, smile politely while boiling with rage inside, and ruminate for hours: *“Did I sound too harsh? Are they upset with me?”*

You spent your entire life trying to be good, reliable, accommodating, and convenient. You endured situations where anyone else would have slammed the door long ago. And then one day, your own body quite literally **knocked the ground out from under your feet**.

Coincidence? Not at all. It is a direct neurophysiological consequence.

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2. The Fourth Trauma Response: Fawn and Auto-Aggression

Everyone knows the three classic evolutionary responses to threat: **Fight**, **Flight**, and **Freeze**. However, psychotherapist **Pete Walker** identified a fourth, deeply insidious trauma response — **Fawn** (People-Pleasing).

The Fawn response is wired in childhood environments where parental love was conditional or emotional volatility made assertiveness dangerous:

> *"If I get angry, argue, or say 'no,' Mom will withdraw her love, Dad will explode, and I will be abandoned. To survive, I must read everyone's micro-expressions the second they walk into the room and make myself completely pleasing and useful."*

What Happens in Your Brain and Body When You Betray Yourself?

Trace the exact biochemical cascade. A boss, partner, or relative dumps another unfair demand onto your plate. You are exhausted to the bone and desperately want to refuse.

1. **Limbic Mobilization:** Your amygdala and hypothalamus instantly register a boundary violation. A surge of **norepinephrine and cortisol** hits your bloodstream, priming your jaw, neck, and shoulder muscles for a "Fight" response (to defend your territory and say a firm "No!").
2. **Prefrontal "Censor" Suppression:** Your conditioned "Good Person" persona slams on the brakes in terror: *"Stay quiet! You can't refuse! They'll think you're selfish, they'll get angry!"*
3. **Vector Inversion (Auto-Aggression):** The sympathetic mobilization energy is already circulating in your blood! Yet instead of releasing it outward, you force a polite smile and say: *"Sure, no problem, I'll take care of it."* Where does that unexpressed adrenergic charge go? It turns **inward against your own body**.

Stage	Healthy Assertiveness	The "Good Person" Response (PPPD Fuel)
Boundary Intrusion	Unfair demand or interpersonal pressure	Unfair demand or interpersonal pressure
Biochemical Surge	Sympathetic energy to defend boundaries	Sympathetic surge + instant prohibition on anger
External Action	Calm, direct refusal: "No, that doesn't work for me"	Forced smile: "Of course, I'll handle it!"
Muscular Fate	Discharge and return to parasympathetic rest	Rock-hard clamping of suboccipital neck muscles, jaw & diaphragm
Vestibular Outcome	Steady equilibrium and clear head	Distorted cervical proprioception, autonomic spike, wave of rocking

Your suboccipital neck muscles turn to stone holding back the words *"Leave me alone!"* Your masseter jaw muscles clench all night. Your brainstem floods with unexpressed sympathetic arousal — and the floor begins to pitch beneath your feet.

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3. Why Dizziness Is Your Body's Somatic "NO"

When you override your emotional boundaries for years, your biological survival instinct eventually stages a coup.

Your subconscious assesses the situation:

> *"The host has lost their mind. They are giving away our life force to everyone around them. They refuse to protect us verbally. If this continues, we will burn out completely. Taking manual control."*

And the brain activates **PPPD**.

Notice how ruthlessly effective this mechanism is:

- Before PPPD, you couldn't say "no" to your demanding mother-in-law, boss, or draining friend without drowning in guilt. Now you have an ironclad medical alibi: *"I can't come help today, my vertigo is flaring up terribly."*
- Before PPPD, you worked 12-hour days without breaks. Now the computer screen swims so badly that you are physically forced to close the laptop.

> ***If you refuse to say "No" with your mouth, your vestibular system will say it for you.***

Chronic dizziness literally takes your legs out from under you so that the people riding on your back are forced to get off.

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4. Three Toxic Myths of the People-Pleaser

To cut off the fuel supply to your neurosis, we must dismantle three childhood beliefs running your autopilot:

Myth 1: "If I am always accommodating, people will love and value me"

The harsh truth of human psychology: **people do not love doormats — people use them**. When you have zero boundaries, others rapidly lose respect for you. You become an appliance — a free therapist, a tireless worker, a convenient caretaker. And the moment you develop PPPD and stop functioning as a convenient appliance, the very people you sacrificed your health for are often the first to grow annoyed: *"Why are you still dizzy? Pull yourself together already!"*

Myth 2: "Saying 'no' or choosing myself is selfish"

Let us define terms accurately:

- **Healthy self-respect** is deciding how to spend your own time and protecting your nervous system.
- **True selfishness (manipulation)** is when *someone else* demands that you sacrifice your health and peace of mind for their convenience! When someone sulks because you politely declined a request, you are not the selfish one — they are.

Myth 3: "I am responsible for other people's feelings and disappointment"

Memorize this foundational law of psychological boundaries: **taking offense is the choice of the person taking it**. If you politely decline to spend your only day off running errands for a relative, and they slam the phone down in anger — that is **their** emotional immaturity and **their** manipulation tactic. You are not obligated to subsidize someone else's tantrums with your nervous system.

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5. The Detox Dictionary: Translating from "Pleaser" to Adult

A people-pleaser over-explains, apologizes, and leaves loopholes for pressure. A grounded adult speaks concisely and uses periods, not question marks.

Scenario	The "Good Person" Reply (PPPD Fuel)	The Grounded Adult Reply (Nervous System Shield)
Asked to work late on Friday	"Oh, my head is spinning so badly today, I'd love to help, I'm so sorry, I just feel awful..."	"No, I can't stay late today. I will pick this up first thing Monday morning."
A friend dumping toxic negativity on the phone	Listening for 60 minutes with a clenched jaw until the room starts spinning	"Hey, I need to run and take care of some things right now. Let's catch up next week."
Pressured into hosting guests or attending an event	"Well, okay, come over..." (then resenting everyone for three days and having panic attacks)	"This weekend doesn't work for me — I'm taking quiet time to recharge. Let's plan for another time."
Guilt-tripping ("You only think about yourself!")	Defending yourself, crying, trying to prove you are a good person	"I understand you're disappointed, but my decision stays the same."

Notice the cardinal rule: **never offer long justifications for your "No."** The moment you start explaining (*"I can't because..."*), a manipulator treats your reason as an opening for negotiation. A calm, brief *"No, that doesn't work for me"* leaves zero handles to grab onto.

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6. The “Selfish Immunity” Protocol: 4 Steps to Reclaim Your Spine

Step 1. The 10-Second Pause Rule

For people-pleasers, the word “Yes” flies out of their mouth before the prefrontal cortex even processes the request — it is an automatic reflex. Install a hard circuit breaker: you are forbidden from answering **any** request on the spot. Your default phrase is now:

> *“Let me check my schedule and get back to you in an hour.”*

During that hour, check in with your body: *“When I picture saying yes to this, do I feel expansion and genuine interest, or a knot in my stomach and tightness in my neck?”* If your body contracts — the answer is “No.”

Step 2. Exposure to Guilt and Disapproval

The real reason you cave in is an intolerance of guilt and other people’s sour faces. Treat guilt the exact same way you treat dizziness during exposure therapy!

- You say “No.”
- The other person acts cold or disappointed.
- A wave of sticky guilt rises in your chest: *“I’m a terrible person, I should apologize and fix it.”*
- **Stop!** Breathe and observe that guilt for 15 minutes without acting on it. It is merely the withdrawal symptom of an old childhood reflex. Endure it 5 times, and the collar of people-pleasing snaps forever.

Step 3. Returning Other People’s Backpacks

Write down a list of responsibilities you are currently carrying for capable, grown adults (your spouse, adult children, coworkers, parents). Start systematically handing their backpacks back to them. Every capable adult has two hands, two legs, and a brain — stop playing Atlas while your own nervous system is in flames.

Step 4. One Daily Hour of “Sacred Selfishness”

Carve out at least 60 minutes every single day where you are officially off-duty to work, chores, and other people’s needs. Spend that hour doing what genuinely restores you, with zero obligation to be “productive.”

💬 Core Insight

> *"Trying to be 'good to everyone' is the fastest way to become a ruthless enemy to your own nervous system. Every time you swallow resentment and betray your boundaries for someone else's approval, your body pays for that performance with neck spasms, adrenaline surges, and lost equilibrium. Stop trying to be convenient — start being alive."*

📝 Homework Assignment (Action Plan)

1. **The Energy Drain Audit:** Write down 3 people or 3 recurring obligations after which your dizziness and brain fog predictably worsen. Those are your primary sources of suppressed auto-aggression.
2. **The "Clean No" Drill:** Within the next 48 hours, say a calm, polite "No" in at least two situations where you would normally cave in. Do not apologize, do not over-explain, and **do not use your health or dizziness as an excuse**!
3. **Permission to Disappoint:** Stand in front of a mirror, look yourself in the eyes, and say out loud: *"I allow other people to be disappointed in me. I am not here to live up to everyone's expectations. My nervous system comes first."*

Module 4: Recovery

Anatomy of a Relapse: Why Recovery Isn't Linear

What happens to neural connections during a relapse, typical triggers for recessions, and why relapse is a litmus test for overload, not reset.

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1. Essence and problems: the nightmare “everything is broken”

You were on the mend. For two weeks you felt great: the unsteadiness was almost gone, your head cleared, you started going to the shops and making plans for your life. You believed that the nightmare was over.

And suddenly, on one ordinary day, it hits you with renewed vigor. The head again becomes “lead”, the floor begins to shake, and the panic returns in full. Your immediate reaction is deepest despair: * “Everything is broken!” Nothing helped! All my efforts were in vain, I returned to the starting point, and I will never get out of this hell!

You feel empty. This state is called **rollback** (setback). And this is the most critical moment on the path to recovery. It is at this stage that most people give up, quit practicing and start visiting doctors again.

2. Physiological explanation: spiral recovery and litmus test

Let's look at the neurobiology of rollback.

When you recover, your new neural connections (the path of peace) are still very thin and fragile. The old highway of fear did not disappear instantly; the brain simply stopped walking along it.

But if you overload your nervous system (didn’t get enough sleep, overworked, got an ARVI or experienced severe stress), the brain loses energy. He becomes lazy to spend resources on maintaining a new, not yet automated rest program. In energy deficit mode, the brain automatically slides into the oldest, deepest, most energy-efficient rut - the anxiety rut.

Recovery from PPPD is **never linear**. A line graph is a fantasy that does not exist in nature. Recovery goes in a spiral: you go up, but at every turn you see the same wall of symptoms. It seems to you: "*I'm walking in circles*". But this is an illusion: you see the same wall, but you are one floor higher. You are no longer the same person who got sick a year ago - you have knowledge, experience and tools.

Typical physiological triggers of kickbacks:

1. **Lack of sleep for 3+ days in a row** (critically reduces the resource of the central nervous system).
2. ** ARVI, flu or cold** (the immune response temporarily increases receptor sensitivity and anxiety).

3. **Change in weather/atmospheric pressure** (with a spasmodic neck, this causes a temporary increase in unsteadiness).
4. **Alcohol** (a hangover is a severe vegetative crisis that resets resting GABA receptors).
5. **Hormonal fluctuations** (PMS in women).

But there is one more trigger that occurs more often than all the others combined:

6. **“I felt better - I quit practicing”** - this is trap number 1. You feel better within 2-3 weeks. You decide: “Well, that’s it, it’s over!”* - and without noticing to yourself, you remove from your life everything that pulled you out. You stop doing Jacobson. You give up on meditation. You stop walking. You return to old thought patterns: * “I owe everything, the children owe it, everything at work must be perfect”*. And a week later - bam, rollback. And you’re in a panic: *“Nothing helped!”*.

No. It actually helped. It was **because** of what you did that made you feel better. And when you stopped, your nervous system returned to the old habits.

Iron rule: Jacobson's relaxation, meditation, cardio and cognitive restructuring are not emergency pills. They are **daily neural hygiene**, like brushing your teeth. You don't stop brushing your teeth when they stop hurting, do you? Same here — daily practice without negotiation or skipping.

3. Cognitive traps: the illusion of zeroing

Main cognitive trap: *"I'm back to square one. My progress is zero."*

This is a lie from your frightened mind. Neuroplasticity cannot be “erased” in one day. The new neural connections that you spent weeks building are still there. They are simply temporarily blocked out by adrenaline noise, like a radio station during a thunderstorm. As soon as the storm subsides, the signal of calm will come through again. A setback is not a reset. This is just a temporary flare-up of the old alarm system.

The second pitfall is trying to “immediately fix yourself” right during a setback. You start feverishly doing exercises, meditating 5 times a day, getting angry and checking your symptoms every 5 minutes. This injects intense anxiety and control back into the system, prolonging the setback for weeks.

4. Point of Support: Insight

> *A setback is not your enemy. It is your ally. It is a biological litmus test showing that your system reached overload. If unsteadiness temporarily increases, your brain is not broken. It is simply signaling: “System is overheated. Please pull over, turn off the engine, and allow us to recharge.” Acknowledge the warning without panic, and allow yourself to slow down.*

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5. Practice: Analysis of the causes of rollback

Our goal is to stop panicking during rollbacks and teach the brain to see cause-and-effect relationships in them, and not a “chaotic breakdown.”

When a pullback hits you, open a notepad and do a written audit of the last 3-5 days:

1. **Sleep:** How many hours have I slept in the last 3 days? Have I fallen asleep with my phone in my hands?
2. **Body:** Was there severe physical fatigue? Neck spasm? Did I drink alcohol or too much coffee?
3. **Psyche:** Were there any conflicts at work or at home? Was I trying to control everything around me?
4. **Practice:** Have I abandoned Jacobson’s relaxation and gymnastics, deciding that “I’m already better”?

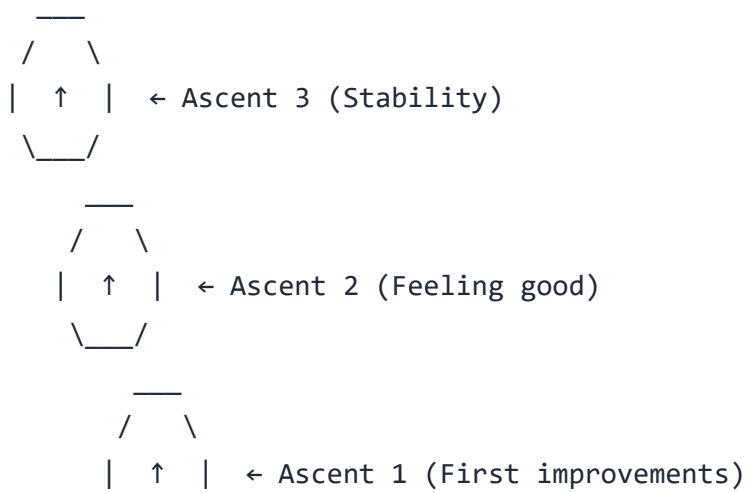
Realize: * “My setback is a natural consequence of [overwork at work / lack of sleep / ARVI]. My body reacts normally. I'm giving him time to recover."*

Homework (DZ):

1. **Read the “Spiral Model” section below** and draw your own spiral: which pullbacks you have already experienced and what happened after each one.
2. Remember your last “bad period.” How long did it last? Compare with the time of “good” periods. Write down the ratio.
3. Make your personal list of 5 triggers that most often cause a rollback. For each, write a protocol of actions.

Spiral model of recovery

Forget about the straight line “bad → good”. Recovery looks like an **upward spiral**:





Each “turn” of the spiral is a cycle: improvement → rollback → improvement at a higher level. Keyword: **at a higher level**. Each new rollback is softer than the previous one, and each new rise is higher.

Here's what it looks like in numbers:

Month	Good days	Bad days	DHI (average)
1	8	22	65
2	15	15	50
3	22	8	35
4	26	4	20

If you're in month 2 and you're having a bad day today, you might think: *"I'm not better, I'm just as bad as at the beginning*". But that's a lie. Because in month 1 there were 22 bad days, and now there are 15. You are progressing, it’s just that your brain, in alarm mode, focuses on the dips and ignores the ups.

Cure: Diary. Every evening, rate the day: 🟢 (good), 🟡 (average), 🔴 (bad). In a month, do the math. The numbers don't lie.

Taxonomy of rollback triggers

Kickbacks don't happen out of nowhere. Everyone has a trigger. Here's the full list:

Physiological triggers:

- **Lack of sleep** (less than 6 hours). The most powerful and most frequent trigger. One bad night's sleep can wipe out a week of progress.
- **SARL/flu.** The immune response activates the same autonomic chain as anxiety. Unsteadiness when you have a cold is normal and not a relapse.
- **PMS/Hormonal Fluctuations** For women, unsteadiness often worsens 3-5 days before menstruation. This is a progesterone decline, not a return of PPPD.
- **Caffeine.** More than 2 cups of coffee per day = increased cortisol levels = increased shakyness.
- **Alcohol.** Even 1-2 glasses of wine in the evening → the next morning anxiety and unsteadiness due to rebound excitation of the central nervous system.

Psychological triggers:

- **Conflict.** Quarrel with a partner, boss, parents. Unexpressed anger → spasm → unsteadiness.
- **Information overload.** A lot of news, a lot of social networks, a lot of other people's problems.
- **Internal criticism.** “I should have gotten better a long time ago” is the most insidious trigger because it looks like motivation, but in fact it creates stress.

Environmental triggers:

- **Weather.** Drop in atmospheric pressure. Yes, it really has an effect, although the mechanism has not yet been fully studied.
- **Seasonal changes.** Autumn and spring are periods of increased vegetative lability.
- **Long sitting indoors.** Physical inactivity + vitamin D deficiency + lack of vestibular stimulation.

Strategy “Storm”: algorithm for surviving relapse

How to wait out a vegetative storm, the three-day rule and the therapeutic power of “Letters to yourself” in moments of falling.

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1. Essence and issues: panic during a storm

When a pullback hits, your brain instantly goes into disaster mode. A wave of thoughts covers you: * “Nothing works! Everything is gone! I will never be cured!”*.

In a panic, you begin to take chaotic actions: feverishly do gymnastics, meditate, call loved ones, frantically search for new symptoms on Google and drink sedatives. You try to bail water out of a sinking boat with a leaky bucket while running around the deck during a storm. But these throwing only intensify the pumping, throwing new portions of adrenaline into your already inflamed nervous system.

The storm cannot be defeated by force or vanity. You can only wait it out.

2. Physiological explanation: the law of conservation of energy of the central nervous system

Neurological backlash is an inflamed, hypersensitive state of your autonomic system. Your brain is temporarily exhausted. If at this moment you begin to get angry, panic and demand immediate recovery from yourself, you are forcing the exhausted system to work at its limit.

It's like trying to drive an overheated engine with the pedal to the floor.

In physiology, this period requires a severe reduction in stimulation. The brain must receive a signal of absolute safety in order to turn off the “fight or flight” mode and start self-regulatory processes.

To do this, we use the **3-day rule** - the period of maximum reduction in external and internal load. We need to remove all the “irritants” that feed the amygdala and give the body a physical resource to restore autonomic balance.

3. Cognitive Traps: “I need to fix myself immediately.”

Main cognitive trap: * “If I relax and lie down now, I will admit defeat and the disease will win. I have to fight!”*.

Fighting symptoms is neurosis. Your resistance, anger, and attempts to “fix it now” create 90% of the tension during a rollback. Remember: **you cannot force the vegetative system to calm down with the**

power of thought. But you can stop preventing her from recovering. Your task during a storm is not to fight the waves, but to drop anchor and drift.

4. Point of Support: Insight

> *The captain of a ship during a storm does not try to bail out the ocean with buckets and does not shout at the waves. He removes the sails, battens down the hatches, throws a reliable anchor and waits for the elements to subside. Do the same. The storm will end because it always runs out of energy. Your task is simply to wait it out.*

---

5. Practice: Storm Protocol

When you are hit by a strong pullback, immediately turn on this algorithm for **3 days**:

Why exactly 3 days?

The physiology of a vegetative storm follows a predictable pattern:

- **Day 1:** Amygdala is on full alert. Adrenaline and cortisol are at their peak. Maximum instability. The brain screams: *"We are dying!"*. Any attempts to "fix yourself" increase panic.
- **Day 2:** If you didn't add any more fuel (googling, panicking, trying to "do Jacobson 5 times in a row"), your cortisol levels begin to drop. Adrenaline receptors are desensitized. The unsteadiness remains, but the panic subsides. The first "windows" of clarity appear, lasting 15–30 minutes.
- **Day 3:** Vegetation begins to return to the background. Shakiness is reduced by 30–50% of peak. Sleep improves. The brain switches from "fight" mode to "wary observation" mode.

Three days is the minimum biological cycle during which the body utilizes excess stress hormones and reboots the vegetative balance. Don't expect instant results. But don't stretch the "gentle regime" for weeks - otherwise it will become an avoidance.

Step 1. Three Day Rule

1. **Double your body practice:** Do Jacobson relaxation 2 times a day (in the morning and before bed). Use the Kuznetsov applicator. Take a warm shower on your neck. We need to release physical tension.
2. **Information vacuum:** For 3 days, completely delete social networks, do not read the news and close all chats. Your brain is now an open wound. Don't pour the salt of other people's emotions and anxieties into it.
3. **Energy saving mode:** Lower the bar of requirements for yourself to zero. Is the house not cleaned? Okay. Dumplings for dinner? Wonderful. Failed an unnecessary task at work? The world won't collapse. Your only goal for these 3 days is to simply live them comfortably.

4. **Prohibition of evaluation and guilt:** Stop checking every 5 minutes whether you are pumping or not. Tell yourself: * “Yes, I’m shaking. This is a rollback. I accept this. We will remain in this state for several days, it will pass.”* Don't blame yourself for the decline - you didn't make a mistake, it's a normal biological process.

Do/Don’t Memo

✅ DO	❌ DON'T
Warm shower on the neck 2 times a day	Google new symptoms
Jacobson morning and evening	Do Jacobson 5 times a day “to be on the safe side”
Walk outside for 20–30 minutes at a calm pace	Run 10 km “to burn off the adrenaline”
Go to bed an hour earlier	Scrolling through your phone until 2am
Eat warm food, drink water	Drink 3 cups of coffee “for vigor”
Tell loved ones: “I need peace now”	Calling friends and discussing symptoms for hours
Watch a light series or listen to a podcast	Read neurotic forums
Accept: “This will pass in 2-3 days”	Screaming: “Why am I feeling bad again?!”

Step 2. Schedule Day 1 (hardest day)

Here is a specific checklist of actions for the first day of the storm:

Morning (7:00–10:00):

- When you wake up, don’t check your head. Stand up. Drink a glass of warm water.
- Contrast shower (30 seconds warm → 15 seconds cool × 3 cycles).
- Light breakfast. No coffee - replace it with herbal tea or cocoa.
- Relaxation according to Jacobson (20 minutes).

Day (10:00–18:00):

- Go outside. Slow walking for 20–30 minutes. Horizon Rule. No headphones.
- Do a simple routine (cook a meal, organize a shelf, watch a movie).
- For each wave of anxiety - Emergency card from Appendix A.
- Do not engage in dialogue with thoughts. “Detached observation” technique.

Evening (18:00–22:00):

- Warm shower on neck and shoulders (5 minutes).

- Kuznetsov applicator (15 minutes).
- Relaxation according to Jacobson (20 minutes). *No screens an hour before bed.
- Go to bed before 23:00.

Step 3. Letter to yourself in case of a rollback

When you are in a panic, your brain, under the influence of adrenaline, lies to you, drawing pictures of incurability. You need an authoritative opinion. And the best authority for you should be **you yourself are in a calm state**.

Here is an example of such a letter - rewrite it in your own words while you are calm:

> *"Hi, it's me—you're fine. I know that right now it seems to you that everything is lost. That the treatment doesn't work. That you'll never get better. But I want to remind you of something:*
>
> *Three weeks ago you went to the store without panicking. You slept fine. You laughed. It was real. I didn't dream about it. And it will happen again - because the neural connections that you built have not gone away. They are simply temporarily drowned out by adrenaline.*
>
> *Now you are in a storm. The storm lasts 2–3 days. No more. Because the adrenaline is running out of fuel. Don't add firewood—don't Google, don't panic, don't try to "fix" yourself. Just wait it out.*
>
> *I believe in you. Because I am you. And I know what we are capable of."*

Open the template 4.2_setback_letter.pdf and rewrite it by hand. Save to favorites on your phone.

At the moment when you feel like "everything is broken," take out your phone and read this letter **out loud**. Trust your calm self, not your panicking self.

Homework (DZ):

1. **Write your personal "Kickback Letter"** on a piece of paper or in your phone notes right now, while you are calm. Include 3 specific facts about your progress (what you were able to do last week that you couldn't do a month ago).
2. Print or handwrite the Do/Don't chart and hang it on your refrigerator.
3. Apply the "Storm" protocol at the first sign of deterioration in health. Record in your notebook what day after the start of the protocol you were released.

New personality and architecture of boundaries: who am I without my illness?

The existential impasse of recovery, why the old personality led to neurosis and how to build boundaries to protect your health.

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1. Essence and issues: the existential vacuum of recovery

When the symptoms of PPPD finally begin to subside, many patients are suddenly faced with an unexpected and frightening problem - an **existential crisis**.

The disease took up 80% of your time and thoughts. You woke up with the thought of being unsteady, planned your day based on your symptoms, discussed your health with loved ones, googled and sat on forums. Illness was your main occupation, your topic of conversation and your identity. You are used to being "the one who is sick and trying to get better."

And so, when the pitching goes away, a ringing emptiness forms inside. A terrible, unconscious question arises: * "Who am I without my illness? What should I talk to people about now? How to fill the free 8 hours of attention a day? Who am I now?

It turns out that being healthy is unusual and even scary. The brain is afraid of this emptiness and may unconsciously try to bring the symptoms back, just to return to the familiar, understandable rut of the "struggle for survival."

2. Physiological explanation: neural identity shift and the cost of polite silence

Recovery from neurosis is not a return to your "old self." This is physically impossible.

That old personality of yours - with all its perfectionism, inability to refuse, habit of tolerating toxic people, sacrificing oneself for the sake of other people's interests and silently swallowing grievances - led you to a dead end where the fuses of the autonomic system burned out. If you try to recover and remain the same reliable "comfortable person", your nervous system will very quickly return you to PPPD.

At the level of neurophysiology, boundaries are a mechanism for protecting your central nervous system from overload.

Every time you accept overtime work you don't want to do, or listen to an hour-long complaint from a toxic friend, your amygdala becomes aroused. You suppress anger and irritation (because you "have to

be good"). Suppressed emotions cause muscle spasms in the neck and diaphragm. Neck spasm cuts off normal blood flow and receptor signals ➔ hello, dizziness.

Building strict personal boundaries is not selfishness, it is **hygiene of your autonomic system**. Your boundaries are a fence around your "video card" that prevents it from overheating from other people's tasks.

3. Cognitive Traps: "If I say no, I will be rejected."

Main cognitive trap: * "I have to justify myself when I refuse. If I just say "no" without a long explanation, people will think I'm selfish, stop loving me and turn away."*

Attempts to soften the refusal lead to the fact that you begin to come up with complex excuses, lie, or end up agreeing, betraying yourself. Remember: **your "no" has value in itself**. You don't need a neurologist's certificate to avoid going to painful family gatherings. You don't have to make excuses to your boss for not working on your legal days off. If people only value you when you are "convenient" and fulfill their requests at the expense of your health, then this is not a relationship, this is exploitation.

4. Point of Support: Insight

> *You can't get better by staying the same. The person you were led you to neurosis. Recovery is not about mending an old fence. This is the demolition of an old rotten house and the construction of a new building with strong walls and locks on the doors. Learn to close doors.*

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5. Practice: Setting strict vegetative boundaries

We move on to the practice of building a new identity and protecting our resources.

Step 1. Declaration of boundaries

Write a list of 4 strict rules that you introduce to protect your nervous system from overload. Remember their wording:

1. **Professional boundaries:** *"I do not work on weekends and do not respond to work messages after 19:00"*.
2. **Family boundaries:** *"I do not go to events that are physically draining and cause anxiety, even if relatives will be offended"*.
3. **Information boundaries:** *"I don't listen to other people's toxic complaints about life for more than 10 minutes. My resource is limited."*

4. **Disease Limits:** *"I no longer discuss my symptoms and frailty with loved ones every day. This feeds my neurosis."*

Step 2. Practice "One Inconvenient Situation"

Your task this week is to complete one border protection action:

- Refuse a request that is difficult or unpleasant for you to fulfill (colleague, friend, relative).
- Make the refusal **short and without excuses**. For example: * "No, I won't be able to help with this on Saturday. I have other plans"* (even if your plan is to just lie on the couch).
- **Bear the guilt.** When you say no, a wave of guilt will rise within you. This is normal - the old neural track resists. Don't run to save the situation. Take a breath and exhale. The feeling of guilt will physiologically subside in 20 minutes, and the feeling of your own strength and preserved vegetative energy will remain with you.

Homework (DZ):

1. Write down in a notebook **5 qualities of the "old personality"** and **5 qualities of the "new personality"**. For each quality, write a specific action that confirms the new identity.
2. Complete the Balance Wheel (below).
3. Set one boundary in each key area of your life.

Manifesto of a new identity

Write a 1-page document - this is not an affirmation or a dream. This is **engineering specification for the new you**. Format:

```
MANIFESTO OF A NEW IDENTITY

Date:_____

I, [name], declare the following:

1. I am a person who [specific quality + specific action].
Example: "I am a person who does vestibular exercises every day
and doesn't miss a single day, even if he feels well."

2. I am NOT [specific old behavior] anymore.
Example: "I no longer Google symptoms after 9:00 pm."
```

```
3. My boundaries: [3 specific rules].
Example: “I don’t take work calls after 7:00 p.m. I refuse
meetings that don't bring me joy. I sleep at least 7 hours."

4. My superpower, born from PPPD: [what did you get].
Example: “I learned to hear my body. I became more aware than 90%
people around. I know my triggers and I know how to work with them.”

5. My vow: [one sentence].
Example: “I will never go back to living on autopilot.”

Signature:_____
```

Fill it out. Hang it on the wall. Re-read once a week.

Wheel of Balance

It is a visual tool for assessing the current state of life. Draw a circle and divide it into 8 sectors:

Sector	Score (1–10)	What needs to change
Health/Body	_	
Career/Job	_	
Finance	_	
Relationships/Family	_	
Friends/Society	_	
Personal growth	_	
Hobbies/Pleasures	_	
Spirituality/Meaning	_	

Give an honest rating from 1 to 10 in each sector. Connect the dots. The resulting figure is your “wheel”. If it looks like an even circle, you are in balance. If it’s on a star, it means that some spheres are sagging and pulling the rest along with them.

Rule: PPPD almost always lives in “sagging” sectors. If “Health” is a 3 and “Work” is a 9, you’re on fire. If “Relationships” is 2, it means that unspoken grievances are accumulating in the muscles.

Boundaries by spheres

For each “sagging” sector of the Wheel of Balance, set **one specific boundary**:

- **Work:** *“I do not respond to management messages after 19:00”*.
- **Relationships:** *“I tell my partner when I need time alone instead of silently getting angry.”*
- **Society:** *“I refuse meetings that do not bring me joy, without explanation”*.
- **Health:** *“I go to bed before 23:00, even if I haven’t finished watching the film”*.
- **Finance:** *“I stop spending money on more “miracle drugs” and “healers””*.

Boundaries are not selfishness. This is infrastructure. Without it, any system will fall sooner or later.

Breaking the Recovery Plateau: The "Good Enough" Trap and Fear of Living Fully

The acute hell is behind you: you go to work, shop at grocery stores, and no longer call ambulances. Yet you are stuck in the gray swamp of a "tolerable life" — afraid of losing sleep, drinking a cup of coffee, boarding a flight, hitting the gym, or changing careers. Here is how to shatter the glass dome and reclaim 100% of your life.

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1. The Swamp of "Eighty Percent"

There is a deceptive fork in the road during PPPD recovery that clinicians rarely warn you about.

When your symptoms were a 10 out of 10, you weren't living — you were surviving. You held onto walls just to walk to the kitchen, cried from exhaustion, and were willing to do whatever it took to recover. You committed to the system, practiced your vestibular drills, walked outside daily, and dismantled your cognitive distortions.

Two or three months passed. The acute storm subsided. Your baseline returned to **75–80% of normal**.

And right here, something strange happens: **you stop pushing**.

You sign an unspoken non-aggression pact with your neurosis:

> *"Listen, let's make a deal: I will live quietly, go to bed strictly at 10:30 PM, avoid coffee, skip loud concerts, never board airplanes, avoid ambitious projects, and never get overly stressed. In return, you only rock me at a mild 2-out-of-10 background level so I can get by. Deal?"*

You become a **person living in bubble wrap**. Technically, you are no longer housebound — you commute to the office and visit the neighborhood grocery store. Yet you are not fully alive either — you treat yourself like a cracked porcelain vase that might shatter if shaken too hard.

Clinically, this is the **Recovery Plateau**. And if you linger here, this "low-power retirement mode" becomes a voluntary lifelong cage.

---

2. The Neurobiology of the "Porcelain Vase": Why Overprotection Preserves Symptoms

Why does shielding yourself from every stressor fail to cure the final 20% of dizziness — and instead preserve it in amber?

The answer lies in the laws of **neural adaptation and homeostasis**:

1. Shrinking the Window of Tolerance

The human nervous system adapts to load just like muscles or the immune system.

- If you lock a healthy person inside a sterile bubble with zero pathogens, within six months their immune system atrophies so severely that a common cold becomes life-threatening.
- When you artificially strip your life of elevated heart rates, occasional late nights, intense emotions, caffeine, travel, and physical fatigue, **your amygdala's alarm threshold drops to the floor**.

Your nervous system becomes fragile and greenhouse-grown. The slightest deviation from your sterile routine (staying up an hour late, riding two extra subway stops, strong wind outside) registers in your brain as an emergency — triggering an immediate setback.

2. Novelty Dopamine Starvation

A predictable, monotone routine along the track of "home — safe office — couch" starves your brain of the primary neurotransmitter of drive and neuroplasticity — **dopamine**. When your life lacks challenge, excitement, travel, passion, and novelty, your under-stimulated prefrontal cortex defaults back to its old habit out of sheer boredom — **scanning your vestibular baseline** (*"How much am I swaying today? Is it a 1 or a 2?"*).

Life Dimension	"Porcelain Vase" Mode (80% Plateau)	"Antifragile" Mode (100% Recovery)
Self-Concept	"I am a fragile patient in remission; I must not overdo it"	"My nervous system is healthy and built to handle full load"
Response to Poor Sleep / Fatigue	Panic: "I only slept 5 hours, today is going to be awful!"	Indifference: "Slept poorly — no big deal, I'll sleep great tonight"
Geography & Plans	Living within a 3-mile radius, declining trips and social nights	Flights, road trips, and spontaneous plans without checking symptoms
Physical Exertion	Cautious walking "just so my heart rate doesn't spike"	Heavy lifting, running, swimming, dancing at full intensity

Life Dimension	"Porcelain Vase" Mode (80% Plateau)	"Antifragile" Mode (100% Recovery)
Brain Outcome	Preserving the "chronic patient" identity for years	Complete erasure of the PPPD memory trace

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3. Five Forbidden Zones That Scare People on the Plateau

Audit yourself against this checklist. If you recognize yourself in even two of these points, you are trapped in the "Good Enough" plateau:

1. **Fear of Sleep Loss & Routine Disruption:** You panic if you stay up past your bedtime. If you wake up at night and get 5 hours of sleep instead of 8, you wake up bracing for dizziness all day.
2. **Fear of Stimulants & Physiological Shifts:** You are terrified to drink an espresso, have a glass of wine at a dinner party, eat spicy food, or step into a hot sauna because *"my blood vessels will dilate, my heart rate will rise, and the spinning will start."*
3. **Fear of Normal Physical Exhaustion:** You are afraid of getting tired! Normal muscular fatigue after a workout, yard work, or a long day is mislabeled by your brain as a "relapse."
4. **Fear of Distance (Vacation Paralysis):** Traveling 500 miles from home, boarding a plane, or hiking in the mountains away from hospitals feels reckless: *"What if I get a flare-up there and ruin everyone's trip?!"*
5. **Ambition Freeze:** You postpone changing jobs, launching a business, moving cities, or dating: *"Not right now, I need to protect my nervous system first."*

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4. The Concept of Antifragility: Why Your System Needs Controlled Chaos

Essayist and mathematical statistician **Nassim Nicholas Taleb** coined a foundational concept — **Antifragility**.

- **The Fragile** (a porcelain teacup) shatters under stress.
- **The Robust** (a cast-iron weight) remains unchanged under stress.
- **The Antifragile** (a living nervous system, bones, muscles, immunity) **grows stronger, more calibrated, and more resilient** when exposed to volatility, load, and controlled stressors!

Your vestibular system and autonomic nervous system are inherently **antifragile**.

To burn off the final 15–20% of lingering background haze and rocking, you must stop coddling yourself and start **deliberately shaking up your sterile comfort zone**. Your brain needs undeniable proof that

you no longer grant it special accommodations for “past PPPD.”

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5. The “Ceiling Breaker” Protocol: 4 Steps from 80% to 100%

Once the acute phase is behind you and you hit the plateau, flip the chessboard. Here is your roadmap back into the ranks of the fully healthy:

Step 1. Officially Close “Project PPPD”

As long as you read dizziness forums daily, watch videos about symptoms, and introduce yourself as “recovering from PPPD,” you remain a patient.

- Set an official graduation date.
- Demote vestibular exercises from a “medical treatment” to a quick 5-minute morning warm-up, and shift your primary focus to real athletics (weightlifting, running, swimming, martial arts, dancing).

Step 2. Stress Inoculation (Deliberate Rule-Breaking)

Stop living like a nursing home resident. Once or twice a week, expose your nervous system to controlled physiological provocations:

- Drink a cup of real coffee at a café and go for a brisk walk, letting your heart beat faster without monitoring it.
- Use a sauna or take contrast showers to retrain your brain to treat vasodilation and temperature shifts as normal pleasure rather than danger.
- When you have a night of poor sleep — **deliberately live the next day at 100% activity**, with zero naps and zero self-pity. Prove to your amygdala: *“Even on 5 hours of sleep, I am strong and capable.”*

Step 3. Geographical Expansion

Snap the leash tethering you to your neighborhood.

- Take a weekend trip to another city.
- Book the flight or train ticket you have been putting off since your symptoms began.
- Yes, during the first 30 minutes at the airport or train station, anticipation might make you sway at a 5 out of 10. Smile at it: *“Good — my brain is expanding its safety map.”* Two hours after arrival, novelty dopamine will wash the residual anxiety away.

Step 4. Reclaiming Big Ambitions (Occupying the Future)

Neurosis feeds on an empty calendar. When you lack a compelling, larger-than-life goal in the real world, your brain makes “monitoring balance” your full-time career. Reignite healthy expansion:

- Sign up for a skill you put on hold (martial arts, dancing, driving, a foreign language, snowboarding).

- Pursue a career leap or launch the project you've been delaying.
- Do what **excites you**, without asking your vestibular system for permission.

💬 Core Insight

> *"You cannot recover 100% while continuing to live like a cautious semi-invalid. The final 20% of dizziness does not disappear from exercises or rest — it burns away in the furnace of a bold, messy, full-throttle life. Stop saving yourself for illness — start spending yourself on living."*

📝 Homework Assignment (Action Plan)

1. **The Postponed Life Inventory:** Write down 5 specific experiences, pleasures, or trips you have avoided during PPPD under the excuse of *"playing it safe"* (coffee, intense gym workouts, concerts, flights, saunas, career moves).
2. **Operation Shatter the Dome:** Pick 2 items from that list and execute them within the next 7 days with zero safety precautions.
3. **Information Graduation:** Cut your consumption of illness forums, symptom chats, and health articles down to zero. Fill 100% of that freed mental bandwidth with real-world goals, athletics, work, and living human beings.

Stepping Into Life: Your Inner Point of Support

A final pep talk on why a "slight rocking" when you're tired is your friend, not a disease, and how to shift your locus of control inward.

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1. The Core Problem: Baby Bird Syndrome

So here you are at the end of this guide. You've done an enormous amount of work: rebooted your body, learned how to break out of adrenaline loops, switched off the inner scanner, and sorted out every area of your life. Your dizziness and anxiety scores have dropped significantly.

But on the threshold of your new life, you might develop **baby bird syndrome** — the fear of falling out of the nest.

You catch yourself thinking anxious thoughts: *"What if the book ends and I'm left alone with my own head? Where do I find support? What's my crutch now that I can't lean on the author's authority?"* You're looking for an external guarantee, a magic seal of approval confirming you're healthy forever. But external guarantees don't exist. The only guarantee now lives inside you.

2. The Physiology: Internal Locus of Control and the Overload Sensor

Recovery from PPPD happens when you shift your **locus of control** (where you place responsibility) from the outside to the inside.

While you were running from doctor to doctor looking for a "magic pill," your locus of control was external. You felt like a helpless victim of circumstance. But now you've become your own neurologist, psychotherapist, and coach. You know the physiology of your symptoms inside and out. You know how your brain works, why your neck goes into spasm, how the VOR reflex operates, and how adrenaline burns off in 5 minutes. You're no longer a "patient." You're an engineer of your own nervous system who found a bug in the software and rewrote the code.

Many people who have fully beaten PPPD and returned to a full life — traveling, exercising, working — sometimes still experience a mild **"rocking"** or micro-symptoms when they're severely overtired.

This is not a sign that the disease is coming back. It's a **healthy physiological response** of the autonomic nervous system to overload.

When a regular person is sleep-deprived, they get a headache or an upset stomach. For you — someone who's been through PPPD — the overload sensor now manifests as a slight feeling of unsteadiness. It's your built-in indicator: *"Hey meatbag, we haven't slept in 2 days and drank a bucket of coffee. Time to go to bed."* This sensor is your ally, protecting you from burning out again. There's no reason to fear it.

3. Cognitive Traps: The Search for Eternal Stability

The main cognitive trap: *"A healthy person never experiences dizziness, anxiety, or fatigue. As long as I have the slightest hint of symptoms, I'm sick."*

This is perfectionism applied to health. Healthy people sway in the wind, get dizzy at heights, get tired, get angry, and feel anxious before important events. Being alive means being dynamic, experiencing fluctuations in energy and emotion. Recovery doesn't mean turning into a liquid-metal Terminator who feels no pain. Recovery means being able to live a full life *alongside* your physiological responses, without making a drama out of them.

4. Point of Support: The Insight

> *I didn't give you a magic pill — no such thing exists. But I gave you something far more powerful — UNDERSTANDING. You're no longer a blind kitten bouncing between clinics. You know the lay of the land. You know where the potholes are and how to avoid them. Go live your life. Fall down, get back up, make mistakes, but keep moving forward. Life is waiting for you beyond this screen.*

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5. Practice: Logging Out of the System

We're wrapping up the program and handing the controls over to you.

Step 1. Final Assessment

Take the HADS and DHI tests again. Compare the results with the scores you recorded at the start of your journey (in Chapter 3). Document your progress mathematically. The cold, dry language of numbers is the best proof for your frightened brain that the system works.

Step 2. A Thank-You Letter to Your Body

Your body endured enormous stress. It swayed and spun not to hurt you, but to protect your psyche from overheating. Write your body a physical thank-you letter. Thank it for its endurance, for weathering your anxiety, your hypervigilance, and your endless internal debates. Promise it that you'll treat it with care and maintain proper autonomic hygiene.

Step 3. A Manifesto for Your New Life

On a separate sheet of paper, write 5 rules you're going to live by from now on. These aren't affirmations — this is your constitution. Here's an example:

1. *I will not endure things that destroy me just to appear "normal."*
2. *I have the right to rest without guilt.*

3. *My needs are no less important than everyone else's.*
4. *A slight unsteadiness when I'm tired is my sensor, not my enemy.*
5. *I will never google symptoms again.*

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6. SOS Card: Action Plan for When Symptoms Return

Print this card. Put it in your wallet, stick it on your fridge, save a screenshot on your phone. You'll need it — not because you're guaranteed to relapse, but because in six months you'll forget the algorithm. And panic is a terrible teacher.

Situation: You've been feeling fine for 2–3 months, and suddenly the unsteadiness, anxiety, or brain fog is back.

Step 1. Don't panic. Don't google. Put the phone down. Seriously — put it down. You already know what Google will show you. You know that forums will drown you in other people's fear. You've been through 23 chapters of this book. You're not a beginner. You're an engineer who saw a yellow warning light on the dashboard — not a red one.

Step 2. Check the hardware. Ask yourself three diagnostic questions:

- *Have I been sleeping properly the last 3 nights?*
- *Have I been overloaded at work or in life?*
- *Have I overdone it with caffeine, alcohol, or screen time?*

In 90% of cases, the answer to at least one of these questions is "no." There's your bug. Not in your head, not in your vestibular system — in your daily routine.

Step 3. Reboot the system.

- Do a full Jacobson Progressive Muscle Relaxation session (Chapter 5).
- Go to bed before 11 PM.
- Cut out coffee for 3 days.
- Go for a 30-minute walk in fresh air.

Step 4. Wait 48 hours. Don't evaluate how you're feeling every 15 minutes. Give your body two full days to reboot. In most cases, the yellow warning light will turn off on its own within 48 hours.

Step 5. If it hasn't passed after 48 hours. Reread Chapter 20 (Anatomy of a Setback) and Chapter 21 (Storm Strategy). They contain the full algorithm for dealing with a prolonged setback. You've done this before — your hands remember.

Step 6. If it hasn't passed after 2 weeks. Only now does it make sense to contact your therapist or doctor. Not sooner. Two weeks is the minimum timeframe to distinguish a normal setback from a

situation that requires outside help.

Remember: you haven't gone "back to square one." You're just standing on the platform waiting for the next train. The tracks haven't gone anywhere. Get on and ride.

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7. Check-In Protocol: Follow-Up Assessments at 3, 6, and 12 Months

In software development, there's a concept called "regression testing" — you periodically run old tests to make sure new code hasn't broken anything. Your nervous system deserves the same approach.

3 months after stabilization

Take the DHI and HADS tests again. Record the results.

- **DHI below 14, HADS below 7/7** — you're in the green zone. Keep living. You can reduce your practices to 3–4 times a week instead of daily, but don't quit altogether.
- **DHI 14–30, HADS 8–10** — yellow zone. You're not declining, but something is eating into your resources. Look at sleep, stress, boundaries. Bring back daily Jacobson PMR.
- **DHI above 30, HADS above 11** — red zone. Something has gone seriously wrong. Reread Chapters 17, 20, and 21. If your scores haven't improved after a month — see a therapist.

6 months

Repeat the assessment. By this point, most people's scores are stably in the green zone. If you're there — congratulations. You can switch to maintenance mode: Jacobson PMR twice a week, cardio three times, mindfulness — by habit.

If your scores are bouncing around — don't beat yourself up. Look at what's changed in your life over the past six months. Often the cause isn't the "disease coming back" but a new stressor you haven't processed yet.

12 months

Final regression test. If your scores are stable — you can officially delete PPPD from your list of problems. You've completed the full cycle. Program closed. But the log files remain — they're called "experience."

Write down all the results in your notebook. In three years you'll open those notes and won't recognize the person who started this book. And that will be the best gift you've ever given yourself.

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8. Decision Tree: When to Actually Worry

I know what you're thinking. *"What if one day it's not the old bug, but actually something new?"* Fair question. Let's break it down.

Familiar pattern — don't worry

If you feel the same things you felt during PPPD — foggy head, mild rocking, the "drunk walk" sensation, brain fog, anxiety, derealization — and it coincides with stress, sleep deprivation, or a cold, then there's a 99% chance it's the old program. You know it by sight. It hasn't changed. It's not going to turn into something else.

It's like a computer virus you've already removed twice. You know its signature. You know what it does. And you know the antivirus. Pull out your SOS card and follow the algorithm.

New symptom — observe

If something appears that's *never* happened before — for example, a shooting pain in your ear, numbness on one side of your face, sudden hearing loss — don't panic, but don't ignore it either. Observe for 3–5 days. If it passes — most likely nothing serious. The body sometimes throws one-off glitches — that's normal for any person, not just those of us who've been through this.

Red flags — see a doctor immediately

Here's the list of situations where you need to actually call a doctor, not read books:

- **Sudden hearing loss in one ear** — this could be acute sensorineural hearing loss, and every hour counts.
- **Double vision** (not blurriness, but actually seeing two images) — a neurological symptom that PPPD does not cause.
- **First-ever rotational vertigo** with nausea and vomiting lasting hours — this isn't PPPD; it could be BPPV, vestibular neuritis, or something else that requires examination.
- **Impaired speech, swallowing, or coordination**, limb numbness — call an ambulance, no discussion.
- **Sudden, most severe headache of your life**, unlike anything you've ever experienced — don't tough it out, don't wait.

Remember a simple rule: PPPD doesn't create *new* symptoms that you've never had before. PPPD replays the same familiar tape. If an unfamiliar tune starts playing — that's a reason to check whether someone swapped the disc.

But. Ninety-five times out of a hundred, it's the old tape. So don't rush to the ER every time you sway a little. Use your brain, not your panic.

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9. An Emotional Farewell: From the Author

I need to tell you something. Not as the author of a book, but as a person who was exactly where you are.

My darkest moment came in the eighth month of the illness. I was lying on the floor in the hallway — literally on the floor, because I couldn't stand, and walking to the couch felt terrifying: it seemed like the walls were swaying and I'd fall on the way there. My wife was at work. My kid was at daycare. I lay on the cold laminate flooring and stared at the ceiling. And one thought kept spinning in my head: *"This is forever. I'll never be normal. I'll be a burden for the rest of my life."*

I know what it's like to google symptoms at three in the morning. To read forums where people write "nothing has helped, I've been suffering for 7 years," and feel the ground disappear from under your feet. I know what it's like to call an ambulance for the fifth time in a month, while the paramedic looks at you with barely concealed fatigue because all your tests are normal but you're "dying" again. I know what it's like to be afraid to walk to the store for bread. I know what it's like to ride the subway gripping the handrail so hard your knuckles turn white.

I went through all of it. Every twist of that hell. And I came out the other side.

Not because I'm special. Not because I got lucky. But because at some point I stopped looking for a wizard to fix me and started fixing myself. Piece by piece. Screw by screw. Every single day.

And you know what the hardest part was? Not the exercises. Not the meditation. Not giving up coffee. The hardest part was believing it could actually end. When you've been drowning in a swamp for months, hope feels like mockery. Someone tells you "It'll pass!" — and you want to punch that person, because they don't understand. They've never been in the swamp.

I've been in the swamp. And I'm telling you: it ends. Not in a day, not in a week. But it ends. The brain is a plastic organ. It rewrites its programs. I'm the living proof.

Now I travel, exercise, work, live a full life. There are days when, after severe sleep deprivation, I feel a slight unsteadiness. I smile and say to myself: *"Hey there, old friend. Time to rest."* And I go to sleep. And by morning it's gone. And that — that is freedom.

I wrote this book so you wouldn't lie on the floor for eight months like I did. So you'd walk this path faster, with a map and a flashlight, instead of groping around in the dark. I poured everything I know into these pages. Everything that helped me. Everything that has helped hundreds of people I've worked with.

Now it's yours. Go.

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10. A Letter to Your Future Self

This is possibly the most important assignment in the entire book. And the simplest in form.

Sit down. Grab some paper. Not your phone — paper. And write a letter to your future self. The version of you who, six months or a year from now, will have a rough day and suddenly feel that familiar rocking and panic that "it's all back."

Write to that person from your current self — the one who just completed 23 chapters, who understands the physiology, who knows how neural pathways work and why adrenaline burns off in 5 minutes.

Here's a rough skeleton. But write it in your own words — other people's words don't work in a moment of panic. Only your own do.

"Hey. If you're reading this, you're having a bad time right now. The rocking is back, or the fog, or the anxiety. I know what you're feeling. I felt the same way when I wrote this.

Here's what I need you to remember:

1. This isn't a new disease. It's an old bug you've already fixed. And you know how.

2. Check your sleep, stress, caffeine. One of them is the culprit.

3. Do your Jacobson PMR. Go to sleep. Wait 48 hours.

4. You went through this once and came out the other side. You'll come out again.

5. Don't google. Don't call an ambulance. Don't read forums. Just live through these two days.

I believe in you. Because I am you. And I know what you're capable of."

Seal the letter in an envelope. Write on the front: **"Open only during a setback."** Put it in a desk drawer. And forget about it.

If in a year you never need to open it — great. If you do — it will work better than any pill. Because inside that envelope won't be some stranger's words from the internet, but the voice of the person who knows you better than anyone in the world. Your own.

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11. Gratitude for the Journey: PPPD as a Catalyst for Growth

I know the following words will trigger resistance in you. Read them anyway.

PPPD was the worst thing that ever happened to you. And at the same time — one of the best.

Wait. Don't throw the book away. Let me explain.

Before the illness, you were living on autopilot. Enduring things that shouldn't be endured. Staying silent when you needed to scream. Smiling when you wanted to cry. Working until you dropped. Ignoring your body, your feelings, your needs. You were convenient for everyone — except yourself.

Your nervous system was screaming at you: "Stop! Enough! I can't take any more!" — and you shut it up with caffeine, willpower, and the phrase "everyone lives like this." So it pulled the emergency brake. That's what PPPD is — an emergency brake that shut down a system you'd been redlining without oil.

Thanks to the illness, you did things you never would have done voluntarily:

- You learned how your nervous system works — better than 99% of people.
- You learned to listen to your body instead of ignoring it.
- You built boundaries and stopped being "convenient for everyone."
- You understood that life is finite and fragile, and you started appreciating ordinary days.
- You became stronger — not physically, but internally. You survived something most people can't even imagine.

You know the old saying — "every cloud has a silver lining"? That's you. You didn't just recover. You became a different person. More aware. More honest with yourself. More alive.

PPPD isn't a prison sentence you served. It's a university you graduated from. With honors. Because you didn't give up.

Life After PPPD: What to Expect

First month after stabilization

You'll notice that you think about dizziness less and less. An entire day might pass without you remembering it at all. Then in the evening you'll catch yourself: *"Wait, I didn't think about unsteadiness all day!"* — and that realization itself might trigger a bit of anxiety. This is normal. Your brain got used to "sentry mode" and isn't used to switching off the scanner.

What to do: Nothing. Just note to yourself: *"This is what recovery looks like. I felt good and I forgot about it. That's the whole point."*

Second month

Forgotten desires will start coming back. You'll want to go somewhere, meet up with friends, go to a restaurant, buy new clothes. This is a wonderful sign — your limbic system has turned off survival mode and switched to living mode. Your brain has stopped hoarding resources for "escaping death" and started channeling them toward pleasure.

Possible trap: You might start doing "too much too fast" — walking for 5 hours, working out every day, staying out late with friends. And a week later, you get a setback. This isn't a catastrophe. It's calibration. Your brain is showing you the new acceptable load range. Just dial it back for a couple of days — and carry on.

Third month and beyond

By this point, PPPD stops being the central theme of your life. You'll remember it the way you remember a bad flu — *"Yeah, it was rough. But it passed."* You'll notice you've become a different person: more mindful, less anxious, better at understanding your body and your limits.

Many people who've been through PPPD develop a natural desire to **help others** — to share their experience, to support those who are just starting out. This is a healthy impulse. But be careful: don't turn helping others into a new form of hypervigilance. You're not obligated to save the world. You're obligated to live your life.

About Residual Symptoms

Let's go over this one more time, because it matters.

For many people, after fully recovering from PPPD, **a mild "overload sensor" remains**. It can show up as:

- A slight feeling of unsteadiness during severe sleep deprivation.
- Micro-swaying during high stress (a conflict at work, an exam, a flight).
- A fleeting "floating" sensation during a cold or when you're sick.

This is **not a PPPD relapse**. This is a normal autonomic response specific to your body when it's overloaded. Some people get headaches under stress, some get stomach cramps — you get a gentle swaying. It's your personal "fuel gauge."

The correct response:

1. **Don't be scared.** *"Oh hey, familiar feeling. Means I've been overdoing it."*
2. **Run the protocol:** go to bed early, cut the coffee, do your Jacobson PMR.
3. **Move on with your life.** It'll pass in 1–2 days.

Homework:

1. **Take the tests, compare the results**, and write down the difference in your notebook.
2. **Write and read aloud your "Thank-You Letter to Your Body."**
3. **Write your Manifesto** — 5 rules for your new life. Put it somewhere visible.
4. **Print the SOS card** from Section 6 and put it in your wallet.
5. **Write the "Letter to Your Future Self"** (Section 10). Seal it in an envelope. Put it in a drawer.
6. **Set 3 reminders** on your phone: at 3 months, 6 months, and 12 months — "Take DHI and HADS, record results."
7. Close this book. Step outside, lift your eyes to the horizon, and take the first step into your new, real, steady life.

Relatives and PPPD: instructions for those who are nearby

How PPPD changes a family, why a partner becomes a crutch or an executioner, and what to do to prevent the disease from devouring your relationship.

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1. Essence and issues: invisible illness at the center of the family

PPPD is an invisible disorder. You don't lie with a cast. You don't have a fever. “I feel bad” is not written on my forehead. From the outside you look absolutely normal - and this is the main problem.

Your loved ones are trapped. They see two different people:

- **You say:** *“I’m shaking, I’m scared, I can’t leave the house.”*
- **They see:** A person who walks, talks, eats normally and looks healthy.

From this gap two typical scenarios are born, and both are destructive:

Scenario 1: “You’re just making it up!” The partner does not believe. * “Stop beating yourself up. Go to work. All your tests are normal. Just stop thinking about it."* You feel misunderstood, alone, and begin to **exaggerate** symptoms so that you can finally be heard. Or you become isolated and stop telling - and the suppressed resentment goes into muscle spasm.

Scenario 2: “Poor thing, let me do everything for you” The partner believes too much. He begins to protect you from everything: * “Don’t go to the store, I’ll go myself. Don't leave the house in case you feel bad. Let's cancel the trip."* Sound like a concern? In fact, it is a **disservice** that cements your neurosis. Your partner unknowingly becomes a **crutch**, without which you are afraid to take a step. And the exposition (Chapter 12) becomes impossible, because there is always a person nearby, ready to “save”.

2. Physiological explanation: mirror neurons and emotional contagion

Anxiety is not just **your** problem. It is literally **contagious** on a neurophysiological level.

There are **mirror neurons** in the brain - cells that are activated not only when you yourself perform an action or experience an emotion, but also when you **observe** it in someone else. When your partner sees you having a panic attack, their amygdala fires in response. Not as strong as yours, but enough to trigger a cortisol release.

What does this mean for the family:

- **Partner burnout.** Living next to a chronically anxious person is like living next to a working alarm system. The partner's nervous system is also overloaded. After 6–12 months, the partner may develop his own symptoms: insomnia, irritability, headaches.
- **Anxiety spiral.** You panic → your partner gets scared → you see his fear → you panic more → he gets even more scared. Two overheated brains throw firewood at each other.
- **Role shift.** From a "partner", a loved one turns into a "nurse" or "therapist". Romance and intimacy are replaced by the function of "making sure he doesn't get sick." Relationships from horizontal (two equal adults) become vertical (one is caring, the other is sick). It kills both relationships and recovery.

3. Four toxic patterns paired in PPPD

Pattern 1: "Crutch"

Your partner does **everything** for you. You stop going to the store, driving a car, meeting friends - because * "why, if he/she can do it for me?"*.

Why it's dangerous: Every time your partner "rescues" you from a situation, he confirms to your brain: *"The situation is really dangerous. You are right to be afraid."* The brain writes: "Supermarket = danger (confirmed by partner's behavior)." Avoidance intensifies. PPPD is cemented.

Pattern 2: "Executioner"

The partner devalues. * "Stop whining. Pull yourself together. People have cancer, and here you are with your dizziness."*

Why is this dangerous: Devaluation is not motivation, it is **a blow to the nervous system**. You feel shame, guilt and loneliness. Suppressing your anger (Chapter 28). The muscle armor is strengthened. The instability is growing.

Pattern 3: "Therapist"

The partner listens to a 2-hour report of symptoms every evening. *"And in the morning it was 7 out of 10, and then it became 5, and then I went into the elevator and it became 8..."*

Why it's dangerous: You feed your CAS (Chapter 10). Every evening "debriefing" with a partner is the same rumination, only with an audience. Your partner gets tired, you get no relief, and both go to bed with elevated cortisol.

Pattern 4: "Hostage"

The entire life of a family is built around the disease. Cancellation of trips. Refusal of guests. No restaurants, movies, travel - *"what if I feel bad"*. The partner and children live in the "tiptoe around the patient" mode.

Why this is dangerous: It enhances secondary gain (Chapter 17). The brain receives reinforcement: * "While I'm sick, the family adapts, I don't need to make decisions, they don't demand anything from me."* Recovery is unconsciously sabotaged because it means returning to responsibility.

4. Cognitive traps

Trap No. 1: "The partner must understand everything without words"

"If he/she loves me, he/she should feel how bad I feel, without explanation".

Truth: your partner is not a telepath. He doesn't feel your dizziness. He doesn't know what PPPD is. He didn't read this book. If you don't explain, he won't understand. This doesn't mean he doesn't love you. This means that you have different information.

Trap #2: "I need to protect my loved ones from my problems"

- "Why load children? Why spoil the mood of your husband/wife? I can handle it myself."*

Truth: if you are silent, your loved ones **still** feel the tension, but do not understand the reason for it. And they fill the void with fantasies: *"He/she stopped loving me"*, *"He's hiding something"*, *"I'm probably to blame"*. Honest communication is better than anxious silence.

Trap #3: "I must fully recover before returning to a normal family life."

Truth: Recovery **occurs within** normal life, not instead of it. If you wait until you are 100% healthy to go to the park with your child, you will never go. Go now. With unsteadiness. With anxiety. This is the exposition.

5. Point of Support: Insight

> *PPPD is not just your disease. This is a disease that runs in your family. If you only treat yourself, and the system of relationships remains the same, you both pedal in place. Recovery is a team game. But the rules of this game need to be **said out loud** - clearly, specifically, without guessing.*

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6. Practice

For you: "Five-minute briefing" instead of a two-hour complaint

Replace the nightly endless analysis of symptoms with a **structured briefing** - 5 minutes, no more:

1. **"Today there was [X] out of 10"** - one digit. Without details, without minute-by-minute analysis.
2. **"I did [X]"** - what exactly did you do to recover (gymnastics, exposure, walk).

3. **“I need [X] from you”** - a specific request. *"Please just hug me"* / *"I need to be alone for 30 minutes"* / *"Tomorrow we'll go to the store together, it will be easier for me if you're there for the first 10 minutes"*.

All. 5 minutes. The partner is not overloaded. You don't feed CAS. Information has been transmitted.

For a partner: “Memo for those who are nearby”

Pass this section on to your loved one. This is written **for him/her**:

Dear loved one.

Your partner is experiencing PPPD - persistent postural-perceptual dizziness. It is a real neurological disorder recognized by WHO. This is not fiction, not hypochondria, and not “just nerves.” Your loved one's brain temporarily misprocesses signals from the body, causing them to feel unsteady, swaying, and anxious, even when standing on a level floor.

Here's what you need to know:

1. **It's not dangerous.** No one dies, faints, or has a stroke from it. It's unpleasant and scary - but not dangerous.
2. **It is treatable.** PPPD is a completely reversible condition. Recovery takes 2–6 months with regular work.
3. **You cannot cure him/her.** It is not your responsibility. You can **support**, but not **heal**. Treatment is his/her job.

What helps:

✅ Do it	❌ Don't
Believe the symptoms are real	Saying “you're making this up” or “pull yourself together”
Ask: “How can I help right now?”	Ask every 5 minutes “how are you?”
Support the exhibition: “Let's go together, I'll be there”	Do EVERYTHING for him/her (go to the store, drive the car)
Live your life: visit friends, do hobbies	Throwing away your life and becoming a caregiver
Limit talking about illness to 5 minutes a day	Listen to two-hour monologues about symptoms every evening
Say: “I believe in you. You can handle it"	Saying: “Maybe you shouldn’t go there?”

✅ Do it	❌ Don't
Be patient: there will be good days and bad days	Angry that “I’ve been on treatment for a month now, but there’s no result”
Take care of **yourself**: if you burn out, there will be no one to help	Ignore your condition for the sake of “caring” for your partner

Most important: The best thing you can do is **live normally**. Go to the movies, plan trips, invite guests. Do not adjust your family life to your illness. Normal life around the patient is a signal to his brain: * “The world is safe. Everything is going as usual. There is no need to panic."*

For both of you: “The Contract for Recovery”

Sit down together and agree on the rules. Write them down. This is not a formality - it is the **architecture** of your journey to recovery together.

Example contract:

```
RECOVERY CONTRACT

Date:_____

We, [Name 1] and [Name 2], agree:

1. TIME TO BE SICK: We discuss symptoms and how you are feeling.
Once a day, in the evening, no more than 5 minutes.
Beyond this time we talk about life, not illness.

2. EXPOSURE: [Name 2] supports exits to difficult places,
but doesn't do EVERYTHING for [Name 1]. We are planning the stairs together
fears and celebrate progress.

3. CRUTCHES: [Name 1] agrees not to ask [Name 2] to perform
tasks that he can perform independently (shopping,
driving, making calls), even if it is alarming.

4. SPACE: [Name 2] has the right to his life, friends
and guilt-free hobbies. Partner burnout is the enemy
recovery.

5. STOP SIGNAL: If one of us feels that the pattern
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violated (crutch, executioner, therapist, hostage) - he says
code word [_______], and we return to the contract.

Signatures: _____________ / _____________
```

How to talk to children about PPPD

If you have children, they **already** feel that something is wrong. Children read their parents' anxiety instantly. If you are silent, the child fills the void with fantasies: *"Mom/Dad is sick with something terrible"*, *"It's because of me"*, *"We're all going to die"*.

What to say (adapt according to age):

- **3–6 years:** *"Mom/dad sometimes gets dizzy, like on a carousel. It's not scary and not contagious. The doctor knows and helps. You are not to blame for anything."*
- **7-12 years:** *"I have a thing called PPPD - my brain sometimes confuses the signals and it feels like the floor is shaking. It's like a bug in a computer - not dangerous, but unpleasant. I do special exercises to fix this. It will get better in a few months."*
- **Teenagers:** Can you let me read Chapter 1. A teenager is able to understand the mechanism. The main thing is to say: * "I'm working on it. I don't need your pity. I need your ordinary life - study, friends, music. The more normal you live, the better I feel."*

Homework (DZ):

1. **Send the "Memo for Someone Nearby" section to your partner/loved one.** If it's awkward, say: *"I read a chapter in the book. There's a piece there just for you. Read it, please – it's 3 minutes."*
2. **Complete the Recovery Contract with your partner.** Post it on the refrigerator or save it in your shared notes. Re-read once a month.
3. **Conduct a pattern audit.** Honestly determine which of the 4 patterns (crutch, executioner, therapist, hostage) is working in your couple. Write down 3 specific examples from the last week.
4. **Introduce the "Five Minute Briefing"** instead of nightly conversations about symptoms. After a week, ask your partner: *"Do you feel better?"*. Most likely yes. And you too.

Case Studies

Recovery stories: living examples of victory over PPPD

How real people with severe, persistent dizziness regained their resilience by changing their habits and thinking.

Case 1. Anton (32 years old, Senior Backend Developer)

- **Symptoms:** Unsteadiness, wobbly head, inability to look at the monitor (visual addiction), panic when scrolling code, muscle block in the neck. Duration of illness: 1.5 years.
- **Trigger:** Deadline + processing + coffee 6 cups a day for 3 months.
- **Trap:** I tried to cure PPPD through "Doctor shopping" - I went through 4 MRIs, an ultrasound scan, donated blood for all markers, took a handful of nootropics and vascular drugs. The result is zero. I spent more than $2,500 on "treatment."
- **How it came out:**

1. **Digitization (Chapter 3):** Passed HADS (alarm 16/21) and DHI (68/100). I saw the numbers and stopped considering myself dying.
2. **Closed the door to the clinic.** Recognized that it was a software failure, not a hardware failure.
3. **Body (Chapters 4-5):** Introduced daily relaxation according to Jacobson - relieved neck spasm in 2 weeks. VOR exercises twice a day.
4. **Key Point:** The first 5 days of VOR training the unsteadiness **increased**. Anton read Chapter 14 (neuroplasticity), realized that the brain is being rewired, and continued.
5. **Thinking (Chapter 16):** Started a SMER diary. Discovered that his main distortion is Catastrophization. In a month I caught 47 catastrophic thoughts and rewrote them.

- **Result:** After 2 months I fully returned to working behind the monitor. HADS-A: 16 → 6. DHI: 68 → 18. Unsteadiness is gone by 95%, rare rebounds when overtired are perceived as a "time to sleep" sensor.

Case 2. Elena (45 years old, chief accountant)

- **Symptoms:** Feeling of rocking "like on the deck of a ship" 24/7, panic attacks in supermarkets, fear of queues, complete refusal to travel on public transport. Duration of illness: 3 years.
- **Trigger:** Death of mother + annual report + conflict with husband - three blows in 2 months. Holmes-Rahe scale: 420 points.

- **Trap:** Total control (excellent student syndrome). All my life I was burdened with family and work, unable to relax. I've been accumulating stress for years. Illness has become a “legal” way to stop.
- **How it came out:**

1. **Audit of areas of life (Chapter 17):** Conducted an honest written audit. I discovered a secondary benefit: illness was the only legal reason to tell my husband and children “I’m tired, do it yourself” and not take on additional work.
2. **Ego and bricks (Chapter 18):** The brick “a good wife does everything herself” is mom’s. Crossed it out. She wrote: “A good wife is one who is alive and well.”
3. **Boundaries and Exposure (Chapter 12):** Established a strict “no” rule for households and management. I built a staircase for the exhibition: from standing on the porch to traveling on the bus. I applied the “Horizon Rule” outside.

- **Result:** Removed hyper-responsibility. After 3 months, she completely restored her vegetation, goes to shopping centers without fear, and rides the subway. DHI: 82 → 22.

Case 3. Maxim (28 years old, marketer, traveler)

- **Symptoms:** Constant dizziness after suffering from otitis, derealization (the world is like behind glass), severe fear of fainting, hypochondria. Duration of illness: 11 months.
- **Trigger:** Acute vestibular neuritis → poor-quality rehabilitation → anxious fixation.
- **Pitfall:** CAS syndrome (persistent internal scanner) and Google addiction. I woke up and checked “how is my head?”, read neurotic forums for 3 hours a day. Over the course of 11 months, I Googled for myself: brain tumor, multiple sclerosis, Parkinson’s disease and aneurysm. Of course, he had none of these diagnoses.
- **How it came out:**

1. **Quarantine (Chapter 11):** Implemented the “Cold Turkey” protocol - blocked all medical sites, deleted chats of neurotics, put on the screensaver: “I’m healthy. I’m not allowed to Google.”
2. **Grounding (Chapter 10):** Every time I turned on the scanner, I did the “5-4-3-2-1” practice.
3. **Exposition and surrender (Chapter 12):** When panicking on the street, I used Frankl’s method of paradoxical intention: *“Come on, spin me around even more, throw me on the asphalt!”*.

- **Result:** The Amygdala quickly calmed down when it lost its supply of fear. After 6 weeks, derealization went away, after 2 months Maxim went on his first vacation by plane. HADS-A: 18 → 5.

Case 4. Olga (55 years old, mathematics teacher)

- **Symptoms:** Chronic unsteadiness, fear of leaving the house unaccompanied, insomnia, constant feeling “I’m going to feel bad.” Duration of illness: 4.5 years.
- **Trigger:** Menopause + retirement + “empty nest” syndrome (children left to study).
- **Trap:** Olga was the hardest case of all. 4.5 years of illness, total avoidance, complete social isolation. She believed that “at my age it’s too late to change anything.” Brick: “I’m 55, my time has passed.”
- **How it came out:**

1. **Basic settings (Chapter 7):** Started with the simplest - 10,000 steps. The first week I only walked around the entrance. The second - to the neighboring house. The third one is to the park. Every day 100 meters more.
2. **Inner Child (Chapter 19):** Found a photo of myself as a child. I burst into tears. For the first time in 4 years I told myself: “You can be afraid. I'm close."
3. **Meditation:** Started with 3 minutes a day. In a month - 10 minutes. After 2 months - 20 minutes. Sleep returned to normal for the first time in 3 years.

- **Result:** After 4 months, Olga rides a minibus alone, goes to the store and signed up for painting courses. Her words: “I lost 4.5 years of my life. But I will live the remaining 30 years differently.”

Case 5. Dmitry (24 years old, freelance designer)

- **Symptoms:** Derealization (”the world is in a fog”), panic attacks 3-4 times a week, fear of going crazy, complete refusal of social life. Duration of illness: 8 months.
- **Trigger:** Protracted COVID-19 + remote isolation + sitting at home for 16 hours every day.
- **Trap:** Dmitry was sure that he was “brain damaged after Covid” and “nothing will come back.” I read scientific articles about long-COVID, found confirmation of my fears and convinced myself that his case was uniquely severe.
- **How it came out:**

1. **Digitization (Chapter 3):** HADS-A: 19/21. DHI: 74/100. Seeing the numbers, I realized for the first time the scale of the problem—and that it was measurable.
2. **Sports (Chapter 13):** Started with simple squats at home. A week later - walking. A month later - run 2 km. Adrenaline, which had been accumulating in tight muscles for years, began to be utilized.
3. **Metacognition (Chapter 15):** Mastered the “worry timer” - allocated 15 minutes a day to anxious thoughts, the rest of the time mentally said: “This is not now. There is a special time for this."
4. **Affirmations (Chapter 13):** Every morning - 5 minutes of affirmations. At first I felt like an idiot. After 3 weeks, I noticed that my morning anxiety decreased by 40%.

- **Result:** After 3 months, derealization was completely gone. Panic attacks - from 4 times a week to 0. HADS-A: 19 → 7. Got a job in the office - for the first time in a year I went out to work full-time.

Case 6. Natalia (33 years old, on maternity leave)

- **Symptoms:** Unsteadiness, "cast iron" head, fear of being alone with the child, fear of dropping the baby due to dizziness. Duration of illness: 14 months.
- **Trigger:** Difficult childbirth + postpartum depression + chronic lack of sleep + mother-in-law pressure.
- **Trap:** "I'm a bad mother if I need help." Natalya refused any help, believing that she had to cope on her own. At the same time, her inner voice constantly said: "What if I drop the child? What if I lose consciousness while I'm feeding?" Every feeding turned into anxiety torture.
- **How it came out:**

1. **Ego (Chapter 18):** Dismantled the "good mother = single mother" brick. This was her mother's attitude. For the first time, Natalya asked her husband to take the child for 2 hours to go for a walk alone.
2. **Neurophysiology (Chapter 7):** I improved my sleep - I agreed with my husband about "on duty." Started sleeping 6-7 hours instead of 4. Morning restlessness decreased by 50% in the first week.
3. **Inner Child (Chapter 19):** Wrote to myself: "You have the right to be tired. A tired mother is not a bad mother. A bad mother is a mother who burned out."

- **Result:** After 2.5 months, the unsteadiness went away. Natalya walks with a stroller without fear, leaves her child with her husband without guilt, and signed up for postpartum yoga. Her words: "I became a better mother when I stopped being perfect."

What unites all these stories?

Look at these 6 cases. Different people, different ages, different triggers. But there is only one pattern:

1. **Digitization** - Each of them started with measurement, not panic.
2. **The body is primary** - everyone started with physical practices (walking, Jacobson, VOR), and not with "reprogramming thinking."
3. **Thinking secondary** - cognitive techniques (SERM, metacognition, affirmations) were activated when the body stabilized.
4. **Root cause** - everyone had a deep-seated false attitude that fueled the neurosis. Removing this setup was more important than removing the shakyness.
5. **Nonlinearity** - each of them experienced setbacks. Not a single one was cured in a straight line "up".

Point of Support: Insight

> *These people were not made of steel. They were afraid just like you are now. They were shaking, sick and stormy. But they stopped feeding their fear and began systematically reinstalling the software. Their victory is proof that your brain can do the same.*

Homework (DZ):

1. **Which story is closest to your case?** Write down 3 key steps of this hero that you need to repeat.
2. Determine what “false brick” this person had and whether you have a similar one.
3. Write a draft of **your own recovery case** in a notebook in the future tense. Describe in detail how you will feel in 3 months, where you will go and what you will do. Let your brain see the end goal.

Appendices

Applications: Your emergency suitcase

Emergency card for panic attacks/unsteadiness, autonomic habit implementation tracker and scales for tracking progress.

Appendix A. Emergency card (Save to your phone)

Read this text out loud or to yourself if you feel a sudden surge of anxiety or unsteadiness on the street:

1. **STOP.** Stop. Don't run. Stopping is a physiological signal to your brain: *"There is no danger, we are not running away from a predator"*.
2. **THIS IS ADRENALINE.** The pulse increased, and the picture swam because the adrenal glands released a hormone. This is normal chemistry. It will **burn completely in 3-5 minutes** if you don't start scaring yourself with thoughts. Wait.
3. **SQUARE BREATHING:** Inhale (4 seconds) ➔ hold your breath (4 seconds) ➔ exhale (4 seconds) ➔ hold your breath (4 seconds). Do 5 such cycles to equalize the balance of oxygen and carbon dioxide.
4. **GROUNDING 5-4-3-2-1:**

- 👁 Find with your eyes **any 5 blue or green objects** around you.
- ✋ Become aware of **4 tactile sensations** (how your feet press on your soles, how the wind touches your cheeks, how your fingers squeeze your keys/phone).
- 👂 Hear **3 different sounds** (the hum of cars, a bird, someone talking).
- 👃 Feel **any 2 smells** (the smell of coffee, asphalt, your jacket).
- 👅 Feel **1 taste** (take a sip of water).

5. **LOOK AT THE HORIZON:** Raise your chin. Shift your gaze to a distant building or tree. Don't look at your feet!
6. **PARADOXICAL SURRENDER:** Tell fear: *"Come on, spin me around even more. I allow this to happen. I'm standing here, I'm safe."*

Appendix B. Monthly CNS habit tracker

Print this table or copy it into a notepad. Check the boxes daily. Remember: discipline is the highest form of self-care.

Habit	1	2	3	4	5	6	7	8	9	10	11	12	13	14	...
Sleep 7-8 hours	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐
Jacobson relaxation	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐
Vestibular gymnastics	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐
10,000 steps (on the horizon)	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐
Digital Detox (No Screens)	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐	☐

Appendix B: How to Measure Progress (HADS and DHI Tests)

To prevent your frightened brain from devaluing the results, translate your progress into the language of numbers.

1. Hospital Anxiety and Depression Scale (HADS)

- **Norm:** 0–7 points.
- **Subclinical anxiety/depression:** 8–10 points.
- **Clinically significant anxiety/depression:** 11 points and above.
- *Your goal:* reduce your anxiety score below 8 points.

2. Dizziness Handicap Inventory (DHI)

- **Mild degree of restrictions:** 0–30 points.
- **Average degree of restrictions:** 31–60 points.
- **Severe degree of restrictions:** 61–100 points.
- *Your goal:* reducing the DHI index by 18 points or more during the period of working on the book (mathematically confirmed clinical improvement).

Appendix D. Exposure Diary

Every time you take a step on the fear ladder (Chapter 12), write down the result:

Date	Situation	Alarm BEFORE (1–10)	What did I do	Anxiety AFTER (1–10)	Ran away?	Notes
._	Went to the supermarket	8	Horizon Rule, stood for 5 minutes	5	No	After 3 minutes it released
._	Ride the subway	9	Breathing "square", 5-4-3-2-1	6	No	Got off at the right station
._						
._						
._						

Key Rule: Column “Escaped?” - the most important. If you stay in the situation despite the fear, you have won. Even if the anxiety has not dropped to zero. The fact of not running away is a rewriting of the amygdala code.

Appendix E. Quick reference guide for technicians

All the techniques from the book in one place - for quick access:

Technique	Chapter	When to use	Time
Relaxation according to Jacobson	5	Daily, morning/evening	20 min
VOR-gymnastics (look at the point)	6	Daily, 2 times a day	5 min
Romberg stance	6	Daily	2 min
Contrast shower	7	In the morning, immediately after waking up	3 min
Morning Meditation (NSDR/Audio)	47	In the morning, after a shower	10 min
Horizon Rule	8	Any time you walk outside	—
Breathing "Square"	Adj. A	During an anxiety attack	2 min
Grounding 5-4-3-2-1	10	When you turn on the internal scanner	2 min
Paradoxical intention	12	In case of panic, exposure	1 min

Technique	Chapter	When to use	Time
Diary of SMER	16	After a disturbing episode	10 min
3rd person description (defusal)	16	When merged with a symptom	2 min
Worry Timer	15	Daily, at fixed times	15 min
Affirmations	13	In the morning, in front of the mirror	5 min
Audit of areas of life	17	One time + monthly review	30 min
Storm Protocol	21	With a strong rollback	3 days

Appendix E. Glossary of Key Terms

Term	Meaning
PPPD	Persistent postural-perceptual dizziness (ICD-11: AA36)
Amygdala	The amygdala is the brain's fear center and triggers the fight-or-flight response
CNS	Central nervous system
Vegetatics (VNS)	Autonomic (autonomic) nervous system - controls automatic processes
Cute	Section of the ANS responsible for mobilization (acceleration of the pulse, vasoconstriction)
Parasympathetic	Department of the ANS responsible for relaxation and recovery
Cortisol	Stress hormone secreted by the adrenal glands
Adrenaline	Fight-or-flight hormone, burns off in 3-5 minutes
Oxytocin	The “attachment hormone,” a cortisol antagonist, is released during self-compassion
VOR	The vestibulo-ocular reflex is the connection between the vestibular apparatus and the eyes
CAS	Cognitive Attentional Syndrome - internal scanner (rumination + monitoring)
CBT	Cognitive Behavioral Therapy
MKT	Metacognitive therapy (according to Wells)

Term	Meaning
PRT	Pain Reprocessing Therapy - pain reprocessing therapy (according to Gordon)
SMER	Diary: Situation → Thought → Emotion → Reaction
DHI	Dizziness Handicap Inventory - Index of Dizziness Disorders
HADS	Hospital Anxiety and Depression Scale - anxiety and depression scale
Proprioception	Sensation of body position in space (sensors in muscles, joints, feet)
Derealization	Feeling of the unreality of the surrounding world (“like behind glass”, “like in a fog”)
Optical flow	Movement of a visual image on the retina when moving
Neuroplasticity	The brain's ability to rebuild neural connections
SSRI	Selective serotonin reuptake inhibitors (antidepressants)
BDNF	Brain-derived neurotrophic factor is a “fertilizer” for neurons

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Point of Support: Insight

> *These tables and lists are not just paper. These are your crutches during the transition to complete independence. Use them until your legs regain their former confidence.*